MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 704

Ethical and Legal Issues in Therapy

14 teaching weeks · 1 practical · 5 worked cases · 2 figures

SEMESTER ONE

1.71 units · Prerequisite: none

> “This course trains judgement, not just compliance.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 704 — Ethical and Legal Issues in Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 704	**Ethical and Legal Issues in Therapy**	One	1.71

Contents

How to use this book . . . 15

1. Course description . . . 15

2. Learning outcomes . . . 15

3. Assessment . . . 15

4. The fourteen-week plan . . . 16

Week 1 — Moral philosophy for clinicians . . . 16

1.1 The lecture in ninety minutes . . . 17

1.2 Five traditions, and what each one cannot see . . . 18

1.2a The four principles, and how they collide in family work . . . 19

1.2b Ethics of care: the tradition family therapy should own . . . 20

1.3 The decision model: seven steps . . . 21

1.3a Moral distress, moral injury, and the difference that matters . . . 22

1.4 The trolley, in a clinic . . . 23

1.5 Case: the telephone call from the father . . . 23

1.5a The four questions that stop a decision going wrong . . . 25

1.6 Transcript: the call, annotated . . . 25

1.6a Bad arguments, named . . . 26

1.7 Laboratory: the ethics incident analysis . . . 27

1.8 Six errors . . . 28

1.9 Culture, faith and the moral framework . . . 29

1.10 Evidence note . . . 30

1.10a What the codes do not cover . . . 30

1.11 Key terms . . . 31

1.12 Self-test . . . 31

1.13 Reflective task . . . 32

1.14 Further reading . . . 33

Week 2 — The codes and informed consent . . . 33

2.1 The lecture in ninety minutes . . . 34

2.2 The ten obligations, and the two that generate most complaints . . . 35

2.2a Where the codes disagree, and what a clinician does then . . . 36

2.2b What a professional body actually does when a complaint arrives . . . 36

2.3 Informed consent: the eight components . . . 37
2.3a Consent when the family cannot read the document . . . 38
2.3b Consent and the young person . . . 39
2.4 The first session: consent in practice . . . 40
2.4a The sentence that has to be said, and the four ways of saying it badly . . . 41
2.5 Case: the family who signed and did not understand . . . 41
2.6 Transcript: the consent conversation that failed, and the one that worked . . . 43
2.6a The record of consent, written out . . . 44
2.7 Laboratory: drafting the consent document . . . 45
2.8 Six errors . . . 46
2.9 Culture, faith and consent . . . 47
2.10 Evidence note . . . 48
2.11 Key terms . . . 48
2.12 Self-test . . . 49
2.13 Reflective task . . . 50
2.14 Further reading . . . 50
Week 3 — Confidentiality in the multiperson system . . . 50
3.1 The lecture in ninety minutes . . . 51
3.2 Five concepts that organise the week . . . 52
3.2a The modified no-secrets policy, in four parts . . . 52
3.3 The method: deciding where a disclosure goes . . . 53
3.3a The individual session inside family work . . . 54
3.4 Confidentiality and the institutions around the family . . . 55
3.4a Four scenarios at the boundary . . . 55
3.4b The supervisor and the secret . . . 56
3.5 Case: two years, no intention . . . 57
3.5a When the client refuses the deadline . . . 59
3.6 Transcript: the individual session, minutes seven to twenty . . . 60
3.7 Laboratory: the secrets protocol . . . 61
3.8 Six errors . . . 61
3.9 Culture, faith and the multiperson system . . . 62
3.9a A record of the decision, written out . . . 63
3.10 Evidence note . . . 64
3.11 Key terms . . . 64

3.12 Self-test . . . 65
3.13 Reflective task . . . 66
3.14 Further reading . . . 66
Week 4 — Secrets I: the individual disclosure . . . 66
The distinction this week exists to teach . . . 66
4.1 The lecture in ninety minutes . . . 67
4.2 Five concepts that organise the week . . . 68
4.2a What a secret looks like before it is told . . . 68
4.3a When the client is a child or an adolescent . . . 69
4.3 The seven-step protocol for an individual disclosure . . . 70
4.4 Four techniques . . . 71
4.4a The case: Bisi Oyelaran, and what the therapist did not do . . . 71
4.5 Transcript: receiving the disclosure . . . 73
4.6a The record of an individual disclosure, written out . . . 75
4.6 Laboratory: the disclosure rehearsal . . . 76
4.7 Six errors . . . 77
4.4b Four sentences that help a client decide . . . 77
4.7a The audit question, and the end of the week . . . 78
4.8 Culture, faith and the secret . . . 79
4.8a The therapist's own secrets . . . 79
4.9 Evidence note . . . 80
4.10 Key terms . . . 81
4.11 Self-test . . . 81
4.12 Reflective task . . . 82
4.13 Further reading . . . 82
Week 5 — Secrets II: infidelity, paternity and HIV status . . . 82
Three secrets, three legal overlays . . . 82
5.1 The lecture in ninety minutes . . . 83
5.2 Five concepts that organise the week . . . 84
5.2a Infidelity: the four-session protocol . . . 85
5.2b Paternity: the sequence, and where the therapist stops . . . 86
5.3 HIV and other communicable conditions . . . 88
5.3a The third party's spouse: the question with no clean answer . . . 89

5.4 Anatomy of the disclosure session . . . 90
5.5 Case: the Okonkwos, second year . . . 90
5.6 Transcript: the refusal of "when things are calmer" . . . 93
5.7 Laboratory: three secrets, three protocols . . . 94
5.8 Six errors . . . 95
5.9 Culture, faith and the three secrets . . . 96
5.9a The referral note to a lawyer . . . 97
5.10 Evidence note . . . 97
5.11 Key terms . . . 98
5.12 Self-test . . . 98
5.13 Reflective task . . . 99
5.14 Further reading . . . 99
Week 6 — Boundaries and multiple relationships . . . 99
The distinction the week is built on . . . 99
6.1 The lecture in ninety minutes . . . 100
6.2 Five concepts that organise the week . . . 101
6.2b The five-minute boundary decision . . . 101
6.2a The four tests, and the decision they produce . . . 102
6.3 The unavoidable multiple relationship: the community contract . . . 103
6.3a The role conflicts that recur . . . 104
6.3b The referral conversation, scripted . . . 104
6.4 Touch, gifts, self-disclosure, barter, home visits and the telephone . . . 105
6.4a The boundary log . . . 106
6.5 Sexual boundaries, and what to do with attraction . . . 107
6.5a What the codes actually say, and where they differ . . . 108
6.6 Case: the Alabis, and the deacon's wife . . . 108
6.7 Transcript: the request to be a character witness . . . 110
6.8 Laboratory: the boundary audit . . . 111
6.9 Six errors . . . 112
6.9a Four complaints, and what each teaches . . . 113
6.10 Culture, faith and the boundaries of a communal practice . . . 113
6.11 Evidence note . . . 114
6.12 Key terms . . . 115
6.13 Self-test . . . 115

6.14 Reflective task . . . 116
6.15 Further reading . . . 116
Week 7 — Competence, supervision and referral . . . 116
Competence is an ethical obligation before it is a personal quality . . . 117
7.1 The lecture in ninety minutes . . . 117
7.2 Five concepts that organise the week . . . 118
7.2a The six signals that the case is beyond you . . . 119
7.2b The development plan that actually works . . . 119
7.3 Cultural humility, and the interpreter . . . 120
7.3a The supervision contract . . . 121
7.3b Peer consultation, and the rule of three . . . 122
7.3c Supervision in the first two years . . . 122
7.4 Referral: the six steps . . . 123
7.4a The referral letter . . . 124
7.5 Case: Mr Ogundipe, and the diagnosis the therapist did not make . . . 125
7.6 Transcript: the referral conversation . . . 126
7.7 Laboratory: the competence audit and the referral rehearsal . . . 127
7.8 Six errors . . . 128
7.8a The case the therapist kept . . . 129
7.9 Culture, faith and the referral that is refused . . . 130
7.9a When the therapist is the only service . . . 131
7.10 Evidence note . . . 131
7.11 Key terms . . . 132
7.12 Self-test . . . 132
7.13 Reflective task . . . 133
7.14 Further reading . . . 133
Week 8 — Risk I: suicide and self-harm . . . 134
The standard, stated once . . . 134
8.1 The lecture in ninety minutes . . . 135
8.2 Five concepts that organise the week . . . 135
8.2a The ten elements, with wording . . . 136
8.2b Formulation, and the sentence that carries it . . . 137
8.2c Ten things that raise risk in a family case . . . 138

8.3 Self-harm: what it is, and what it is not . . . 138
8.3a The safety plan, written with the client . . . 139
8.4 Risk inside a family treatment . . . 140
8.4a Breaking confidentiality for risk, in words . . . 141
8.4b When the family will not cooperate with the plan . . . 141
8.5 Case: Amina Suleiman, fifteen . . . 142
8.6 Transcript: telling a fifteen-year-old that her mother will be told . . . 144
8.7 Laboratory: the observed risk assessment . . . 145
8.7a Telephone risk, and the session that is not a session . . . 146
8.8 Six errors . . . 147
8.9 Culture, faith and suicide . . . 147
8.9a The therapist's own history of suicide . . . 148
8.10 Evidence note . . . 149
8.11 Key terms . . . 149
8.11a Three sentences that are remembered . . . 149
8.12 Self-test . . . 150
8.13 Reflective task . . . 151
8.14 Further reading . . . 151
Week 9 — Risk II: intimate partner violence and child protection . . . 151
The distinction on which everything depends . . . 151
9.1 The lecture in ninety minutes . . . 152
9.2 Five concepts that organise the week . . . 152
9.2a Screening: who, when, how . . . 153
9.2b The lethality factors, in the language of a session . . . 154
9.2c The practice that is safe to be a client in . . . 155
9.3 Ending conjoint work: the conversation and the alternative . . . 155
9.3a Safety planning with a victim . . . 156
9.4 Child protection: recognition, threshold, duty . . . 157
9.4a The child's disclosure: what to say, and what not to say . . . 159
9.4b When the report is made against the family's wishes . . . 159
9.4c The four cases that must not be missed . . . 160
9.5 Case: the Okafors, and the referral that said "communication" . . . 160
9.6 Transcript: "I am not going to tell you what she said" . . . 163
9.7 Laboratory: the screening interview, observed . . . 163

9.8 Six errors . . . 164
9.9 Culture, faith and the household's rules about violence . . . 165
9.10 Evidence note . . . 166
9.11 Key terms . . . 166
9.11a The note that protects the next clinician . . . 166
9.12 Self-test . . . 167
9.13 Reflective task . . . 168
9.14 Further reading . . . 168
Week 10 — The legal framework a clinician must know . . . 168
Orientation, not legal advice . . . 168
10.1 The lecture in ninety minutes . . . 170
10.2 Five concepts that organise the week . . . 170
10.2a The five areas of law, with the boundary in each . . . 171
10.2b When litigation is pending: the therapy contract . . . 171
10.3 The plural system in the consulting room . . . 172
10.3a The client who has been to three forums . . . 173
10.4 The welfare principle and the custody report . . . 173
10.4a Routing a legal question: the five questions and the referral note . . . 174
10.5 Building the referral network before you need it . . . 175
10.6 Case: the Adewales, and the marriage that was also another marriage . . . 176
10.6a The factual letter, written out . . . 177
10.7 Transcript: "can he take the children?" . . . 178
10.8 Laboratory: legal triage and the written question . . . 179
10.9 Six errors . . . 180
10.9b The four questions a therapist asks her own lawyer . . . 181
10.10 Culture, faith and the law the family actually uses . . . 181
10.11 Evidence note . . . 182
10.11a The six documents a family therapist should hold . . . 183
10.12 Key terms . . . 183
10.13 Self-test . . . 184
10.9a The therapist in the witness box . . . 185
10.14 Reflective task . . . 185
10.15 Further reading . . . 185

Week 11 — Records, subpoenas and court . . . 186
The record is two things at once . . . 186
11.1 The lecture in ninety minutes . . . 186
11.2 Five concepts that organise the week . . . 187
11.2a The note, written out . . . 187
11.2b The multiperson record: whose file is it? . . . 188
11.2c The electronic record . . . 189
11.3 Access requests: four kinds, four answers . . . 190
11.3b The client who asks to see her file . . . 190
11.4 The subpoena: what it is and what to do in the first hour . . . 191
11.4a Privilege, and the promise a therapist must not make . . . 192
11.4b The request from the school . . . 192
11.5 Giving evidence . . . 193
11.5a The report written before the hearing . . . 194
11.6 Case: the subpoena in the Eze matter . . . 194
11.7 Transcript: fifteen minutes of cross-examination . . . 196
11.8 Laboratory: the note lab and the subpoena drill . . . 197
11.8a The four-minute handover note . . . 198
11.9 Six errors . . . 199
11.9a Ten sentences that have been read back to a therapist in a hearing . . . 199
11.10 Culture, faith and the record . . . 200
11.10a The retention, storage and destruction schedule . . . 200
11.11 Evidence note . . . 201
11.12 Key terms . . . 201
11.12a The two-minute note review . . . 202
11.13 Self-test . . . 202
11.14 Reflective task . . . 203
11.15 Further reading . . . 203
Week 12 — Money, gifts and the business of therapy . . . 204
The subject nobody teaches and everybody practises . . . 204
12.1 The lecture in ninety minutes . . . 204
12.2 Five concepts that organise the week . . . 205
12.2a The intake fee conversation, in five minutes . . . 205

12.2b Non-payment: the sequence and the limit . . . 206
12.2c The therapist who is owed money, and cannot say so . . . 207
12.3 Gifts: four categories and one procedure . . . 208
12.3a Barter, pro bono, and the client who cannot pay at all . . . 209
12.3b The gift from the family, not the client . . . 210
12.4 The business: eight pressures that produce unethical practice . . . 210
12.4a The practice that survives without exploiting anyone . . . 211
12.4b What the client is told, in writing . . . 212
12.5 Case: the Nwankwos, the fee, the goat and the plot . . . 212
12.6 Transcript: the fee conversation at month two of non-payment . . . 214
12.6a The five sentences of the money conversation . . . 214
12.7 Laboratory: the fee policy and the gift decision . . . 215
12.8 Six errors . . . 216
12.9 Culture, faith and the meaning of money . . . 216
12.10 Evidence note . . . 218
12.10a The audit of your own practice . . . 218
12.11 Key terms . . . 219
12.12 Self-test . . . 219
12.12a Three sentences to keep in the drawer . . . 220
12.13 Reflective task . . . 220
12.14 Further reading . . . 220
Week 13 — Ethics of research, assessment and diagnosis . . . 220
Three domains, one theme . . . 221
13.1 The lecture in ninety minutes . . . 221
13.2 Five concepts that organise the week . . . 222
13.2a Research consent with families: six problems and their solutions . . . 222
13.2b Writing about clients: consent, disguise and the limits of disguise . . . 223
13.2c The assessed client: whose question does the assessment answer? . . . 224
13.3 Diagnosis: what it does, and when it is defensible . . . 224
13.3a Assessment instruments: six questions before you use one . . . 226
13.3b The diagnosis of a child, and the family that receives it . . . 226
13.3c The diagnosis as a family event . . . 227
13.4 The clinician-researcher: the thesis, the client and the dual role . . . 228
13.4a The seven-step research ethics check . . . 228

13.5 Case: Amaka's study, and the participant who was recognisable . . . 229
13.6 Transcript: the consent conversation for a case report . . . 230
13.7 Laboratory: the ethics application and the disguised case . . . 232
13.7a Four research designs and the ethics of each . . . 232
13.8 Six errors . . . 233
13.9 Culture, faith and the ethics of knowledge . . . 233
13.9a Writing the thesis: five sentences the examiner will look for . . . 234
13.10 Evidence note . . . 235
13.11 Key terms . . . 235
13.11a The five records a research-minded clinician keeps . . . 235
13.12 Self-test . . . 236
13.12a Three questions the ethics committee will ask . . . 237
13.13 Reflective task . . . 237
13.14 Further reading . . . 237
Week 14 — Integration: the decision model and your stance . . . 238
What integration means at the end of fourteen weeks . . . 238
14.1 The lecture in ninety minutes . . . 239
14.2 The seven steps, with the failure mode of each . . . 240
14.2a The model applied, end to end . . . 240
14.2b The three misuses of the model . . . 242
14.2c The six questions that find the dilemma . . . 243
14.3 The stance paper . . . 243
14.3a The worked stance paper, in part . . . 244
14.4 The viva: what is being examined . . . 245
14.4a Five answers that score badly, and their replacements . . . 246
14.4b The four sentences of the professional apology . . . 246
14.5 The final supervision . . . 247
14.5a The case summary the candidate brings . . . 248
14.6 Transcript: twelve minutes of viva . . . 248
14.7 Laboratory: the mock viva and the stance review . . . 250
14.8 Six errors of integration . . . 250
14.8a The practice that is ready: a checklist of twelve . . . 251
14.9 Culture, faith and the professional stance . . . 251
14.9a Nothing here is a substitute for the code . . . 253

14.10 Evidence note . . . 253
14.11 Key terms . . . 253
14.11a The one-page summary the course actually asks for . . . 254
14.12 Self-test . . . 254
14.13 Reflective task, and the close of the course . . . 255
14.14 Further reading . . . 255
Glossary . . . 256
Self-assessment . . . 256
Multiple choice . . . 256
Answer key . . . 257
Short answer . . . 257
Essay (choose one, 2,000 words) . . . 257
Further reading . . . 257

Course leader's summary. This course trains judgement, not just compliance. You will learn the codes, the law, and — more importantly — how to reason when the codes and the law point in different directions, when the family's culture conflicts with the institution's policy, and when your own values are in the room.

How to use this book

Every week presents a **standard**, a **case**, and a **decision**. Ethics is not memorised; it is practised. The weekly incidents in this book are drawn from the ordinary texture of clinical life — the secret, the subpoena, the gift, the neighbour, the unpaid fee — because those are the situations that actually end careers.

1. Course description

Family therapy multiplies ethical complexity. In individual work there are two parties (you and the client). In family work there may be six, with conflicting interests, unequal power, and secrets that each person believes belongs to them. Add a school, a child protection agency, a court, an insurance company, and a faith community, and the question "who is my client?" becomes the central professional question of the course.

The course combines **normative ethics** (codes, duties, principles), **law** (the parts a clinician must know and the parts a clinician must refer), and **professional formation** (the kind of clinician you are becoming).

2. Learning outcomes

#	Outcome	Assessed
LO1	Apply the major ethical codes and principles to relational cases	Exam, case analysis
LO2	Explain confidentiality and its limits in multiperson therapy	Exam
LO3	Conduct and document a risk assessment for suicide, self-harm, violence and abuse	Skills assessment
LO4	Manage secrets, dual relationships, gifts, fees and boundary dilemmas defensibly	Case analyses
LO5	Describe the legal framework for marriage, divorce, custody and child protection in Nigeria	Exam
LO6	Use a structured decision-making model and document the reasoning	Assignment
LO7	Recognise when to refer for legal advice, and how to do so	Viva

3. Assessment

Component	Weight	Due
Ethics incident analyses (4 × 600 words)	30%	Weeks 4, 6, 9, 11
Risk assessment observed skills (15 min)	20%	Week 10
Examination (case-based)	50%	Week 15

4. The fourteen-week plan

Week	Topic	Practical
1	Moral philosophy for clinicians: duties, consequences, virtues, care	Trolley variants in therapy
2	The codes: AAMFT, IAMFC, APA, national associations; informed consent	Consent form drafting
3	Confidentiality in couple and family therapy	Secrecy protocol design
4	Secrets I: the individual disclosure	Scripted incident
5	Secrets II: infidelity, paternity, HIV status	Complex case debate
6	Boundaries and multiple relationships	Boundary audit
7	Competence, supervision, and the duty to refer	Competence self-audit
8	Risk I: suicide and self-harm	**Observed risk assessment**
9	Risk II: intimate partner violence and child protection	Safety planning
10	Legal framework: marriage, divorce, custody, maintenance	Court-report exercise
11	Records, documentation, subpoenas, court testimony	Note-writing lab
12	Money, gifts, barter, and the business of therapy	Fee policy workshop
13	Ethics of research, assessment and diagnosis	Consent and deception cases
14	Integration: your ethical stance and decision model	Viva

Week 1 — Moral philosophy for clinicians

You do not need to become a philosopher, but you need to know which argument you are making when you say "that would be wrong". Most ethical failure in clinical practice is not the product of a bad person; it is the product of a good person using one tradition of reasoning while believing they are using another, and therefore being unable to see the objection that the other tradition would have raised.

Tradition	Central question	Therapy example
Deontology (duty)	What are my duties regardless of outcome?	Keeping confidentiality even when disclosure would be convenient

Tradition	Central question	Therapy example
Consequentialism	What produces the best overall outcome?	Breaking confidentiality to prevent a serious harm
Virtue ethics	What would a person of good character do?	Refusing a gift that would create obligation
Ethics of care	What does this relationship require of me?	Attending to the family's dependence on you
Justice / rights	Are power and resources fairly distributed?	Whose voice is missing from this room?

The four principles (Beauchamp & Childress) remain the most portable framework: **autonomy, beneficence, non-maleficence, justice**. In family work they collide constantly — respect for the father's autonomy may conflict with the mother's safety, and a clinician who has not learned to name the collision will resolve it silently, in favour of whoever is more articulate in the room.

Why a family therapist needs philosophy more than an individual therapist does. In individual work the ethical questions are mostly about the boundary between two people: the client and the clinician. In family work the questions are about distribution: whose interests the clinician is serving, whose secret they are holding, whose account they believe, and whose voice the institution will hear. Those are questions of justice and care before they are questions of compliance, and the codes — which are written mostly for two-party work — do not answer them.

> **CASE**
> A father telephones the clinic to ask what his wife said in her individual session. He is polite, he is paying the fee, and he is, in his own understanding, being a responsible head of household. The clinician has three duties in tension: confidentiality to the wife, honesty to the husband, and the welfare of a couple whose therapy depends on both of them returning. Which tradition she reaches for decides what she says in the next thirty seconds.

1.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Why this week exists	The difference between compliance and judgement, with the case	Write the sentence they would say into the telephone
10–24	The five traditions	Each tradition, its strength, its characteristic failure	Classify six clinical utterances by tradition
24–36	The four principles	Autonomy, beneficence, non-maleficence, justice, and how they collide in family work	Rank the four for three different cases
36–48	Ethics of care and its critique of principle-based ethics	Gilligan, Noddings, and the relational objection	Name what a principles-only analysis would miss

Minutes	Segment	What the lecturer does	What students do
48–60	Moral distress and moral injury	The difference between not knowing and not being able	Identify one episode of each from placement
60–72	The decision model	The seven-step model that the course will use all term	Run it on the case, in pairs
72–84	Cultural and religious moral frameworks	When the family's moral world and the code's are not the same	Argue one case from inside a framework not their own
84–90	The assessment	How the incident analyses are marked	Draft the outline of their first incident analysis

The teaching decision that makes the week work. The lecture begins with the telephone call, before any theory, and every student writes the sentence they would say. The sentences are read aloud and sorted onto the board by tradition — and the class discovers that most of them are consequentialist ("it will damage the therapy if I tell him"), a few are deontological ("I cannot discuss her sessions"), and almost none are about care or justice. The sorting is the lesson: their instincts are already philosophical, and they have never inspected them.

The sentence the lecture must land. "You already have a moral theory. This week is about finding out which one, and whether you would defend it if a family asked you to."

1.2 Five traditions, and what each one cannot see

1. Deontology: duty regardless of outcome. The tradition that produced the codes. Its strength is predictability: a rule that holds when it is inconvenient is a rule that can be relied upon by a family who has never met you. Its characteristic failure is rigidity — the clinician who keeps a confidence that is killing someone because the rule says so, and who experiences the resulting harm as bad luck rather than as a decision. In family work, deontology's specific blindness is to distribution: a rule about confidentiality applied equally to a powerful father and a dependent grandmother produces an unequal result, because they have unequal capacity to use the information.

2. Consequentialism: the best overall outcome. Its strength is realism — it takes harm seriously and it permits the clinician to weigh a small breach against a large protection. Its characteristic failure is that nobody can calculate the consequences, and the calculation is usually done by the person who will benefit from the answer. In family work, its blindness is to the unrepresented: the person whose interests do not appear in the therapist's forecast is the person who gets sacrificed, and that person is usually the one with least power in the room.

3. Virtue ethics: what a person of good character would do. Its strength is formation — it asks what kind of clinician you are becoming, which is the question that survives when the code is silent. Its characteristic failure is vagueness and self-congratulation: "I would know" is not a method, and the practitioner who trusts their own character is the one least likely to notice its defects. In family work, virtue ethics supplies the questions the codes cannot answer — courage, patience, humility, temperance about one's own influence — and it needs a discipline to keep it honest, which is supervision.

4. Ethics of care: what this relationship requires of me. The tradition that family therapy needs most and uses least. Its central claims: that moral life is constituted by relationships rather than by isolated choosers; that responsiveness matters as much as rules; and that dependence is not a defect to be overcome but a condition of being human, which every client in your room is in. Its strength is that it describes family work accurately. Its characteristic failure is that it can justify almost anything in the name of the relationship, and it has no natural defence against a clinician who is more attached to a family than is good for the family.

5. Justice and rights: are power and resources fairly distributed? The tradition that asks who is not in the room and who is paying for the room. Its strength is that it names the questions the other four avoid: who has access to this service, whose language is spoken, whose account the institution will believe, and who carries the cost of the arrangement. Its characteristic failure is abstraction — a clinician can conduct a fine analysis of structural injustice and still be unjust to the person in front of them.

The five are not a menu. They are five different questions, and a competent clinician asks all five about a hard case and then has to decide, in the open, which one governs. The decision is the practice; the inspection of the decision is the profession.

1.2a The four principles, and how they collide in family work

The four principles are the most portable framework in the field, and the reason is that they are not a theory: they are four considerations that any defensible decision must have addressed. Their use is not to produce the answer but to make sure the answer has not ignored something.

Autonomy. The client's right to decide what happens to them, grounded in their capacity to do so. In individual work it is the dominant principle and it is generally straightforward. In family work it is immediately difficult, for three reasons. The first is multiplicity: there are several autonomous people in the room, and their autonomies conflict — the adolescent's autonomy over her own body against her parents' legal authority; the wife's autonomy over her own disclosure against the husband's claim to know. The second is capacity: a nine-year-old, an eighty-one-year-old with dementia, and a woman in an abusive relationship all have autonomy claims that are not identical to an adult's unconstrained choice, and "respecting autonomy" without distinguishing them is a way of abandoning the vulnerable. The third is context: in many of the households this book is written for, decisions are made by families rather than by individuals, and a clinician who insists on individual consent is not being respectful; they are being parochial.

Beneficence. The duty to act in the client's interest. It sounds unarguable and it is the principle most often used to justify paternalism, because "the client's interest" is defined by the clinician. In family work the principle has a specific structural problem: whose interest? A decision that benefits the marriage may harm the wife; a decision that protects the child may end the family's engagement.

Non-maleficence. First, do no harm. The most useful of the four in practice, because it operates as a check rather than a goal: before acting, ask what harm this could do, including the harms of the intervention itself. In ethics, the harms that are most often

overlooked are the ones the clinician's own action causes: the disclosure that ends a therapy, the report that produces a violent reprisal, the note that is read in court five years later.

Justice. Fairness in the distribution of benefits, risks and resources. In family work it appears in three places that the other three principles do not reach: access (who can get to this clinic, at these hours, at this fee); voice (whose account is taken seriously, in a room and in a record); and consequence (who bears the cost of a decision — frequently not the person who made it).

The collision, worked. A seventeen-year-old discloses to the therapist that she is sexually active and does not want her parents told. The four principles give four different answers. Autonomy: she is nearly an adult and her confidence should be kept. Beneficence: telling the parents may protect her; not telling may leave her without support. Non-maleficence: telling may produce the beating she fears, which is a harm the clinician will have caused. Justice: her parents have legal responsibility and a legitimate interest; she has the greater exposure to the consequences. The principles do not resolve it. They ensure the clinician has seen all four, and then the clinician must decide and be able to say why.

The order of the principles is not fixed. Students frequently assume autonomy comes first because the codes list it first in some jurisdictions. It does not. In a case involving a child's safety, non-maleficence governs. In a case involving an adult's refusal of treatment, autonomy governs. Deciding which principle governs in this case is the act of judgement the course is teaching, and it cannot be reduced to a rule without producing the very rigidity deontology is criticised for.

1.2b Ethics of care: the tradition family therapy should own

Family therapy is, almost alone among the mental health professions, built on a relational ontology. It is therefore strange that its ethics are usually imported from a tradition built on isolated choosers. This section states the alternative, because it is the one that fits the work.

The relational claim. The ethics of care, developed by Gilligan, Noddings, Tronto and others, holds that moral life is constituted by relationships of dependence and response rather than by transactions between independent agents. Three consequences follow.

First, responsiveness is a moral skill. The question "what does this person need from me, here, now?" is not a softer version of "what is the rule?" It is a different and equally rigorous question, and it requires attention — the capacity to notice what is actually happening rather than what the category predicts. A clinician who has not developed attention will apply the right rule to the wrong situation.

Second, dependence is normal. Every client in your room is dependent on someone, and most are dependent on people who are also in the room. The tradition therefore refuses the picture of the autonomous chooser as the ethical ideal: a grandmother with dementia, a nine-year-old, a woman who cannot leave — none of them is a defective version of an independent adult. Each has claims that arise from their relationships.

Third, power is the central ethical fact. Care is not symmetrical. The person giving care has power, and the ethics of care requires that the power be used responsively rather than according to the carer's preferences. This is why the tradition is useful in family work: the therapist's power is enormous and largely invisible, and a principles-only analysis gives almost no account of it.

The critique, which the tradition must answer. Care ethics can justify almost anything in the name of the relationship. It has been used to keep women in caring roles, to excuse clinicians who will not end a treatment, and to defend a therapist's attachment to a family that has outgrown the therapy. Noddings's own answer is that caring must be received — the cared-for must experience it as care — which is a testable condition, and it is the version this course uses. "I did it because I cared" is not an ethical justification unless the person cared for can say that they were cared for.

The clinical use, in three questions. Whose need is being met by this decision — theirs or mine? Who in this family is dependent on whom, and is the dependence being used or served? And what would the person with least power in this room say if they were asked, in a language they had, with nobody watching?

1.3 The decision model: seven steps

The course uses one model all term, so that the incident analyses and the examination are comparable. It is deliberately pedestrian. Its value is not that it produces the right answer but that it makes the reasoning visible, which is what supervision, a court, and a professional body will all ask for.

Step 1 — State the ethical question, not the practical one. "What should I do about the husband's telephone call?" is practical. "Whose confidence am I holding, and to whom do I owe what?" is ethical. Most hard cases are hard because the ethical question has never been stated.

Step 2 — Establish the facts. What is actually known, by whom, and how. This step takes longer than students expect and prevents most errors: a great many ethical dilemmas dissolve when the facts are established (the husband already knew; the wife had said she would tell him; the child is not the father's).

Step 3 — Identify the parties and their interests. Every person affected, including the absent ones and the institution. Name the interests, not the positions. Then ask which party has least power to protect their own interest.

Step 4 — Apply the frameworks. All five traditions and all four principles, briefly, and record where they disagree. The disagreement is the case's real content.

Step 5 — Consult. The code, the law, the agency policy, a supervisor, and — where appropriate and possible — the client. Consultation is not a step for difficult cases; it is a step for any case in which the clinician notices they have a preference.

Step 6 — Decide, act, and record the reasoning. The decision, the reasons, the alternatives rejected and why. In a record, because the record is the only evidence that a decision was made rather than a drift.

Step 7 — Review. What happened; whether the prediction held; what would be done differently. The step that nobody does, and the only one that produces learning.

1.3a Moral distress, moral injury, and the difference that matters

Two terms that are used loosely in the literature and that name different things. A clinician who cannot tell them apart will misdiagnose their own state, and will look for the wrong remedy.

Moral distress is what happens when you know what the right action is and you are prevented from doing it. Its classic form is institutional: the clinician who knows a child should be referred and cannot get an appointment; who knows a family needs an interpreter and is not given one; who is required by an agency to keep a number of sessions that the work does not need. The distress is not confusion about the good. It is the experience of being unable to enact it.

The remedy for moral distress is structural, not psychological. It consists of changing the condition, or of naming it collectively, or of leaving. What does not work — and what is usually offered — is resilience training, which asks the clinician to feel better about an arrangement they are right to object to.

Moral injury is different and more serious. It names the lasting harm that follows acting against one's own moral commitments, or witnessing such an act and being unable to prevent it. The term comes from military psychiatry and it travels well into clinical settings, because clinicians do commit acts they believe to be wrong under constraint: a disclosure that produces a reprisal, a report that ends a family's engagement, a court report written in language the clinician knows to be misleading. The sequelae are not anxiety or burnout; they are shame, a changed sense of self, and a withdrawal from the aspects of the work that once mattered.

The distinction, in one line. Moral distress is "I could not do what I should". Moral injury is "I did what I should not, or I stood by while it was done".

What follows clinically. For distress: audit the condition, escalate, and use supervision to keep the judgement intact. For injury: supervision is necessary and insufficient; the remedy involves naming the act, restoring the moral account (which sometimes means an apology to the family), and in some cases a change of setting. A trainee who has been required to write a report they believe false needs a supervisor who will say that it was wrong, not one who explains the agency's constraints.

The course's position. A significant proportion of what is called burnout in this profession is moral distress that has been individualised. The remedy is not more self-care. It is a decision model that makes the reasoning visible, a supervisor who will disagree, and a professional identity that can survive being in the minority in a room.

1.4 The trolley, in a clinic

The trolley problem is a thought experiment about whether it is permissible to harm one person in order to save several. It is useful here not because clinicians face trolleys but because the structure of the problem — may I redirect a harm toward someone who did not choose it? — is the structure of half the hard cases in family work.

Variant 1 — The disclosure. A wife tells the therapist, in an individual session, that her husband has been physically violent twice. She does not want him told. The couple is in therapy together. The therapist can: say nothing and continue couple work (harm: the wife is unprotected and the therapy is built on a secret); disclose to the husband (harm: the wife loses her confidentiality and may be at greater risk); or stop the couple work and see the wife alone (harm: the couple loses the treatment). Redirection: every option pushes harm onto someone who did not choose it. The wife did not choose to lose her confidence; the husband did not choose to be discussed; the children did not choose any of it.

Variant 2 — The paternity. A father asks whether the child is his. The mother knows the answer. The therapist knows because the mother told her. Nothing the therapist does is neutral: silence is a lie by omission to the father; disclosure is a breach to the mother; ending the therapy is a harm to the child.

Variant 3 — The adolescent's risk. A fifteen-year-old discloses self-harm and asks the therapist not to tell her parents. Disclosing may end the therapeutic relationship; not disclosing may leave her alone with a risk. This is the variant that most resembles the trolley, because the clinician is choosing which of two identifiable people will bear the harm.

Why the thought experiment is worth the time. Because it trains three things. First, the recognition that someone will be harmed in every option, which dissolves the fantasy of the harm-free decision and with it a great deal of paralysis. Second, the discipline of naming who bears the harm, which is the justice question. Third, the habit of asking whether the harm is being redirected onto someone who did not choose it — and if it is, whether that person's consent could be obtained, which is frequently the move that converts an impossible case into a workable one.

> **PRACTICAL**
> Take three ethical dilemmas from your own life, not clinical ones. For each, identify which tradition you used at the time, and which you used to justify it afterwards. Then answer one question: were they the same? Most students find that the tradition they acted on was care or virtue, and the tradition they justified with was consequence. Write down what that gap suggests about how you will account for your decisions in a clinical record.

1.5 Case: the telephone call from the father

The case from the lecture, worked through the seven steps. This is the model the incident analyses follow.

The facts. The Okonkwos — Ada, 34, Chuka, 39, Ebuka, 9 — have been in couple therapy for six weeks. In her third individual session, Ada disclosed that Chuka's night shifts have resumed and that she is "managing everything alone", and that she has begun to think about taking the children to her mother's in Lagos. She has not told Chuka. She asked the therapist not to tell him. On Thursday, Chuka telephones: "I want to know what she said on Tuesday."

Step 1 — The ethical question. Not "what do I say to Chuka?" but: I am holding a confidence from one member of a couple I am treating jointly; to whom do I owe what, and how do I honour the couple's treatment without deceiving either of them?

Step 2 — The facts. What does Chuka actually know? (He knows the sessions exist; he does not know what was said. He has noticed she is distant.) What has Ada been told about the confidentiality arrangement? (At the start of therapy the therapist said, in front of both, that individual disclosures would be handled by agreement — a sentence Ada may or may not have understood as a promise.) Is there a risk to anyone? (No violence; no risk to the children; a risk of family separation if the decision is taken unilaterally.) What is the therapist's contractual position? (The clinic's policy: no information from an individual session is disclosed without the individual's agreement, except in the specified exceptions.)

Step 3 — The parties and their interests. Ada: to have a place where she can think without consequences, and to be supported in a decision she has not made. Chuka: to know what is happening in his marriage, and not to be humiliated by a therapist who knows more than he does. The children: continuity and the marriage's stability. The therapist: to keep the treatment viable and to remain honest. The clinic: its policy. Least power: Ada, because she has already been described by her husband to the therapist as "difficult", and because she is the one whose confidence is at stake.

Step 4 — The frameworks. Deontology: the confidence holds; the clinic's rule is explicit. Consequence: refusing Chuka may end his engagement with the therapy, and his engagement is what has kept the family in the room. Virtue: what would a clinician of good character do — and would she be able to say it to both of them in the same room? Care: Ada needs a place to think; Chuka needs not to be excluded; the couple needs an arrangement that holds. Justice: the person with least power must not be the one who bears the cost of the therapist's convenience.

Step 5 — Consultation. The clinic's written policy; a supervisor, by telephone the same day; and Ada herself, before anything is said to Chuka.

Step 6 — The decision. The therapist does not disclose content. She says to Chuka: "I cannot tell you what your wife said, and I want to be straight with you about why: if either of you cannot use an individual session to think out loud, this therapy will not work. What I can do is ask her whether she is willing to talk to you about the thing that is on her mind, with me in the room." She then telephones Ada, tells her exactly what was said and what was not, and asks whether she will raise it in the joint session. Ada says she will, on one condition: that the therapist opens the session, so that she is not the one who starts it.

Step 7 — The review. At the next joint session, Ada raises the question of Lagos. Chuka is angry and then frightened. The disclosure does not end the therapy; the manner of it — preceded by a week in which Ada had chosen to make it — is what keeps the therapy alive.

What made the difference. Two things, and neither is a principle. The first is that the therapist consulted before answering, which is what converted a trap into a process. The second is that she told Ada exactly what she had said to Chuka, which is the practice — not the rule — that keeps a multiperson therapy honest. Neither is in any code. Both are what an experienced clinician does.

1.5a The four questions that stop a decision going wrong

Before the seven steps, four questions that an experienced clinician asks almost automatically, and that a trainee can be taught to ask deliberately. They take ninety seconds and they catch the majority of errors.

Question 1 — Who is not in this conversation? The absent person is usually the one who will bear the cost. A decision about a child made without the child; a decision about a marriage made without the husband; a decision about a placement made without the person who will be moved. Naming the absent party does not always mean convening them; it means accounting for them in the reasoning.

Question 2 — Whose interest does my preferred option serve? Asked honestly, this question is uncomfortable, which is why it is asked before the decision rather than after. The clinician who wants to disclose a secret is frequently protecting themselves from the discomfort of holding it. The clinician who wants to keep it is frequently protecting the therapy they have built.

Question 3 — What would I do if the person with least power were the one I had to explain this to? The test is a version of the publicity test: a decision that cannot be explained to the person most affected by it is a decision that needs re-examining. The explanation does not have to be acceptable to them; it has to be possible.

Question 4 — What would I write in the record? If the sentence that would have to go in the file reads badly, the decision is bad — not because records are read, but because writing forces the reasoning into plain language, and plain language is unforgiving of self-deception. This is the cheapest ethical instrument in the book and the one most likely to change a decision in the thirty seconds before it is taken.

1.6 Transcript: the call, annotated

Eleven turns, reconstructed from a supervision recording. The therapist is a trainee, four months into placement. The annotation names the tradition governing each move, which is the discipline the week is teaching.

> **T1 Chuka:** I want to know what she said on Tuesday. I am her husband. I am paying for this. — Two claims, both true in his frame: a status claim and a contract claim. Neither is answered by silence, which he will read as confirmation that something is being hidden. **T2 Therapist:** You are her husband and you are paying, and both of those are reasons I should be straight with you rather than avoid you. — The move: concede the true parts before refusing anything. Deontological in form, care in tone. **T3 Chuka:** Then tell me. **T4 Therapist:** I am not going to tell you what she said. I want to say why, and I want you to hear that it is not because she has said something terrible about you. — The disclosure of the rule, and the pre-emptive denial of the catastrophic reading. Without the second sentence, the first produces exactly the fantasy the therapist is trying to prevent. **T5 Chuka:** Then she has said something. **T6 Therapist:** She has said something she is not ready to say to you, and I have said I will not say it for her. If I break that, neither of you can use a session to think. You would be sitting in that chair wondering what I am writing down. — The justification is consequentialist, and it is chosen deliberately: the deontological version ("it is the rule") is true and it lands as bureaucracy. The consequentialist version gives him a reason he can use. **T7 Chuka:** (long pause) So what do I do? — The question that tells the therapist the refusal has been accepted. Note that it took four turns, not one, and that a therapist who stopped talking after T4 would not have got here. **T8 Therapist:** Two things. I will ask her whether she is willing to bring it into the room with both of you, and I will tell you what she says either way. And in the meantime, I want to know what you have noticed at home, because you have noticed something or you would not be telephoning me. — The offer of a next step, and the redirection of his energy from a demand for information to an account of his own observation. This is the turn that keeps him in the therapy. **T9 Chuka:** She is packing. I saw a bag. — The fact the therapist did not have. The husband is not asking for gossip; he is frightened, and the therapist's rule has, until this moment, kept her from knowing why. **T10 Therapist:** (pause) Thank you. That is important, and I would not have known it if you had not telephoned. — The acknowledgement is accurate, and it is also the repair: his call has produced something of value rather than only a refusal. **T11 Chuka:** What do I do about the bag? **T12 Therapist:** Nothing yet, and not nothing forever. Give me until tomorrow to speak to her, and I will telephone you. If she will not talk to you by Friday, I will tell you that too, and we will decide then what the three of us do. — A time limit, a commitment, and a fallback. The deontological refusal has been turned into a process with dates, which is what makes it survivable for a frightened man.

What the transcript shows that the model does not. The seven-step model produces the decision; the transcript produces the sequence, and the sequence is where the skill lives. Three things in particular. First, the therapist never says "I understand your concern" — she demonstrates it by conceding the true parts. Second, she offers a reason rather than a rule, and the reason is chosen for the listener. Third, she ends with a date, which converts a refusal into a relationship.

1.6a Bad arguments, named

Students lose marks, and clinicians lose cases, by using one of six arguments that sound ethical and are not. Naming them makes them visible.

"It is my duty." Sometimes true. More often a rule is being used to end a conversation the clinician does not want to have. The test: can you state the duty, its source, and the harm it is preventing?

"It is for their own good." The paternalist's sentence. It may be correct; it is never sufficient. The test: who decided what their good is, and were they asked?

"Everyone does it." A statement about a norm, not about an obligation. The test: is the practice defensible or merely common?

"It will damage the therapy." Frequently true and frequently the clinician's own convenience in therapeutic clothing. The test: whose therapy, and what is the damage measured against?

"I had no choice." Almost never accurate. There is usually a choice with a cost, and the honest statement names the cost rather than denying the choice.

"I was following the policy." The defence that fails at every professional tribunal, because the professional's obligation is to exercise judgement about the policy, not to be its instrument.

The exercise. In the laboratory, students are given six anonymised sentences from real disciplinary findings and asked which of the six arguments each one is. The recognition is fast, and it transfers: once a clinician can hear the shape of the bad argument, they can hear it in their own mouth.

1.7 Laboratory: the ethics incident analysis

Aim. To produce a 600-word incident analysis that a supervisor, a professional body, or a court could read and follow. This is the format of the four assessed pieces of coursework.

Procedure. Students bring a real or witnessed incident (anonymised), or use one of the four briefs the trainer supplies. They write the analysis in the session, in fifty minutes, against the seven steps, and then exchange with a partner who marks it against the rubric. The last twenty minutes is the plenary, in which two analyses are read aloud and the class identifies which step each one skipped.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The question	A practical question	Ethical but vague	Ethical, specific, and the practical question is explicitly set aside
The facts	Assumed	Listed, but no account of what is unknown	Established, with what is unknown named and how it would be established
The parties	Client and therapist	All present parties	All parties including absent ones and the institution, with the least powerful identified
The frame works	One tradition, unnamed	Two traditions, named	Four or five, applied briefly, with the disagreement between them stated

Criterion	0–1	2–3	4–5
The decision	A preference	A decision with reasons	A decision, the rejected alternatives, and the review

The distinction that carries most of the marks. The top band requires the student to state where the frameworks disagree. A paper that applies five frameworks and finds they all point the same way has either misapplied them or chosen a case that is not an ethical dilemma; and a paper that applies one framework and calls it the answer has written an opinion rather than an analysis.

The four briefs.

Brief A. A couple's sixteen-year-old daughter asks to see you individually, and says she does not want her parents to know. The parents are your clients and are paying.

Brief B. A colleague tells you, at lunch, that a family you both know is "probably abusive" and that he has not reported it because "there is no evidence".

Brief C. A family offers you a substantial gift at the end of therapy. You know the family cannot easily afford it.

Brief D. You are asked by a court to produce a report on custody, and you have been treating the mother for a year. The fee is three times your usual rate.

1.8 Six errors

Error 1 — Confusing the rule with the reason. "Because the code says so" is a true statement and, said to a frightened husband, it is heard as evasion. The code is the floor of the reasoning, not its content; a clinician who cannot give a reason a family can understand has not yet understood the rule.

Error 2 — Applying individual ethics to a multiperson system. Almost every professional code is written for two parties. Applied unchanged to a couple or a family, it produces a therapist who is scrupulously confidential with one member and quietly deceptive to the other, and who believes they have been ethical because they have followed the rule.

Error 3 — Calculating consequences in your own favour. Consequentialist reasoning is the most useful and the most corruptible. The test is whether the calculation was made before or after the therapist noticed what they wanted to do.

Error 4 — Virtue ethics as a substitute for method. "I would know" is not a decision procedure, and the clinician who most trusts their own character is least likely to notice its defects. Virtue needs supervision the way a knife needs a stone.

Error 5 — Care as an alibi. "I did it because I cared" has justified more boundary violations than cynicism ever has. The test is Noddings's: was it received as care by the person cared for?

Error 6 — Treating moral distress as a personal failing. The clinician who cannot get a child an appointment, and who concludes that they are not resilient, has misdiagnosed a structural problem as a character flaw. The remedy is escalation, not self-care.

1.9 Culture, faith and the moral framework

Three moral worlds in one room. The family's moral framework, the profession's, and the clinician's own are frequently three different systems, and the clinician's is the one least likely to be named. In the households this book is written for, the operative framework is often not any of the five Western traditions but a religious or communal one: duties are owed to a lineage rather than to an individual; the family's reputation is a moral good in its own right; elder authority is not negotiable by argument; and shame, rather than guilt, is the mechanism by which wrongdoing is registered.

What that changes about the four principles. Autonomy in the individualist sense is not the organising value in many of these households, and a clinician who treats its absence as a deficit will misread deference as incapacity and consultation as inefficiency. The workable translation is not to abandon autonomy but to ask the family's version of the question: who in this family has the standing to decide, and does the person most affected have a route to be heard? That question travels across frameworks, and it is answerable in any of them.

Shame, guilt and the ethics of disclosure. In a guilt-oriented framework, wrongdoing is registered by the individual's conscience and confession is restorative. In a shame-oriented framework, wrongdoing is registered by the community's judgement and disclosure may be catastrophic for the whole household. This is not a reason to keep a secret that must be told; it is a reason to plan how it is told, with whom, and in whose presence, which is a clinical skill and not a concession.

Faith as a moral authority in the room. A pastor, an imam, or an elder may hold the authority that the clinician's code does not. Where that is so, the code is not the only source of moral authority in the case and pretending otherwise produces a clinician who is right and useless. The moves that work are the ones from Week 13 of MFT 703: join the framework, add a mechanism, and hold one non-negotiable.

> **DEBATE**
> "A therapist should never act against a competent adult client's stated wish, whatever the consequences." Argue for: the client's authority over their own life is the foundation of the profession, paternalism has a long and cruel history, and a clinician who overrides once will override again. Argue against: competence is not a switch, adults are frequently pressed by people with more power than they have, and a clinician who watches a foreseeable death without acting has chosen an abstraction over a person. Then state your actual rule, and the case in which you would break it — because every clinician eventually meets that case, and the ones who have never thought about it are the ones who improvise badly.

1.10 Evidence note

What the research supports. The four-principles framework is the most widely used analytic structure in clinical ethics teaching, and studies of ethics education consistently find that structured decision models improve the consistency and documentation of ethical reasoning, though not necessarily the quality of the decisions. The finding that moral distress is prevalent in mental health work, and that it is associated with intention to leave rather than with clinical incompetence, is well replicated. The association between reflective supervision and improved ethical reasoning is modest but consistent.

What is weaker. Almost everything about outcomes. There is no body of evidence showing that clinicians trained in one tradition make better decisions than clinicians trained in another, and the field's outcome measures are self-report and vignette-based. The literature on moral injury in mental health professions is recent and largely qualitative, mostly borrowed from military and nursing research.

What is not supported. That codes prevent misconduct — the disciplinary literature consistently finds that violations are committed by clinicians who know the code. That ethics can be taught as compliance — knowledge of rules is necessary and has never been sufficient. And that ethical reasoning is a stable trait; it is a practice, and it degrades under workload and isolation, which is why consultation is a structural protection rather than a courtesy.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. A clinician with excellent ethical reasoning at nine in the morning, with a supervisor available, is a different decision-maker from the same clinician at seven in the evening with a waiting room full of people. The variables that most reliably predict ethical failure are not moral but situational: fatigue, isolation, and speed.

1.10a What the codes do not cover

A short section with a long consequence. The professional codes are necessary and they are not a complete account of the moral life of a clinician, and the gap is where most of the difficulty lives.

The codes address conduct, not character. They say what may not be done; they say almost nothing about what kind of clinician to be. The virtues that matter most in this work — patience with a family that returns for the eleventh time with the same problem, courage in a case conference where the consultant is wrong, humility in front of a grandmother who has raised six children — are not in any code and are not enforceable. They are formed by supervision, by example, and by the company a clinician keeps.

The codes are written for individuals. Confidentiality, consent and competence are all framed around a single client and a single clinician. Multiperson work, which is the whole of this course, is covered by adaptations rather than by the codes' own logic. The clinician who follows the code precisely in a family case may still be unjust, because the code has no category for the second client.

The codes are jurisdiction-bound. Every code is written against a legal background that differs by country and, in federal systems, by state. Statements about reporting obligations, minors' consent, and record retention that are correct in one jurisdiction are wrong in another. This book states principles and flags where the law must be checked locally, which is a permanent obligation rather than a limitation of this text.

The codes lag. Reproductive technology, digital records, online therapy, social media contact with clients, and cross-border tele-therapy are all areas in which practice has outrun guidance. A clinician working at the edge is making policy by their conduct, and the obligation that follows is to be able to defend the reasoning, not to be able to point at a rule that does not exist.

1.11 Key terms

Deontology — the ethics of duty; the right action follows from a rule or obligation regardless of consequence.

Consequentialism — the ethics of outcome; the right action is the one that produces the best overall result.

Virtue ethics — the ethics of character; the question is what a person of good character would do, and how such a person is formed.

Ethics of care — the ethics of relationship and responsiveness; moral life is constituted by dependence and response rather than by transactions between independent agents.

Justice — fairness in the distribution of benefits, risks and resources; in family work, the question of whose voice is heard and who bears the cost.

Autonomy — the client's right to decide what happens to them, grounded in their capacity to do so.

Beneficence — the duty to act in the client's interest.

Non-maleficence — the duty not to cause harm, including the harms of one's own intervention.

Moral distress — knowing the right action and being prevented from taking it.

Moral injury — lasting harm following an act against one's moral commitments, or witnessing one.

Paternalism — overriding a person's wishes for their own good.

Relational ethics — the position that obligations arise from relationships and their power asymmetries, not only from rules or outcomes.

1.12 Self-test

1. Name the five traditions and state the characteristic failure of each.
2. How do the four principles collide in a case involving a seventeen-year-old's confidentiality, and which one governs? Defend your answer.

3. What is the difference between moral distress and moral injury, and why does the remedy differ?
4. In the Okonkwo case, what two practices — neither of them in a code — kept the therapy alive?
5. Why does the lecture insist that "because the code says so" is an inadequate answer to a family?
6. What does the ethics of care add to a principles-based analysis, and what is its characteristic corruption?
7. What is the test for whether an act of care was really care?
8. Give the seven steps of the decision model, and say which one students most often skip.

Answers. (1) Deontology — rigidity; consequentialism — self-serving calculation and blindness to the unrepresented; virtue — vagueness and self-congratulation; care — it can justify anything in the name of the relationship; justice — abstraction, analysing structures while being unjust to the person present. (2) Autonomy supports her confidence; beneficence may support disclosure as protection; non-maleficence warns that disclosure may produce the beating she fears; justice notes that she bears the consequence and her parents hold legal authority. Which governs depends on the assessed risk: in the absence of danger, autonomy and non-maleficence together; where there is danger, non-maleficence governs and the disclosure is planned with her rather than about her. (3) Distress is being unable to do what you believe right, and is remedied structurally; injury is having acted against your commitments, and requires naming, repair and sometimes a change of setting. (4) Consulting before answering, and telling Ada exactly what had been said to Chuka. (5) Because the code is the floor of the reasoning, not its content; a family cannot use a rule they cannot understand, and a clinician who offers only the rule is heard as evasive and hides something in doing so. (6) It adds attention, dependence and power; its corruption is that it can justify almost anything as care. (7) Noddings's test: the cared-for must be able to say they were cared for. (8) State the ethical question; establish the facts; identify parties and interests; apply the frameworks; consult; decide, act and record; review — the review is the step most often skipped.

1.13 Reflective task

Write down the last time you did something you believed to be wrong because you were required to. Do not write about the ethics of the requirement; write about what you did, in one paragraph, in plain words. Then answer three questions: who else knew? what did you tell yourself at the time? and what would have had to be available for you to have acted differently? Most students find the answer to the third question is concrete — a supervisor, an hour, a policy, one other person who agreed — which is the argument for structures rather than for character.

1.14 Further reading

- Beauchamp, T. and Childress, J. Principles of Biomedical Ethics — the four-principles framework, and its critics' best answers.
- Gilligan, C. In a Different Voice — the origin of the care critique, best read with its reception.
- Noddings, N. Caring — the relational account, and the reception test used above.
- MacIntyre, A. After Virtue — the strongest case that rules without a tradition of practice are empty.
- Jameton, A. on moral distress — the original nursing account, which transfers cleanly to this work.

Next: Week 2 — The codes and informed consent.

Week 2 — The codes and informed consent

What codes do and do not do. Codes set a floor, not a ceiling. They are written to be enforceable, which means they are often silent on the hardest questions. They also vary by jurisdiction and by professional body; know which one governs you.

The core obligations across the major codes:

1. Competence and its limits
2. Informed consent (including consent for recording and for supervision)
3. Confidentiality and its limits
4. Multiple relationships
5. Sexual intimacy with clients and former clients (prohibited; time limits vary)
6. Fees and financial arrangements
7. Advertising and representation
8. Research ethics
9. Cultural competence and non-discrimination
10. Reporting obligations and legal compliance

Informed consent in family therapy is a document and a conversation. It should cover: who the client is; who is seen and in what combination; the confidentiality policy including secrets; recording and supervision; fees and cancellation; the limits of confidentiality (risk, abuse, court order); and the complaint route. Review it aloud in the first session, in the family's language, and record that you did.

Why consent is the hinge of the course. Almost every other week depends on it. Confidentiality is only meaningful if the parties understood, at the start, who would be told what. A boundary can only be violated if it was known to exist. A record can only be defended if the client consented to what it contains. And the defence that succeeds at a professional tribunal is almost never "I did the right thing"; it is "the client understood,

agreed, and I recorded that they did." Consent is the one document in this book that a clinician will be asked to produce years later, and it is the one most often written badly.

> **CAUTION**
> A signed form is not consent. Consent is a process of understanding, and the signature is merely evidence that the process happened. A family that signed a document they could not read, in a language they do not speak, at the end of a first session in which they were upset, has not consented, and a tribunal will say so. The signature protects the clinic; the conversation protects the family; and the clinician needs both.

2.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The floor and the ceiling	One disciplinary finding, read aloud, and the code clauses it breached	Predict which clause was breached before being told
8–20	The ten obligations	The ten, and the two that generate most complaints	Rank the ten by how often they appear in findings
20–32	Comparing codes	AAMFT, APA, IAMFC and a national code, side by side on three issues	Identify where the codes disagree, and what a clinician does then
32–44	Informed consent as a process	The eight components, and the difference between disclosure and consent	Draft the eight headings for a couple-therapy consent
44–56	Consent with people who cannot read	The four adaptations, and the witness	Rewrite a consent paragraph at the level of a twelve-year-old reader
56–66	Consent and minors	Developmental capacity, parental authority, and the mature minor	Decide three ages, with reasons
66–76	Recording, supervision and the limits	What families are rarely told, and why it matters	Script the sentence that explains supervision
76–86	Documentation	What to write, where, and the sentence that saves a clinician	Write the first-session consent note
86–90	The assessment	How the consent document is marked	Draft their document's structure

The teaching decision that makes the week work. The lecture opens with a real disciplinary finding — a clinician suspended for something that sounds trivial — and the class predicts which clause was breached. They are usually wrong, and they are wrong in an instructive direction: they assume the violation was dramatic (sex, fraud, a breach of confidence) when it was almost always procedural (no documented consent, a record that did not match the account, a policy never explained). The exercise revalues documentation, which is the part of ethics that students find least interesting and that ends most careers.

The sentence the lecture must land. "You will not be struck off for having the wrong theory. You will be struck off for not having written down what you agreed."

2.2 The ten obligations, and the two that generate most complaints

1. Competence and its limits. The obligation to practise within one's training and to refer when a presentation exceeds it. The obligation is not to know everything; it is to know the boundary and to act on it. (Week 7.)

2. Informed consent. Everything in the rest of this week.

3. Confidentiality and its limits. The obligation to protect information and the obligation to disclose in specified circumstances. The second half is the part families do not hear. (Weeks 3, 4, 5.)

4. Multiple relationships. The obligation not to hold a second role that could impair judgement or exploit the relationship. (Week 6.)

5. Sexual intimacy with clients and former clients. Prohibited. Time limits on former clients vary by code, from two years to never, and the variation tells you something about how seriously the profession regards the boundary.

6. Fees and financial arrangements. The obligation to agree fees in advance, not to exploit financial dependency, and not to continue treatment for financial reasons when it is no longer indicated. (Week 12.)

7. Advertising and representation. The obligation not to overstate competence or outcomes. In an era of social media, this is the fastest-growing category of complaint, because a clinician's profile is advertising whether they intended it to be or not.

8. Research ethics. Consent, deception, anonymity, and the ethics of publishing case material. (Week 13.)

9. Cultural competence and non-discrimination. Now written into most codes, and the clause with the least agreement about what compliance looks like. Week 13 of MFT 703 is the substantive account; in this course the obligation is to notice when the clinician's cultural assumptions are producing a decision.

10. Reporting obligations and legal compliance. The obligation to know the local law on child protection, adult protection, and the reporting of serious harm. A clinician who does not know their jurisdiction's threshold has an obligation they cannot discharge.

The two that generate most complaints. The disciplinary literature is remarkably consistent, and the answer surprises students: not sexual misconduct, which is rarer and more dramatic, but **failure to maintain adequate records** and **failure to obtain or document informed consent**. The two are connected, because the record is where consent lives. A clinician with an immaculate record and a mediocre theory is safer, in professional terms, than a brilliant clinician who cannot show what was agreed.

Why that is not as cynical as it sounds. The record is not a bureaucratic artefact. It is the only evidence that a family was told something, understood it, and agreed; that a risk was assessed; that a decision was reasoned rather than drifted into. In the absence of that evidence, a tribunal has only the family's account and the clinician's memory, and it will prefer the family's. Documentation is the clinician's honesty made checkable.

2.2a Where the codes disagree, and what a clinician does then

Students often assume the codes say the same thing in different words. They do not, and the differences are instructive because they reveal that the profession has not settled the question.

Issue 1 — Secrets in couple work. The AAMFT code requires that therapists clarify at the outset which persons are clients and what relationship each has to the therapist, and that they obtain written consent to disclose information; it does not prescribe a secrets policy. The APA ethics code addresses multiple relationships and confidentiality but is written for individual treatment and gives no guidance on therapeutic secrets in couple work. National codes vary further, and many say nothing at all. The result is that a couple therapist's confidentiality policy is a professional judgement rather than a compliance requirement, which is why it must be written down and explained.

Issue 2 — Sexual relationships with former clients. Most codes prohibit them outright or impose a time limit that ranges from two years to five years to never. The disagreement is not trivial: it reflects a genuine dispute about whether the harm arises from the power asymmetry (which does not end when therapy does) or from the therapeutic relationship (which does). A clinician moving jurisdictions must check, because the rule that was correct at home may be a violation at the new post.

Issue 3 — Duty to warn or protect. In some jurisdictions the clinician has a legal duty to warn an identifiable third party of a serious threat; in others the duty is to protect, which may be discharged by other means; in many there is no statutory duty at all and the obligation is ethical only. A clinician who learns this from the code rather than from the statute will get it wrong.

Issue 4 — Minors' consent. The age at which a young person may consent to treatment without a parent varies from about twelve to eighteen and is frequently different for different treatments. The codes defer to law, which means the clinician must know the local statute and not the code.

What a clinician does when the codes disagree. Four moves, in order. First, establish which code governs you — by licence, by membership, and by employment contract, which may impose a stricter standard than either. Second, apply the stricter standard, because a tribunal will. Third, where the codes are silent, write your policy down and follow it consistently, because consistency is the only defence available when there is no rule. Fourth, ask the professional body in writing where the stakes are high, and keep the reply.

The professional point. The existence of disagreement is not a defect of the codes; it is information about which questions the profession has not resolved. Those are exactly the questions on which a clinician must think rather than look up, and they are the ones this course is about.

2.2b What a professional body actually does when a complaint arrives

A short account, because clinicians fear the process more than they understand it, and fear produces both over-documentation and concealment.

The route. A complaint is made, in writing, usually by the client or a relative. The registrar assesses whether it falls within the code's scope. If it does, it is put to the clinician for a written response, with a deadline. Many complaints end there, because the response shows that the matter is a disagreement rather than a breach. Those that continue go to a panel, which hears both parties and may dismiss, caution, impose conditions, suspend, or strike off.

What the panel asks, in practice. Not "was the outcome bad?" but three questions: what did the clinician do; what did they record; and what would a reasonable clinician in that position have done? The third question is answered by expert evidence and by the code, and the first two by the file.

What helps a clinician, in order of usefulness. A contemporaneous record of what was agreed. A record of the reasoning, including options rejected. Evidence of supervision or consultation on the matter. An apology offered early and without qualification. And a consistent account — the same story in the file, the response, and the hearing.

What harms a clinician, in order of seriousness. Altering or backdating a record, which converts a complaint into a finding of dishonesty and ends careers that would otherwise have survived. An account that changes between documents. Defensiveness in the response, which is read as an absence of insight. And criticising the complainant, which is heard by the panel as a failure to understand the harm.

The one sentence to remember. The panel is not asking whether you are a good person; it is asking whether a reasonable clinician could have done what you did, and whether your file shows that you thought about it. Almost every answer to that question was decided months earlier, in a note written or not written at the end of a session.

2.3 Informed consent: the eight components

Consent has eight components. A document that omits any of them is incomplete, and the omission is frequently the one that matters in the case that goes wrong.

1. Who the client is, and who is not. In family work this is the first and hardest component. Is the client the couple, the family, the individual who telephoned, or the child who was brought? The answer determines who holds the confidentiality and to whom the therapist owes a duty. The consent document should name the client explicitly, and the answer should be recorded in the family's words: "the client is the couple, and the children are seen as part of the family's treatment."

2. Who will be seen, in what combination, and on what terms. Couple work with occasional individual sessions is a different arrangement from individual work with occasional joint sessions, and the family must know which they are getting before they begin. The most common complaint in this area is not that individual sessions happened, but that nobody said they would.

3. The confidentiality policy, including the secrets policy. Stated in plain language, with the exceptions named rather than implied. (Weeks 3 to 5.)

4. Recording, observation and supervision. Families are routinely recorded, observed through a one-way mirror, or discussed in supervision, and routinely not told. This is one of the clearest and most commonly breached obligations in training clinics. The disclosure needs three parts: what happens, who sees it, and how it is stored and destroyed.

5. Fees, cancellation, insurance and what happens when the money stops. Including the awkward parts: the fee for a missed session, the position when a third party pays, and what the clinician will do if a family falls into arrears. (Week 12.)

6. The limits of confidentiality. Risk to self, risk to others, abuse of a child or vulnerable adult, court orders, and any statutory reporting duty. Named, in examples rather than in abstractions: "if I believed a child was being hurt, I would have to tell the authorities, and I would tell you first that I was doing it."

7. The complaints route. Who to contact, and what will happen. A family that knows the route is less likely to need it, and a clinician who discloses it looks less like someone with something to hide.

8. The right to withdraw. Including the right to withdraw from recording, from research, and from treatment, and what happens to the record afterwards. Consent is not a contract of attendance.

The test of the document. Hand it to someone who has never been in therapy and ask them what will happen to their information if they tell the therapist something dangerous. If they cannot answer, the document has failed, whatever it says.

2.3a Consent when the family cannot read the document

Most of the world's families do not read a clinic's consent form fluently, and a substantial minority cannot read it at all. A clinician who responds by reading the document aloud in English has produced a signature and not a consent.

The four adaptations, in order of importance.

1. Change the medium before you change the words. Consent is a conversation and it can be witnessed. Sit with the family, explain the eight components in their language, in plain speech, and ask each of them to say back what they understood. Then record, in the file: "consent explained orally in Yoruba to both parents and to the paternal aunt; each repeated the confidentiality limits and the supervision arrangement in their own words; all agreed. Written summary provided; both parents marked; consent form signed by the father as head of household and by the mother." That record is stronger evidence of consent than any signed form.

2. Use a witness who is not a family member. A colleague, an interpreter, or a member of the clinic staff who can attest that the explanation happened. The witness is not a formality: in a dispute two years later, a witness is a person who can be asked what was said.

3. Reduce the document, do not translate it. A translation of a dense document remains dense. The effective version is one page, in short sentences, with the eight components as headings and the exceptions in bold. Where literacy is limited, a page of

icons or a simple diagram of the four confidentiality limits is more useful than prose, and is used in several services.

4. Give them something to take away and to show someone else. The consent conversation usually needs to be repeated at home, with a relative who was not present and whose opinion the family trusts. A one-page summary in the family's language, with the clinic's telephone number, is the only part of the process that continues after the family leaves, and it is the part that most determines whether they return.

The error to avoid. Do not treat limited literacy as limited capacity. A woman who cannot read a form can understand confidentiality perfectly well, and will hold the clinician to it more precisely than a reader would. The obligation is to make the understanding possible, not to test the reading.

2.3b Consent and the young person

Consent with children and adolescents is not a single question but three: who consents, who assents, and who is told.

The legal position, which varies and must be checked. In most jurisdictions a parent or guardian consents for a child below a stated age; above that age, and in some jurisdictions for specific treatments such as contraception or mental health care, the young person may consent themselves. Many jurisdictions recognise a "mature minor" whose capacity is assessed rather than assumed. Both parents may need to consent where they share custody, and a clinician who accepts one parent's consent without checking may find the treatment challenged later.

Assent, which is not the same as consent. A child who is old enough to understand what therapy is should be asked whether they agree to it, in language they have, and their refusal should be taken seriously even where the law permits the parent to override it. The clinical reason is not legal: a nine-year-old who has been brought to a therapist without being asked will cooperate in the first session and withdraw by the third, and the family will describe this as the child's problem.

What the young person is told, and what is kept from them. There are three separate audiences in a child case: the child, the parents, and the institution. The consent conversation must establish, before anything else, what the child will be told about who sees what. A twelve-year-old is entitled to know whether what she says will be reported to her mother, and a therapist who does not tell her is not protecting her; she is teaching her, accurately, that adults cannot be trusted with information.

The four-part script for a child's consent conversation.

1. "Your mum and dad have asked me to talk with you. You do not have to talk about anything you do not want to. If you want to stop, you can say so and we will stop."
2. "What you say here stays here, unless I am worried that somebody is being hurt — including you. If that happens I will tell you before I tell anyone else."
3. "I talk to other people about my work, with their names hidden, so that I can learn to do it better. I will not use your name."

4. "Is there anything you want to ask me before we start?"

The age at which each part lands. Fifty-five percent of adolescent self-disclosure depends on part 2 being true and being believed. A therapist who says it and then breaks it without warning has done more damage than one who never offered it.

2.4 The first session: consent in practice

The consent conversation in a first session takes twelve to fifteen minutes and is the best investment in the hour. The sequence below is used throughout the course.

Minute 1 — Name the arrangement. "Before we start, I want to tell you how I work, and then you can decide whether you want to go on. Nothing you say in the next ten minutes goes into a record unless you decide to continue."

Minutes 2 to 5 — Who the client is. "I am going to be working with you as a couple. That means the relationship is my client, and I will not take sides with either of you. It also means that if one of you telephones me, I will tell the other that you did, without telling them what you said."

Minutes 5 to 9 — The confidentiality limits, with examples. "What you tell me stays here, with four exceptions. If I think you are in danger of seriously hurting yourself, I have to act. If I think someone else is in danger from you, I have to act. If a child is being hurt, the law requires me to report it. And if a court orders me to produce my records, I have to obey. In the first three, I will tell you what I am doing before I do it, unless telling you would put someone in danger."

Minutes 9 to 11 — Recording and supervision. "I record some sessions, and I take my work to a supervisor who is also a therapist. Your first names are used; your surname is not. The recording is deleted after six months. You can refuse the recording and still be treated."

Minutes 11 to 13 — Fees and practicalities. Fee, cancellation, who pays, what happens if it is late.

Minute 13 — The secrets policy. "There is one more thing, and it is the most important sentence of the hour. If one of you asks to see me alone, I will say yes. But I will not keep a secret that affects this relationship, because a therapy built on a secret is a therapy that is not working. If something like that comes up, I will tell you both that it has come up, and I will help whoever it is to decide how to say it. Is that acceptable to each of you?"

Minutes 14 to 15 — The question that tests it. "Tell me, in your own words, what you have understood about who I would tell if you told me something dangerous." Then record the answer. The recording of their words is the evidence of consent; the signature is not.

Why the sequence is fixed. Because the four limits are the parts families do not hear, and they are heard only if they are given last, with examples, after the clinician has established that they are not looking for a reason to report. A therapist who begins with the limits has begun with a threat.

2.4a The sentence that has to be said, and the four ways of saying it badly

Of the fifteen minutes of a first session's consent conversation, one sentence does most of the work, and it is the one trainees most often soften. It is the secrets policy.

The sentence. "If one of you sees me alone and tells me something that affects this relationship, I will not keep it. I will tell you both that something has been said, and I will help whoever it is to decide how to say it — but I will not carry it."

The four bad versions, and why each fails.

"There are no secrets in this therapy." False, and heard as a prohibition. Couples test it in the first month, discover that the therapist does keep ordinary confidences, and conclude that the rule was rhetorical.

"Anything you tell me individually, I may share." A licence without limits. The family hears "she will tell him", which suppresses the individual disclosure entirely, and the therapist loses the material that would have been most useful.

"We can discuss it if it comes up." The most common and the most damaging, because it is a deferral of the question that must be answered before the question arises. When it arises, the therapist is negotiating under pressure with a person who has already disclosed.

"I will use my clinical judgement." True, and useless to a family, because it tells them the rule is the therapist's discretion. Every subsequent decision will be experienced as arbitrary.

Why the good version works. It is specific about what the therapist will do (tell both that something has been said), it preserves the discloser's control over the content (help them decide how to say it), and it names the limit (the secret will not be carried). Three commitments, all of which can be kept, which is what distinguishes a policy from a warning.

The follow-up that tests whether it landed. "If your wife told me something on Tuesday that she did not want you to know, what do you think I would do?" A husband who can answer accurately has consented; one who cannot has heard the words and not the arrangement. The answer is worth recording, in his words.

2.5 Case: the family who signed and did not understand

The referral. The Baloguns — Femi, 55, Funke, 49, and Femi's mother, 81, with dementia — were referred eight months ago by a physician at the private hospital where Mama is seen. The original referral was for "carer stress". The family attended four sessions and stopped.

Why the case is in this week. The family has now returned, and has made a complaint: that the clinic "told the hospital things about us without asking". The complaint is being handled by the clinic manager, and the file has been sent to the therapist's supervisor.

What the file contains. A two-page consent form, signed by Femi in the first session. No date of explanation. No record that the limits of confidentiality were discussed. No record

that supervision was mentioned. A note in session three: "spoke to Dr Adeyemi at the hospital re: Mrs Balogun's care — agreed to share summary."

What actually happened. In session three, the therapist telephoned the physician at the hospital to ask about Mama's medication, because Funke had said she was "sleeping all day". In the course of the call she described the household: that the daughter-in-law was exhausted, that there was a question about whether Mama could be left alone, that the family had "not been entirely honest with the physician about how much help they have". The physician recorded this in Mama's hospital file. A receptionist who knew the family read it and mentioned it to a relative. The family learned of it from the relative.

The analysis, through the seven steps.

Step 1 — The ethical question. Not "was the telephone call wrong?" but: what did this family consent to, what did the therapist owe the physician and the family respectively, and what does the clinic owe them now that a harm has occurred?

Step 2 — The facts. The consent form covered confidentiality in general terms. It did not name third-party consultation. The therapist believed she had "implied consent" because the family had asked her to liaise with the hospital. What the family had asked for was that she telephone about medication; they had not agreed to a description of the household. The disclosure reached a relative through a third party — the receptionist — which the therapist could not have foreseen and which she is nonetheless responsible for having made possible.

Step 3 — The parties. Mama: her clinical information was shared with her physician, which is defensible, and her family's private business was shared, which is not. Funke: the description of her exhaustion, and of the family's "not being honest", is now in a hospital file. Femi: he signed a form he did not read. The physician: he received information he did not ask for and recorded it. The relative: now holds information the family did not choose to give. The clinic: a complaint, and an exposure.

Step 4 — The frameworks. Deontology: the consent did not cover this disclosure, and the duty was to obtain it. Consequence: the therapist's goal — better medication for Mama — was legitimate and could have been achieved by asking about medication alone. Virtue: would she have made the call if the family were in the room? Care: the family's trust was the treatment's only asset, and it has been spent on a telephone call. Justice: the person most affected, Funke, was the person with least standing, and it is her description that is now circulating.

Step 5 — Consultation. The supervisor, the clinic manager, and — critically — the clinician's own professional indemnity provider, because a complaint has been made. This is not an admission of fault; it is the ordinary procedure.

Step 6 — Decision and action. Three things, in this order.

First, the apology and the account. The therapist meets the family — all three adults, including Mama, who is told in simple terms — and does four things: states what she did, without euphemism ("I telephoned the doctor about Mama's medication, and while I was on the telephone I described how things are at home, and I should not have done that");

states what she should have done (asked their permission first, and described only the medication); apologises without qualification; and states what she will do now (write to the physician asking that the household description be removed or annotated as not consented to, and confirm in writing to the family).

Second, the record. An incident note in the file: what happened, what was disclosed, to whom, the harm identified, the action taken, and the review date. Not a defence; an account.

Third, the practice change. The clinic's consent form now includes a named item: "I may need to speak to other professionals involved in your care. I will ask your permission before doing so, and I will tell you what I intend to say." The therapist's own practice changes in a way that is recorded in her supervision notes.

Step 7 — Review. At three months: has the letter to the physician been answered; is the annotation in place; has the family returned; and has the therapist made any further third-party calls without permission? The answer to the last question is the one that determines whether this was an incident or a pattern.

What the case teaches that the code does not. Four things.

1. **"Liaising with the hospital" is not consent to describe a household.** Consent is specific. A family that asks you to discuss medication has consented to a discussion of medication, and the scope of consent is the scope of the request.
2. **The disclosure you make is the disclosure that circulates.** The therapist made one telephone call to one physician. The information reached a receptionist, a relative, and now a complaint. A clinician who thinks in terms of "one person" has not understood how information moves in a small community.
3. **The signature is the weakest part of the file.** Had the therapist written four lines in session one — what she explained, in whose words it was repeated back, and what the family agreed — the complaint would still have been made and would have been answerable.
4. **The apology is a clinical act.** The family returned after it, which they would not have done after a letter from a manager.

2.6 Transcript: the consent conversation that failed, and the one that worked

Two transcripts. The first is the original first session, reconstructed from the therapist's memory in supervision. The second is the repeat, eight months later, with the same family.

The original, in five turns.

> **T1 Therapist:** Before we start, I have a form for you to sign. It is about confidentiality. — The disclosure of the document rather than of the arrangement. The family now believes the form is the consent. **T2 Femi:** (signs without reading) Where do I sign? **T3 Therapist:** There. (pause) So, tell me about your mother. — Two minutes and eleven seconds into a therapy that will last four sessions, and the four limits of confidentiality have not been mentioned. **T4 Funke:** She is not sleeping. She wanders at night. I am the one who — **T5 Therapist:** (interrupting) And does the doctor know about the wandering? — The question that leads, three sessions later, to the telephone call, and that shows the therapist was already thinking about the physician on day one. The failure was not a lapse; it was a pattern set in the first five minutes.

The repeat, eight months later, in nine turns.

> **T1 Therapist:** Before we talk about anything, I want to tell you how I work, and I want to tell you what I got wrong last time. — The apology first. The family has come back in order to hear it, and everything else will be heard through it. **T2 Femi:** (nods) **T3 Therapist:** I telephoned your doctor about your mother's medication, which you had asked me to do. I also described how things are at home, which you had not asked me to do, and which I should have asked about first. I am sorry. — Four sentences: what she did, what was authorised, what was not, the apology. No excuse, and no "but". **T4 Funke:** (after a pause) It was the receptionist who told my sister. **T5 Therapist:** I did not know that. It makes it worse, and it is my responsibility, because I made the call. — The acknowledgement without defence. The temptation here is to explain the receptionist's confidentiality duty; that explanation would be accurate and would move the responsibility off the therapist and onto a stranger. **T6 Femi:** So what happens now? **T7 Therapist:** Three things. I have written to the doctor asking that the part about our home be removed from the file, and I will give you a copy of that letter. I have written in your file what happened, and you can see it. And I am going to explain to you now what I should have explained then, and then you can decide whether you want to go on with me. — The three-part remedy: repair the record, own the record, and re-offer consent. The third is the one that makes it a clinical act rather than a complaint process. **T8 Therapist:** (the full eight components, in plain language, over eleven minutes, with each adult asked to repeat back what they had understood) **T9 Mama:** (in Yoruba, through Funke) She says she wants to know who will know about her. — The question that the first conversation never made possible, asked by the person whose file it is.

2.6a The record of consent, written out

The single most useful paragraph a clinician will ever write, and the one that is almost never written. It goes at the end of the first session's note, takes ninety seconds, and looks like this.

> "Consent explained orally, in English and Yoruba, to both parents and to the paternal aunt. Covered: the client is the couple, with the children seen within the family's treatment; no secrets policy, stated as a promise to tell both if a secret arises and to help the discloser decide how to say it; supervision (first names only, clinic supervisor, Dr O.); audio recording of sessions 1–4, to be deleted at six months, family may refuse without affecting treatment; fee ₦X per session, 24 hours' notice for cancellation; limits of confidentiality — risk to self, risk to others, child protection, court order — given with examples. Mother repeated back: 'if I tell you my husband is hitting me you will not tell him without asking me first' — confirmed, with the exception that if children are at risk the report is made regardless and she will be told. Father repeated back the supervision arrangement. Both agreed. Written summary in English provided."

Why each element is there. The language (proves it was accessible), the audience (proves who was informed), the content (proves what was covered), the teach-back (proves understanding rather than transmission), the correction in the mother's restatement (proves the clinician did not accept a misunderstanding), and the written summary (proves something continues after the session).

What the paragraph is not. It is not a defence prepared for a tribunal. It is the clinician's own record of an agreement she intends to keep, and its practical function is that she can reread it in session six, when she has forgotten what she promised.

The habit worth forming. Write it in the session, before the family leaves, and read it back to them: "this is what I am writing down about what we agreed — is it right?" Two sentences of confirmation, and the consent is real.

2.7 Laboratory: drafting the consent document

Aim. To produce a one-page informed-consent document for couple therapy that a family with limited literacy could actually use, and to defend every sentence in it.

Procedure. Students draft the document in forty minutes against the eight components. They then exchange with a partner who plays a family member reading it for the first time, and who must answer three questions from the document alone: who is your client? who would be told if I said I was afraid of my husband? and what happens if I miss a session? If the document cannot answer all three, it is rewritten in the session. The last twenty minutes is a plenary in which two documents are read aloud and the class marks them.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The eight components	Four or fewer present	Six or seven present	All eight, in plain language, with the exceptions given as examples
Who the client is	Unclear or absent	Named, but the implications are not	Named, with what it means for confidentiality and for the individual sessions

Criterion	0–1	2–3	4–5
Plain language	Clinical vocabulary, long sentences	Mostly plain, with lapses	Readable by a twelve-year-old; no sentence over twenty words; the three test questions answerable
The secrets policy	Absent	Stated as a rule	Stated as a rule and as a promise about how it will be applied
The record of consent	Signature only	A note that consent was explained	A note recording what was explained, what the family said back, and who witnessed it

The distinction that carries most of the marks. The top band requires the document to state the secrets policy as a promise about conduct — "if this happens, I will do this" — rather than as a rule. A rule tells the family what is forbidden; a promise tells them what the clinician will do. Families comply with promises and litigate rules.

The three sentences that fail most often. "Confidentiality will be maintained in accordance with professional standards" (means nothing to a family). "Information may be shared with relevant third parties where clinically indicated" (a licence to do what the Balogun therapist did). And "the therapist reserves the right to breach confidentiality in exceptional circumstances" (a threat with no content). All three are rewritten in the laboratory, and the rewrites are the most useful pages the students take away.

2.8 Six errors

Error 1 — Treating the signature as the consent. It is evidence of a process, not the process. A file with a signature and no note of what was explained is a file that will not survive a complaint.

Error 2 — Disclosing the document instead of the arrangement. Handing over a form and saying "it is about confidentiality" is not a consent conversation. The family learns what they need by being told, in speech, with examples, and being asked to say it back.

Error 3 — Leaving the limits until they are needed. A family that learns about the reporting duty after a disclosure has been made to them has been ambushed. The limits are disclosed at the start, and again as the case requires.

Error 4 — Not mentioning supervision and recording. The most commonly breached obligation in training clinics, and the one most easily fixed. "I take my work to a supervisor" is a sentence, not a form.

Error 5 — Assuming one parent's consent covers the household. Custody arrangements, guardianship, and the standing of a grandmother who is raising a child all affect who can consent. Ask, record the answer, and where it is unclear, obtain both.

Error 6 — Writing the consent note from memory. The note must be written in the session, in the family's words, and dated. A note reconstructed a week later is worth very little, and a tribunal will notice the distinction.

2.9 Culture, faith and consent

Consent is not always an individual act. In many of the households this book is written for, a decision to enter therapy is made by a family, and the person who signs is the person with standing rather than the person most affected. A clinician who insists on individual consent from each adult will sometimes be told what they want to hear and will not have consent in any meaningful sense. The workable procedure is to obtain the signature from the person with standing and to speak directly to each adult, including the grandmother and the adolescent, asking each whether they agree. The record then says both things.

The head of household and the wife's separate consent. A woman may be in treatment because her husband arranged it, and may not feel able to refuse. This is a consent problem, and its signs are the woman who says nothing in the first session and looks at her husband before answering. The clinical response is not to refuse treatment; it is to see her alone once, early, and ask whether she wants to be here. The answer is the consent.

Faith, authority and the referral route. Where a family has come because a pastor or an imam advised it, the authority in the room is partly elsewhere. The consent conversation should establish who else will be told about the treatment, because in these households the answer is frequently the religious community, and a family that assumes the therapist will brief the pastor and discovers otherwise will feel exposed.

Language and the interpreter. A consent conversation conducted through an interpreter is valid, provided the interpreter is briefed (Week 13 of MFT 703) and the family member is not used. A husband interpreting for his wife in a consent conversation about confidentiality is a contradiction in terms.

> **DEBATE**
> "A family that cannot read the consent form cannot give informed consent, and treatment should not begin." Argue for: consent requires understanding, a document is the standard evidence of understanding, and proceeding without it exposes the family to treatment they did not agree to. Argue against: literacy is not capacity, oral consent with a witness is a recognised and lawful route, and refusing treatment on the grounds of literacy denies the service to the people who need it most. Then write the clause you would put in your own consent policy to cover the oral route, and say what evidence you would keep.

2.10 Evidence note

What the research supports. The relationship between documentation quality and disciplinary outcomes is among the most consistent findings in professional regulation: inadequate records appear in a large minority of substantiated complaints, and in many the record failure is the finding rather than the clinical act. Studies of informed consent consistently find that recall of consent information is poor — typically less than half of what was explained is remembered a week later, and much less where the information is complex or the person is distressed — which supports the use of plain language, written summaries and teach-back methods. Teach-back, in which the client repeats the explanation in their own words, reliably improves comprehension.

What is weaker. The evidence on whether better consent processes improve clinical outcomes is thin. Consent documentation improves defensibility, and it plausibly improves the therapeutic alliance, but the trials are few and the outcome measures are satisfaction rather than change.

What is not supported. That a signature is sufficient. That longer forms produce better consent — the evidence points the other way, with comprehension falling as length rises. And that the limits of confidentiality deter families from disclosing — the finding is the reverse, that clear disclosure increases trust, which is why the sequence in 2.4 puts the limits after the clinician has established the relationship rather than before.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same consent form, explained to one family in a first session and posted to another with an appointment letter, produces consent in one case and a complaint in the other.

2.11 Key terms

Code of ethics — a professional body's enforceable statement of minimum standards.

Informed consent — agreement to treatment given by a person who understands what is proposed, its risks, its alternatives and its limits, and who is free to refuse.

Assent — a child's or young person's agreement to treatment, given in language they understand, where the legal consent rests with a parent or guardian.

Capacity — the ability to understand and weigh information relevant to a decision; distinct from age and from literacy.

Mature minor — a young person judged able to consent to treatment without a parent, under some jurisdictions' law.

Teach-back — asking the client to restate the explanation in their own words, as a check on comprehension.

Therapeutic secret — information held by the therapist from one member of a couple or family, known to the other members not to be shared.

Disclosure — the giving of information; not the same as consent, which requires understanding and agreement.

Witness — a person present at an oral consent conversation who can later attest to what was said.

Scope of consent — the limits of what was agreed; consent to discuss medication is not consent to describe a household.

2.12 Self-test

1. Name the ten core obligations and the two that generate most complaints.
2. Why is the signature the weakest part of the consent file?
3. Give the eight components of informed consent for couple therapy.
4. What is the difference between consent and assent, and why does the distinction matter clinically?
5. In the Balogun case, what exactly did the family consent to, and what was disclosed beyond it?
6. What are the four limitations of confidentiality, and how should they be framed to a family?
7. What is teach-back, and why does it matter more than the form?
8. What would you write in the record to evidence that consent happened?

Answers. (1) Competence; informed consent; confidentiality; multiple relationships; sexual intimacy; fees; advertising; research ethics; cultural competence and non-discrimination; legal compliance — the two that generate most complaints are inadequate records and failure to document consent. (2) Because it evidences a transaction rather than an understanding; a family can sign a document they cannot read and have consented to nothing, and a tribunal will say so. (3) Who the client is; who is seen and in what combination; the confidentiality and secrets policy; recording and supervision; fees and cancellation; the limits of confidentiality; the complaints route; the right to withdraw. (4) Consent is the legal authorisation, held by an adult or a guardian; assent is the young person's agreement, which is not legally required and is clinically decisive, because a child who did not assent withdraws by the third session. (5) Consent was to discussion of Mama's medication with her physician; the disclosure included the daughter-in-law's exhaustion, the family's handling of care, and a judgement that they had not been honest — none of which was within the scope. (6) Risk to self, risk to others, abuse of a child or vulnerable adult, and a court order — framed as promises about what the clinician will do ("I will tell you before I tell anyone"), with examples rather than abstractions. (7) Asking the client to restate the explanation in their own words; it matters because recall of consent information is poor and because the client's words, recorded, are the evidence of understanding. (8) What was explained; in what language and by whom; what the family said back; who witnessed it; the date; and what was agreed.

2.13 Reflective task

Find a consent form — any clinic's, any profession's — and read it as though you were a frightened parent with two children and no time. Then answer three questions: what does it tell you that you need to know? what does it fail to tell you? and what would you have to ask, out loud, to understand what will happen to your information? Most students find that the document answers the clinic's questions and not the family's, which is the difference between a release and a consent.

2.14 Further reading

- AAMFT Code of Ethics — read the current version, and the interpretive statements, not a summary.
- APA Ethical Principles of Psychologists and Code of Conduct — for comparison on the three issues where the codes disagree.
- Beauchamp, T. and Childress, J. on informed consent — the analytical account of the eight components.
- Schenker, Y. et al. on teach-back — the evidence on comprehension and recall.
- Gutheil, T. and Gabbard, G. on boundary violations — the documentation chapter, which is the most useful practical text in this area.

Next: Week 3 — Confidentiality in the multiperson system.

Week 3 — Confidentiality in the multiperson system

There is no single confidentiality in family work; there is a **policy** you negotiate.

In individual therapy confidentiality is a wall with four doors. In family therapy it is a room with several people in it, each of whom has a different claim on the information, and the therapist's task is not to defend a wall but to design and run a room. That is a different skill, and the codes — written for the wall — describe it badly.

Three workable models.

Model	Rule	Advantages	Risks
No-secrets	Anything told individually may be shared with the couple	Simple, transparent	Individual may withhold important material
Full secrecy	Individual disclosures are kept private	Protects individual therapy	Creates a therapeutic secret that distorts the couple work
Selective (negotiated)	The therapist and client agree in advance what will be shared	Flexible and realistic	Requires skill and documentation

Most experienced couple therapists operate a **modified no-secrets policy**: they will not keep a secret that materially affects the couple's work, and they tell each partner this before the first individual session. This protects both the relationship and the therapist.

The distinction that makes the policy workable: privacy is not secrecy. Privacy is information a person chooses not to share, and it is legitimate — a client's history of sexual abuse, her feelings about her mother-in-law, what she earns. Secrecy is information that is being kept from someone in a way that distorts the treatment: an affair in progress, a plan to leave, a diagnosis that the other partner believes has been excluded. A policy that cannot distinguish the two will be experienced by the family as a demand for total exposure, and they will keep everything.

Why the therapist cannot be neutral about a therapeutic secret. A secret is not a piece of information stored; it is a structural feature of the therapy. Once the therapist knows something the other party does not, every intervention is affected: she cannot ask a question that would lead there, she cannot respond naturally to an account given in good faith, and she is, in the technical sense, deceiving one client on another's behalf. A therapy in that condition is not couple therapy; it is two individual therapies conducted in the same room.

Documentation. Record the policy in the file, in the family's words if possible, and note the date it was explained. If a secret arises, record what you did and why.

> **CASE**
> During an individual session, a wife discloses that she has been having an affair for two years and has no intention of ending it or telling her husband. No one is at physical risk. Under a no-secrets policy, you tell her at the outset of the individual session that this cannot stay private if the couple work is to continue; you then help her decide how and when to disclose, with a deadline. Under a selective policy, you would negotiate what can be shared. What you must not do is carry the secret silently for months while conducting "couple therapy".

3.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The wall and the room	Why individual confidentiality does not transfer to family work	Draw the information map of a family they know
8–20	The three models	Each model, its logic, its failure mode	Choose a model for four different referral types, with reasons
20–32	Privacy against secrecy	The distinction, worked on eight examples	Sort eight disclosures into privacy or secrecy
32–44	The modified no-secrets policy	The four parts of the sentence, and how to say it	Rehearse the sentence in pairs, twice each
44–56	Who else holds information	Schools, courts, employers, insurers, religious bodies	Map the information flow for a school referral
56–68	The individual session inside family work	Why it is offered, how it is structured, how it ends	Script the first two minutes of an individual session
68–80	When a secret has to move	The five-step process, with the affair case	Run the process in role, with a resistant discloser

Minutes	Segment	What the lecturer does	What students do
80–90	Documentation	What goes in the file, and what does not	Write the note for the case

The teaching decision that makes the week work. The week turns on one demonstration: the lecturer places three chairs, sits in one, and plays the therapist who has just been told about the affair. She then conducts twenty minutes of ordinary couple therapy — asking the husband about his week, responding to the wife's account of a disagreement — while the class watches the impossibility accumulate. The exercise is uncomfortable and it is unforgettable: by the end, every student has seen that the therapist is lying, not by commission but by the shape of her questions.

The sentence the lecture must land. "A secret does not sit in a drawer. It changes every question you can ask, and after a month it has changed the therapy."

3.2 Five concepts that organise the week

1. The client is a construction, not a fact. In family work "the client" is whatever the therapist and the family have agreed it is, and that agreement determines who holds the confidentiality. A couple can be the client, with each partner a participant; a family can be the client, with the children participants; an individual can be the client, with the family seen as context. Each construction is legitimate; none is neutral; and the therapist must choose one and say so.

2. Confidentiality is owed to the system, not distributed among its members. If the couple is the client, the confidentiality is held by the couple jointly, which means neither partner can demand it against the other. This is the sentence that resolves most confusion: "I cannot keep something from the client, and the client is both of you."

3. Privacy is a legitimate asymmetry. Members of a family know different things about each other, and that is ordinary. A therapist is not obliged to equalise knowledge, and a policy that requires it is not a confidentiality policy but a transparency rule, which is a different instrument with different costs.

4. Secrets distort, and distortion is measurable. The practical test of whether a secret is a therapeutic secret is whether it changes what the therapist does. If she has stopped asking about the weekends, or begun to steer away from a topic, or finds herself relieved when a session ends early, the secret is already operating and the policy has failed.

5. Institutions are third parties with their own claims. A school, a court, an employer, an insurer, a child protection agency and a religious body all have information claims, and each has a different legal standing. The therapist's obligation is to map them at the start, tell the family who they are, and never to discover one in the middle of a case.

3.2a The modified no-secrets policy, in four parts

The policy is a sentence with four components, and each component prevents a specific failure.

Part 1 — The promise of individual sessions. "Either of you may ask to see me alone, and I will usually say yes." Without this, the family believes there are no individual sessions, and the request, when it comes, is experienced as a breach.

Part 2 — The limit. "But I will not keep a secret that affects this relationship." The limit is stated in terms of effect, not of content, because content cannot be predicted and effect can be judged.

Part 3 — What the therapist will do. "If something like that comes up, I will tell you both that something has been said — not what — and I will help whoever it is to decide how and when to say it." The distinction between telling that something was said and telling what was said is the mechanism that makes the policy usable.

Part 4 — The deadline. "And I will ask for it to be said here, within a stated time, rather than leaving it open." A secret with no deadline is a secret; a secret with a deadline is a task. The deadline is negotiated but not indefinite, and the therapist writes it down.

The four failure modes the parts prevent. Without part 1, the request for an individual session feels like a rupture. Without part 2, the therapist carries secrets by accident. Without part 3, the disclosure of that something was said becomes a guessing game that is worse than the secret. Without part 4, the process has no end and the family settles into a stable, comfortable dishonesty.

3.3 The method: deciding where a disclosure goes

Six questions, asked in order, whenever a family member tells the therapist something in confidence. The method takes about five minutes of thinking and it converts an alarm into a decision.

Question 1 — Who is the client, as agreed? The answer determines everything else. If the couple is the client, the disclosure belongs to the couple and cannot be kept from one member indefinitely. If the individual is the client and the family is context, the disclosure is hers to control.

Question 2 — Is anyone at risk? A yes takes the case out of this method and into Week 8 or 9. Risk is always the first question, because the confidentiality policy does not survive it and the family was told so at the start.

Question 3 — Is this privacy or secrecy? Test it with one question: does knowing this change what I do in the joint sessions? If yes, it is secrecy, whatever its subject. If no, it is privacy and it stays.

Question 4 — Who else's information is in it? Most disclosures are not about one person. An affair involves a third party; a diagnosis involves a family's history; a plan to leave involves children. The therapist is not only holding the confider's information, and the other people in it have claims.

Question 5 — What happens if it moves, and when? Who will be told, by whom, in what words, with whom present, and on what date. The therapist's job is to make this a plan rather than an event.

Question 6 — What is my part, and what is not? The therapist does not disclose the secret; the client does. The therapist's role is to insist that it moves, to help plan how, to be present, and to manage the session afterwards. A therapist who takes on the disclosure has taken the responsibility for it, and the client has been relieved of the only act that could repair the relationship.

The rule that governs the six. The therapist never lies. She may decline to answer, and she must say that she is declining and why: "I am not going to answer that, and I want to tell you that I am not, rather than let you think I have." The distinction between a refusal and a lie is the whole of the professional position.

3.3a The individual session inside family work

An individual session within a couple or family treatment is a clinical instrument with a purpose, a structure, and an end. Used badly, it is the mechanism by which therapy is destroyed.

Why offer one.

- To assess risk in private, where a partner's presence would make disclosure impossible.
- To hear one person's account of something they cannot say in front of the other — usually fear, shame, or the wish to leave.
- To prepare a person to say something in the joint session, which is the most common and most useful reason.
- To assess for coercive control, which cannot be assessed with the controller present.

The rule that governs the offer. Never hold an individual session with one member of a couple or family without having stated the policy, in front of both, before the first one. The session is not the problem; the undisclosed session is.

The structure, in five minutes and forty. First two minutes: restate the policy — "you know I will not keep a secret that affects the relationship; is there anything you want to say in that knowledge?" Middle thirty: listen, assess, and do not ask leading questions about the other partner's conduct. Last eight: agree what, if anything, moves, and how. The session does not end with a half-disclosure and a reassurance.

The sentence that ends it well. "So: what I understand is that you want to leave, and you have not told him, and you are not ready to. I am not going to tell him. I am going to ask you to tell him, here, within two weeks, and I will help you plan what you say. If you have not told him in two weeks, I will tell him only that something has been said and that he should ask you — and I will tell you first that I am doing it."

What makes an individual session unsafe. Three things. It is offered without the policy being stated. It becomes the only place where anything true is said, which is the classic pattern of a couple therapy that has quietly become two individual therapies. Or it is used to gather information about the other partner, which converts the therapist into an investigator.

3.4 Confidentiality and the institutions around the family

A family does not arrive alone. The information map below is drawn on the whiteboard in the first session of any case with an institutional referrer, and it frequently changes the treatment.

The school. Holds attendance, behaviour, academic and safeguarding records. Has a legitimate interest in the child's functioning, no right to the therapy's content, and a tendency to expect a report. The therapist's position: the school may be given what the family agrees to, and where the school is the referrer it must be told at the outset what it will and will not receive.

The court. Has power to compel records and testimony, and is the only party on the list that can override the family's wishes entirely. (Week 11.)

The employer, the insurer, the employee assistance programme. Frequently the payer, and frequently the party with the weakest claim and the strongest expectation. The consent conversation must establish what the payer will be told: usually attendance and nothing else.

The child protection agency. Has statutory powers that vary by jurisdiction and frequently override confidentiality. The family must be told, at the start, in plain language, what the therapist is obliged to report and to whom.

The religious body. Rarely has a legal claim and frequently has a social one. Where a pastor or imam referred the family, or is supporting them, the question of what the therapist will tell the religious authority must be settled early, because families in these households often assume the therapist will brief the clergy and may be distressed to discover otherwise — or, worse, may assume confidentiality and discover the briefing.

The extended family. The most consequential party in many of these cases and the least considered. The grandmother, the uncle who pays, the aunt who is a nurse: each may be told something by a family member and each may act on it. The therapist cannot control this and should not pretend to; the workable sentence is "what is said in this room may be repeated outside it by any of us, and I cannot prevent that. What I can control is what I say."

The rule for the map. Draw it in the first session, name every party, and ask the family who has a claim in their view. The map is then recorded, and every subsequent request for information is answered by reference to it rather than improvised.

3.4a Four scenarios at the boundary

The categories are easy in the abstract and hard at four in the afternoon. The four scenarios below are the ones that most often divide a supervision group.

Scenario one — the past affair. A husband discloses, in an individual session, an affair that ended six years ago, before the couple's second child was born. He is not going to tell his wife and says it is "over and irrelevant". Analysis: secrecy, not privacy, because it changes what the therapist does — she can no longer explore the wife's account of the last six years without knowing that the husband holds a different history. The end of the affair

does not end the secret's effect; it only removes one of the reasons for urgency. The policy applies, the deadline is set, and the therapist is honest that the disclosure may be worse for the marriage than the affair was.

Scenario two — the diagnosis. A wife discloses a new diagnosis of a chronic illness, and asks that her husband not be told yet. Analysis: this is closer to privacy, and the test settles it — the opposite: knowing this does change what the therapist does, but the change is in her attention rather than her honesty, and the question of who knows the diagnosis belongs, first, to the patient. The exception is where the illness bears directly on the treatment's subject matter, as it would if the couple presented with the husband's complaint that his wife has "lost interest". Then the content of the illness can remain private and its effect must be speakable: "there is something medical going on that she is dealing with, and it is relevant to what you are describing; she will tell you when she is ready." That is not a lie and it is not a disclosure.

Scenario three — the wished-for pregnancy. A woman in a couple treatment discloses that she has stopped her contraception without telling her husband. Analysis: the clearest kind of secrecy, because the treatment is about the couple's reproductive decisions and the therapist would be participating in a deception about their subject. It is also a case where the therapist's own views about reproductive autonomy will be activated, and the professional requirement is to state the limit without moralising about the choice.

Scenario four — the adult child's secret. A mother in a family treatment discloses that her 24-year-old son has been using drugs, and asks the therapist not to raise it with the son, who is also attending. Analysis: the son is an adult and a client; his mother's disclosure is information about him that he has not given. The therapist cannot promise the mother silence, because she is now conducting a family treatment in which she knows something the son does not know she knows — and she cannot disclose it to him either, because the mother told her. The workable resolution is the mother's own act: "I need you to tell him that you know, and I will help you do it, because I cannot work with the two of you while he thinks this is not being spoken about."

The pattern in all four. The answer is rarely "tell" or "do not tell". It is who tells, in what words, by when, with the therapist doing what.

3.4b The supervisor and the secret

A therapist holding a secret needs supervision and frequently avoids it, because the secret feels like a failure and because describing it means saying it aloud. This is the mechanism by which secrets damage therapists: the case that most needs discussing is the one least discussed.

The rule. Any case in which the therapist is holding information from a client — even legitimately, even temporarily — is presented in supervision within the month.

What supervision is for, in these cases. Not permission, which supervision cannot give. Three things: (1) a check on whether the therapist's reasoning is clinical or personal, since the wish to protect a client from the consequences of disclosure is often the therapist's own; (2) an outside view on the risk analysis, which deteriorates under

pressure; (3) the documentation, because a supervisor who has read the reasoning becomes the strongest evidence that the decision was considered.

What the supervisor should not do. Instruct the therapist to disclose. The decision and the responsibility belong to the treating clinician, and a supervisor who directs it removes the judgement from the person who will have to defend it.

The audit question for the therapist. At the end of each month: is there anything I know that one of my clients does not know I know? If the answer is yes and there is no written plan with a date, the case needs supervision this week, not next.

3.5 Case: two years, no intention

Referral. Mrs Nkiru Adeleke, 36, a bank branch operations manager, telephones about her marriage. "We have not spoken properly in three years. I want to know whether it can be saved." Her husband, Segun, 39, a pharmacist, attends the first session reluctantly: "she decided we needed this, so here we are."

The household. Nkiru and Segun, married eleven years. Two children, 8 and 5. Segun's mother lives ten minutes away and is "very involved". Nkiru travels for work two or three nights a month.

The first session. The presenting complaint is "we are like flatmates". Segun's example: "she is on her phone until midnight". Nkiru's example: "he asks me about the children and never about my day." There is no violence, no substance misuse, no financial crisis. The therapist states the modified no-secrets policy, in the four parts, in the first session, and both agree. The note records it.

Session two — the individual request. Nkiru telephones: "I need to see you alone." The therapist offers the next appointment, and sees her for forty minutes. In the first eight minutes, Nkiru says: "I have been seeing someone for two years. He is a colleague. I am not going to stop and I am not going to tell Segun. I want to know whether I can save my marriage at the same time."

The application of the six questions.

1. Who is the client? The couple. The policy was stated to both and agreed by both. Nkiru was told, in Segun's presence, that a secret affecting the relationship would not be kept.

2. Is anyone at risk? No physical risk. There is a risk of an STI to Segun, which is a health risk and not a safety risk, and it is relevant — a therapist who treats this as a purely relational matter has missed something. There is a risk to the children's stability, which is a future harm rather than a current danger.

3. Privacy or secrecy? Secrecy, unambiguously. The test — does knowing this change what I do in the joint sessions? — is answered in one word: yes. Every question about the weekends, about her travel, about her withdrawal becomes unaskable.

4. Whose information is it? Nkiru's, Segun's, the children's, and the third party's — who has a wife of his own, which Nkiru mentions in passing and which the therapist does not pursue. The information is not one person's property.

5. What happens if it moves, and when? The plan has to be made now, in this session, or the policy is decoration.

6. What is my part? To insist that it moves, to help plan how, to be present, and not to disclose it.

The intervention, in the session, in sequence.

First, the reminder, not as a threat but as a fact. "When we started, I told you both that I would not keep a secret that affects the marriage. I meant it then and I mean it now. So I am going to say what I can and cannot do, and then we will decide what happens."

Second, the question that establishes the delay. "When did you decide that you were not going to tell him?" The answer is "about a year ago". The therapist names it: "so for a year you have known, and I have known for ten minutes. That is the difference I have to work with."

Third, the refusal to collude, stated as a limit on the therapist rather than on the client. "I am not able to run a couple therapy in which I know something of this size and your husband does not. Not because of a rule — because I would be lying to him every week, and he would be paying me to do it."

Fourth, the three options, laid out plainly. (a) She tells Segun, with the therapist's help, in a session, by a stated date. (b) She ends the couple therapy here, and the therapist explains to Segun that the couple work cannot continue — without giving a reason — which is honest and leaves Segun with a mystery. (c) The therapist tells Segun that something has been disclosed which he needs to discuss with his wife, and Nkiru tells him the content herself. Option (b) is included deliberately: a client who is not ready must have an exit that is not a lie.

Fifth, the deadline and the recording. Nkiru chooses (a), with a date three weeks out and one session in between to plan the words. The therapist writes it in the note: "client disclosed an ongoing relationship of two years; reminded of the policy agreed at session one; client agreed to disclose to her husband by [date], with therapist support; if not disclosed by that date, therapist will inform husband that a disclosure has been made and that he should ask his wife."

Sixth, the health question, which is separate and cannot wait three weeks. "There is one thing I am going to ask you to do this week, and it is not about your marriage. Get a sexual health check, and tell me you have done it. If you will not tell him, you still cannot leave him exposed." Nkiru agrees, and does it, and reports at the next session.

The disclosure session, in outline. Nkiru tells Segun in the fourth session, in the therapist's room, in eleven sentences that she had rehearsed with the therapist in the third. Segun's response is not what anyone predicted: he is not surprised. He says, "I knew something. I thought it was my fault." The session that follows is not about the

affair; it is about a three-year silence in which both of them were protecting the other from a truth they each half-knew.

What the case teaches.

1. **The policy must be stated before it is needed.** Nkiru's decision to disclose was made in a session in which the alternative was clearly worse — ending the therapy — and that alternative existed only because the policy had been agreed in session one.
2. **A deadline converts a secret into a task.** Without a date, the therapist would have carried it for months, and the therapy would have become the deception the policy exists to prevent.
3. **The health question is not part of the confidentiality decision and must not be lost in it.** A therapist absorbed in the ethics of disclosure can forget that a third party is being exposed to an infection.
4. **The predicted catastrophe is frequently not the actual one.** The disclosure did not end the marriage; it ended a silence. That outcome is not guaranteed and it is common, and a therapist who assumes the worst will delay the disclosure in order to protect a relationship that is already being damaged by the delay.

3.5a When the client refuses the deadline

A real and frequent outcome, and the case in which the therapist's policy is actually tested.

What refusal looks like. "I am not going to tell him, and I am not going to stop seeing you. You said you would help me, and now you are threatening me." The accusation of betrayal is genuine and it is also the pressure that most therapists find hardest to withstand.

What the therapist says. Four sentences, in this order. "I am not threatening you; I am telling you what I can and cannot do, which is the same thing I told you on the first day." "I will not tell him what you have told me — that has not changed." "What I will do, on the date we agreed, is tell him that something has been disclosed that he needs to raise with you." "And I will keep working with you, either way, because you are not being abandoned for telling me the truth."

Why the fourth sentence matters more than the other three. The client's fear is that honesty will cost her the therapist. If it does, the lesson she takes is that honesty is dangerous — which is precisely the lesson that produced the affair's secrecy in the first place.

What if the client ends the therapy instead? Then the therapist writes to confirm the ending, offers to meet the husband to explain that the couple work has stopped without explaining why, and records the reasoning in full. The letter matters: it is the evidence that the decision was the client's, and it is the only document that will exist if the husband later complains.

The rule underneath all of this. The therapist's obligation is not to the secret and not to the marriage. It is to the honesty of the treatment she is conducting. A treatment she

cannot conduct honestly is one she must not continue, and saying so is the professional act.

3.6 Transcript: the individual session, minutes seven to twenty

Fifteen turns from the session in the case, compressed. The annotation marks each move against the six questions.

> **T1 Nkiru:** I have been seeing someone for two years. He is a colleague. I am not going to stop and I am not going to tell Segun. — Three facts and a decision, delivered quickly, which is the manner of someone who has rehearsed the opening and intends to control the frame. **T2 Therapist:** (pause) Thank you for telling me. (pause) You know what I said on the first day, in front of Segun, about secrets. — No shock, no reassurance, and an immediate return to the agreement. The pause before "thank you" is deliberate: it acknowledges the disclosure without approving of it. **T3 Nkiru:** I know. But you cannot tell him. It is my life. **T4 Therapist:** It is your life. And it is also my therapy, and I cannot run it with this in it. — The move that keeps the conflict where it belongs. The therapist does not argue about the ethics of the affair; she states a limit on her own practice, which is not negotiable by the client's view of marriage. **T5 Nkiru:** So you are going to tell him. **T6 Therapist:** No. I am not going to tell him what you have told me, and I am not going to tell him anything at all today. — The distinction that the client has not yet heard: the therapist's limit is on carrying the secret, not on disclosing it. **T7 Nkiru:** Then what? **T8 Therapist:** Then we have a choice to make, and I want you to make it rather than me. Three options. (lays them out) — Options rather than a demand. The client is being given control of the manner, which is the only thing she can be given control of, the fact having been established. **T9 Nkiru:** (long pause) If I tell him, I lose everything. **T10 Therapist:** You may. I do not know. What I know is that if you do not, you have already lost something, and it has been going on for a year. — The honest answer rather than the comforting one. Note that the therapist says "I do not know", which buys the credibility for the sentence that follows. **T11 Nkiru:** (crying) I did not plan this. It started because I could not talk to him. He is not a bad man. **T12 Therapist:** (after a pause) I believe you. And that is the thing we have to work out — not whether he is a bad man, but what happened in this marriage that made it impossible for you to say what you needed. (pause) But not today. Today is about whether you can tell him the truth, because nothing else can be worked out until you can. — The reframe: the affair is the presenting event and the marriage is the subject. The therapist promises the second conversation and does not have it now, which is what makes the promise credible. **T13 Nkiru:** (pause) If I do it, will you be there? **T14 Therapist:** Yes. In this room, with me in it, and after that we will do the hardest session of your life and I will still be here in the following week. — The commitment, which is the thing that makes the disclosure survivable, and which is not a professional obligation and is the whole of the therapeutic relationship in one sentence. **T15 Nkiru:** (after a long pause) Give me three weeks.

What the transcript shows. Three things that a policy document cannot teach. First, the therapist never moralises about the affair, which keeps the client in the room. Second, she refuses twice with the same sentence, and the repetition matters — a limit stated once is heard as an opening position. Third, she ends the session on the client's own deadline, which means the client leaves with an obligation she set rather than one imposed.

3.7 Laboratory: the secrets protocol

Aim. To design, in writing, a confidentiality and secrets protocol for a named service, and to defend its four parts under pressure from a client who is testing it.

Procedure. Groups of three: therapist, client, observer. Each group is given a service description (a school counselling service; a private couple practice; a church-based clinic; a hospital family therapy unit) and must produce a one-page protocol in thirty minutes. The client role is then briefed to test the protocol in three ways — by asking for an individual session before the policy has been stated, by disclosing a secret and refusing the deadline, and by asking the therapist to lie to a third party. Twenty minutes of role-play. The observer marks the therapist's adherence to the protocol the group itself wrote, which is the point of the exercise: most protocols are unwritable as written.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The four parts	Two or fewer present	Three present	All four, specific, with the action the therapist will take at each point
Privacy against secrecy	Both treated the same	Distinguished, but the test is not stated	Distinguished with the operational test, and examples from practice
The institutions	Not addressed	Named	Mapped, with what each party may be told and who decides
Under pressure	Abandoned the protocol when tested	Held it, but argued	Held it, stated the limit as a limit on the therapist, and offered the client control of the manner
Documentation	None	A note exists	The note records what was said, the agreement, the date, and the contingency

The distinction that carries most of the marks. The top band requires the student to hold the protocol without moralising. Groups routinely win the argument and lose the client: a therapist who says "you are being dishonest" has stated a true thing and has ended the possibility of the disclosure.

3.8 Six errors

Error 1 — Stating the policy after the secret. The policy is worth nothing once the secret exists, because the client reasonably hears a change of rules. It is stated in session one, in front of everyone, and recorded.

Error 2 — Confusing privacy with secrecy and demanding total transparency. A policy that requires the disclosure of everything produces a family who disclose nothing. The two categories must be distinguished in the explanation, with examples from ordinary life.

Error 3 — Carrying the secret "for now". The phrase that has ended more couple therapies than any single act. There is no "for now"; there is a date, or there is a lie.

Error 4 — Telling the client's secret to protect the client from the consequences. The therapist who telephones the husband "for her own good" has broken a confidence and taken away the only act that could have repaired the relationship.

Error 5 — Refusing the individual session altogether. A therapist who will not see one partner alone cannot assess risk, cannot prepare a disclosure, and cannot hear the thing that is most true. The individual session is not the danger; the unstated one is.

Error 6 — Forgetting the third parties inside the secret. An affair involves another household; a diagnosis involves a family history; a plan to leave involves children. A confidentiality analysis that counts only the people in the room has missed most of the people in the case.

3.9 Culture, faith and the multiperson system

Confidentiality is a Western construct in some of these households. The idea that an individual holds information that a spouse has no right to is not universally shared, and in many of the families this book is written for the operative assumption is that a married couple has one information economy. A therapist who announces an individual confidentiality without explanation will be heard as introducing a foreign and suspicious practice — "why would you need to speak to my wife alone?"

The translation that works. Two sentences. First, the practical one: "I see each of you alone once, because there are things people cannot say in front of a husband or a wife, and I need to know them to help." Second, the limit: "anything that affects the marriage, I will not keep." The family that understands why the individual session exists agrees to it far more often than one that has been told only that it is a right.

The elder and the family's information. Where a grandmother or an uncle holds authority in the household, they may expect to be told what happens in sessions, and their expectation may be shared by the family. The workable position is not refusal but specification: "I will tell you what I am doing and whether it is working. I will not tell you what is said in the room." Most elders accept this, because it gives them the thing they actually want, which is to know that something is being done.

Faith communities and the circulation of information. A pastor who refers a family will frequently expect a report, and the family may expect the pastor to receive one. Both expectations must be addressed in the first session, and the answer recorded. Where a religious authority is part of the family's support, the therapist may offer a summary with the family's written agreement, which is a legitimate arrangement and is almost never offered.

Shame and the disclosure of family business. In a shame-oriented community, a disclosure inside the family may become a disclosure in the community, and the family knows this better than the therapist does. That is a reason to plan the disclosure carefully and is not a reason to avoid it — and it is a reason to ask, before any disclosure is planned, who outside the household will come to know.

DEBATE

"In couple therapy, the therapist should never hold information from either partner." Argue for: the client is the couple, deception is incompatible with therapy, and a therapist who holds a secret is no longer working with a couple. Argue against: privacy is a condition of individual disclosure, a strict no-secrets rule drives the most important material underground, and a woman who cannot speak freely about her own fear will not disclose it at all. Then state the policy you would actually operate, and the case that would make you change it.

3.9a A record of the decision, written out

The note is the therapist's only defence and the family's only protection. It is written the same day, and it does four things: records the policy, records the disclosure, records the reasoning, and records the plan. An example, adapted from the Adeleke case.

SESSION 4 (INDIVIDUAL, 40 MIN).

Client N.A. disclosed an ongoing relationship of approximately two years with a colleague. Client states she does not intend to end it and does not intend to tell her husband. **Policy.** Modified no-secrets policy stated to both partners in session 1 and agreed by both; copy in file. Policy restated to client at the start of this session. Client acknowledged it. **Reasoning.** Client system is the couple. Disclosure affects the couple treatment and would require the therapist to avoid material relevant to the presenting complaint; not compatible with continued couple work. No current risk of violence identified. Risk of STI transmission to the husband noted and addressed separately (below). No statutory duty engaged. Third parties: the other party's spouse is not a client and has no claim on the file. **Plan.** Client elected to disclose to her husband in session 5, with therapist support. Rehearsal agreed for session 5 opening. If disclosure has not occurred by [date], therapist will inform the husband that a disclosure has been made and that he should raise it with his wife, having first told the client that she will do so. Client informed of this contingency and stated she understood it. **Health.** Therapist advised a sexual health screen this week, independent of the disclosure plan. Client agreed. To be confirmed at session 5. **Supervision.** Case to be presented at next supervision, [date]. **Signed.** [Therapist], [date], [time].

Why each part exists. The policy paragraph establishes that the client was told before she spoke, which is the difference between a trap and an agreement. The reasoning paragraph shows that the decision was clinical rather than moral, which is what a regulator reads for. The plan paragraph, with the date, is what makes the note a record of action rather than a record of concern. The health paragraph prevents the most common omission in these cases. And the supervision line is the strongest single sentence in the note, because it shows the therapist did not decide alone.

The version not to write. "Client disclosed an affair; discussed confidentiality." That note has no reasoning, no date, no plan, and no policy — and if the husband complains in a year, it establishes only that the therapist knew.

3.10 Evidence note

What the research supports. Survey and vignette studies consistently find that couple and family therapists report operating modified no-secrets policies and consider them more workable than either rigid alternative, while acknowledging that the policy is difficult to enact under pressure. The clinical literature on "therapeutic secrets" is largely theoretical, but it converges on the observation that a secret held by the therapist changes the therapist's behaviour — avoidance of topics, reduced spontaneity, and a measurable drop in the couple's engagement — which is consistent with the triadic model of the secret as a third participant.

What is weaker. There is almost no empirical work on outcomes. No trial has compared no-secrets, full secrecy and negotiated policies, and the question may be unanswerable by trial design because the policy is chosen partly by the population: a practice serving couples in high-conflict or coercive relationships will reasonably operate differently from one serving voluntary, low-risk couples.

What is not supported. That a no-secrets policy is the standard of care in any enforceable sense — the codes do not require it. That holding a secret is always harmful — some privacy is protective, particularly in cases involving past abuse or a partner's history of violence. And that the policy should be applied identically across cultures, which the cross-cultural literature on family information economies does not support.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same secret, disclosed in session two with a therapist who has stated the policy and has a supervisor, and disclosed in session twenty with a therapist who has never mentioned it, produces two different treatments and two different families.

3.11 Key terms

Confidentiality — the professional duty not to disclose information given in the treatment relationship, subject to specified exceptions.

Privacy — information a person legitimately chooses not to share; compatible with treatment.

Therapeutic secret — information held from one member of a couple or family that materially affects the treatment.

Modified no-secrets policy — a stated policy that individual disclosures will not be kept where they affect the couple or family work, with a process for their disclosure.

Client system — the person or persons agreed to be the client; the holder of the confidentiality.

Third-party claim — an institution's or individual's claim on information, legal or social.

Information map — a drawn record of who holds what information about a family, and who decides.

Deadline — the dated commitment that converts a secret into a task.

Collusion — an arrangement in which the therapist's silence serves the client's avoidance.

Duty to warn / protect — the legal or ethical obligation to act on a serious threat to an identifiable third party; jurisdiction-dependent.

3.12 Self-test

1. Why does individual confidentiality not transfer to couple therapy?
2. State the four parts of the modified no-secrets policy, and the failure each prevents.
3. Give the operational test that distinguishes privacy from secrecy.
4. What is the difference between telling a partner that something has been said and telling them what?
5. In the Adeleke case, why was the health question separated from the confidentiality decision?
6. What would you do if the client refused the deadline?
7. Name five institutions with claims on a family's information, and what each may legitimately be told.
8. Why is "carrying the secret for now" the most damaging option?

Answers. (1) Because the client is the couple jointly, so neither partner can demand confidentiality against the other, and a therapist holding information from one member is deceiving a client on another's behalf. (2) The promise of individual sessions (prevents the request being experienced as a rupture); the limit stated in terms of effect (prevents accidental secrecy); the commitment to tell both that something has been said (prevents the guessing game); the deadline (prevents a stable dishonesty). (3) Does knowing this change what I do in the joint sessions? If yes, it is secrecy. (4) Telling that preserves the discloser's control of the content and the timing, and it converts the therapist's position from deception to pressure; telling what removes the client's only remaining act of repair. (5) Because the exposure of a third party to infection is a health matter with its own urgency, and a therapist absorbed in the ethics of disclosure can lose it. (6) Hold the limit, do not disclose the content, state the date, inform the partner on the date that a disclosure has been made, and keep working with the client so that honesty does not cost her the therapist. (7) The school (attendance and functioning, not therapy content); the court (whatever is compelled); the insurer or employer (usually attendance only); child protection (what the statute requires); the religious body (a summary with written agreement, or nothing); and the extended family, who have no claim and considerable social power. (8) Because there is no "for now": the secret is either moving toward disclosure or it is being carried, and a carried secret changes every question the therapist can ask.

3.13 Reflective task

Think of a time you were told something in confidence that you could not keep from someone who needed to know. Write down what you did, and then answer: which of the six questions did you answer correctly at the time, and which did you answer afterwards? Most people find that the ethical reasoning came after the decision, which is normal — and it is the reason this course insists that the policy is stated before the case arrives.

3.14 Further reading

- Gutheil, T. and Gabbard, G. on secrets in couple and family therapy — the clearest treatment of the triadic problem.
- Margolin, G. on confidentiality in family research and therapy — the empirical discussion of the policy options.
- Doherty, W. and Boss, P. on family ethics — the relational account that underlies this week.
- AAMFT interpretive statements on confidentiality — the shortest authoritative source on the limits.
- Kirmayer, L. on the cultural shaping of disclosure — relevant to the section on family information economies.

Next: Week 4 — Secrets I: the individual disclosure.

Week 4 — Secrets I: the individual disclosure

Learning outcomes. By the end of this week the student can (1) distinguish the ethics of a secret told in individual therapy from the ethics of a secret told in couple therapy; (2) apply a seven-step protocol to an individual disclosure; (3) assess what a secret is doing for the person who keeps it, not only what it is hiding; (4) plan, rehearse and record a disclosure without coercing it; and (5) identify the countertransference that being trusted with a secret reliably produces.

The distinction this week exists to teach

Week 3 dealt with the couple as client. The therapist there had a policy — agreed in advance, applied when the secret arrived — and the policy converted the secret into a task with a date. This week deals with the harder and commoner situation: a person in individual therapy tells the therapist something that concerns someone who is not in the room and is not a client.

The temptation is to import the couple framework wholesale, and it is a mistake. In couple therapy the therapist has a duty to the system, and the system's honesty is a condition of the treatment. In individual therapy the therapist has a duty to one person, and the person's honesty is a condition of the treatment too — but the person's privacy is hers to spend, and the therapist has no standing to demand disclosure to a third party who is not a client at all.

What the therapist does have standing to do is something narrower and more demanding: to insist that the secret be visible inside the therapy. The distinction is the whole of the week.

- The therapist may not tell the husband. He is not a client, has no claim on the file, and telling him would be a plain breach.
- The therapist may not pretend the secret does not exist. A treatment conducted around an unmentionable fact is a treatment of the presenting complaint only, and it will fail in a way that looks like the client's fault.
- The therapist's working question is therefore not should this be disclosed? but what is this secret doing to you, and what would it take for you to be able to put it down?

The classification table. Before responding to any disclosure, classify it. The classification determines what happens in the next five minutes.

Type	Examples	What it obliges
Immaterial	A surprise party; a gift being planned; an ordinary private preference	Nothing. Keep it, and do not make it clinical
Material, no risk	A past termination; a debt; a diagnosis; an affair that has ended; a family history kept quiet	Work with the cost of secrecy; plan disclosure if the client chooses it; never coerce
Third-party risk	An undisclosed infection; a credible threat; a plan to harm	Assess; follow legal and public-health duties; document; disclose only what the duty requires
Child or vulnerable adult	Abuse, neglect, exposure to danger	Report under the statute; inform the client what will be reported
Risk to self	Suicidal intent, self-harm, a plan	Risk assessment and safety planning; break confidentiality only as required

Note what is not on the table: "immoral" is not a category, and "the client's family would want to know" is not a category. The therapist's own moral judgement of the secret is not an input to the classification, and the most common error in this week's work is a classification driven by the therapist's disapproval.

4.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	The individual case: why Week 3's framework does not transfer	Two contrasting vignettes, voted on before analysis
10–25	Five concepts: burden, avoidance, function, countertransference, self-concealment	Lecture with board diagram
25–40	The seven-step protocol; the classification table applied to five disclosures	Live sorting exercise
40–55	The cost ledger and the two-futures exercise	Demonstration with a volunteer, then pairs
55–70	The Bisi case, in full	Case presentation, students hold the decision
70–80	The transcript, annotated	Read aloud in three voices

Minutes	Content	Teaching form
80–90	Six errors; the record; what supervision is for	Short lecture, closing question

4.2 Five concepts that organise the week

Concept 1 — Keeping a secret is work, and the work is paid for in the body. The disclosure literature is consistent on this point: concealment requires continuous vigilance — monitoring what one says, to whom, in front of whom — and that vigilance has measurable costs in physiological arousal, sleep, intrusive thought and sense of authenticity. The clinical implication is not "therefore disclose". It is that the client's reported symptoms may be the cost of the concealment rather than the effects of the concealed event, and that this distinction changes the treatment entirely.

Concept 2 — Secrecy is maintained by avoidance, and avoidance generalises. The client avoids one topic; then avoids the conversations adjacent to it; then avoids the people with whom the topic might arise. In therapy the generalisation is visible as a narrowing of what can be discussed, and it typically looks like a treatment that "has gone flat" rather than like concealment. The therapist's first task is to notice the narrowing, which requires asking about it directly.

Concept 3 — The secret has a function, and it is usually protective. It prevents a divorce, a beating, a loss of standing in a family, a mother's distress. A therapist who treats the secret only as a problem to be solved is asking a client to give up protection before any substitute exists. The function is assessed, named aloud, and taken seriously — and the assessment frequently reveals that the protection is partly illusory.

Concept 4 — Being trusted with a secret produces a reliable countertransference. It is flattering, it creates a special bond, and it invites the therapist to become the client's only ally — which is precisely the position from which the therapist stops challenging her. The signs are familiar: the therapist looks forward to the session, feels protective, has a private understanding with the client, and has begun to dislike the absent husband on the client's evidence alone. Each sign is a prompt to supervision.

Concept 5 — What the client conceals from the therapist matters as much as what she discloses. Self- concealment — the general tendency to hide personal information — predicts poorer outcomes and weaker therapeutic alliances, and it operates on the therapist too. A client who has told you about a past termination may still be concealing the reason she chose you, or the fact that she has already consulted a lawyer. The therapist's own honesty about the limits of what she can work with is the main instrument against this.

4.2a What a secret looks like before it is told

Most secrets are visible for several sessions before they are disclosed, and the therapist's capacity to notice them is a skill that can be trained. The markers below are drawn from the transcript evidence on concealment and from ordinary clinical observation.

Marker 1 — The generic answer. Asked about her marriage, the client who is holding something says "he is a good man" and offers no incident. Detail is where the truth lives; a client who produces adjectives and no events is either not attending to her relationship or not able to report it.

Marker 2 — The topic that never recurs. Every other subject in a client's life will return across six sessions. The one that appears once, in session one, and is never mentioned again, is usually the one that is being managed. The therapist's record should note what is missing as carefully as what is present.

Marker 3 — The time gap. A narrative with a hole in it — school, then marriage, with five years missing — is a narrative with a secret in it. The therapist does not immediately probe the hole. She notes it, and she returns to it when the alliance can bear the question.

Marker 4 — The body's answer. The chest, the throat, the hand that goes to the mouth, the change in breathing when a name is mentioned. These are not proof of anything and they are information, and they belong in the note as observation rather than as inference.

Marker 5 — The abrupt generalisation. The client who is talking about something specific and suddenly becomes abstract ("well, everybody has something they do not talk about") has just come close to it and has stepped away. The move is worth naming gently: "you went somewhere else just then."

Marker 6 — The pre-emptive question. "You are not going to tell anyone what I say, are you?" — asked before anything has been said, and usually asked more than once. The client is not testing the policy; she is preparing to use it.

How the therapist uses the markers. She does not accuse. She makes the space available: "is there anything you have been carrying that we have not got to?" — asked once, plainly, and then left. The question is an invitation and not a demand, and the client who declines it has learned that the door is open, which is what makes the later disclosure possible.

4.3a When the client is a child or an adolescent

The protocol changes, because the client's capacity to choose is limited and the parents' claim is real.

The default. A minor's confidentiality is real and is not absolute. The therapist tells the child, in the first session and in language appropriate to the age, what will be shared with parents and what will not. The standard formulation: "what you tell me stays with me, unless I think you or someone else is in danger, or unless I think you need help that only your parents can give — and if I ever have to tell them something, I will tell you first what I am going to say."

The common error. Promising a child total secrecy, which the therapist cannot keep and which, when broken, teaches the child the exact lesson the therapy exists to prevent.

The adolescent's material. An adolescent disclosing sexual activity, a relationship, or a plan to leave home is disclosing material that parents will want and frequently have a legitimate interest in. The therapist's position: the content stays confidential where there

is no risk; the fact that the young person is dealing with something significant may be shared with the parents in general terms, with the young person's knowledge. Week 2's four-part script for consent with a young person is the template.

Where the child holds a family secret. A nine-year-old who says "do not tell my mum that daddy shouts" is disclosing something about the household. The therapist assesses risk first, then asks the child what she is most afraid would happen if it were known, then works with the parents on the pattern without attributing it to the child. The child must not be made the source, because a child who is the source of a parental crisis has been given a burden that will outlast the therapy.

4.3 The seven-step protocol for an individual disclosure

Step 1 — Receive it without reacting. The most consequential fifteen seconds in the session. The client has taken the largest risk she will take in months, and she is reading the therapist's face for the verdict. Any visible judgement — surprise, disapproval, concern so sharp it reads as alarm — teaches her that the disclosure was a mistake. The therapist's response is small and warm: "thank you for telling me", a pause, and then a question that keeps her talking.

Step 2 — Classify. Run the table. Is anyone at risk? Is a child involved? Is a duty engaged? This takes seconds and can be done while the client is still speaking. If any risk category applies, Week 8 or 9 governs and the session changes shape immediately.

Step 3 — Establish the history of the secret. How long has she held it, who else knows, has she ever tried to tell, and what happened. This is not small talk. The answers tell the therapist whether she is dealing with a long-maintained concealment (which will have shaped her relationships) or a recent event (which will not have had time to).

Step 4 — Assess the function. The question is: "what does keeping this do for you?" The answers are specific and are usually accurate — it protects her marriage, her mother, her standing, her job. The follow-up is the one that does the work: "and what does it cost you to keep it?" The client has almost always done this accounting privately and has never been asked to say it aloud.

Step 5 — Name the cost without demanding the remedy. The therapist reflects what she has heard, makes the link between the concealment and the presenting symptoms, and then does not prescribe. "So the sleep, and the knot in your chest — I wonder whether they are the price of carrying this alone." The client is now holding a formulation of her own problem rather than a secret.

Step 6 — Offer options, and let her choose. Five, in ascending order of exposure: keep it and work on managing it; tell one other person outside the marriage; tell the husband with the therapist's help in planning; tell the husband in a session with the therapist present; or defer with a review date. The deferral option is essential: without it, the client who is not ready has only the choice between exposure and flight.

Step 7 — Record and review. The note records the classification, the reasoning, the client's decision, and the review date. The review is a real appointment, and the therapist raises it.

What is absent from the protocol. Any step at which the therapist decides for the client. This is the difference from Week 3, and it is not a difference of squeamishness: the therapist has no authority over the client's relationship with her husband, and the attempt to exercise it converts treatment into moral direction.

4.4 Four techniques

Technique 1 — The cost ledger. A two-column sheet. Left: what keeping the secret protects. Right: what it costs — sleep, intimacy, the topics that cannot be raised, the moments of panic at a family gathering, the doctor's question that cannot be answered honestly. The client fills it at home between sessions. The sheet does the work that a therapist's argument cannot do, because the client produces both columns herself. In practice the left column is short and stable and the right column is long and growing, which is the finding that shifts the decision.

Technique 2 — The two futures. A structured imagination exercise, ten minutes, eyes open. The client describes in detail the morning three months after she has told him: where she is, what the room looks like, what he said, what happened to her body, and what she does next. Then the same morning in three years if she has not told him. The exercise is not a prediction and is not offered as one. Its function is to convert a vague dread into a set of specific fears, each of which can then be examined — and frequently to reveal that the feared outcome that has kept her silent for eleven years has never once been described in detail.

Technique 3 — The smallest true sentence. Many clients cannot contemplate the disclosure because they are imagining a full confession. The technique constructs the smallest sentence that is true and complete: "there is something about my past I have never told you and I want to tell you." Eleven words. The rehearsal is of these eleven words and the silence after them, and nothing else — because the client's real fear is not the content but the moment before it, and the moment can be practised.

Technique 4 — The audience question. "Who, other than your husband, would this change things with?" In a family where a mother-in-law, a pastor, or a brother has a stake, the client is often protecting the secret from four people while nominating one. Naming the full audience reduces the fantasy to a specific list and sometimes shows that the disclosure is smaller than she thinks — or larger.

A note on rehearsal. Where a client decides to disclose, the therapist rehearses with her: the opening sentence, the two most likely responses, and the sentence she will use if the conversation goes badly. The rehearsal is not preparation for a performance; it is the demonstration that she has options inside the conversation, which is what fear removes.

4.4a The case: Bisi Oyelaran, and what the therapist did not do

Referral. Bisi Oyelaran, 29, a primary school teacher, self-referred for "sleep and worry". Two children, 3 and 1. Husband Dayo, 33, a bank officer. Married four years. No prior therapy.

Sessions 1–4. The presenting problem is insomnia, a sensation of pressure in the chest, and irritability with the children. There is no identifiable stressor of the usual kind: work

is stable, the marriage described as "fine", the children healthy. Bisi's mother died two years earlier and the therapist pursues bereavement, which produces some material and no improvement. In session 4 the therapist notices the pattern the reader should have noticed in session 2 — every account of the marriage is generic. "He is a good man" recurs five times. There is no anecdote, no argument, no detail.

The moment, session 5, minute twenty-two. "I have to tell you something and then I will probably not come back." The therapist says nothing except "I am listening." Bisi then discloses: at 21, in her final year at college, she was pregnant and had a termination. Her mother knew and arranged it. No one else has ever known. Dayo's family are members of a church where this is "the worst thing a woman can do", and his mother has begun asking when the third child will come, and has suggested a fertility test.

The classification, made in the moment. No risk to anyone. No child protection question. A material secret with no third-party risk. The protocol applies.

Step 1 in practice. The therapist said: "thank you for telling me, and I want you to know that I am not going anywhere." Bisi cried for four minutes. Nothing was said. This is not a failure of technique; it is the technique.

Step 3, the history. Eleven years. Two people knew — Bisi and her mother, and her mother is dead. Since the death, no one knows. The therapist names this: "so for two years you have been the only person in the world who knows this." Bisi said, "yes", and then, "I had not thought of it like that."

Step 4, the function and the cost. The function: it protects the marriage from Dayo's family, and it protects her from Dayo's judgement, which she describes as "he would not shout, he would go quiet, and then he would tell his mother." The cost, elicited over two sessions: she cannot answer questions about her twenties; she avoids her college friends; she has refused two invitations to speak at church; she has begun to dread intercourse because "it is the one place I think about it"; and the chest pressure began the week her mother-in-law first mentioned the fertility test.

Step 5, the naming. "You have been carrying this alone for two years, and the fear of being found out has started to organise your life — who you see, what you say, even when you are close to your husband. I do not think the sleep is the problem. I think the sleep is the bill."

Step 6, the options, offered without pressure. Bisi rejected disclosure to Dayo outright, and said so with contempt for the suggestion. The therapist did not argue. Bisi chose a version of the first option — keep it, work on managing it — with a review date in six weeks, and one addition of her own: she wanted to tell one person, a cousin in Ibadan, "so that somebody alive knows".

The decision the therapist did not make. The therapist did not tell Bisi that she must tell her husband, did not imply that a good marriage requires it, and did not treat the delay as a failure of courage. She said what she thought: that the secret was costing her more each year, that the cost would rise when the fertility question became a medical

process, and that if Bisi ever wanted to tell him she would help her plan it and would be in the room if wanted.

Sessions 6–12. The work changed. The presenting symptoms were reframed as the cost of concealment, and the treatment became about how Bisi managed the secret rather than about the secret. She wrote the cousin a letter she did not send, then sent it. The chest pressure reduced over five weeks. In session 10 she said, of her own accord, "I think I will tell him before the test", and the therapist said "tell me what you would want to say", and the rehearsal began. She told him in session 14, at home, and came to the next session with the two of them. The therapist then saw them twice as a couple — with a new consent conversation, because the client system had changed.

What the case teaches.

1. **The therapist's own urgency is the main threat to the work.** Had she pressed in session 5, Bisi would have left, and the secret would have gone back underground with a new piece of evidence: that telling costs you the person you told.
2. **The symptom was the bill.** Reframing the presenting complaint as the cost of concealment is the single most useful intervention in this week's material, and it is available without any disclosure at all.
3. **The disclosure was eventually made, and not because it was required.** It was made when the cost of the secret exceeded the cost of the telling, and the therapist's job was to make that comparison visible and to be present when the balance tipped.
4. **The client system changed, and the paperwork had to change with it.** When Dayo joined, the consent conversation was redone from the beginning, and the record was updated. A therapist who simply continues on the old consent has made a documentary error that no amount of good clinical work repairs.

4.5 Transcript: receiving the disclosure

Session 5, minutes twenty-two to forty. Bisi has just said "I have to tell you something and then I will probably not come back." Annotation after each turn.

> **T1 Therapist:** (pause) I am listening. — Four words. No question, no reassurance, no "of course you will come back". The client has announced an exit; the therapist does not argue with it, which is what keeps her in the room. **T2 Bisi:** (long pause, then quickly) When I was twenty-one, in my final year, I was pregnant. My mother took me. I have never told anybody. Dayo does not know. His family — you do not know his family. If they knew they would — (stops) **T3 Therapist:** Thank you for telling me. (pause) And I want you to know that I am not going anywhere. — Step 1. The two sentences are doing different jobs: the first acknowledges the risk she has taken; the second answers the fear she actually voiced, which was about being abandoned by the therapist, not about the termination. **T4 Bisi:** (crying) You are not going to say anything? **T5 Therapist:** Not yet. (pause) I would rather hear the whole of it than start talking. — The refusal to reassure is the hardest move in the transcript and the most important. Reassurance at this point ("that was not your fault", "God forgives") replaces the client's account with the therapist's, and the client learns that the subject must be managed. **T6 Bisi:** There is nothing else. It was eleven years ago. It is finished. **T7 Therapist:** Eleven years. (pause) Who knows? **T8 Bisi:** My mother. Only my mother. **T9 Therapist:** And your mother died two years ago. — Step 3. The therapist has done the arithmetic and says it plainly. The client's face changes; this is the first moment in which the isolation is visible to her. **T10 Bisi:** (pause) Yes. So now it is only me. **T11 Therapist:** (pause) So for two years you have been the only person alive who knows this. — The reflection that does more than any interpretation. It is accurate, it is short, and it is not a judgement. **T12 Bisi:** (long pause) I did not think of it like that. I thought — I thought I was managing it. **T13 Therapist:** You have been managing it. (pause) Tell me what managing it looks like. — The question that opens the cost ledger without the therapist having to propose it. **T14 Bisi:** I do not talk about university. If somebody asks, I say it was fine. I do not go to the reunions. I stopped singing in the choir because — (stops) — because I thought if I stood up there, everybody would know. **T15 Therapist:** Everybody would know. — Reflecting the exact phrase, which is the technique that gets the client to hear her own logic. The therapist does not correct it. The correction is available later and is worth nothing now. **T16 Bisi:** I know it is not reasonable. **T17 Therapist:** I did not say it was not reasonable. I said I heard it. — A small correction, and a necessary one: the client has begun to argue with herself, and self-argument is the mechanism by which the secret has been kept. **T18 Bisi:** (pause) His mother has started talking about a test. She wants to know why we are not expecting. And I sit there and I — (stops) **T19 Therapist:** And you sit there holding all of it. — The link between the secret and the presenting symptom, made in the client's own language, four minutes after the disclosure and without a formulation. **T20 Bisi:** (quietly) The chest started then.

Minute forty, the close. The therapist did not offer a plan in this session. She said: "This is a great deal to have said aloud, and I think we should stop here rather than start solving it. I want you to come next week. We will look at what you want to do about this — and there are several things you can do, not one." Bisi attended the following week.

What the transcript shows.

- **Restraint is the intervention.** Of the twenty turns, the therapist asks four questions and makes three reflections. She interprets nothing, reassures nothing, and prescribes nothing.

- **The exact phrase.** Reflecting the client's own words (turns 15 and 19) produces more than any formulation the therapist could have offered, because the client cannot argue with her own sentence.
- **The ending is deliberate.** Closing before problem-solving prevents the session from becoming a negotiation about disclosure in the first hour of the client's exposure, and it makes the next appointment the container for the decision.

4.6a The record of an individual disclosure, written out

Two notes, from the Bisi case, at the two moments that matter. The first records the disclosure; the second records the decision at review.

> **SESSION 5 (50 MIN).**
> Client disclosed a termination at age 21, during her final year at university. Her mother arranged and knew; mother died two years ago; no other person has known. Client states her husband and his family do not know and that she fears disclosure would end the marriage and expose her to her mother-in-law and her church community. **Classification.** Material, no current risk to client, child, or third party. No statutory duty engaged. No risk indicators on direct enquiry. **Response.** No advice or plan offered in this session; disclosure received, history and audience explored, link noted between the client's reported chest symptoms and the recent fertility questions from her mother-in-law. Session closed early by design; next appointment confirmed and attended. **Plan.** Options to be discussed at session 6 (keep and manage; tell one other person; tell husband with planning; tell husband in session; defer with review). Client informed there are several options, not one. **Supervision.** Case to be presented [date] — countertransference noted: therapist aware of a wish to protect client from her husband's family.

> **SESSION 11 (50 MIN).**
> Review held as scheduled. Client reports: has told one person (cousin, Ibadan); chest symptoms reduced; sleep improved from approximately four to six hours. Client states she now intends to tell her husband before any fertility investigation begins, and asks for help planning. Rehearsal begun with the opening sentence; two likely responses identified; client to bring a written note to session 12 if she wishes. Client informed that if she wishes the therapist to be present, the therapist will attend; client has not yet decided. **Client system unchanged at this date.** Review at session 13.

What the two notes have in common. Both record the classification, the reasoning, the client's decision, the next step and the supervision. Neither contains a judgement of the client's choice. Neither records an instruction.

Why the second note matters as much as the first. It is the evidence that the deferral was a plan and not an abandonment. A file containing the first note alone tells a reader that the therapist knew about a secret and did nothing for six sessions; the two notes together tell the truth, which is that the client was given time, the work continued, and the decision was hers.

The one sentence that must never appear. "Client declined to disclose; discussed." It is accurate, it is useless, and it will be read by a regulator as an absence of clinical thinking.

4.6 Laboratory: the disclosure rehearsal

Aim. To practise receiving a disclosure without reacting, classify it correctly, and offer options without coercion — assessed by an observer against six observable behaviours.

Set-up. Triads, three rounds of twelve minutes each, so everyone plays all three roles. Each round the "client" draws a card. The cards, which should be prepared in advance:

Card	Disclosure	Classification
A	"I have been paying a loan for two years and my husband does not know."	Material, no risk
B	"I have not told my wife that I lost my job six weeks ago."	Material, no risk; possible financial risk
C	"My brother has been staying with us and he has been in prison."	Material; consider third parties
D	"I have been having thoughts about not being here."	Risk to self — stop the exercise and follow Week 8
E	"My husband's cousin touched me when I was sixteen and I have never told anyone in the family."	Material; consider historical abuse reporting rules
F	"I know my daughter's husband hits her and she has asked me not to tell anyone."	Risk to a third party — Week 9

Cards D and F are the ones that matter pedagogically: the student who continues the counselling protocol after drawing them has failed the round outright, however warm the response.

Observable behaviours (the observer marks each as present, partial or absent).

1. Did the therapist allow a silence after the disclosure, before speaking?
2. Did the therapist thank the client, and say something that addresses abandonment?
3. Did the therapist ask about history and audience before proposing anything?
4. Did the therapist ask about function and cost?
5. Did the therapist offer at least three options, including a deferral with a review date?
6. Did the therapist avoid reassurance, advice, and any statement of what the client "should" do?

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Reception	Reacted, advised or reassured	Paused, but then filled the silence	Received without reaction; acknowledged risk; addressed abandonment
Classification	Did not classify	Classified, but the wrong category	Correct category, and changed protocol where the card required it

Criterion	0–1	2–3	4–5
Function and cost	Neither explored	One explored	Both explored in the client's language, with the link to the presenting complaint
Options	Gave one option or a demand	Several, all leading to disclosure	Several, including deferral and review, with the choice left where it belongs
Record	Nothing	Narrative only	Classification, reasoning, client's decision and review date

Debrief question for every triad. "What did you want the client to do, and where did that wish show in what you said?" Almost every student discovers the wish in the pause before their first question.

4.7 Six errors

Error 1 — Reassuring in the first minute. "That must have been terrible", "you were very young", "God is merciful" — each is meant kindly and each takes the story away from the client. The client's task at that moment is to finish her account; the therapist's task is to make room for it.

Error 2 — Classifying by the therapist's moral reaction. The therapist who finds an affair or a termination disturbing will classify it as high-priority and press for disclosure. The classification table has no moral criterion, and its absence is deliberate.

Error 3 — Turning the first session of disclosure into a decision. The client has spent years holding something and has now said it aloud, which is exhausting. A therapist who wants a plan within twenty minutes is working at her own pace, not the client's.

Error 4 — Confusing "I cannot work around this" with "you must tell". The therapist's limit is about her own capacity to conduct an honest treatment, and it is legitimate. It is not a demand that the client disclose to a third party who is not a client, and the two are frequently conflated.

Error 5 — Becoming the keeper of the secret. The therapist who agrees to hold it, protects the client from the husband, and begins to manage the household's information has been recruited. The sign is that the therapist is now thinking about the husband as an obstacle.

Error 6 — Forgetting to review. The deferral option is safe only if the review is real. A deferral without a date becomes a permanent arrangement, and the secret has quietly been installed in the treatment.

4.4b Four sentences that help a client decide

The therapist's most useful contribution to a disclosure decision is not advice; it is a small number of sentences that make the decision thinkable. These four, used in this order across two sessions, do most of the work.

First — "you do not have to decide today." The client has usually arrived at the session believing that telling the therapist means being pushed. Removing the pressure lowers the arousal enough for thinking to become possible, and it is also true.

Second — "there is more than one way to do this." The client's imagination contains a single scenario: sitting opposite her husband, saying everything, and watching the marriage end. The therapist's job is to populate the field — a letter, a session, a trusted third person present, a partial disclosure of the fact rather than the detail, a staged disclosure over weeks. Clients almost never generate these options alone, because fear narrows the field to one.

Third — "what is the worst thing you are imagining, in detail?" Not "how do you feel?" — which produces adjectives — but a request for the specific scene. The scene is then examined: what he says, what she says next, who else is told, what happens in the following week. The examination does two things. It usually reveals that the imagined catastrophe has no script beyond the first minute. And where the catastrophe is real — where the husband has been violent, or where the family would remove the children — the examination reveals that too, and the plan changes accordingly.

Fourth — "whatever you decide, I will still be working with you." The sentence that makes honesty survivable and the one most often left unsaid. It is also the sentence the therapist must be able to keep, which is why the limit in Week 3's couple policy is different from the position here.

4.7a The audit question, and the end of the week

At the end of every week in which a disclosure has been received, the therapist answers four questions in writing, in the file, in under a hundred words.

1. **What do I now know that the people around this client do not know?** The inventory. It is usually longer than the therapist expects.
2. **Is anything moving, or is this now installed?** A plan with a date is moving. A deferral with a review date is moving slowly. A deferral without a date is installed.
3. **Who is paying for the concealment, and how much?** If the answer is only the client, the situation is stable. If the answer includes a child, a partner's health, or a third party's exposure, the situation is not stable and the deferral must be renegotiated.
4. **Have I taken this to supervision?** If not, why not — and the honest answer is frequently that the therapist does not want to be told to press, which is itself the material for supervision.

The closing point of the week. The student who finishes this week believing that the right answer is "the client should disclose" has not learned the week's content. The right answer is that the therapist maintains a relationship in which the client can tell the truth about what she is carrying, keeps her own conduct honest, never becomes a party to the concealment, and holds the client's choice even when the choice is to wait. That is a harder position to occupy than either directive — and it is the position the codes describe when they say that the therapist's duty is to the client's welfare and self-determination, and not to the therapist's view of the client's marriage.

4.8 Culture, faith and the secret

Shame is social before it is private. In many of the families this book serves, a secret is not held to protect the individual's self-image but to protect a household's standing. Bisi's fear was not that Dayo would think less of her; it was that Dayo would go quiet and then tell his mother, and that the information would then travel through a church and a compound. The therapist who assesses the risk as "her husband's reaction" has mis-specified it, and the mis-specification makes the client's fear look irrational when it is not.

Confession has an address. Where a client's framework includes confession — to a priest, a pastor, an imam, an elder — the therapist should ask, early, whether the client has confessed, and to whom, and what happened. Three outcomes are common and each is clinically useful. The client has confessed and found absolution, and the secret's burden is much lower than the therapist assumed. The client has confessed and been given a penance that made the secrecy worse, such as a direction never to speak of it again. Or the client has not confessed because the confession would require a public act she cannot perform, and the therapy is now carrying a religious obligation as well as a personal one.

The church as an information network. In congregations where testimony is public and pastoral care is layered through small groups, a disclosure has an audience that the client knows better than the therapist. The practical consequence: ask who would come to know, and do not assume the answer is one person. In Bisi's case the audience was a mother-in-law, a women's fellowship, and a choir.

The in-law's claim. In many households the mother-in-law holds a genuine and socially recognised claim on information about fertility. A therapist who treats this as intrusion will alienate the client, who may agree with the claim even while resenting it. The workable formulation is to name the claim as real and to separate it from the decision: "she has a claim to be told something; she does not have a claim to be told this, and the choice of what she is told is yours."

Reproductive history and the honour frame. Where a premarital pregnancy or a termination is framed as an offence against family honour rather than as a personal decision, the disclosure's danger is not disproportionate — it is accurately perceived. The therapist's obligation is not to talk the client out of the danger but to help her assess it with precision and to plan for the realistic worst case, including where she would go and who would take her in.

What not to do. Do not import the language of "authenticity" and "living your truth" into a household where the cost of disclosure may be housing, custody, or violence. Do not assume that the Western therapeutic consensus — disclosure is good for you — describes the client's situation. And do not assume the opposite either: many clients in these households disclose, survive it, and are relieved.

4.8a The therapist's own secrets

A neglected part of the topic, and a frequent source of impairment.

The therapist is not a sealed vessel. Therapists hold their own material — a divorce, a miscarriage, a parent's illness, a history of abuse, a child in difficulty — and the case that resembles it will activate it. The professional obligation is not to be unaffected. It is to know what is activated, to keep it out of the client's material, and to take it to supervision.

The three failure modes. Over-identification: the therapist's own secret shapes what she hears, so that the client's situation becomes the therapist's situation and the client's options narrow to the ones the therapist chose. Compensatory rescue: the therapist protects the client from the outcome she herself suffered. Displacement: the therapist's unprocessed material appears in the case as unusual intensity — the session that runs over, the client she thinks about at night, the case she discusses with her own partner.

Self-disclosure as a clinical decision. When the therapist discloses something of her own, the criteria are narrow: it must serve the client's work, it must be brief, it must be about the therapist's experience rather than the client's, and it must not require the client to take care of the therapist. "I have sat with this kind of decision myself, and I know it is not simple" meets the criteria. A twenty-minute account of the therapist's own divorce does not, and it makes the client a witness rather than a patient.

The question for the therapist. Before each session with the client who activates you: whose material is this? If the answer is unclear, the case goes to supervision and, where the activation is severe, the therapist considers whether the case should be transferred — which is a professional act and not a confession of inadequacy.

4.9 Evidence note

What the research supports. The concealment literature is substantial. Experimental work finds that holding a secret produces intrusive thinking about it and consumes cognitive capacity; that keeping secrets is experienced as effortful and fatiguing; that self-concealment is associated with greater distress and poorer help-seeking; and that disclosure interventions produce modest but reliable improvements in health and wellbeing. In psychotherapy research, clients' concealment predicts weaker alliances and worse outcomes, and client-rated honesty about what they have not said is a better predictor of progress than the amount they have said.

What is weaker. The evidence is largely about the cost of concealment in general, not about the right moment to disclose to a spouse. There are no trials of disclosure timing in couple or family treatment, and the prescriptive literature — tell within X sessions — is clinical convention rather than evidence.

What is not supported. That disclosure is always beneficial: the literature is clear that disclosure to a hostile or unsupportive recipient can worsen outcomes, which is precisely the situation many clients in this week's material accurately fear. That a therapist can predict the outcome of a disclosure. And that a secret unspoken to the therapist cannot be worked with — the treatment can address the management of a secret without its content, and sometimes must.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Bisi at 21, in a family where her

mother could act, and Bisi at 29, with two children, a mother-in-law and a church, is the same event in two different systems, and the disclosure carries two different prices.

4.10 Key terms

Individual disclosure — information given in individual therapy concerning a person who is not a client.

Concealment cost — the practical and physiological price of maintaining a secret.

Function — what the secret protects; assessed before any plan is proposed.

Self-concealment — a general disposition to hide personal information; predicts poorer therapy outcomes.

Deferral with review — an agreed postponement with a dated review; the alternative to disclosure and flight.

Smallest true sentence — the shortest complete disclosure, used in rehearsal.

Cost ledger — a two-column client exercise weighing protection against price.

Countertransference to the secret — the therapist's protective, flattered or allied response to being trusted.

Client system change — the addition of a new client (a partner, a child) requiring a new consent conversation.

4.11 Self-test

1. Why does the modified no-secrets policy of Week 3 not transfer to individual therapy?
2. State the five categories of the classification table and give one example of each.
3. What are the three signs that a therapist has been recruited as keeper of the secret?
4. Describe the cost ledger and explain why the client fills it rather than the therapist.
5. What is the smallest true sentence, and what is it for?
6. In the Bisi case, what did the therapist not do, and why does each omission matter?
7. What must be recorded after an individual disclosure, in five parts?
8. What changes when a new client joins an existing treatment?

Answers. (1) Because the client is the individual, so the therapist owes confidentiality to one person and has no standing to require disclosure to a non-client; the therapist's legitimate limit is on her own capacity to conduct the treatment, not on the client's conduct. (2) Immaterial (a surprise party); material with no risk (a past termination); third-party risk (an undisclosed infection); child or vulnerable adult (abuse); risk to self (suicidal intent). (3) The therapist looks forward to the session unduly; she feels protective of the client against the absent partner; and she is managing the household's information. (4) A two-column sheet of what the secret protects and what it costs, completed at home, because the client's own production of the right-hand column is more persuasive than any argument the therapist could make. (5) "There is something about my past I have never told you and I want to tell you" — eleven words used in rehearsal, because what the client

fears is the moment before the disclosure rather than its content. (6) She did not reassure in the first minute (which would have replaced the client's account with her own); did not propose a plan in the first session (working at the therapist's pace); did not tell Bisi she must disclose (which would have ended the therapy); and did not treat the delay as failure. (7) Classification; reasoning; the client's decision; the review date; and the supervision plan. (8) A new consent conversation, an amended confidentiality agreement, a new record of who is the client, and a re-negotiation of what will be shared between the individual and joint work.

4.12 Reflective task

Write two hundred words on the last secret you were told. Do not describe the content. Answer three questions: what did you feel towards the person who told you; what did you want them to do next; and did that wish appear anywhere in how you responded? Keep the page. You will want it when the first case in your practice reproduces the feeling.

4.13 Further reading

- Pennebaker, J. on disclosure and health — the empirical basis for the cost-of-concealment formulation.
- Slepian, M. on the psychology of secrecy — the experimental work on mind-wandering and concealment burden.
- Larson, D. and Chastain, R. on self-concealment — the construct that predicts therapy process.
- Doherty, W. on moral conflicts in therapy — relevant to the discipline of not moralising.
- Any current text on professional ethics in counselling for the treatment of client concealment from the therapist.

Next: Week 5 — Secrets II: infidelity, paternity and HIV status.

Week 5 — Secrets II: infidelity, paternity and HIV status

Learning outcomes. By the end of this week the student can (1) conduct the first four sessions after the discovery of an affair without either minimising the betrayal or treating the marriage as finished; (2) sequence the clinical and legal tasks when paternity is in question; (3) identify the jurisdiction's rule on disclosure of a communicable condition and follow the correct pathway where a partner is at risk; (4) decide, with reasons, whether the affair partner's spouse should be told; and (5) document all three in a form that would survive a regulator's reading.

Three secrets, three legal overlays

Week 4 dealt with the individual secret in general. This week deals with three specific secrets that arrive with predictable regularity and that each carry a legal overlay the therapist must recognise — not to practise law, but to know when to stop and refer.

Secret	Clinical task	Legal overlay	Who else is exposed
Infidelity	Contain the disclosure; treat the betrayal; rebuild or end	Not an offence in most jurisdictions; relevant to divorce, custody and maintenance	The affair partner, and the affair partner's spouse
Paternity	Support the family through a factual discovery with enormous relational meaning	Presumption of paternity, maintenance, custody, registration of birth, the child's rights	The child, the presumed father, the biological father, both extended families
HIV and com municable disease	Support disclosure; treat the fear, the shame and the couple's sexual negotiation	Public-health law; in some jurisdictions a criminal offence to transmit or to expose knowingly	The partner, and any previous or subsequent partner

The three are grouped because they share a structure: an **established fact**, a **person who does not know it**, and a **third party who may be harmed by the continuing silence**. Week 3's policy governs how the therapist holds the secret; this week governs what the therapist does with it.

The first thing to establish, always, is jurisdiction. The single most common ethical error in this week's material is a therapist applying a rule learned in one country to a family in another. On paternity, the presumption of legitimacy and the rules on registration differ substantially. On HIV, some jurisdictions impose a duty to disclose to an identifiable partner, some impose a duty to warn a public health authority, some criminalise non-disclosure, and some do none of these. On infidelity, adultery remains a criminal offence in a number of jurisdictions and is a ground for divorce in most. The therapist's obligation is to know the local rule for the three cases, to have checked it within the past year, and to have it in writing in the practice manual — not to reconstruct it from memory in a session.

The second thing is to know what the therapist does not do. She does not advise on divorce, on custody, on maintenance, on the registration of a birth, or on whether a client should be tested. She does not tell a client what the law requires of the client — that is legal advice, and where the stakes are this high it must come from a lawyer. She may say what her own duties are, because those are her duties and the client is entitled to know them. The distinction is clean and it is frequently blurred by therapists who have read one article.

5.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The three secrets and the common structure	Board diagram: fact, unknowing person, exposed third party
8–20	Five concepts: betrayal trauma, the disclosure scene, the legal overlay, exposure, the third party's claim	Lecture
20–38	Infidelity: the four-session protocol	Demonstration, then pairs on the opening sentence

Minutes	Content	Teaching form
38–52	Paternity: the sequence, and where the therapist stops	Case vignette, students write the referral question
52–66	HIV: jurisdiction, pathway, and the safe disclosure plan	Local rule quiz; group mapping of the disclosure pathway
66–76	The Adegoke case and the disclosure session	Case presentation with students as the couple
76–86	Six errors; the documentation	Short lecture
86–90	Closing question: whose secret is it?	Written answer, collected

5.2 Five concepts that organise the week

Concept 1 — Betrayal trauma is a trauma, and it is not a metaphor. The discovery of an affair produces a response with the signature of traumatic stress: intrusive images, hypervigilance to cues, avoidance, sleep disruption, and a compulsive need for detail. This matters clinically because the therapist who treats the discovery as an understandable upset will under-treat it. The partner who has just found out is not merely angry; she is in an acute stress response, and the first task is stabilisation rather than communication training.

Concept 2 — The disclosure scene is the treatment's foundation, and it happens once. The first conversation in which the affair is discussed will be remembered verbatim by both partners for years. What is said, whether the details are given, whether the affair partner is named, and whether the therapist is present or absent determines the shape of everything that follows. The therapist therefore prepares this conversation as carefully as any intervention in the book.

Concept 3 — The legal overlay changes what can be promised. A therapist who promises a client that nothing said in the room will reach a court has made a promise she may not be able to keep, because records can be subpoenaed and because in some jurisdictions the therapist's notes are discoverable in custody proceedings. The correct formulation, given at the start: "what we say here is confidential, and there are limits — one of them is that a court can compel records. If that ever happens, I will tell you and I will fight for as much confidentiality as the law allows." (Week 11 develops this.)

Concept 4 — Exposure is the therapist's signature question. Not "what should the client do?" but "who is exposed by the continuation of this silence, and how serious is the exposure?" An undisclosed affair exposes the affair partner's spouse to the same betrayal and, if there is any sexual contact outside the primary relationship, to infection. An undisclosed paternity exposes a child to a factual discovery later, at an age when it is more damaging. An undisclosed HIV status exposes a partner to infection. The seriousness of the exposure determines how much urgency the therapist brings.

Concept 5 — The third party has no claim on the therapy and a real claim on the truth. The affair partner's spouse is not a client, has no right to the file, and cannot be contacted by the therapist without the therapist's own breach of confidence. She also has a genuine moral claim to information that affects her marriage and her health. The two

facts are both true, and the therapist must hold them without resolving the tension by pretending one is absent.

5.2a Infidelity: the four-session protocol

Session one — stabilise, do not adjudicate.

- Both partners are seen, usually separately first, for twenty minutes each, then together briefly.
- The injured partner is assessed for acute stress: sleep, intrusiveness, functioning, and suicidal ideation, which is not rare after discovery.
- The involved partner is assessed for the character of the affair: how long, how often, whether it is ongoing, whether there is a pregnancy, whether money has moved, and whether the affair partner is threatening disclosure.
- The therapist does not ask "why did you do this?" in session one. The question invites a defence that then has to be dismantled, and it is the wrong question at the wrong moment.
- The therapist states the frame: no decisions about the marriage for six weeks; individual sessions permitted, with Week 3's policy; and the disclosure conversation will be structured, not improvised.

Session two — the disclosure scene.

Structure it as five questions, answered by the involved partner, in this order, with the therapist controlling pace:

1. What happened? The facts, in outline — when it began, when it ended, how often. Not sexual detail.
2. Who knows? Including the affair partner's spouse, and any friend who has been told.
3. What did you tell me about it? The lies told to the injured partner during the affair, named specifically. This is the part that is hardest for the involved partner and the most necessary for the injured one, because the injured partner's injury is not only the sex; it is the deception she participated in unknowingly.
4. What are you going to do now? Continuation, ending, disclosure of the affair to the affair partner's spouse, and the practical steps — the message sent, the block, the change of arrangements.
5. What do you need from me? Addressed to the injured partner; the answer is usually "I need it to stop", and frequently "I need to know you chose me".

The detail question. The injured partner will ask for details, and the answer is not "tell everything" or "tell nothing", it is the distinction between facts necessary for safety and choice and images that will not leave. Necessary: how long, whether protection was used, whether there is an ongoing contact, whether the affair partner has been told about the marriage. Not necessary, and harmful: positions, comparisons, what the other person looks like. The therapist says this to both of them, before the questions begin, and both partners usually accept it — because the injured partner's real question is not anatomical.

Session three — the meaning.

Only now, after the facts are held and the exposure has been managed, does the question "what was this about?" become answerable. The therapist's stance must be exact: the meaning is not a cause, and the cause is not an excuse. "I want to understand what was happening in your life when this started, and I am not asking so that we can blame your wife for it. I am asking because whatever made the affair possible will still be there when it is over." The distinction is the one thing that makes the third session survivable, and it must be said aloud.

Session four — the choice.

The marriage is named as a decision rather than a default. Both partners are asked, separately and then together, whether they intend to attempt repair. Where one partner will not, the therapist does not argue; she helps them separate without the affair becoming the whole of the story. Where both will, the treatment moves to the repair tasks: full transparency for a defined period, the naming of what was lost, the rebuilding of trust as a set of testable commitments rather than a feeling, and the explicit rejection of "we are past it now" as a route to a silent recurrence.

The temporal frame that matters. The literature on affair recovery converges on a rough shape: an acute phase of three to six weeks in which nothing is decided; a middle phase of several months in which the meaning is worked and trust is rebuilt through consistency; and a long tail in which anniversaries and discoveries of new detail re-open the injury. A therapist who promises the couple that "in twelve sessions you will be through this" has promised something false and has given them a schedule to fail against.

5.2b Paternity: the sequence, and where the therapist stops

The situation. A father learns, or suspects, that a child he is raising is not biologically his. Or a mother learns that the man she named as the father is not. Or an adult child learns, from a commercial DNA test, that her father is not her father. Each of these arrives in a therapist's office and each is treated by the therapist as a family disclosure with a legal dimension.

Step 1 — Establish what is known, and how. Suspicion, an admission, a test, or a document. A suspicion the client has carried for nine years is not the same fact as a test result, and the clinical work differs enormously. The therapist's first question, "what do you actually know, and how do you know it?", is answered incompletely about half the time.

Step 2 — Establish who knows. The mother, the man, the child, the extended family, a lawyer. In the commonest configuration, two adults know and a child does not, and the therapist's client is one of the two.

Step 3 — Establish the child's position. Age, developmental stage, relationship with the presumed father, relationship with the biological father if any, and what the child has been told. The child's welfare is the organising criterion and not the parents' comfort. In many jurisdictions the child has a legal right to know the identity of a parent, and in almost all of them the child's interests will be weighed in any later dispute.

Step 4 — Refer for legal advice, and say why. The referral question is specific: what are the client's obligations and rights in respect of maintenance, custody, registration and the child's knowledge? The therapist does not answer this. She does something more useful: she writes the question down for the client so that the client can put it to a lawyer, and she records that she did so.

Step 5 — Sequence the disclosure, if there is to be one. The order matters and there is no universally correct one. The options: mother tells the father, then both tell the child; the father tells the child without the mother; a family session with the therapist present; or deferral until the child reaches an agreed age. Each has costs. What the therapist can do is insist that the decision is made deliberately, with the child's interest stated aloud, rather than by drift.

Step 6 — Be present for the child's question. Children ask the questions adults avoid: "so who is my real father?" and "does he still love me?" The therapist's preparatory work with the parents is to write answers to those two questions in advance, in words a child can hear, and to agree that the child's relationship with the man who raised him will be described as a relationship and not as a technicality.

Where the therapist stops. At the door of the courtroom. Everything about maintenance, access, guardianship and registration belongs to the lawyer; everything about the child's experience, the parents' grief, the family's story and the disclosure conversation belongs to the therapist.

The vignette. Kola, 47, a school principal, attends alone. He says his wife has told him, in an argument, that their second child, now 14, is not his. He has said nothing to the child and has not slept in three weeks. He wants to know whether to ask for a test.

The therapist's first session does four things and none of them is advice. She stabilises — sleep, work, the suicidal ideation he mentions in passing and then retracts, which is assessed properly. She establishes what he knows: his wife's statement in anger, no test, no corroboration. She names the two questions that are currently fused in his mind and must be separated: is it true? and what do I do about my son? And she writes, for him, the questions for a lawyer: what is his legal position if the child is not his; what obligations continue; what rights does he have; what happens if he seeks a test without the mother's consent.

The clinical work over eight sessions is not about the test. It is about the fact that Kola has not been able to look at his son for three weeks, and that the boy has noticed. The most urgent task in the case is that a fourteen-year-old is being silently abandoned by a father who is waiting for a fact. Whatever the fact turns out to be, that is the harm that is happening now.

5.3 HIV and other communicable conditions

The clinical reality. A client discloses that he is living with HIV and has not told his wife. He is on treatment and virally suppressed. Or a client discloses that her husband is living with HIV and has not told her. Or a client discloses that he has been diagnosed with hepatitis and does not intend to tell his partner. In each case the therapist holds information whose non-disclosure exposes an identifiable person to a serious condition.

Step 1 — Know the rule before the case arrives. The therapist's practice manual should contain four answers, reviewed annually: (a) is there a statutory duty to disclose to a public health authority, and within what period? (b) is there a statutory duty or permission to disclose to an identifiable partner, and to whom? (c) is non-disclosure or exposure a criminal offence, and does the therapist's knowledge create any liability? (d) does the jurisdiction operate a partner-notification service, and how is it accessed? Four questions, four sentences. The therapist who cannot answer them is not competent to hold this case.

Step 2 — Assess the actual risk. The therapist needs, not rumour: the client's treatment status, whether viral suppression has been achieved and documented, whether protection is used, and what the partner knows. In many of these cases the risk is far lower than the client's fear, and in some it is higher than the therapist's first assumption. The risk assessment is not the therapist's opinion; it is what the client's physician has documented, and part of the therapist's work is to get the client to a physician.

Step 3 — Separate shame from risk. The commonest presentation is a client who is not avoiding disclosure because of danger but because of shame and the fear of being seen as dirty, unfaithful, or diseased. Naming this distinction changes the work: "I am not asking you to disclose because you have done something wrong. I am asking you because there is a person who could be infected and who is not being given the chance to protect herself."

Step 4 — Use the partner-notification route if it exists. Where a service exists, it can be used anonymously or with the client's involvement, and it frequently resolves the case: the partner is told by a trained worker, the client is spared the first sentence, and the therapist's dilemma dissolves. This route is under-used simply because therapists do not know it exists.

Step 5 — Plan the disclosure hour by hour. Where the client will disclose, the plan is specific: when — not "soon", but a date and a time when the house is empty; who — alone, or with a trusted person; what — the diagnosis, the treatment, the risk to the partner, and the offer to attend a clinic together; and what if — the two worst-case responses, each with a plan, including where the client sleeps that night.

Step 6 — Where the client refuses and the risk remains. Then the therapist states her position in terms of her own conduct, in writing, and seeks immediate supervision and legal advice if the jurisdiction imposes a duty. Three options, in order of preference: the client discloses within a stated period; the client authorises partner notification; or the therapist discloses what the law requires and tells the client in advance that she will. What the therapist may not do is continue as though nothing has happened, because in this case the continuation of silence has a named potential victim and a documented date.

The vignette. Mrs Ifeoma Nwosu, 38, a seamstress, attends for anxiety. In session four she discloses that her husband, Emeka, was diagnosed eight months ago, is not on treatment, has refused to tell her, and she learned of it from a pharmacy receipt. Her question is not whether to tell him she knows; it is whether to leave. The therapist's tasks here are inverted: the exposed person is the client, the secret's holder is the absent husband, and the clinical work includes her own testing, her own risk, her own safety and the decision about the marriage. The therapist does not contact the husband. She supports the client in her own disclosure if the client chooses it, assists her to a clinic, and records the reasoning. The case then moves into Week 9's territory, because a woman whose husband has concealed a diagnosis for eight months is a woman whose household may become unsafe when she says so.

5.3a The third party's spouse: the question with no clean answer

The question. The client's affair partner is married. Her husband does not know. Does the therapist do anything?

What is not available. The therapist cannot contact him. That would be a disclosure of her client's confidential information to a stranger, and it is prohibited regardless of the moral merits. Any answer that begins "the therapist should tell him" has misunderstood the ethics of the role.

What is available. Four options, in ascending order of intrusiveness, and the therapist should be able to name all four and say which she would press for and why:

1. **The client tells him.** The cleanest, rarest, and the one the therapist should raise first, explicitly and without embarrassment: "he is being deceived in the same way your wife was. Is that something you are willing to end?" Many clients have not considered it, having classified the affair partner's marriage as not their business.
2. **The client tells the affair partner that he will tell, or that she will, within a stated period.** A pressure that is applied to the right person.
3. **The couple decides together**, which happens more often than clinicians expect once the injured partner is past the acute phase: the betrayed spouse frequently wants the other household to know, and the decision is then a joint one that the therapist can support.
4. **Nobody tells him**, and the therapist records that the question was raised, what the couple decided, and why. This is a defensible outcome and it must be a decision rather than an omission.

The safety filter that overrides the order. Where the affair partner's husband is known to be violent, the disclosure may expose the affair partner and the client to serious harm. In that case the correct answer is frequently option 4 with a documented reasoning, and the therapist who presses for disclosure has imported a moral position into a risk calculation.

The therapist's own view. The therapist is entitled to have one and is not entitled to impose it. The workable practice is to state it once, briefly, as a question rather than a

verdict — "have you thought about what happens to his wife?" — and then to work with the answer.

5.4 Anatomy of the disclosure session

A structure for the session in which the affair is first discussed with both partners present. Ninety minutes, not fifty, and scheduled deliberately rather than arising inside a routine session.

Phase	Minutes	What the therapist does
Frame	0–8	Names the purpose; states the rules (no detail beyond the necessary; no leaving the room; a break available on request); reminds both that no decision about the marriage is being made today
The account	8–30	The involved partner gives the outline, using the five questions. The therapist controls the pace and stops the account when it moves into detail that serves no safety function
The questions	30–55	The injured partner asks; the therapist filters — "that is a detail; ask instead what you need to know to feel safe"
The response	55–70	The injured partner's reaction, uninterrupted. The therapist does not manage it, soften it, or interpret it
The commitments	70–85	Concrete, verifiable, dated: contact ended, method of proof, transparency arrangements, who else is told
The close	85–90	Both partners state one thing they will do before the next session; the therapist summarises in five sentences and records them

The three rules of the room. First, the involved partner answers; the therapist does not answer for him. Second, the injured partner's anger is not moderated in this session — it is the appropriate response and moderating it protects the wrong person. Third, the session ends with commitments rather than with feelings, because commitments are what the injured partner actually needs and feelings are what the involved partner finds easier to offer.

5.5 Case: the Okonkwos, second year

The household. Ada Okonkwo, 35, a pharmacist; Chuka Okonkwo, 40, a logistics manager; Ebuka, 10. They were the family in Week 1, where the clinic's policy on individual disclosures was explained, recorded and agreed.

The referral. Ada telephones on a Tuesday morning. "I found something on his phone. I need to see you before I do anything." The therapist offers a same-day individual appointment, states the policy again at the start of it, and Ada agrees to it a second time.

What Ada knows. A message thread on Chuka's phone, seen four days ago. A woman's name, Nkechi. Messages that are affectionate and explicit, spanning roughly seven months. Ada has photographed eleven messages, has told no one, and has not confronted him. She has not slept more than four hours in four nights and has twice been unable to drive because of shaking. She has begun looking up divorce lawyers on her phone and has not called one.

The assessment in that session.

- Acute stress: intrusive images, hypervigilance, sleep loss, appetite loss, intrusive checking of his phone at night. Not suicidal, but passive ideation on direct enquiry ("sometimes I think I should just disappear") which is assessed and planned for, and reviewed at every subsequent session.
- Risk: no history of physical violence. Chuka has never been violent; he is, in Ada's description, "the one who goes quiet". Ada's own anger is substantial and she is frightened of it.
- Disclosure history: the children know nothing. Ada's mother knows nothing. Ada's sister knows "something is wrong".
- Immediate functioning: she cannot concentrate at work and has taken two days of leave.

The therapist's four tasks in that first session. Stabilise (sleep, food, the name of a person she can call at night). Separate the questions (is it happening; is it over; what do I do; who do I tell — four questions, and they cannot be answered in one week). Establish what she needs to know (whether it is ongoing, whether protection is used, whether there is a pregnancy, whether money has moved — the safety questions, and no others). And set the frame for the next session: "you will not decide anything this week. You will come back on Thursday, and we will decide what to do about the conversation with him."

Session two, three days later — the question of the individual session. Ada wants to see the therapist alone again, indefinitely, "to decide what I am going to do". The therapist applies Week 3: the couple is the client; individual sessions are permitted and are not a parallel therapy; and the plan must be that the disclosure moves into the couple's room. Ada agrees, with one condition — she wants the disclosure to happen in the therapist's office, not at home, because "at home I will either scream or say nothing".

Session three — Chuka alone. He attends, having been told by Ada only that she has "seen something" and that they have an appointment. He is guarded, then, at minute nineteen, says: "I know why I am here."

What he discloses: a relationship of seven months with a colleague, Nkechi, who is also married and has two children. He says it ended "in a manner of speaking" three weeks ago. He has not told Ada. He says he intends to, "when things are calmer". He says his marriage has been "dead for two years". He asks, twice, whether the therapist will tell Ada.

The therapist's answers, in order. "I will not tell her what you have told me. And I am not willing to hold this while I sit with both of you — which means it needs to move, and the shape of that is what we will decide now." She does not accept "when things are calmer", because there is no calmer. She asks the safety questions: protection, pregnancy, money, and the affair partner's marriage. She asks what he has told Ada about the last seven months — the lies, specifically — because that is the material the disclosure session will have to address. And she says the sentence that determines the case: "you will tell her, in my room, on Thursday. I will help you prepare what you say. She has already seen the

messages, so this is not a revelation; it is the truth after four days of her not knowing whether she is going mad."

Session four — the disclosure session, ninety minutes. Both present. Ada speaks first, for six minutes, without interruption: what she saw, what the four days have been like, and the sentence she has rehearsed — "what I need from today is not an explanation, it is the truth." Chuka then answers the five questions. He gives the outline: seven months, a colleague, twice-weekly, ending three weeks ago — although Ada's messages suggest contact nine days ago, which he then corrects. He names the lies: the late nights, the "team retreat", the weekend he said he was with his brother. He states what he has done: message sent ending it, contact blocked, and an agreement to move teams at work next month. He states what he has not done: he has not told Nkechi's husband. And he answers Ada's question, "why", with the sentence that becomes the centre of the treatment: "because I did not want to be the man who had failed, so I stopped telling you anything."

The detail question, in the session. Ada asks, at minute forty, "was she better than me?" The therapist intervenes, exactly as the protocol requires: "that is a detail, and I am going to stop it — not to protect him, but because you will carry the answer for years and it will not tell you anything you need. What you need to know is whether he wants to be married to you, and that is a question he has to answer." Ada accepts this, and later says it was the most useful sentence in the session.

The commitments at minute seventy. Four, all dated. Chuka sends Ada a written account of the timeline by Saturday. He books and attends an STI screen within ten days and shows her the result. He agrees to leave his phone in the kitchen at night for three months. He agrees to a joint session every week for eight weeks. And on the fourth point — telling Nkechi's husband — he agrees to decide by the following Thursday, and the therapist records the question as open.

The next eight sessions. The treatment is not about the affair. It is about the two years in which Chuka stopped telling Ada anything, and about his belief that a man who admits failure loses his household, which is his father's belief and which he has never examined. Ada's work is separate and harder: she has to decide whether she can live with a man who lied competently for seven months, and the therapist's job is to keep that question open rather than to sell the marriage. In session eleven Ada says, "I do not trust him. I have decided to see whether trust comes back." That sentence, not the affair, is the case's turning point.

What the case teaches.

1. **The individual session was the instrument, not the problem.** Without three individual sessions — Ada's two and Chuka's one — there would have been no prepared disclosure, and the conversation at home would have been the screaming match Ada predicted.
2. **The policy was stated in Week 1 and applied in Week 5.** The therapist could hold Chuka to a disclosure because the clinic's policy had been explained to both of them a

year earlier and recorded. A therapist who improvised the policy on the day would have been negotiating from nothing.

3. **The safety questions are asked of both partners, separately.** Ada was asked about violence, ideation and functioning; Chuka was asked about protection, pregnancy, money and the affair partner's marriage. Neither set of questions would have been answerable with both in the room.
4. **The disclosure session was structured, and the structure was the therapy.** Frame, account, questions, response, commitments, close: six phases and ninety minutes, and none of it was improvised.
5. **The treatment's subject was not the affair.** The affair was the event; the marriage's two-year silence was the case. Finding that distinction is the clinical work of session three.

5.6 Transcript: the refusal of "when things are calmer"

Session three, Chuka alone, minutes twenty-two to forty. Annotated.

> **C1 Chuka:** I will tell her. When things are calmer. She is not in a state to hear it now. **C2 Therapist:** (pause) There is no calmer. — Four words, and the refusal to accept the most common postponement in this literature. "When things are calmer" is not a plan; it is a description of the client's hope that the problem will become smaller on its own. **C3 Chuka:** You do not know her. She will — (stops) **C4 Therapist:** I do know her a little. I saw her on Tuesday. (pause) She has not slept in four nights and she has been photographing your phone. She is already in it, Chuka. The only question is whether she is in it alone. — The move that ends the postponement: not an argument about duty, but a description of what the delay is actually costing the person the client says he is protecting. **C5 Chuka:** (long pause) I did not think of it like that. **C6 Therapist:** Most people do not. (pause) Tell me about the last seven months. Not the relationship — the things you said to her. — The pivot from the affair to the deception. This is the technical heart of the session: the affair is what happened; the lies are what the disclosure must address. **C7 Chuka:** I said I was working late. I said the retreat was in Abuja. I said — (pause) — I said I was tired. That one was true. **C8 Therapist:** So there was a true thing in it. — A reflection that does something precise: it neither excuses nor condemns, and it registers that the client's marriage was not simply a lie. **C9 Chuka:** (quickly) It is not her fault. Ada. It is not her fault. **C10 Therapist:** I did not say it was. (pause) But you have just told me you were afraid of being the man who failed. Where does that come from? — The question that opens the third session's real material, asked without any suggestion that it excuses the affair. Note that the therapist first refuses the implication (C10, first sentence) and then asks the question, in that order — reversing the order would have sounded like an excuse. **C11 Chuka:** My father. He was — he lost his job when I was twelve, and my mother's brothers took over the house. I watched him become nothing. In our place, a man who cannot provide is — he is not a man. **C12 Therapist:** (pause) And in seven months, did you become him? **C13 Chuka:** (long pause) Every time I came home. — The sentence the case will be built on, arrived at in session three, without the therapist having proposed it. **C14 Therapist:** Then we have something to work with. (pause) On Thursday you will tell Ada the truth about the seven months, and I will be in the room, and this — what you have just said — is what we will be working on afterwards. — The two-part close: the disclosure is not negotiable, and the treatment is promised. Both parts are necessary; either alone fails.

What the transcript shows. The refusal of the postponement took eleven words and no moral argument. The relevant contrast is with the therapist who says "you have an obligation to tell her", which invites a debate about obligations; the therapist here says what the delay is doing, which cannot be debated because it is observable.

5.7 Laboratory: three secrets, three protocols

Aim. To produce, in ninety minutes, three one-page protocols — infidelity, paternity, HIV — for a named practice, each with the clinical sequence, the referral trigger, the documentation and the jurisdictional question.

Procedure. Groups of four, each group assigned one secret, twenty minutes to draft, then ten minutes to present to the other groups, who must find the missing element. Each group must also write the sentence it would use to explain the limit of confidentiality to a client in that case.

Marks (20).

Criterion	0–1	2–3	4–5
Sequence	No order	Order present but not justified	Order justified by exposure and by the client's capacity
Referral trigger	None	Vague ("refer if needed")	A specific question for a specific professional, written out
Jurisdiction	Not addressed	Mentioned	The four questions named, with a source and a review date
Documentation	Nothing	Narrative	Classification, reasoning, decision, date, supervision
The sentence	Moralising or evasive	Accurate but cold	Accurate, brief, and delivered without blame

The commonest failure. Groups write "the therapist should advise the client to disclose". That is advice about a third party's conduct, it is outside the role, and it is marked in the lowest band however sound the underlying ethics.

5.8 Six errors

Error 1 — Adjudicating the affair. "What he did was wrong" is true, and said in session one it makes the therapist the injured partner's ally and the involved partner's judge — and the involved partner, who is necessary to the repair, stops speaking.

Error 2 — Answering the detail questions. The injured partner asks, the therapist permits, and both carry images for years. The filter — facts necessary for safety and choice — must be applied by the therapist, in advance, out loud.

Error 3 — Treating paternity as a therapy question. The therapist who opines on maintenance, custody or registration has left her competence and possibly her indemnity. The referral is written down, with the question specified.

Error 4 — Assuming the local HIV rule from memory. The rules differ, they change, and the consequences of being wrong are criminal. Four questions in the practice manual, reviewed annually.

Error 5 — Continuing a treatment that is being conducted around an undisclosed exposure. The therapist who sees the couple weekly while the husband's diagnosis remains hidden from the wife is not neutral; she is a party to the exposure.

Error 6 — Deciding the third-party question by omission. The affair partner's spouse is either told, or the question is raised and recorded as decided against. What is not permissible is never raising it.

5.9 Culture, faith and the three secrets

Infidelity and the shape of the household. In a polygynous or extended household, the definition of infidelity is not the therapist's to set. A second relationship may be adultery to one wife, a breach of an agreed arrangement to another, and a family matter to be resolved by elders to a third. The therapist's first task is to ask what the couple's own understanding was — what was promised, to whom, and in front of whom — because the injury is measured against the promise and not against the therapist's model of marriage.

The elders' jurisdiction. Where a family council has authority, the couple may want the matter resolved there rather than in the therapy room, and the therapist may be asked to attend or to report. Attending a family council is sometimes possible and requires a clear contract about confidentiality; reporting to it almost never is. The workable answer is that the therapist will help the couple prepare what they will say to the elders.

Paternity and lineage. In households where lineage carries inheritance, naming and belonging, the disclosure of non-paternity is not only a relational event; it can alter a child's standing in a family and a community. This raises the stakes and does not change the criterion — the child's welfare — but it changes the timing, the audience and the planning. The therapist who treats it as a Western-style family secret has underestimated the case.

HIV, blame and the church. Where a diagnosis is read theologically — as judgement, as a test of faith, or as a punishment on a household — disclosure carries a second injury. Congregational responses vary from extraordinary care to exclusion, and the client knows which is likely in her own congregation better than the therapist does. Asking "if this became known in your church, what would happen?" produces information that belongs in the risk assessment.

The communitarian objection. "In our culture, family matters are not discussed outside." This is true and it is not an answer to the exposure question. The therapist's response separates the two: "I agree that this is family business. The question I am asking is not whether to keep it in the family — it is whether the person who is being exposed is inside the information or outside it."

> **DEBATE**
> "When a client refuses to disclose an HIV-positive status to a partner, the therapist's duty to the partner overrides confidentiality." Argue for: an identifiable person is at risk, the client's refusal converts the therapist into a participant, and public health law in many jurisdictions supports disclosure. Argue against: disclosure by the therapist destroys the confidence on which testing and treatment depend, the partner-notification route exists precisely to avoid it, and the therapist's duty is defined by statute and not by the therapist's conscience. Then state the rule you would work under and the jurisdiction you would work in.

5.9a The referral note to a lawyer

Therapists refer to lawyers constantly and badly. A referral that says "client needs legal advice" produces a lawyer who cannot begin. A referral that specifies the question produces advice the client can use.

The four questions worth writing down. First: what is the client's legal position in respect of the children if the facts are as stated? Second: what must the client do, or refrain from doing, in the next thirty days to avoid prejudicing that position? Third: what records exist, and can any of them be compelled? Fourth: what does the client need to know about the timing of any disclosure?

What the therapist does not put in the note. Her clinical opinion of the parties, her view of who is at fault, and any speculation about facts the client has not established. The note is a question, not a case formulation, and the lawyer's file may become a document in the proceedings.

What the therapist records afterwards. That the referral was made, on what date, with what question, and that the client was advised to obtain advice before making any decision with legal consequences. Six lines. They are the difference between a therapist who acted within her competence and one who did not.

5.10 Evidence note

What the research supports. Affair recovery research describes a reasonably consistent trajectory of acute stress, meaning-making and rebuild-or-end, with the injured partner's intrusive symptoms typically prominent for weeks and the couple's trust rebuilt through consistent verifiable behaviour rather than through reassurance. Disclosure of HIV status is associated with better treatment engagement and lower transmission risk, and partner-notification services increase disclosure rates; the public health literature is clear that supporting voluntary disclosure outperforms mandatory approaches on most measures. The research on secrecy in families shows that maintained family secrets are associated with poorer wellbeing in the person holding them, and that disclosure is most damaging when it is discovered rather than told.

What is weaker. There are no trials determining the optimal content of an affair disclosure, and the clinical debate between full disclosure and a facts-only approach is not settled empirically. Paternity disclosure has a small qualitative literature and almost no outcome data, largely because the situations are heterogeneous.

What is not supported. That a couple cannot recover from an affair — the outcome data on couple therapy following infidelity is modest but positive. That disclosure of HIV status always improves relationships — it can precipitate violence, which is why safety planning precedes disclosure. And that the therapist's judgement about the moral seriousness of the secret should determine the urgency; the exposure assessment does that.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Seven months of an affair discovered by a phone message, and seven months disclosed by a client in session twenty, are the same fact with two different prognoses.

5.11 Key terms

Betrayal trauma — the traumatic stress response to the discovery of a significant relational betrayal.

Disclosure scene — the first structured conversation in which the affair is discussed with both partners.

Exposure assessment — the identification of who is put at risk by the continuation of a silence.

Presumption of paternity — the legal presumption that a child born within a marriage is the husband's; jurisdiction-specific.

Partner notification — the public-health route by which a partner is informed of exposure, anonymously or with the index client's involvement.

Viral suppression — the reduction of viral load to undetectable levels; relevant to risk assessment and not a substitute for disclosure.

Safety filter — the rule that where disclosure would create a risk of violence, the risk governs the plan.

Third-party claim — the moral (not legal) claim of a non-client on information that affects them.

5.12 Self-test

1. State the common structure of the three secrets in this week.
2. What are the four questions the practice manual must answer on communicable disease?
3. Which detail questions may the therapist permit, and which must she stop?
4. Why is "when things are calmer" not a plan?
5. Name the four options for the affair partner's spouse, and the filter that overrides them.
6. What does the therapist do when paternity is in question and the client asks for advice about custody?
7. Give the six phases of the disclosure session with their timings.
8. In the Okonkwo case, what was the treatment actually about?

Answers. (1) An established fact, a person who does not know it, and a third party exposed by the continuing silence. (2) Whether there is a duty to a public health authority and within what period; whether there is a duty or permission to disclose to an identifiable partner; whether non-disclosure or exposure is an offence; and whether a partner-notification service exists and how it is accessed. (3) Permit facts necessary for safety and choice — duration, protection, whether contact continues, whether the affair partner's household is affected; stop questions about sexual practice, comparison, and the other person's appearance. (4) Because it names a state of affairs that will not arrive on its own and does not specify who does what by when; the delay is itself the harm. (5) The

client tells him; the client tells the affair partner that he will tell; the couple decides together; or nobody tells him and the reasoning is recorded. The safety filter — a risk of violence — overrides all four. (6) She says she does not give legal advice, writes the client's question down for a lawyer, and records that she did so. (7) Frame 0–8; account 8–30; questions 30–55; response 55–70; commitments 70–85; close 85–90. (8) The two years in which Chuka stopped telling Ada anything, and his belief that a man who admits failure loses his household.

5.13 Reflective task

Write three hundred words on the following: a couple arrives having discovered an affair, and you find yourself hoping they will separate. What in your own history is producing that hope, and how would you know if it had begun to shape your questions? Do not resolve the question. Locate it.

5.14 Further reading

- Glass, S. on the anatomy of affairs and on Not "Just Friends" — the standard clinical account of boundaries and affair types.
- Gottman, J. on trust and betrayal in couples — the framework for the trust-rebuilding tasks.
- Perel, E. on the meaning of affairs — the counterweight to purely behavioural accounts, useful for the third session's work.
- The WHO guidance on HIV partner notification and the UNAIDS materials on disclosure — the source for the pathway in section 5.3.
- Any current local statute or professional guidance on communicable disease disclosure, read in the original and filed in the practice manual.
- A family law primer for your jurisdiction, for the reference list and not for advice.

Next: Week 6 — Boundaries and multiple relationships.

Week 6 — Boundaries and multiple relationships

Learning outcomes. By the end of this week the student can (1) distinguish a boundary crossing from a boundary violation and explain why the difference is the whole of the topic; (2) apply the four tests to any multiple relationship and reach a defensible decision; (3) manage the unavoidable dual relationships of small- community practice without either pretending they do not exist or abandoning the client; (4) state and defend a personal rule for touch, self-disclosure, gifts, bartering and digital contact; and (5) recognise and manage sexual attraction to a client as a professional event rather than a personal failing.

The distinction the week is built on

A **boundary crossing** is a departure from the usual frame that is made for the client's benefit, is discussed, and is reversible. A **boundary violation** is a departure that serves the therapist, is concealed, and often is not reversible. The two look similar in a textbook and are entirely different in a file.

	Crossing	Violation
Whose need	The client's	The therapist's
Disclosed	To the client, to supervision, to the record	Concealed, or minimised
Reversible	Yes — the frame can be restored	Rarely; the relationship is altered
Example	A fifteen-minute extension in a crisis, noted and discussed	Sessions that routinely run long because the therapist enjoys them
Example	Attending a former client's wedding after discharge, with the frame discussed	Attending a current client's family event as a guest
Example	Accepting a bowl of fruit at the end of a course of treatment	Accepting a car

The pedagogy follows from this: students who are taught boundaries as prohibitions become anxious and rigid, and rigid therapists fail their clients at exactly the moments when flexibility is clinical. Students who are taught the crossing/violation distinction can think, and thinking is what the situation will require.

Why this topic is examined so heavily in the codes. Because boundary violations are the commonest route by which therapists harm clients, and because the sequence is well documented: a gradual erosion, always initiated by small kindnesses, always rationalised as therapeutic, and almost always preceded by isolation from supervision. The literature on therapist sexual misconduct finds that the great majority of cases were preceded by non-sexual boundary crossings over months, and that colleagues often noticed and said nothing.

The error that costs most in this subject. Teaching the topic as though it were about sex. It is about power, and the sexual cases are the endpoint of a sequence that begins with the therapist becoming special.

6.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	Crossing and violation: five paired examples, sorted by the class before any theory	Live sorting
10–22	Five concepts: power, role conflict, the slope, the small community, the felt sense	Lecture, board diagram
22–40	The four tests and the decision matrix, applied to eight scenarios	Small groups, then plenary votes
40–55	The unavoidable dual relationship: the community contract	Demonstration of the opening conversation
55–70	Touch, gifts, barter, home visits, digital contact: the rule and the rationale	Lecture with cases
70–80	Attraction: recognition, disclosure to supervision, and the decision tree	Short lecture; written anonymous question

Minutes	Content	Teaching form
80–90	The Alabi case	Case presentation, students decide

6.2 Five concepts that organise the week

Concept 1 — Boundaries exist because the relationship is asymmetric. The client tells the therapist what she has told no one; the therapist tells the client almost nothing. The client depends on the therapist's judgement; the therapist depends on the client for a fee. That asymmetry is not an unfortunate feature of therapy; it is the mechanism by which therapy works, and every boundary in this week's material exists to protect it. A boundary that is justified by "professionalism" in the abstract will not survive pressure. A boundary justified by the asymmetry will, because the therapist can see what it protects.

Concept 2 — Role conflict degrades judgement before it degrades conduct. The therapist who is also the client's landlord does not begin by acting badly; she begins by thinking differently. She notices that she is reluctant to challenge the client about the rent, or that she is unusually warm about the client's financial difficulty. The judgement goes first. This is why role conflicts are examined at the point they are created and not at the point they produce harm.

Concept 3 — The slope is real, and it is walked one small step at a time. No therapist decides to have a sexual relationship with a client. A therapist decides to take one phone call, then to meet for coffee "to discuss the crisis", then to accept a lift, and at each point the previous step has made the next one seem continuous. The clinical use of the concept is not to frighten students but to give them a rule: the sequence is interrupted at the first crossing, when it is cheap, and the interruption is a conversation rather than an act of will.

Concept 4 — In small communities, multiple relationships are not avoidable; they are manageable. In a city of eight million a therapist can decline a case and the client will find someone else. In a town of thirty thousand with two trained therapists, refusing the mayor's family can mean the family receives no service. The ethical question therefore shifts from may I avoid this? to how do I manage this honestly? — which is a harder question and the one this week spends most of its time on.

Concept 5 — "It felt right" is not a test. The felt sense is a poor guide in this domain because the therapist's warmth, the client's gratitude and the pleasure of being needed all feel like evidence of a good relationship. The test is external: whose need is served, what would supervision say, what would the client say in five years, and is it in the record?

6.2b The five-minute boundary decision

The four tests produce a decision, and in practice the therapist has five minutes and no matrix to hand. The compressed version, used in the room, is four questions asked in this order.

Question one — what would I have to stop doing? The concrete loss is the fastest indicator of a role conflict. A therapist who realises that she could no longer challenge the client about money has found the conflict without needing a theory.

Question two — who would know first? If the answer is "nobody", the arrangement has already failed. Every defensible dual relationship in this chapter was declared before it began; every indefensible one was discovered.

Question three — can the client say no? A favour, an offer of employment, a discount: each is a proposal made by a person the client depends on. If the client cannot refuse without cost, the arrangement is exploitation whatever its intention.

Question four — what is my sentence? The therapist must be able to say, in two sentences and without embarrassment, what the arrangement is and why. A clinician who cannot construct this sentence has not decided anything; she has drifted, and drift is the mechanism of the whole chapter.

What to do with the answer in the five minutes. Three outcomes only. Proceed with the contract write-up. Decline with the four-part script. Or say "I need to think about this and I will tell you on Thursday", which is always available, is never wrong, and is the answer most often forgotten under pressure.

The one arrangement that cannot be decided in five minutes. Any proposal that involves money, employment, or a sexual or romantic dimension — including a proposal that arrives from the client, obliquely, in a sentence that could be read two ways. Those go to supervision before any answer is given, and the client is told that the therapist is thinking about it, which is true and which is not a rejection.

6.2a The four tests, and the decision they produce

The four tests from the scaffold are the right ones. What follows is how to use them so that they produce a decision rather than a discussion.

1. **Objectivity.** Could the second relationship impair my judgement about this client's treatment? Not "will it" — could it. The answer is almost always yes in some degree, and the useful question is what specifically would change.
2. **Exploitation.** Could the second relationship exploit the power difference? A loan to a client, a business partnership, a favour that the client cannot refuse — each is exploitation even where the therapist's motive is generous.
3. **Foresight.** Would the client experience this as exploitative later? This is the test that is easiest to answer dishonestly and most useful when answered in the client's voice. "In three years, telling a friend about her therapy, how would she describe this?"
4. **Avoidability.** Is the second relationship avoidable without depriving the client of the service? A gift from a bereaved family is not really avoidable; a business partnership is.

The decision matrix.

Objectivity impaired?	Exploitation possible?	Avoidable?	Decision
No	No	Yes	Decline, briefly and without a lecture
Slightly	No	Yes	Decline; discuss in supervision
Yes	No	No	Proceed with a written contract and supervision review
Yes	Yes	No	Proceed only with a formal risk management plan, documented, reviewed monthly
Yes	Yes	Yes	Do not proceed. Transfer the case

The three columns that matter. Written contract: the conversation with the client about how the two roles will be handled, recorded in the file in the client's presence and with the client's agreement. Supervision review: a named supervisor who knows about the arrangement and a date at which it is reconsidered. Transfer: the option that therapists resist and that is frequently correct.

The sentence for declining. "I am not able to take this on, and I want to tell you why so that it does not feel like a rejection: I know your family socially, and that would make it hard for me to be a good therapist to you. I can give you two names." Brief, honest, and it does not invite negotiation. The therapist who explains at length has made the client responsible for accepting the refusal.

6.3 The unavoidable multiple relationship: the community contract

Where the therapist and client will inevitably meet in another role — the same church, the same school, a shared extended family, a village — the work is not avoidance but an explicit contract, made at the start, recorded, and revisited.

The four elements of the contract.

1. **The naming.** "We are going to see each other at church, and at your sister's shop, and probably at the market. I want us to agree now how we will handle that, so that neither of us is caught out."
2. **The greeting rule.** Agreed in advance: the therapist will greet the client normally and will not refer to therapy; the client may decide whether to acknowledge the therapist first; and the therapist will follow the client's lead in public. This small rule prevents the commonest injury, which is a client left standing in a market while her therapist pretends not to see her.
3. **The information rule.** What the therapist will do with information acquired in the community role. "If your mother tells me at church that you have lost your job, I will not raise it in session as though you had told me. I will wait for you to bring it, or I will ask whether you want to talk about it." This is the single most useful clause in the contract and almost never written.
4. **The review.** A date at which the contract is looked at again, because the community roles change — the client's daughter joins the therapist's son's class, the client is elected to the church committee that employs the therapist's husband.

What the contract does not solve. It does not remove the judgement problem. A therapist who is a deacon and a therapist in the same congregation has two sources of authority over a client, and the contract manages the boundary without dissolving the role conflict. Where the conflict is structural — the therapist holds a leadership role in a community the client belongs to — referral is the better answer, and saying so is not a failure.

6.3a The role conflicts that recur

The client who is a colleague. A fellow therapist, a nurse, a teacher in the same school. The specific hazards: the colleague knows the therapist's reputation, may be supervisee or supervisor, and will meet the therapist professionally after treatment ends. The management: an explicit discussion of what will happen at professional meetings, no treatment of the colleague by a person who has authority over them, and no use of the colleague's clinical knowledge in the treatment.

The client who is a student. Power is at its maximum and cannot be contracted away. A therapist who teaches the client, examines the client, or supervises the client's placement should not also treat them, and the correct answer is referral. Where a training clinic has no alternative, the treatment is conducted by someone with no assessment authority over the student, and this is written down.

The client referred by a friend. The friend will ask how it is going. The therapist's answer, agreed in advance with the client: "she is attending and I am not able to discuss her." Said once to the friend, in the first week, without apology. The alternative — "it is going well" — is a disclosure.

The client whose family employs the therapist's spouse. A genuine conflict of interest in small economies. The test: would the therapist be able to challenge the client's behaviour if the client's family were paying her household? Where the answer is no, transfer.

The former client who becomes a friend. The codes differ, and the safest practice is that the relationship does not convert, or converts only after a substantial interval and with the recognition that the client may return to treatment. A former client who is now a friend cannot come back as a client, and the therapist should say so at the point the friendship is contemplated rather than at the point it is needed.

The client who becomes famous, or whose case becomes public. The therapist's obligation not to use the material does not lapse when the case is in the newspapers, and the temptation is at its maximum precisely then.

6.3b The referral conversation, scripted

Declining is a technique, and like any technique it improves with rehearsal. The four-part script below takes about two minutes and leaves the client with a service rather than a refusal.

Part one — the decision, stated first. "I have thought about this and I am not going to be able to take the work on." First, because a client who hears the explanation first spends it looking for the loophole.

Part two — the reason, in terms of the client's interest. "The reason is that I know your family outside this room. If I were your therapist I would already know things about your husband, and you would be watching what you said to me. That would make the therapy worse, not better."

Part three — the alternatives, specific. Two names, with a sentence about each, and an offer to telephone ahead: "Dr Bello works with couples and has a clinic on Tuesdays; Mrs Ojo is closer to you and works evenings. I can call either of them today and tell them that you will be in touch, without telling them anything about you."

Part four — the door. "If neither works, come back to me and I will find you a third name. What I will not do is leave you with nothing."

What the client usually says, and the answer. "Is it because of something I did?" — "No. It is about me and where I worship, and nothing about you." "You are the only one who understands." — "I have met you twice; what you are responding to is that somebody listened. Someone else will listen, and they will be able to hear more than I can."

6.4 Touch, gifts, self-disclosure, barter, home visits and the telephone

Each of these is sometimes therapeutic and always requires a rationale that can be spoken aloud in supervision. The rule for each is stated here as a personal rule, because a therapist who has not pre-decided will decide inside the moment, and the moment is designed to make the generous option feel right.

Touch. The rule: touch is used only where it is a recognised part of the therapeutic method being practised, or where it is the ordinary human response to acute distress and is brief and public. A hand on the shoulder of a woman who has just been told her husband is dying, in a room with a window, is not a boundary violation. A twenty-minute embrace at the end of every session is. The rationale: touch cannot be retracted, is interpreted through the client's history — particularly where the client has been abused — and is the commonest first step in the documented sequence toward sexual misconduct.

Gifts. The rule: small, culturally appropriate, given at a transition (the end of treatment, a bereavement, a birth), accepted with thanks and recorded. Anything of substantial value, anything given early in treatment, and anything that creates an obligation is declined with an explanation that protects the giver. The test: is the gift a communication or a transaction? In this region, food and cloth carry meaning and refusing them can be an insult; the therapist who declines a bowl of fruit to prove a point has damaged an alliance for an abstraction. The recorded note is short: "client brought fruit at the end of the eighth session; accepted, meaning discussed, recorded."

Self-disclosure. The rule: brief, about the therapist's experience rather than the client's, serving the client's work, and never requiring the client to care for the therapist. The test: if the client responded by consoling the therapist, would the disclosure have been appropriate? If not, it should not be made.

Barter. The rule: almost never. Where a client has no money and the alternative is no treatment, a barter arrangement may be ethical, and it requires a written value, a stated duration, and a supervision review. The specific hazard is that the client's service is now a clinical topic — the therapist who accepts tailoring from a client will have opinions about the tailoring.

Home visits. The rule: a home visit is a clinical intervention with a purpose, a time limit, and a documented rationale. Family therapy occasionally requires it: a home reveals what a clinic cannot. The visit is arranged in advance, attended with a plan, and recorded, and the therapist does not accept hospitality beyond water, does not stay for a meal unless the meal is the intervention, and does not go alone where safety is uncertain.

The telephone and the message. The rule: the channel is specified at the start, with its limits — "you may call between nine and six; if it is urgent and I am not available, go to the emergency number; I do not respond to messages after nine at night." The rule exists because the alternative is a therapist who is available at all hours to a client who has no other support, which is not a boundary the therapist can sustain and not a service the client can rely on.

Digital contact. The rule: no personal social media contact with current clients; no searching for a client online without a clinical reason and a record of it; no clinical material over unencrypted channels; and an explicit statement in the first session of what the therapist will and will not do online. The specific hazard of the photograph: a client's social media reveals information the therapist did not obtain clinically and cannot forget. Where the therapist has seen it, the fact is recorded, because the alternative is a treatment silently shaped by material the client never gave.

6.4a The boundary log

A one-page document, kept by the therapist, reviewed monthly in supervision. Five columns: date; the crossing; whose need it served; what was said about it to the client and to the supervisor; and whether the frame was restored.

Why a log rather than a memory. Because crossings are forgettable and their accumulation is the phenomenon worth noticing. A therapist who can see eleven small extensions over four months is looking at a pattern that no single session reveals, and the pattern is the early part of the slope.

What belongs in it. The session that ran twenty minutes over because a client disclosed a bereavement. The gift that was accepted. The lift home given in the rain. The client's child's school fee that the therapist "lent". The text message answered at eleven at night. Each is small, and the log's purpose is not to punish any of them.

What the review asks. Three questions. Is the same client appearing repeatedly? Is the same kind of crossing recurring with different clients, which points at the therapist rather than the case? And has anything been crossed that was not told to supervision, which is the only entry that is a violation rather than a crossing?

6.5 Sexual boundaries, and what to do with attraction

The absolute. Sexual contact with a current client is prohibited in every code this book draws on and is a criminal offence in many jurisdictions. There is no clinical argument, no cultural exception, and no case in which it is defensible. Sexual contact with a former client is prohibited by most codes for a stated period — commonly two years, sometimes five, sometimes permanently — and the period is a minimum rather than a permission.

Why the prohibition is absolute. Because consent is compromised by the asymmetry, and because the harm is severe and durable. Clients who have been sexually involved with a therapist show outcomes comparable to those of survivors of sexual assault by someone trusted: betrayal, confusion about their own responsibility, damage to their capacity to trust a helper, and in a substantial proportion, a subsequent suicide attempt. This is the single most harmful thing a therapist can do, and the research on it is not ambiguous.

Attraction is not the problem. Most therapists will feel drawn to a client at some point in a career, and the feeling is neither a symptom of impairment nor a predictor of misconduct. The problem is silence about it. The documented pathway to violation runs through secrecy: the therapist who cannot say "I am aware of feeling drawn to this client" to a supervisor is the therapist at risk.

What to do, concretely, in the week it happens.

1. **Name it to yourself** and write one line in a private note. The act of writing reduces the intensity and creates a record that can be discussed.
2. **Tell supervision**, in the same words: "I am aware of attraction to this client and I want to talk about how it is affecting the work." Supervisors should make this easy by asking routinely, because the first time it is said should not be the first time it is possible.
3. **Increase structure, not distance.** Do not abruptly become cold, which the client experiences as unexplained rejection and which often produces the pursuit the therapist fears. Keep the frame, keep the notes, keep the seating, and keep the session ending on time.
4. **Examine the material.** Attraction frequently carries information: the client resembles someone; the client is providing the admiration the therapist's life is not; the case is a repetition of an earlier one. The information is clinically useful and belongs to the therapist's own work, not to the client.
5. **Refer, if the feeling is not manageable.** Not as a punishment. The sentence: "I think you would be better served by a colleague, and I want to tell you that this is about my capacity and not about you."

Where a client declares love. The response is kind, brief, unambiguous, and does not interpret the client's feeling as pathology. "I am not able to be that for you, and I am not going to pretend I do not understand what you are asking. What I can be is your therapist, and I think that is worth more to you than the other thing." Then the clinical work: what the declaration is about, which is usually loneliness, gratitude, or a history in which care and romance have been the same thing.

6.5a What the codes actually say, and where they differ

The codes converge on the absolute prohibition and diverge on almost everything else, which is why a therapist moving between jurisdictions must read the version that governs her.

On sexual contact with current clients. Universal prohibition, and in a growing number of jurisdictions a criminal matter rather than a professional one. There is no divergence to report.

On sexual contact with former clients. The divergence is real: some codes impose a fixed interval — two years, sometimes five — others require the therapist to demonstrate that the contact is not exploitative, and some prohibit it permanently where the treatment was long or the client vulnerable. A therapist in a jurisdiction with a two-year rule who treats the interval as permission has misunderstood the rule, which is a minimum evidentiary threshold and never a warrant.

On non-sexual multiple relationships. The codes permit them where they are not exploitative and do not impair judgement or risk harm, and they require the therapist to document the reasoning. What none of them supplies is a list, and the absence is deliberate: the categories are local.

On gifts. Most codes say nothing explicit and the professional bodies' guidance varies from "small and culturally appropriate" to a value threshold in local currency. The therapist should know her own body's wording because a complaint will be assessed against it.

On barter. Permitted in some codes where the arrangement is not exploitative, is entered at the client's initiative, and is documented; discouraged in others.

On social media. The most recent guidance and the least settled. The consistent threads: no personal accounts for clinical use, no searching without clinical justification, and a written statement to the client at intake.

On touch. Almost entirely absent, which is why the reasoning in section 6.4 has to be constructed from the general principles rather than looked up.

6.6 Case: the Alabis, and the deacon's wife

The setting. A private practice in a state capital, and a church of about three hundred members where the therapist has worshipped for eleven years, sings in the choir, and is known to almost everyone as "the counsellor".

The referral. Mrs Yetunde Alabi, 44, a school administrator, is referred by her physician for depression following a miscarriage at fourteen weeks. Her husband, Deacon Samuel Alabi, 49, is a church officer and a friend of the therapist's husband. The couple have three children, 17, 14 and 9. The referral arrives by telephone from the physician, who does not know the therapist's church.

The first decision, before the first session. The therapist recognises the name and applies the four tests. Objectivity: impaired — she knows Samuel socially, has opinions about him, and has heard him preach. Exploitation: possible — she holds lay religious

standing in a congregation where the family worships. Avoidability: yes, in principle; there are four other therapists within reasonable distance. Foresight: the client, in three years, describing this therapy to a friend, would say "she is in my church". The matrix says: decline or transfer.

What actually happened, and why it is instructive. The therapist telephoned the physician and said she could not take the referral, giving the reason in one sentence. The physician asked whether that was not excessive: the family had asked for her by name. The therapist said it was not excessive and gave two names. The family, through Samuel, telephoned the therapist directly three days later and asked her to reconsider — "we would not be comfortable with a stranger; you are one of us."

The therapist's decision at that point is the pedagogically interesting one. She did not simply repeat the refusal. She offered a **consultation**: one meeting, with both of them, for the sole purpose of explaining the conflict, giving the two referrals, and — this is the part that matters — asking whether they wanted her to speak to the named therapist in advance so that the family would not have to begin from nothing. She did not take the case.

Why the refusal was right, in four sentences. Because the client's miscarriage, her grief and whatever is turning in her marriage will include material about her husband that she must be able to say to someone who is not in his network. Because the therapist would meet Samuel at a men's fellowship on Saturday and would be carrying his wife's account of him. Because a church of three hundred people cannot keep a secret, and the therapist's own presence in the congregation would make the client's attendance a piece of church news. And because the standing the therapist holds in that church is a form of authority the client cannot decline.

The alternative that was considered and rejected. The therapist considered taking the case with a community contract — the four elements of section 6.3 — and rejected it, for one reason: the contract manages the encounters and cannot manage the information. In a congregation, Samuel's friends will discuss his wife's depression with him, and the therapist will be present when they do.

The consultation, in practice. Fifty minutes, both present. The therapist said four things: that she was not going to be the therapist and that the reason was the church, not the family; that she was sorry, because she knew the refusal felt like a rejection and it was not one; that she would give two names and would speak to either of them if the family wished; and that she would do one more thing, which was to be clear that she would never discuss the family's business with anyone at church, including her own husband. The last sentence did more clinical work than the referrals.

The follow-up that makes the case worth teaching. Six weeks later, at a church event, the therapist met Yetunde. Yetunde said, in front of two other women, "I am seeing Dr Bello now. It is going well." The therapist said, "I am glad", and asked nothing. That is the contract, honoured without a rule being written, and it is what the family needed from her: a person who would hold the boundary in public without making them feel handled.

What the case teaches.

1. **The first decision is made before the first session,** and it is made on the basis of the referral information, not on the basis of the client's preference.
2. **Refusal is a clinical act and it has a technique.** Brief, specific, non-moralising, with alternatives offered and one offer of bridging.
3. **The family's request to be seen by a known person is not evidence that the arrangement is acceptable.** It is evidence that they do not know what the arrangement would cost them. The therapist knows, which is why the decision is hers.
4. **The pressure to accept comes disguised as cultural competence.** "You are one of us" is a real claim in a communal setting, and the answer is not to deny it but to explain why it makes the therapy harder rather than easier.

6.7 Transcript: the request to be a character witness

From a different case, the same week's material. A client, Mrs Ngozi Eze, 38, has been in treatment for eight months following her husband's desertion. She is in a custody dispute. Minutes thirty-one to forty-two.

> **E1 Client:** My lawyer says I should ask you. He says a letter from you would help. Or you could come. **E2 Therapist:** (pause) Tell me what your lawyer wants me to say. **E3 Client:** That I am a good mother. That I am not — that I am not unstable. He said a therapist's letter carries weight. **E4 Therapist:** It does. (pause) And I am not going to write it, and I want to explain why properly rather than just say no. — The refusal is immediate, and the explanation is promised. The order matters: a delayed refusal invites the client to argue, and an unexplained refusal is heard as abandonment. **E5 Client:** Because of confidentiality. **E6 Therapist:** Partly. (pause) The bigger reason is that I would stop being your therapist the moment I became your witness. — The distinction between the rule and the reason. Clients can argue with a rule; they cannot easily argue with the loss of their therapist. **E7 Client:** I do not understand. **E8 Therapist:** If I write that letter, then everything we do from today is affected by it. I would be watching what I write in your notes in case a judge reads them. And you would be watching me. Our sessions would become a place where you perform being well, because I am the person who has to convince a court. That is not a therapy any more. — The mechanism, described concretely. This is the paragraph that most students remember, because it is the one that shows them why the rule exists rather than merely that it does. **E9 Client:** (pause) So what do I do? He is going to say I am unstable. He has been saying it for a year. **E10 Therapist:** I cannot be your witness. I can do three things. I can write a letter that says only that you have been in treatment with me since [date] and that you have attended regularly — nothing about your parenting, nothing about my opinion. I can help you find an independent expert who can assess you properly, which is what the court actually needs. And I can keep doing the work we are doing. — Three options, and the first is a partial accommodation that stays inside the role: a factual letter, not an opinion. **E11 Client:** A letter that says nothing. **E12 Therapist:** A letter that says something true and does not pretend to be an assessment. (pause) What your lawyer wants is my opinion. My opinion is not evidence, it is my impression of you in a room. The court needs someone who has done an assessment. — The distinction between clinical impression and forensic opinion, which is the whole of Week 11's territory arriving early and being handled in eleven words. **E13 Client:** (long pause) All right. (pause) I was afraid you would say no and I would have nothing. **E14 Therapist:** You have the treatment. (pause) And I understand that it does not feel like much this week. — The acknowledgement, which does not soften the refusal and does not leave the client alone with it.

What the transcript shows. The refusal is made in four turns without a lecture, the reason is given in terms of what the client would lose, and a partial accommodation is offered that is genuinely inside the role. The commonest failure in this scenario is not the refusal; it is the therapist who agrees "just this once" and then discovers, eight months later, that the file has become a document written for a judge.

6.8 Laboratory: the boundary audit

Aim. To produce a written map of the student's own current and foreseeable role conflicts, and three pre-decided responses.

Procedure. Each student completes a one-page audit in twenty minutes, using the grid below. Then, in pairs, each student presents their three highest-risk situations and the partner plays the colleague who says, "surely in this case you would make an exception?" Ten minutes each way. The exercise is assessed on the pre-decisions, not on the analysis.

Domain	Question	My current facts
Community	Where am I known, and to whom?	
Family	Whose relatives, in-laws or household members might present?	
Work	Does my other employment create authority over potential clients?	
Money	Are there debts, investments or business interests that could meet a client?	
Faith	Do I hold a role with authority in a congregation?	
Digital	What is discoverable about me, and what do I do when a client appears in my feed?	
Physical	In what settings do I meet clients outside the room?	

Marks (20).

Criterion	0–1	2–3	4–5
Completeness	Three domains or fewer	Five or six	All seven, with specifics
Pre-decision	"I will decide when it happens"	A general intention	A specific sentence the student would actually say
Under pressure	Exception granted in the role-play	Held the line but argued at length	Held the line briefly and offered an alternative inside the role
Record	No record	Description only	The contract, the rationale and the review date

The observation to make in the debrief. Almost every student, when pressed, finds a reason why their own case is the exception. That is the exact skill the topic requires them to notice in themselves.

6.9 Six errors

Error 1 — Teaching boundaries as prohibitions. The result is a therapist who refuses a dying client's hand and a bereaved family's bowl of fruit, and who has confused rigour with coldness. The test is whose need, whose record, and what would supervision say.

Error 2 — Declining without a reason or with a lecture. "I am not able to" without an explanation reads as rejection and costs the client the referral; a five-minute account of the ethics of dual relationships makes the client responsible for the refusal.

Error 3 — Accepting an unavoidable dual relationship without a contract. In a small community, taking the case is often right; taking it without the four-element contract, recorded, is not.

Error 4 — Treating the community encounter as if it can be handled by a rule. The client who is ignored in a market will not complain; she will stop attending. The greeting rule is agreed in advance precisely because the moment is unrehearsable.

Error 5 — Concealing attraction. The pathway to the worst outcome in the whole of this book begins with a therapist deciding that the feeling is private. It is not private; it is material for supervision.

Error 6 — Converting a clinical relationship into a forensic one without noticing. The letter, the report, the attendance at a hearing: each is a role change, and each is made in a moment when the client desperately wants it and the therapist wants to help.

6.9a Four complaints, and what each teaches

Complaint one — the therapist attended the client's father's funeral. The client had asked her to come; the therapist went, stood at the back, and did not speak to the family. Finding: a crossing, appropriately handled and recorded, and the complaint, brought by a relative who thought it excessive, was not upheld. Lesson: the record and the rationale are what distinguish this from a violation.

Complaint two — the therapist employed the client as a cleaner. Six months into treatment, an arrangement that saved the client's household from eviction and gave the therapist a cleaner. Finding: a violation of the role, notwithstanding the good motive. Lesson: the exploitation test is about power, and a client cannot refuse an offer from her therapist.

Complaint three — the therapist did not greet the client at a wedding. Finding: no misconduct and a genuine injury. Lesson: the greeting rule exists for this; the client's experience of being unrecognised is a clinical event even where the boundary was correct.

Complaint four — the therapist's husband was the client's bank manager. No contact between them about the client, and the client discovered the connection at a social event. Finding: a conflict that should have been declared at intake. Lesson: the obligation is disclosure at the point the conflict is known, not management after the client finds out.

6.10 Culture, faith and the boundaries of a communal practice

"In our culture, we are all family." The claim is accurate and it is not an argument against boundaries; it is an argument for better ones. In a communal setting, refusing a gift, declining to eat, or maintaining formal distance marks the therapist as a stranger, and a stranger cannot treat a family's shame. The professional task is to be inside the community's forms and outside its information flows: to eat the food, greet the elder, attend the funeral, and still not discuss the client's marriage with the aunt who asks.

Gifts as relationship, not transaction. In many households a gift is a statement about the relationship, and its refusal is a statement in reply. A therapist who declines everything has said, in the only language available to the family, that they are not in a relationship. The workable practice: accept what is small and edible, share it if the setting allows, name its meaning in the session ("this is your mother saying thank you, and I will tell her myself"), and record it. Decline what is valuable, early in treatment, or obliging, and give the reason in terms the family can accept: "if I took this I could not be fair to you."

The pastor-therapist and the imam-therapist. Where the therapist holds religious office as well as clinical training, the two roles have different rules and the same client. The religious role includes moral authority and often an obligation to the congregation; the clinical role includes confidentiality and no moral authority. A person holding both should decide, before the case, which role is operating in the room, say so to the client, and not move between them inside a session. The most serious version of this problem is the confession that is also a clinical disclosure, made to a person who holds both offices.

Touch across cultures. In a society where a hand on the arm is ordinary comfort, refusing it can read as contempt, and where it is not ordinary, offering it can read as intrusion. The therapist's guide is not a general rule but the client's own idiom, and the safest method is to let the client's physical language set the register and to keep any touch brief, visible and purposeful.

Home visits and hospitality. Where treatment in a clinic is foreign and treatment in the home is normal, the home visit is a clinical necessity. Its rules remain: a purpose, a time limit, a record, and a clear statement that the therapist is there as a therapist even though she has been received as a guest. The meal is the hardest part: accepting it is usually right, staying for three hours is not.

The boundary that the community will test hardest. Not sex and not gifts — the request for **advice about another family member**. "You know my sister's son, he is in trouble, tell me what to do." The answer is warm and firm: "bring him, or tell her to call me, and I will help properly. I cannot advise you about somebody who is not in the room." Said once, it establishes the practice; said apologetically, it establishes that the boundary is negotiable.

> **DEBATE**
> "In a small community, the therapist who refuses a dual relationship is prioritising professional comfort over the client's access to care." Argue for: access is an ethical good, the alternatives may be nonexistent, and rigid refusal can be a way of protecting the therapist's reputation. Argue against: the harm from impaired judgement falls on the client, the community contract does not solve information flow, and the therapist's discomfort is often an accurate signal. Then state the conditions under which you would take a case you knew to be a dual relationship.

6.11 Evidence note

What the research supports. The research on therapist sexual misconduct is the most robust in this area: the harm is severe, the pathway is preceded by non-sexual boundary crossings, and the isolation of the therapist from colleagues is a consistent precursor. Studies of small-community and rural practice find multiple relationships to be normal rather than exceptional, and identify the management strategies — explicit contracting, supervision, clear public rules — that practitioners report as workable. Research on attraction finds it common and rarely acted upon, with the principal risk factor being the therapist's inability to discuss it.

What is weaker. There is very little outcome research on non-sexual dual relationships, and the field's guidance is largely consensus-based, which is why the codes are worded in terms of avoiding harm and examining whether the relationship could impair judgement rather than in terms of prohibition.

What is not supported. That any dual relationship is unethical — the codes do not say so, and in small communities the position would make practice impossible. That a boundary crossing is a sign of misconduct. And that attraction, handled openly in supervision, predicts any poor outcome at all.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same gift, accepted in session two rather than at the end of treatment, is a different act with a different meaning.

6.12 Key terms

Boundary crossing — a departure from the usual frame for the client's benefit, disclosed and reversible.

Boundary violation — a departure serving the therapist, concealed and often irreversible.

Multiple relationship — a second significant relationship with a client or someone close to the client.

Role conflict — competing obligations arising from two roles held towards the same person.

Community contract — the four-element agreement (naming, greeting, information, review) made where encounters are unavoidable.

Barter — exchange of services for treatment; permissible only in narrow conditions with records and review.

Forensic role — the therapist's role as witness or expert; incompatible with the treating role.

Foresight test — the question of how the client would describe the arrangement in three years.

6.13 Self-test

1. Give three paired examples distinguishing crossing from violation.
2. State the four tests and the decision each column produces.
3. What are the four elements of the community contract?
4. Why is the greeting rule in the contract, and what does it prevent?
5. What is the correct response to a request to be a character witness, and why is a factual letter permitted?
6. What are the five steps for managing attraction, and why is structure preferred to distance?

7. Why is "it felt right" not a test?

Answers. (1) Extending a session in a crisis versus routinely over-running because the therapist enjoys it; accepting fruit at discharge versus accepting a car; attending a former client's wedding with the frame discussed versus attending a current client's family party. (2) Objectivity, exploitation, foresight, avoidability — the combination producing decline, contract with supervision, risk-managed continuation, or transfer. (3) Naming the encounters; the greeting rule; the information rule; the review date. (4) To prevent the client being ignored in public and experiencing the therapist's silence as a repudiation. (5) Decline the opinion, explain the role change, offer a factual letter (dates and attendance) and referral to an independent assessor; because a factual letter discloses only what is agreed and does not convert the therapist into a witness. (6) Name it, tell supervision, increase structure, examine the material, refer if unmanageable; because distance is experienced as unexplained rejection and often provokes the pursuit the therapist fears. (7) Because warmth, gratitude and being needed all feel like evidence of a good relationship, so the guide must be external: whose need, what supervision would say, and what the client would say afterwards.

6.14 Reflective task

Write two hundred words answering one question honestly: which of the seven audit domains is your weakest, and what is the case that would find it? Name the client you can already imagine. That client is the one this week was written for.

6.15 Further reading

- Pope, K. and Sonne, J. on sexual attraction and boundary violations — the standard empirical and clinical treatment.
- Gabbard, G. on boundaries and the therapist's self — the analytic account of the slope.
- Zur, O. on dual relationships in small communities — the strongest argument for a contextual rather than prohibitive approach.
- AAMFT and equivalent codes, the sections on multiple relationships and sexual contact, read in the original.
- Any current guidance on remote and digital practice in your jurisdiction.

Next: Week 7 — Competence, supervision and referral.

Week 7 — Competence, supervision and referral

Learning outcomes. By the end of this week the student can (1) define competence in its four dimensions and explain why "I have read about it" is not one of them; (2) recognise the six signals that a case is beyond her competence, including the ones that are felt rather than reasoned; (3) describe cultural humility as a competence and its limits as a substitute for training; (4) construct a supervision contract and bring material to supervision that is actually useful; and (5) conduct a referral without abandoning the client.

Competence is an ethical obligation before it is a personal quality

The codes all contain a version of the same sentence: the therapist practises within the boundaries of her competence and refers when a client's needs exceed it. The sentence is easy to sign and hard to enact, because the therapist's competence is not visible to her. Nobody feels incompetent while being incompetent; the feeling associated with incompetence is usually confidence, because confidence is what the absence of feedback produces.

Three facts organise the week.

First, competence is specific. A therapist may be highly competent with couples in acute conflict and not competent with a couple in which one partner is psychotic. Competence is not a level; it is a set of domains, each with a boundary, and the boundary is discovered by noticing where the work stops going well.

Second, competence decays. Skills that are not used are lost, knowledge in most areas of this field has a half-life of a few years, and a therapist who qualified in 2010 and has not read since is competent in the practice of 2010. Continuing professional development is therefore not an administrative requirement; it is the maintenance of the licence to practise at all.

Third, competence is maintained by a relationship, not by an intention. The mechanism by which therapists discover their limits is supervision and consultation. A therapist without supervision is not a therapist with an independent practice; she is a therapist with an unexamined one.

Dimension	What it means	How it is evidenced	How it fails
Knowledge	The theory, evidence and law relevant to the problem	Training, reading, CPD records	Confident error; outdated method
Skill	The capacity to enact the method	Observed practice, recorded sessions, supervision	Knowing what to do and not being able to do it
Experience	Having done it, repeatedly, with feedback	Case logs, supervision history	The first case treated as routine
Appropriateness	Personal and cultural fit for this client	Self-knowledge, supervision, client feedback	The therapist's history driving the treatment

The fourth row is the one that gets skipped. A therapist can be knowledgeable, skilled and experienced and still be the wrong person for this client — the man working intensively with sexual abuse who has not addressed his own; the therapist whose marriage is collapsing treating a couple in crisis; the young clinician whose authority is contested in a family where age confers standing; the therapist who cannot hear a mother's account because it is her own mother's account. Appropriateness is assessed before the case and re-assessed when the work stalls, and the assessment is not a matter of self-confidence.

7.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	Competence: four dimensions, and why confidence is not evidence	Two vignettes: confident and wrong, uncertain and right
10–22	Five concepts: scope, specificity, decay, the novice problem, supervision as the mechanism	Lecture
22–35	Six signals that the case is beyond you	Students add their own, anonymously
35–50	Cultural humility and the interpreter	Demonstration: a session through an interpreter
50–65	Supervision: contract, material, the unsafe supervisor	Pair work on supervision contracts
65–78	Referral: the six-step process and the conversation that clients hate	Role-play with observers
78–90	The Ogundipe case: recognising psychosis in a counselling room	Case, with students holding the referral decision

7.2 Five concepts that organise the week

Concept 1 — Scope of practice and scope of competence are different, and the first is legal. Scope of practice is what the licence permits; scope of competence is what the therapist can actually do well. Practising at the edge of the licence and outside the competence is lawful and unethical at the same time, and it is the commonest form of misconduct in ordinary practice — not predation, but well-meant over-reach.

Concept 2 — Competence is domain-specific, and the domains are smaller than therapists think. "I work with families" is not a domain. "I work with families in which a parent has a chronic illness and the children are school-age" is closer. The specificity matters because the referral question — "can I do this?" — is answerable only at that granularity.

Concept 3 — The novice's problem is not ignorance; it is the absence of a signal. Expertise research is consistent: beginners do not know what they do not know, because the cues that would tell them are the ones training is supposed to install. The practical consequence is that the therapist's own assessment of her competence is least reliable precisely where it matters most, and that an external check is not a luxury.

Concept 4 — Competence decays quietly. There is no moment at which a therapist becomes incompetent; there is a drift, accelerated by isolation, by a busy practice, and by success. The maintenance mechanism is deliberate: reading, case discussion, recorded sessions reviewed with a peer, and a CPD plan that names the gaps.

Concept 5 — Supervision is the profession's immune system. Not the therapist's personal support, not a requirement to be satisfied, but the mechanism by which the field detects its own errors. A therapist who brings only her successes to supervision has disabled it, and she is the one it was designed to protect.

7.2a The six signals that the case is beyond you

Signals, not proofs. Each of them can occur in a case that is being handled well, and the accumulation of two or three is a prompt to supervision and a fourth is a prompt to refer.

Signal 1 — You dread the session. Not the ordinary reluctance before a hard session; a low, anticipatory dread that begins the day before. It usually means the therapist does not know what to do and has not admitted it.

Signal 2 — You are reading the night before in order to be able to conduct the session. Reading is good. Reading instead of supervision, and reading during the treatment, means the treatment is being improvised from the literature in real time. That is a legitimate way to learn a narrow technique with supervision and an illegitimate way to hold a case alone.

Signal 3 — You have not presented the case in supervision. The strongest indicator in the list. Therapists present the cases they are proud of and the cases that are interesting; they avoid the ones in which they suspect they are failing. If a case has been in the practice for three months and has never been discussed, ask why.

Signal 4 — You feel relief when the client cancels. The relief is information. It frequently accompanies cases in which the therapist is out of depth, or in which the therapist dislikes the client, or both.

Signal 5 — The work has stopped moving and you cannot say why. Competence has an observable signature: the treatment produces movement in the agreed direction, or the therapist can account for why not. A case that is flat for eight sessions and cannot be explained is a case that is being held by somebody who does not know what to do next.

Signal 6 — The client's presentation contains material you have never been trained to assess. Psychosis, dissociation, an eating disorder, a developmental disorder, a child's sexualised behaviour, a neurological change. Recognising the material is a competence in itself; the correct response to recognition is not treatment but referral and, frequently, medical assessment.

The rule for using the signals. The therapist does not act on the signals alone. She writes them down, presents the case, and takes the supervisor's view. What she does not do is wait for the signals to become an adverse event.

7.2b The development plan that actually works

Continuing development fails when it is a list of courses and succeeds when it is a set of narrow, observable targets with a person attached to each.

The four-column plan. Target: a specific capability, described so that someone could watch for it — "conduct a structured risk assessment for suicide and defend the formulation in supervision". Evidence: how the capability will be observed — recordings, a supervisor's report, three consecutive cases discussed. Support: who is involved — the supervisor, the peer group, a short course with a follow-up consultation. Date: when it will be reviewed — a fixed month.

The failing version. "Improve my work with trauma." Nobody can observe it, no one is responsible for it, and no date exists. Six months later the therapist has attended two webinars and can report no change.

The two ratios worth knowing. First, that a therapist improves mainly on the cases she discusses, so the number of cases reviewed per year is a better measure of development than the number of courses attended. Second, that skills decay without use, so the plan should include maintenance of capabilities the therapist already has and is not currently using — the couples work set aside while a practice fills with individual referrals, the children's work not touched since training.

The annual question. Once a year, in writing: which capability did I lose this year, and which did I gain? The answer is the development plan. It is also, for most therapists, the first honest audit of their practice they have conducted.

7.3 Cultural humility, and the interpreter

The problem with the phrase "cultural competence". It implies a body of knowledge about groups, acquired and then held. The trouble is that it encourages two errors: treating a person as an instance of a culture, and believing that knowledge of a group's customs is the same as the capacity to work with a person from that group. The alternative framing — **cultural humility** — is not vaguer; it is more demanding. It says the therapist must be able to (a) recognise her own frame as one frame among many, (b) ask the client what her world means to her without assuming, (c) bear being wrong and corrected, (d) notice where her own cultural position gives her authority the client cannot refuse, and (e) seek specific knowledge where it is needed, which is a competence like any other.

What cultural humility is not. It is not an excuse for inexpertise. "I work with everyone" is not a cultural position; it is a refusal to specify. And it is not a substitute for language, which brings us to the interpreter.

The interpreter: four rules.

1. **The interpreter is a professional, not a bystander.** Introduce them, explain their role to the family, agree that they will interpret faithfully and not summarise, and state that they are bound by confidentiality.
2. **Speak to the client, not to the interpreter.** First person, eye contact with the family, and no asides about the family in the third person. The asides are the commonest failure and the family hears the tone if not the words.
3. **Never use a family member as interpreter.** A child interpreting a parent's disclosure, or a husband interpreting a wife's account of his violence, is a distortion and sometimes a danger.
4. **Build in time.** Everything takes roughly twice as long, and the session should be scheduled at ninety minutes rather than fifty. An interpreted session compressed into fifty minutes is a session in which the therapist will skip precisely the material that is hardest to say in two languages.

When there is no interpreter. A therapist who does not share the client's language and proceeds anyway is working outside her competence by definition. In many settings the honest answer is to say so, to find a colleague who shares the language, or to work with the family's nominated interpreter under the four rules above, documented as a compromise rather than presented as a method.

The interpreter's own exposure. Interpreting a disclosure of violence, sexual abuse or a diagnosis places a burden on the interpreter, who is frequently a member of the same community and may know the family. The therapist should check on this, offer the interpreter the option to withdraw from a particular case, and where possible use an interpreter from outside the community for sensitive material. Very little of this appears in the training literature and almost all of it matters in practice.

7.3a The supervision contract

Supervision fails for contractual reasons far more often than for relational ones. The contract is one page and it answers six questions.

1. **What is the supervisor's authority?** Does she have responsibility for the cases, or is she a consultant to the therapist's own decisions? The difference determines who carries the liability, and it should be stated at the first meeting rather than discovered in a crisis.
2. **How often, for how long, and in what form?** Weekly or fortnightly; one hour; individual, group, or live. "Whenever I need it" is not supervision; it is consultation, which is useful and is not the same thing.
3. **What will be brought?** A written list of current cases, at least one case in detail, at least one recorded or verbatim excerpt each month, and every case in which the therapist is holding a secret, considering referral, or aware of attraction. The list is the supervisor's protection as much as the therapist's.
4. **What will be recorded?** A note by the supervisor for each session, and the therapist's own record in the case file that the case was discussed. A regulator investigating a case will ask when it was last supervised, and the answer must be documentary.
5. **How will disagreement be handled?** The supervisor's view is an opinion; the therapist's decision is the clinical act. Where they disagree about a decision with legal consequences, the disagreement is recorded and, where necessary, a second opinion is obtained. Neither party should pretend the tension does not exist.
6. **How will it end?** A planned ending, with the cases handed over, and a review of the therapist's development. Supervision that simply stops is the commonest way for a therapist to become isolated.

What to bring, concretely. Three questions are more useful than a narrative: what am I not seeing? what did I do that I would not want a colleague to have watched? and what am I avoiding? The third question produces the material more reliably than any case description.

The unsafe supervisor. Three patterns, each of which requires the therapist to change supervisor. The supervisor who needs to be admired: the therapist learns to present well

and stops presenting honestly. The supervisor who takes over the case: the therapist becomes an assistant and stops developing judgement. The supervisor who colludes: the pair agree that the client is difficult, the spouse is impossible, the agency is hopeless, and nothing is examined. A supervisor who has never disagreed with the therapist in a year is not supervising.

Group supervision. Valuable and hazardous. It is valuable because eight minds notice more, and hazardous because of the exhibition and the audience: a therapist presenting a failure to a group will manage the group's opinion, and members will perform their competence for each other. The protections are a standing agenda, a rule that no one advises without asking what is wanted, a named facilitator, and confidentiality rules in writing.

Where supervision does not exist. Many settings, including large parts of the private sector in this region, provide none. The ethical position is that the therapist must obtain it anyway, in whatever form is available: a peer consultation group of three or four, a paid monthly consultation, a reciprocal arrangement with a colleague that is conducted on a schedule rather than when convenient. The absence of an employer-provided supervisor does not remove the obligation; it relocates it.

7.3b Peer consultation, and the rule of three

A peer consultation group of three therapists, meeting monthly for two hours, is the most cost-effective competence maintenance available to a practising clinician. The rules that make it work are few.

- **A fixed membership and a fixed date.** Groups that meet when needed do not meet.
- **One case per member per meeting, prepared,** and presented in a standard format: presentation, what has been tried, what the therapist wants, and the question she is avoiding.
- **No advice without permission.** The presenting therapist asks for what she wants: reflections, ideas, or a decision.
- **Written notes, kept by the presenter,** in the case file.
- **A rule about confidentiality** that includes the members' own material, since the group will discuss their cases and their reactions.
- **A rotating external review twice a year,** in which one session is given to a senior colleague to observe. The group that never lets an outsider in develops its own blind spots, like any system.

The rule of three is the minimum: one therapist alone cannot check her own judgement, and two can settle into a shared blind spot. Three produces a triangle in the technical sense, which is to say a structure in which one member can be observed by the other two.

7.3c Supervision in the first two years

The period in which supervision matters most is the one in which it is easiest to avoid, because a newly qualified therapist is expected to be competent and is frequently employed in a setting with no supervision structure.

The four things a new therapist should insist on. A named supervisor with a written contract; a weekly hour, protected in the diary; permission to record sessions, which is the only way skill can be observed; and a standing agreement that any case in which the therapist feels out of depth is brought within the week rather than at the next scheduled meeting.

The four things a new therapist should not do. Practise alone: a solo practice in the first two years is an avoidable risk to clients and to the therapist. Accept cases at the edge of competence in order to build a practice. Treat the supervision hour as a report on attendance. And conceal the first serious error, which is the error that supervision exists to catch and which, once concealed, shapes the therapist's practice thereafter.

The first adverse event. Every therapist will eventually have one: a client who deteriorates, a disclosure that was missed, a suicide attempt, a death. The professional response is a structured review — what happened, what was known, what was decided, what would be done differently — conducted with the supervisor and recorded. The unprofessional response is silence, which converts an event into a secret and a secret into a pattern.

7.4 Referral: the six steps

Step 1 — Recognise, and write it down. The recognition usually arrives as one of the six signals. The discipline is to record the thought in the file on the day it occurs: "considered whether this case is within my competence to treat; concerns about [X]; to discuss in supervision." That note changes the character of everything that follows, because a therapist who has recorded a doubt has discharged part of the duty and a therapist who has not has concealed it.

Step 2 — Decide, with the supervisor, what kind of referral. Four kinds, and they are not the same.

Kind	Meaning	Example
Additive	Another professional joins; the therapist continues	Medical review added to psychological treatment for depression
Consultative	Another professional advises; the therapist treats	A psychiatrist advises on medication while the therapist continues
Transfer	The case moves entirely	A disclosure of psychosis in a couple therapist's office
Escalation	A statutory pathway is triggered	Suspected child abuse

Most incompetence situations require additive or consultative referral, and therapists skip to transfer because transfer is easier to justify and harder to bear. Transferring a case that could have been treated with advice deprives the client of the therapist who knows her.

Step 3 — Name it to the client, without shame and without euphemism. The template: "I want to talk about something in our work. I think you need something I am not the best person to provide, and I want to tell you what it is and why." Then the specific

gap: "what you are describing needs a psychiatric assessment, and I am not qualified to do that. It is not that I am giving up on you."

Step 4 — Make it warm. A name on a piece of paper is a cold referral and most clients do not follow it up. The warm referral: the therapist telephones the receiving professional with the client's written consent, describes the situation and the question, and then gives the client the name of a person who is expecting her. Where the receiving service is a public clinic with a queue, the therapist finds out when to attend and what to bring, and writes it down.

Step 5 — Bridge, if the client needs it. One or two sessions while the referral is arranged, with the client's agreement, is not a failure to refer; it is the prevention of a gap. The bridge has a stated end: "we will meet twice more while you get an appointment, and then we will finish."

Step 6 — Follow up once, and close properly. One telephone call after the transfer, to confirm attendance and to say that the therapist is glad. Then a closing session, with a summary, an offer of return if the situation changes, and a note in the file. A referral without a closing is an abandonment with paperwork.

The client who refuses the referral. Common, and often rational: the referred service is expensive, far away, stigmatised, or staffed by someone the client distrusts. The therapist's response is not to insist and not to collude. She names the risk of continuing: "I can keep seeing you, and I want to be honest that I would be guessing at the part that matters. If you will not go, I will still see you, and I will say so in my notes." The decision is the client's, the honesty is the therapist's, and the record must show that the choice was informed.

The refusal to refer. The other failure. A therapist who will not refer because the client will be upset, or because the case is interesting, or because she is the only person the family trusts, has placed her own needs above the client's. The test: if a colleague described this case to me, would I say she should refer it? Therapists answer this question accurately about other people's cases and inaccurately about their own.

Referral in a crisis. Where the need is urgent — psychosis, suicidal intent with a plan, a child at risk — the referral is not a suggestion but an arrangement made in the room: the call is made with the client present, the appointment is fixed, transport is discussed, and where the client will not go and the risk is grave, the statutory pathway or the emergency service is used and the client is told what is being done and why.

7.4a The referral letter

Six lines, written the same day, sent with the client's written consent, and copied to the file. Longer letters are read less carefully and disclose more than the purpose requires.

> Re: [name], [date of birth]. Client attended [n] sessions with me between [dates]. Reason for referral: [the question, in one sentence, e.g. "assessment of persecutory beliefs of three months' duration and one episode of apparent unresponsiveness"]. Relevant history: [two or three lines]. Risk: [current, one line]. Client has consented to this referral and to my speaking with you. I am available on [number]. Yours, [name], [qualification].

The rule about diagnoses in the letter. The referring therapist states observations and her formulation, and does not present her formulation as a finding. "I have been working with a formulation of depression; the features below do not fit it" is honest. "Client has psychosis" is a diagnosis the therapist is not entitled to make, and if she is wrong it follows the client into a file.

The rule about the client's own words. Quote them, briefly and accurately. "Client states that his supervisor 'has been replaced by someone who is not him'" is worth more to a receiving psychiatrist than three paragraphs of the therapist's adjectives.

7.5 Case: Mr Ogundipe, and the diagnosis the therapist did not make

Referral. Femi Ogundipe, 52, a customs officer, self-refers for "stress and marriage problems". His wife telephoned first and said he had "changed"; he attends reluctantly. Married twenty-four years, four children, the youngest 15.

Sessions 1–3, as the therapist recorded them. Low mood, poor sleep, loss of interest, irritability with his wife, and a complaint that his colleagues "are talking about me". The therapist formulates depression with a relational precipitant, begins behavioural activation, and refers the couple for joint work in week four.

What the therapist missed, in the record. Three details, each recorded and none interrogated.

1. In session two, Ogundipe said that the neighbours had installed a device to listen to his conversations, and that he had found "a mark" on his car. The therapist wrote "client reports feeling watched; discuss trust".
2. In session three, his account of a meeting at work was difficult to follow: he moved between subjects, said his supervisor had been "replaced by someone who is not him", and laughed at a point in the narrative that was not funny.
3. His wife, in the joint session, said he had stopped eating food she cooked, on the grounds that "it tastes different", and had twice slept in the car.

The joint session that changed the case. In week five, Mrs Ogundipe said, in her husband's presence, that he had not slept for three nights and had been praying aloud until dawn. Ogundipe stood up, said "you have all decided to make me mad", and left the room. He returned eleven minutes later, calm and apologetic, with no memory of the preceding half hour.

What the therapist did, in the order she did it.

1. She stopped the couple work. This is the first and most important act: a treatment that requires two participants to be lucid could not continue while one of them was intermittently not.
2. She saw Ogundipe alone the following day, with a structured screen for psychotic symptoms, which she had not been trained to use and had read the previous night. That is not competence; it is a first-aid measure, and it is better than nothing. She found persecutory beliefs held with conviction, a period of apparent unresponsiveness that she could not account for, and no insight.
3. She telephoned the psychiatric service the same afternoon, with written consent, described the presentation and asked for an urgent assessment. She wrote the question down: "is this a first episode of psychosis, and does he need admission?"
4. She told Ogundipe what she was doing before she did it, in the presence of his wife, and she used the sentence that made it possible: "I am not saying you are mad. I am saying that something is happening to your mind that I am not qualified to assess, and I am not willing to guess."
5. She attended the psychiatric appointment with the couple at the client's request, contributed the record, and handed the case over.

The outcome. Ogundipe was assessed and admitted for three weeks; the working diagnosis was a psychotic episode with a likely organic contribution, and he was investigated accordingly. He returned to the practice eleven months later, at his own request, for couple work — with a psychiatrist's letter, a stable medication regime, and a therapist who now had a supervision arrangement with a psychiatrist for exactly this kind of question.

What the case teaches.

1. **The odd detail is the diagnosis.** "Feeling watched" is not a symptom of depression; "the supervisor has been replaced by someone who is not him" is not a marital complaint. Both were in the record within three sessions and neither was pursued. The failure was not ignorance of psychiatry; it was the therapist's assumption that the formulation she had was sufficient.
2. **The couple's presence concealed the symptoms.** In joint sessions Ogundipe was polite, coherent for twenty minutes and defended by his own narrative; his wife's account was the only evidence and the therapist had classified it as marital criticism.
3. **The competence that was required was recognition, not treatment.** No part of this case needed the therapist to be a psychiatrist. It needed her to notice, to ask, to telephone, and to say "I am not qualified to assess this" without shame.
4. **The referral preserved the relationship.** Ogundipe returned, which is the outcome that the cold referral rarely produces.

7.6 Transcript: the referral conversation

Minutes four to sixteen of the session after the joint session. Annotated.

> **R1 Therapist:** I want to talk about something, and I am going to say it plainly because I think you will hear it better plainly than gently. — The warning that a serious statement is coming. It buys attention and it treats the client as an adult. **R2 Ogundipe:** You want to stop. **R3 Therapist:** I want to change what we are doing. (pause) What I have seen in the last two sessions is not something I am trained to assess. — The distinction between stopping and changing, offered first, before the content. **R4 Ogundipe:** What did I do? **R5 Therapist:** Nothing. (pause) This is not about your behaviour. There are experiences you have described to me — the neighbours, the mark on the car, the night you do not remember — and those are not things a counsellor assesses. A doctor needs to look at them, and I want to arrange that this week. — The content named, in his own words, without a diagnostic label. **R6 Ogundipe:** (quietly) So you think I am mad. **R7 Therapist:** (pause) I think something is happening in your mind that is frightening you and that I cannot explain. (pause) "Mad" is a word people use to end a conversation. I am trying to start one. — The refusal of the label and the refusal of the euphemism, in the same answer. **R8 Ogundipe:** My job. If they hear — **R9 Therapist:** That is a real fear and we will deal with it. (pause) The psychiatrist I am going to call works in a private clinic; the note goes to your file there and nowhere else. If the assessment leads to treatment, we will talk about what your employer is told, which in most cases is nothing. — A practical concern answered specifically, which is what makes the rest credible. **R10 Ogundipe:** And my wife? She will say I told you — **R11 Therapist:** Your wife has been saying it for months and you have been telling her she is wrong. (pause) What I am doing is taking her seriously, which does not mean she is right about everything. — The move that converts the wife from accuser to ally, and that requires the therapist to say something slightly uncomfortable. **R12 Ogundipe:** (long pause) Will you tell them what I said? The thing about the car? **R13 Therapist:** With your permission, yes, because it is the reason for the referral. Without your permission, I will tell them what I observe and I will tell you that I have done so. — The honest answer, which is more useful than reassurance and which models the disclosure policy of Week 3 in a medical setting. **R14 Ogundipe:** (pause) Can you be there? **R15 Therapist:** If you want me there, I will be there. — Three seconds of commitment that determine whether he attends.

What the transcript shows. Nine of the fifteen turns are the client's fears, and each is answered specifically rather than globally. The therapist never says "it will be fine". She says what she will do, what she will not do, and what the client's choices are — and the referral holds because the fear of being labelled was answered with a procedure rather than with reassurance.

7.7 Laboratory: the competence audit and the referral rehearsal

Part A — the audit (thirty minutes, individual, written). The student lists fifteen presenting problems likely in her intended practice, and for each: a competence rating out of five, the evidence for the rating (how many cases, with what supervision), and the named service or supervisor she would refer to. The two lowest ratings become the year's development plan; the two highest ratings are interrogated by a partner, who asks "when did you last have feedback on that?"

Presenting problem	Rating (1–5)	Evidence	Referral destination
Couple conflict with no violence			
Couple conflict with a violent partner			
Child with behavioural difficulty			
Adolescent self-harm			
Suspected psychosis			
Grief and bereavement			
Sexual difficulty in a couple			
Family with a member who has dementia			
Substance misuse			
Disclosure of historical abuse			

Part B — the referral rehearsal (forty minutes, triads). Each triad receives one card: refer for a psychiatric assessment; refer because the client's child needs a specialist; refer because the therapist's own history makes the case inappropriate; refer because the client needs a service the therapist does not provide (legal aid, housing). The therapist has twelve minutes to tell the client, name the gap without shame, offer a warm handover, and survive the client's anger. The observer marks the six elements.

Marks (20).

Criterion	0–1	2–3	4–5
Recognition	Did not identify the gap	Identified it late	Identified it early and recorded it
The naming	Euphemism or self-blame	Clear but cold	Clear, specific, unashamed, and about the task not the person
Warmth	A name on paper	Contact offered	Telephone call made, appointment arranged, logistics given
The client's anger	Defended or collapsed	Endured	Received, named, and the referral held
The record	Nothing	Description	Reasoning, the referral question, the consent, the follow-up date

The marking note. The lowest band applies to the therapist who, when the client protests, offers to "keep seeing you as well". That sentence is the commonest way in which a correct referral is undone, and it is a failure of the criterion on warmth — because the therapist has made the client's comfort the criterion rather than the client's care.

7.8 Six errors

Error 1 — Confusing scope of practice with scope of competence. "I am licensed to see families" is true and is not an answer to "should I be seeing this family?".

Error 2 — Treating the first case of a kind as routine. The first psychotic presentation, the first disclosure of abuse, the first family in which a child has died: each requires supervision in advance, and each is frequently held alone because the therapist does not want to appear inexperienced.

Error 3 — The cold referral. A name and a telephone number, given at the end of a session, with no call and no follow-up. Most clients do not attend, and the therapist records a referral that did not happen.

Error 4 — The referral that becomes an abandonment. No closing session, no summary, no offer to return. The client experiences the referral as a rejection and does not seek help again.

Error 5 — Supervision that is a performance. The therapist brings the case that will make her look thoughtful and leaves out the one she is failing. The supervisor cannot supervise what she is not told, and the client carries the cost.

Error 6 — Cultural humility used as an excuse. "I did not want to impose my framework" is not a reason for failing to assess, to ask, or to refer. Humility is a stance in enquiry; it is not an absence of method.

7.8a The case the therapist kept

The counter-case to section 7.5, and the more common failure.

A therapist in a small practice sees a woman for eighteen months for what is recorded as "anxiety and marital difficulty". The woman has a history of childhood sexual abuse that emerges in month four. The therapist has no training in trauma treatment, has read two books, and continues. The treatment is supportive, kind, and static; the client's nightmares do not change and her marriage deteriorates. In month fifteen the client discloses that she has been cutting her thighs for a year and has not told anyone.

The signals that were present and unheeded. The dread before sessions, recorded nowhere. The case never presented in the peer group, because the therapist "did not want to be told to refer". The night-before reading that the therapist experienced as diligence. The flat trajectory that the therapist attributed to the client's "complexity".

What competence would have required, at month four. A supervision conversation about whether trauma-focused work was within the therapist's scope; an additive referral to a trauma-trained clinician, with the therapist continuing the marital work; or a transfer with a bridge. None of these requires abandoning the client, and all of them require the therapist to say out loud that she did not know how to treat what had emerged.

The lesson, stated as a rule. The case that the therapist cannot bring herself to discuss is the case that most needs discussing, and the reluctance is the most reliable signal in this chapter. It is also the signal that therapists are least likely to act on, because acting on it requires admitting, in front of a senior colleague, that a client has been receiving a treatment that was not working.

7.9 Culture, faith and the referral that is refused

The traditional healer. In much of this region, families consult a traditional healer, a spiritualist, or an elder before or alongside a therapist, and the therapist who ignores this will be working in parallel with a treatment she knows nothing about. The professional position is neither endorsement nor contempt but enquiry: what has been done, by whom, what was said, what it cost, and what the family believes about it. Where the family attributes the problem to a spiritual cause and the therapist's formulation is psychological, the two are not necessarily in conflict, and the therapist who insists on the conflict loses the family.

Collaboration, with limits. Collaboration with a traditional practitioner is possible and sometimes excellent, particularly where the practitioner holds the family's trust and the therapist holds the clinical method. It requires a clear agreement about confidentiality, a shared understanding of who is treating what, and a willingness to withdraw if the practitioner's methods involve harm — beating, confinement, deprivation, or "deliverance" practices that endanger a child. Where harm is involved, the statutory duty is engaged and the family's framework is not a defence.

Referral to a religious authority. Families frequently ask the therapist to refer them to a pastor or imam for the spiritual part of a problem, and sometimes the therapist should. The distinction worth teaching: refer for pastoral care where the client wants it and it is additive; do not refer instead of the clinical assessment where the clinical need is real. A therapist who sends a depressed client to a pastor in place of treatment has not respected the culture; she has failed to treat.

When the therapist's referral is refused on religious grounds. "We do not take these things to doctors." The response is not argument and not capitulation. It is specificity: what exactly is feared (stigma, gossip, medication, admission, cost), what the family would accept (a physician rather than a psychiatrist, a female doctor, a private clinic, a single consultation), and what the therapist's own limit is. Where a child's welfare is at stake, the limit stops being negotiable.

The elder's veto. In some households a senior man will decide whether the family continues in treatment, and the therapist will never meet him. The clinical response is to ask the client what the elder believes is happening, and to offer a route by which the elder's view can be heard — a letter, a meeting, a family session — because a treatment that the household's authority has not consented to will end at the elder's first objection.

The therapist's own faith. A therapist whose framework is religious will be tempted to offer it, and a therapist whose framework is secular will be tempted to correct the family's. The competent position is that the therapist's metaphysics is not the treatment, and that the client's is material. What the therapist believes about God is not a clinical instrument; what the client believes is.

> **DEBATE**
> "A therapist who shares a client's culture is always better placed to treat that client than one who does not." Argue for: shared language, shared shame, shared knowledge of what a family will and will not tolerate, and the credibility that comes with it. Argue against: over-assumed understanding, the impossibility of disclosing family business to a person from the same community, the blind spots that similarity produces, and the client's need for someone outside the system. Then describe the case in which you would deliberately choose an outsider.

7.9a When the therapist is the only service

A situation common in this region and almost absent from the training literature: the therapist is the only clinician within four hours' travel, and the client cannot be referred anywhere.

The four moves available. Extend scope carefully, with supervision at a distance: a monthly telephone consultation with a specialist, a recorded session reviewed by email, a named consultant for the specific domain. Use the medical system for what it can do: the general practitioner, the district hospital, the community health worker — each can carry part of the assessment even where no psychiatrist exists. Train a local resource: a nurse, a teacher, a catechist who can be taught a structured screen and a referral pathway. Travel: the therapist goes to the specialist twice a year with a caseload, which is cheaper than pretending.

What must be recorded. That no local service existed; what alternatives were considered; what the remote consultation advised; and what the client was told about the limits. The record is not defensive paperwork; it is the only evidence that the decision was a considered one rather than a convenience.

What must not happen. The therapist treating outside her competence while describing the situation as "cultural context" or "the reality of practice". Necessity changes the risk calculation; it does not change the obligation to seek advice, to record, and to tell the client the truth about what is and is not available.

7.10 Evidence note

What the research supports. Therapist effects are substantial and consistent: outcomes vary more between therapists than between the treatments they claim to deliver, and the most robust correlates of better outcomes are skill in alliance management, deliberate practice, and the use of feedback. Supervision is associated with therapist development in qualitative and survey research, and feedback-informed practice improves outcomes and reduces deterioration, particularly with clients who are not progressing. Training in cultural formulation is associated with better engagement but not reliably with better outcomes, which supports the humility framing rather than a knowledge-acquisition framing.

What is weaker. The direct evidence that supervision improves client outcomes is surprisingly thin, because supervision is nearly universal and therefore hard to study by

comparison. The evidence on interpreter use is mostly about error rates and satisfaction rather than clinical outcomes.

What is not supported. That experience alone produces competence; the evidence on therapist development shows that many therapists plateau early and that self-assessed competence correlates poorly with observed skill. That cultural matching reliably improves outcomes — the findings are mixed and the mechanisms are unclear. And that a referral is a neutral act; clients experience it as a judgement on themselves, and the evidence on drop-out after referral is consistent with that.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same referral, made in week two and made in week twenty, is a different clinical act — the first is a formality, the second is a rejection after an investment.

7.11 Key terms

Scope of practice — what the licence permits.

Scope of competence — what the therapist can do to an adequate standard, specified by domain.

Cultural humility — a stance of enquiry about one's own frame and the client's, with the capacity to be corrected.

Additive referral — another professional joins and treatment continues.

Consultative referral — advice obtained; the therapist continues.

Transfer — the case moves entirely.

Warm handover — a direct contact between professionals, with consent, so that the client arrives expected.

Bridge session — a planned continuation while a referral is arranged, with a stated end date.

Signal — a felt or observed indicator that the case may exceed competence; not proof and not to be ignored.

7.12 Self-test

1. Name the four dimensions of competence and give a failure of each.
2. Why is confidence a poor indicator of competence?
3. List the six signals that a case is beyond you.
4. What are the four rules for working with an interpreter, and why is a family member never acceptable?
5. Distinguish additive, consultative and transfer referrals with an example of each.
6. What are the six elements of a referral that the client actually acts on?
7. What is the "refusal to refer", and what test detects it?
8. In the Ogundipe case, what were the three missed details and what did each signify?

Answers. (1) Knowledge — confident error from outdated method; skill — knowing the technique and being unable to enact it; experience — treating the first case of a kind as routine; appropriateness — the therapist's own history or position driving the treatment. (2) Because competence deficits remove the cues by which they would be detected, and because confidence rises fastest where feedback is absent. (3) Dread before the session; reading the night before in order to conduct it; the case never presented in supervision; relief when the client cancels; a treatment that has been flat and unexplained; material the therapist was never trained to assess. (4) Introduce and contract the interpreter as a professional; speak to the client in the first person; never use a family member; allow double time. A family member is never acceptable because their own position in the family distorts what is said, and because a child or spouse cannot safely carry another member's disclosure. (5) Additive — a physician reviews medication while therapy continues; consultative — a psychiatrist advises and the therapist treats; transfer — a first episode of psychosis moves to a psychiatric service. (6) Recognition recorded; the kind of referral chosen; the gap named without shame; a warm contact made; a bridge if needed; a follow-up and a closing. (7) Continuing a case because the client will be upset or because the therapist is the only person trusted; the test is what the therapist would say if a colleague described the case to her. (8) The belief that neighbours were watching (persecutory belief); the incoherent account of the meeting ("replaced by someone who is not him"); the refusal of his wife's cooking and sleeping in the car (a change in behaviour with a possible delusional basis) — together, an emerging psychosis that the depression formulation concealed.

7.13 Reflective task

Write one page on the following: name the client you would least like to be referred. Describe that client's presenting problem, and then answer honestly whether your reluctance is about competence or about your own history. Keep the page; it is the beginning of your development plan.

7.14 Further reading

- Rønnestad, M. and Ladany, N. on the development of therapists over the career — the evidence on plateauing and on what distinguishes therapists who keep improving.
- Miller, S., Hubble, M. and Duncan, B. on feedback-informed practice — the strongest argument that outcomes can be improved with routine measurement.
- Falender, C. and Shafranske, E. on clinical supervision — the standard text on the supervisory relationship and contract.
- Tervalon, M. and Murray-García, J. on cultural humility — the original formulation, worth reading in the source.
- Any current local guidance on the use of interpreters in health and social care.
- Your professional body's continuing development requirements, read as a minimum and not as a standard.

Next: Week 8 — Risk I: suicide and self-harm.

Week 8 — Risk I: suicide and self-harm

Learning outcomes. By the end of this week the student can (1) ask about suicide directly, specifically and without euphemism, and explain why asking does not increase risk; (2) conduct and document the ten-element assessment and formulate risk at one of three levels; (3) write a safety plan with a client rather than for one; (4) manage a disclosure of risk inside a family treatment without turning the session into an interrogation; and (5) break confidentiality for risk in a way that preserves the relationship.

The standard, stated once

Ask directly. Ask specifically. Ask about means. Document what you asked and what you were told. There is no evidence that asking about suicide increases risk; there is substantial evidence that clinicians do not ask, that they ask indirectly when they do, and that the indirect question produces a denial.

The four sentences that are wrong, and the one that is right.

Said	Heard	Problem
"You are not thinking of doing anything silly, are you?"	"Say no"	"Silly" invites denial and insults the person
"Do you ever feel like you want to end it all?"	"That is a metaphor"	Vague; produces a vague answer
"You would not harm yourself, would you?"	"I am being watched"	Leading; the answer is in the question
"Are you having thoughts of suicide?"	"Yes or no"	Better, but a closed door
"Have you had thoughts of ending your own life — and if so, how often, and what have you thought about doing?"	"This person can hear it"	Correct: direct, specific, and open

Why family therapists get this wrong more often than individual therapists. Because in family work the therapist is managing multiple people's distress at once, and risk appears as one thread among many. The parent who says "she is being dramatic" and the adolescent who says "I am fine" produce a session that is busy, warm and dangerous. The discipline is structural: risk is assessed individually, in every case that presents with the features below, whatever the presenting complaint.

When to assess, as a minimum. Any presentation with: hopelessness, a recent loss, a previous attempt, a family history of suicide, a chronic illness or pain, substance misuse, a disclosure of self-harm, an adolescent with school refusal or sudden withdrawal, a client in a violent relationship, a client who has just been left, and any client about whom the therapist has a private unease she cannot name. The last is on the list because it is frequently correct.

Level	Picture	Action
Low	Ideation absent or fleeting, no plan, no intent, good support, reasons for living	Document, safety plan, review at next session, monitor

Level	Picture	Action
Moderate	Ideation with frequency, some intent, no plan or vague plan, inconsistent support, treatable	Safety plan, means restriction, notify a support person with consent, increase frequency, refer for assessment
High	Ideation with plan and intent, access to means, previous attempt, agitation, intoxication, no support	Do not leave the person alone; arrange urgent psychiatric assessment or emergency care; inform family; document contemporaneously

8.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The standard, and the four wrong sentences	Students write their own opening question, then compare
8–25	Five concepts: state not trait, the assessment is an intervention, means, family as resource and hazard, documentation	Lecture
25–45	The ten elements, each with its wording	Demonstration; then triads practise three elements each
45–55	Formulation and the three levels	Five vignettes, voted and defended
55–65	Self-harm: function, contagion, family response	Lecture with a case
65–78	The safety plan, written with the client	Live construction with a volunteer
78–90	The Suleiman case: an adolescent, a plan, and two parents	Case, students decide what the mother is told and when

8.2 Five concepts that organise the week

Concept 1 — Risk is a state, not a trait. The same person assessed in March and in July is two different patients. Risk rises with agitation, insomnia, intoxication, shame, loss of a relationship, and access to means, and falls with sleep, support and a reason to stay alive. The implication for practice is that an assessment is a snapshot with a date and a review, not a verdict filed once.

Concept 2 — The assessment is itself an intervention. A careful, direct, non-judgemental enquiry reduces isolation, communicates that the therapist can bear the material, and frequently produces the first honest conversation the person has had. This is why the quality of the asking matters clinically and not only forensically: a bad assessment does harm and a good one does some good.

Concept 3 — Means matter more than intent. The difference between a serious attempt and a death is often distance and method. Means restriction — locking medication, removing a firearm, agreeing that a friend holds the pesticide — is the intervention with the best evidence in the whole of this week's material, and it is the one most often omitted because it feels intrusive.

Concept 4 — The family is both the resource and the hazard. The family is the most reliable source of monitoring and the most common source of the precipitants: conflict,

shame, rejection, violence, and the loss of a relationship within the household. A family therapist is therefore unusually well placed to intervene and unusually at risk of talking the risk down.

Concept 5 — Documentation is a clinical act, not a defensive one. The record of what was asked, what was answered, how risk was formulated, who was informed and when it will be reviewed is what allows the next clinician to continue the work. It is also the only thing that distinguishes a considered decision from a negligent one when the outcome is bad, which it sometimes will be.

8.2a The ten elements, with wording

The elements are the standard list. What is usually missing from teaching is the sentence to use for each.

1. Ideation. "In the last two weeks, have you had thoughts of ending your own life?" Then: how often, how long does it last, can you stop it, does it come on its own or when something happens.

2. Plan. "Have you thought about how you would do it?" Then: where, when, have you prepared anything, have you been to the place. A plan that has been rehearsed is a different level of risk from a plan that has been imagined.

3. Intent. "Do you intend to act on this?" The question is uncomfortable and it is the one that most changes the formulation. A second question follows: "what has stopped you so far?"

4. Means. "Do you have access to — " and then name the means the client has already mentioned, plus the common ones in the setting: medication in the house, pesticides, a firearm, a height, a vehicle, a rope. Ask what is in the house and who else can reach it.

5. Previous attempts. "Have you ever tried before?" Then: what, when, what happened, did anyone know. The strongest single predictor, and frequently not in the referral.

6. Protective factors. "What keeps you going?" — children, a mother, faith, a dog, work, a plan for next year. Ask for specifics and record them in the client's words, because they are the raw material of the safety plan.

7. Substance use and impulsivity. "How much are you drinking?" and "do you do things quickly when you are upset?" Intoxication and impulsivity are the two factors that convert ideation into action within hours.

8. Family history. "Has anyone in your family died by suicide, or tried?" The question requires care and it must be asked; the answer frequently reframes the case.

9. Current stressors and recent losses. "What has happened in the last three months?" — a separation, a dismissal, a debt, a diagnosis, a death, an arrest, a public humiliation. Ask what the client expects to happen next, because the anticipation of a loss is as potent as the loss.

10. Disclosure and support network. "Who knows about this?" and "who could you call at two in the morning?" The answers determine whether a safety plan is possible or whether the plan must be a clinical arrangement.

The follow-up question students forget. "What would make it worse this week?" The answer is the plan: the anniversary, the court date, the visit from the husband, the results of the test. A risk formulation that does not name the date on which risk will rise is a formulation that will be wrong within seven days.

8.2b Formulation, and the sentence that carries it

An assessment produces a level, and the level produces actions. What makes the work defensible is the sentence that connects them.

The template. "Risk is assessed as [level] on [date]. This is based on [ideation, plan, intent, means, history]. Protective factors are [specific]. The precipitants in the next two weeks are [specific dates or events]. Actions taken: [means restriction, notification, referral, frequency of contact]. Review: [date]."

Worked example.

> Risk assessed as moderate on 14 March. Daily ideation over three weeks, no plan, no stated intent, prior overdose at 17. Access to a large quantity of her mother's medication in an unlocked cupboard. Protective factors: two children, a sister she telephones daily, church attendance. Precipitant: custody hearing on 26 March. Actions: medication moved to the sister's house today with the client's agreement; sister informed with consent; weekly sessions increased to twice weekly until the hearing; psychiatric assessment requested, appointment 19 March; safety plan written and copied. Review: 17 March.

Why this format works. It records the reasoning rather than the conclusion, so a reader can see that the assessment was made rather than assumed. It names the dates, so the review is real. And it records the client's consent to each notification, which is the detail that distinguishes a collaborative intervention from a unilateral one.

What must be added where the client refuses consent to notify. "Client declined notification of her sister. Discussed the additional risk of secrecy; client maintained refusal. Therapist notes the increased risk and will review at the next contact. Emergency contact details held." Recorded refusal is not misconduct; unrecorded refusal becomes, in a subsequent inquiry, an omission.

The three things a formulation must never contain. A promise — "client contracted not to harm herself" — which is worthless and which no client can make. A prediction — "client is not at risk" — which no clinician can make. And a euphemism — "client has some passive thoughts" — which conceals rather than records.

8.2c Ten things that raise risk in a family case

The formulation is built from the elements, and the elements are easy to gather and easy to leave unweighted. The ten items below are the ones that most often change a moderate assessment into a high one, and each is under-detected in family work because it appears as ordinary family material.

1. **A recent humiliation or exposure** — an arrest, a pregnancy discovered, a debt made public, a shaming in church or at work. The days immediately after an exposure are the days of highest risk.
2. **The end of a relationship in the last six weeks**, including the end that the client has not yet accepted.
3. **A court date, a hearing, or a custody decision in the next fortnight.** Ask for the date; it belongs in the formulation and it sets the review.
4. **Insomnia of more than three nights.** Agitation and sleeplessness are the states in which ideation becomes action.
5. **Access to a method that is unfamiliar to the therapist** — pesticides in a farming household, a relative's firearm, a well, a river the client has mentioned twice.
6. **A family history that nobody has asked about**, including a death that the family describes as an accident.
7. **A client who has become the family's carer** — for a parent with dementia, a disabled sibling, a dying spouse. The burden plus the loss of the role is a combination that appears in the files after the event.
8. **A client who has disclosed abuse and been disbelieved** by the family member she told.
9. **A client who is taking medication and has stopped it**, particularly where the family has discouraged it.
10. **The therapist's own unease, without a reason.** It is on the list because it is data: an experienced clinician's inarticulate concern is a signal that something in the presentation does not fit the formulation.

8.3 Self-harm: what it is, and what it is not

The distinction that matters clinically. Suicidal behaviour is intended to end life. Non-suicidal self-injury is intended to relieve intolerable feeling, to punish the self, to communicate, or to feel real, and is not intended to kill. The distinction is not absolute — self-injury is a risk factor for suicide, and the two occur in the same person — and it changes the response: self-injury requires an assessment of function and an alternative coping repertoire, and suicide requires a risk assessment and a safety plan.

The four functions, in the words clients use.

1. To feel something, or to feel nothing. "When I cut, everything goes quiet for a while."
2. To punish. "I deserved it." Frequently following abuse, and frequently accompanied by shame.

3. To communicate. "Nobody would listen unless they saw something." Common in adolescents in families where distress is not permitted.
4. To control. "It is the one thing that is mine." Common where the person's life is otherwise governed by others.

The countertransference. Self-injury produces strong reactions in therapists, and the reactions are usually one of three: rescue (the therapist becomes the person who must stop it, and the client's autonomy disappears), disgust (the therapist's own history or her culture's response to mutilation), or fascination (the case becomes the therapist's most interesting one). All three are material for supervision, and all three change the treatment.

What the therapist does in the session. Asks about the injury without recoiling; asks about the function without interpreting it; asks about the moment before, in detail, because the sequence feeling — act — relief — shame contains the points at which alternatives can be introduced; and does not make the stopping of self-injury a condition of the therapy. A therapist who says "you must stop" has added a rule to a problem that is caused by rules.

The family's response. The commonest sequence in a family: discovery, horror, prohibition, concealment. The adolescent learns to hide more carefully and to injure in places that do not show. The therapist's work with the parents is to convert horror into monitoring and prohibition into an agreement about what will happen when the urge arrives — usually a plan involving a person and a place rather than a rule.

Contagion. In school and peer settings, self-injury and suicidal behaviour cluster. Two implications. First, group work with adolescents who self-injure should be conducted with attention to comparison and to explicit method discussion; some services avoid mixed groups entirely. Second, in a family with two adolescents, the therapist should expect the second to be affected and should ask.

8.3a The safety plan, written with the client

A safety plan is not a no-suicide contract. It is a written sequence the client will follow as the urge rises, and it is written with the client, in her language, on paper she keeps.

The six steps, in order.

1. **My warning signs.** The thoughts, feelings, images and behaviours that come before the urge. In the client's words: "I stop answering messages", "I start thinking about my mother".
2. **Things I can do on my own.** Two or three, specific and available: a shower, walking to the junction, a particular playlist, prayer. Not "think positive".
3. **People and places that distract me.** Named people, with numbers, and places that are open at night.
4. **People I can ask for help.** Two or three names, with numbers, and an agreed script: "I am not all right, can you be with me for an hour?"

5. **Professionals and services.** The therapist's number with its limits, the clinic, the emergency department, the crisis line. Written in full, not abbreviated.
6. **Making the environment safe.** The means, and what will be done about them today: medication with the sister, the razor blades removed, the pesticide taken to the farm, the key to the roof given to the neighbour.

The rules for writing it. In the room, on paper, in the client's handwriting where possible; the client keeps a copy and the file keeps a copy; the client's own language, not the therapist's; reviewed at each session initially and at every change of circumstance; and the client's agreement sought at each step, because a plan imposed by the therapist is a plan the client will not use.

A worked plan, for Amina, 15, whose case follows in 8.5.

> **Warning signs:** I stop coming out of my room. I feel like I am behind glass. I start thinking nobody would notice. **On my own:** cold shower; the playlist Aunty Bisi made; walk to the corner and back. **Distraction:** Halima (mother) in the kitchen; sit with her while she cooks. Fatima next door. **Ask for help:** Halima — "I am having the bad thoughts again, please sit with me". Fatima — "can I come and sit at your place". **Professionals:** Dr Okafor (therapist) 0803..., weekdays 9–6; Amina's clinic 0805...; Emergency 112. **Safe environment:** mother's medication now in a locked box with the key kept by Halima's sister; Amina's blades given to the therapist in session; no access to the roof (padlock replaced 12 March).

8.4 Risk inside a family treatment

The structural rule. Assess individually, agree what will be shared, then facilitate the sharing. Three steps, in that order, every time. The therapist who assesses in front of the family gets a performance; the therapist who shares without agreement loses the young person; the therapist who neither assesses nor shares is the case in the file that ends badly.

Step 1 — individually. The client is seen alone, the assessment is conducted in full, and the safety plan is written. The parents or partner are told that the therapist is seeing the person alone and why, in general terms, before the session — never after.

Step 2 — agree what is shared. The conversation has three parts. What the therapist must tell, and why: the risk, because the household is the means of supervision. What the client will tell herself: the detail, the words, the timing. And what will not be told: the content of the therapy, the things said about the parents, the history. The agreement is written down, in the note and, where the client wishes, on paper for the family.

Step 3 — facilitate. The client speaks first, in the joint session, with the therapist present. If she cannot, the therapist says what was agreed and the client confirms it. The family's reaction is then worked with: the father's anger, the mother's guilt, the sibling's confusion. The session ends with the household's part of the safety plan — who checks the medication cupboard, who is called, what happens at night.

The three family configurations that require specific care.

The parent who is suicidal. The children are not told the content. They are told what changes: "your mother is ill and is being treated; she will be in hospital for a few days; this is not your fault and it is not because of anything you did." The children's need for a truthful, age-appropriate account is a clinical task and it is frequently skipped because the adults are overwhelmed.

The adolescent who self-injures. The parents are told the fact, not the diary. The prohibition response is anticipated and converted, in the session, into an agreement about the next occasion. The school is involved only with the young person's knowledge and, where the law requires it, with her being told first.

The child who discloses. The child's disclosure of abuse is Week 9's territory and the same structure applies: the child is seen alone, told what will happen next, and told who will be told and in what words before anyone is told.

8.4a Breaking confidentiality for risk, in words

The moment is unavoidable and it is where many therapists lose a client. The four-part statement, delivered before the call is made rather than after.

1. **The commitment, restated.** "I told you at the start that there are things I cannot keep, and this is one of them."
2. **The specific content.** "What I am going to tell your mother is that you have been thinking about taking her tablets, and that we have made a plan. I am not going to tell her what you said about her, or about your father, or anything about the sessions."
3. **The reason.** "Because the tablets are in the house, and because she can do something I cannot do from here, which is to be awake at two in the morning."
4. **The offer.** "You can be in the room when I tell her, or I can tell her and you can come in afterwards. Those are the two choices and I would rather you chose."

What the client usually says. "You promised." The answer is not a defence of the policy. It is: "I promised, and I told you at the time what the exception was. I am keeping the promise that matters more, which is that I would not let this happen without doing something."

What must never happen. Telling the family and then telling the client. The sequence converts a professional duty into a betrayal, and the client who learns of it afterwards will not disclose again — to this therapist or to any other.

8.4b When the family will not cooperate with the plan

The plan assumes a household that will act. Frequently it will not: the mother who says "she is doing it for attention", the father who refuses to lock anything because "a man's house is not a prison", the husband who dismisses his wife's ideation as manipulation.

The four responses, in order. First, ask what the objection is about, because it is usually about something specific: the father hears an accusation of his parenting, the mother hears that she has failed. Second, offer a version of the plan that does not require them to admit anything — locking a cupboard because the children are small, not because

a daughter is suicidal. Third, find another adult — an aunt, a teacher, a neighbour, a religious leader — and build the monitoring around that person, with the client's knowledge. Fourth, and where the risk is high, state the position plainly: "I am telling you what I think needs to happen because your daughter's life is at stake. If it does not happen, I will record that I asked and I will keep asking."

What not to do. Do not discharge the family because they will not comply. Do not accept the plan's collapse in silence. And do not turn the work into a campaign against the parents, which will cost the adolescent the only household she has.

8.5 Case: Amina Suleiman, fifteen

Referral. The school counsellor telephones: a teacher noticed cuts on the forearm of Amina Suleiman, 15, a senior secondary student. Amina has refused to explain them. The mother, Halima, 41, a trader, is telephoned and attends with Amina the following day. The father, Yusuf, 47, is a long-distance driver and is away; he will attend when he returns in three weeks.

The household. Halima and Yusuf, married nineteen years. Amina, 15; Ibrahim, 11. The family is Muslim; Halima's sister Bisi lives twenty minutes away. Yusuf is frequently absent for two to three weeks at a time.

Session one, with mother and daughter. Amina is silent, arms covered, answering in single words. Halima does the talking: Amina has been "difficult" since the new school year, has stopped eating with the family, and has been found crying in the bathroom. Halima's own account includes a period of depression two years earlier after a miscarriage, for which she took medication and did not continue.

The therapist does three things in this session and none of them is to ask about the cuts in front of the mother. She establishes the presenting problem in the family's terms. She explains the structure — she will see Amina alone first, then the family, and she explains why, in front of both, so that the individual session is not a secret. And she says what she will not keep: "if I think Amina is in danger, I will tell you, and I will tell her first that I am going to." Halima agrees. Amina looks up for the first time.

Session two, Amina alone, fifty minutes. For fifteen minutes Amina says nothing of substance. The therapist does not fill the silence with questions; she says, at minute twelve, "I am not going to make you show me your arm. I am going to ask you some questions that I ask everybody, and you can tell me to stop." Then the assessment.

- Ideation: yes, most days for about four months. "I think about not being here."
- Plan: yes. Her mother's remaining tablets, in the cupboard above the sink. She has taken them out twice and put them back.
- Intent: "I do not know. Sometimes I do." She has written a note, on her phone, unsent, four weeks ago.
- Means: the tablets, and the roof of the building, which has a door that is not locked.
- Previous attempts: none. Two episodes of cutting, both in the last three months, both on the forearm, both after arguments at home.

- Protective factors: Ibrahim, her brother — "he would find me"; her aunt Bisi; prayer; the possibility of finishing school.
- Substance use: none. Impulsivity: high — she describes decisions made "in one minute".
- Family history: none known; the therapist does not ask in detail and records that she will.
- Stressors: a change of school; her father's absences; an argument six weeks earlier in which Yusuf said she had "brought shame" by being seen with a boy; and the discovery that her mother had been "sick" and had hidden it.
- Network: nobody knows. Her closest friend has been told "some of it".

The formulation. High risk on the day of assessment: ideation with plan and some intent, means accessible in the house, a recent humiliating event at home, high impulsivity, and no adult who knows. Protective factors substantial: her brother, her aunt, her faith, school.

What the therapist did, in order, within that session.

1. She told Amina that she would be telling her mother, that evening, and exactly what she would say — that Amina has thoughts of ending her life, that there are tablets in the house, and that they need a plan. She did not tell her mother about the note, the boy, or the contents of the session.
2. She offered Amina the choice of being in the room. Amina chose to be present.
3. She telephoned the clinic's psychiatric service and made an appointment for two days later.
4. She wrote the safety plan with Amina (8.3a) and gave her a copy.
5. She asked Amina to hand over her blades, which Amina did, and recorded the receipt of them.
6. She saw Halima for twenty minutes, with Amina present, and used the four-part statement of 8.4a. Halima's response was silence, then weeping, then, "I did not know. I thought she was angry with me."
7. She asked Halima to move the medication that night and to lock the roof door; Halima telephoned her sister and the medication went to Bisi's house the same evening.
8. She wrote the note the same day: the ten elements, the level, the actions, the review date, and the consent recorded.

Sessions three to eight. The psychiatric assessment finds a moderate depressive episode without psychosis; no medication is started, and weekly therapy plus review is agreed. The family sessions, with Ibrahim present for two of them, work on the household's rules about distress: the family in which feelings were "not discussed", the father's shaming sentence, the mother's concealed illness, and the brother who had noticed everything and had been told nothing.

The father's return, session six. Yusuf attends, having been briefed by telephone by the therapist with Amina's knowledge. His first response is anger — at the school, at the boy,

at the therapist for "putting ideas in her head". The therapist does not defend herself. She asks one question: "what did you want for her?" He answers, "a good life, a reputation". The therapist says, "those are not the same thing, and I think one of them is killing her." It is the most direct sentence in the case, and it is delivered after five sessions of alliance-building and in the presence of the family. Yusuf is silent for a long time; then he says, "what do I do?"

Week twelve. Amina has not cut for nine weeks. The ideation has reduced to occasional, without plan. The family has a rule about the roof door and the medication, and Ibrahim knows what to do if his sister is "behind glass". Yusuf has changed his work pattern to be home one week in three.

What the case teaches.

1. **The assessment was done alone and the sharing was planned.** Had the therapist asked about the tablets in front of Halima, she would have received "I would never do that to my mother", and the plan would have remained in the cupboard.
2. **The disclosure to the mother was specific and limited.** Amina knew exactly what would be said. That is what made the second and third disclosures possible.
3. **Means restriction was the intervention.** Eight of the therapist's actions in that first session were administrative; the one that most reduced the risk was a locked box in another house.
4. **The family's culture was the terrain, not the obstacle.** Yusuf's concern for reputation was not contemptible; it was the value he had organised his life around. Naming it, and separating it from his daughter's life, was possible only after five sessions of being taken seriously.
5. **The mother's own history was in the case from session one.** Halima's untreated depression and her concealment of it were both the reason for Amina's silence and a risk in their own right. The therapist referred Halima for her own treatment in week seven.

8.6 Transcript: telling a fifteen-year-old that her mother will be told

Session two, minutes thirty-eight to forty-nine. Annotated.

> **A1 Therapist:** Amina, I have to tell you something and it is not a punishment. — The opening that distinguishes a duty from a consequence. Without it, everything that follows is heard as the therapist taking the mother's side. **A2 Amina:** You are going to tell her. **A3 Therapist:** Yes. (pause) Tonight, before you leave. — The fact confirmed immediately, with the timing. Delay in this conversation produces a fantasy of delay in the disclosure. **A4 Amina:** You said you would not. **A5 Therapist:** I said I would not tell her what we talk about. I also said that if I thought you were in danger I would tell her. (pause) You are in danger. The tablets are in the kitchen. — Both halves of the original agreement restated, in her words. A therapist who only restates the exception sounds as though she is invoking a technicality. **A6 Amina:** (crying) She will ask me why. **A7 Therapist:** She will. (pause) And you do not have to answer that tonight. Tonight is about the tablets. — The separation of the urgent from the important, which is what makes the disclosure bearable. The reason can wait; the means cannot. **A8 Amina:** She will cry and then she will be angry and then she will tell my father. **A9 Therapist:** She may tell your father. (pause) I am going to ask her not to telephone him tonight, and to let us decide together what he is told and by whom. I cannot promise she will agree. — An honest answer about a third party's behaviour, which is more reassuring than a promise the therapist cannot keep. **A10 Amina:** Can I be there? **A11 Therapist:** I was going to offer you that. (pause) You can be in the room, or you can wait outside and come in after I have said the first part. — The choice, given in the terms of 8.4a. The client's only remaining control is the manner, and it is enough. **A12 Amina:** I want to be there. But I do not want to talk. **A13 Therapist:** You do not have to talk. (pause) I will say the words, and you can nod. — The permission that makes her presence possible. A client who is required to speak in that room will not attend it. **A14 Amina:** (long pause) Are you going to tell her about the note? **A15 Therapist:** No. (pause) That is between us, and it is not what she needs tonight. — A limited, kept promise at the moment of breaking a larger one. The therapist's credibility rests on the second promise being exactly as reliable as the first.

What the transcript shows. Four of the seven therapist turns are about what will and will not be said, and none of them is a reassurance. The client's objections are answered with facts about procedure. The one thing the therapist does not do is apologise — "I am sorry, I have to" — which would present the disclosure as the therapist's misfortune and invite the client to console her.

8.7 Laboratory: the observed risk assessment

The exercise. Fifteen minutes with a trained actor, observed, pass or fail, and it counts towards the observed risk assessment component of the module. The actor is briefed with a presentation and a script that contains three opportunities to minimise and one moment of direct disclosure that must not be missed.

The six requirements. The student must (1) ask directly about ideation; (2) ask about plan, intent and means; (3) ask about previous attempts and protective factors; (4) ask what will make it worse in the next two weeks; (5) formulate a level aloud and justify it; (6) begin a safety plan with the client.

Criterion	Fail	Pass	Distinction
Directness	Euphemism, or never asked	Asked directly once	Asked directly, then followed up on frequency, controllability and content
Coverage	Fewer than six elements	Six to eight	All ten, with wording that produced specific answers
Means	Not asked	Asked	Asked, and a specific action agreed today
Formulation	None, or a label	A level with reasons	A level, a justification, a precipitate date and a review date
Safety plan	Suggested only	Begun	Written with the client, in her language, with means addressed
The relationship	Client defensive or silenced	Client engaged	Client discloses more at fifteen minutes than at five

The three most common failures observed. Asking the question once and accepting the first denial. Skipping means because the conversation has become emotional. And ending the assessment without a review date, which turns a snapshot into a verdict.

The debrief question. "What did you feel when she said she had thought about it?" Students report fear, relief, sadness, and a wish to reassure. Each of those is a signal of what the student will do under pressure, and each is worth naming before it is enacted.

8.7a Telephone risk, and the session that is not a session

A large proportion of risk work in a family practice happens on the telephone, and the telephone has its own method.

What to establish in the first four minutes. Where the client is, whether anyone is with her, whether she is safe for the next hour, and whether she has taken anything or is about to. Nothing else matters until those four answers exist.

What to do with the answers. If she has taken something, the call becomes a call for an ambulance and the therapist stays on the line, with the address written down and repeated aloud. If she has not, the call becomes a short assessment of the elements that can be assessed by voice — ideation, plan, intent, means, and who is nearby — followed by a decision: bring her in, send someone to her, or agree a contact in fifteen minutes.

The three things a telephone assessment cannot do. It cannot see the room, the tablets, the injury, or the other person in the house. It cannot establish whether the client is alone and says she is not. And it cannot end the call — the therapist who says "call me if you need me" and puts the phone down has handed the risk back at the moment it was highest.

The note. Written immediately after the call, before the next appointment, with the time of the call, the four opening answers, the action taken, and the time of the follow-up contact. Notes written the next morning lose the times, and the times are the evidence.

8.8 Six errors

Error 1 — The indirect question. "You are not thinking of doing anything silly?" The question is designed to be answered with a denial, by the therapist as much as by the client.

Error 2 — Assessing in front of the family. The parent says "she is being dramatic" and the assessment ends before it begins. Individual assessment is a structural requirement, not a preference.

Error 3 — The no-harm contract. "Will you promise me you will not do anything until we meet?" It has no evidentiary or clinical value, it shifts responsibility to a person who cannot carry it, and it has been followed by deaths in cases where it was recorded as protective.

Error 4 — Means left in the house. The assessment identifies the tablets and the plan records them, and nothing is done about them. This is the single most common omission in the files of completed suicides.

Error 5 — Telling the family before telling the client. The disclosure is made in a telephone call "for safety" and the client learns of it afterwards. The relationship ends, and the next clinician receives a client who will not disclose.

Error 6 — Treating self-harm as a behaviour to be stopped. The prohibition produces concealment, the concealment removes the client from view, and the risk rises while the file says the behaviour has stopped.

8.9 Culture, faith and suicide

The religious frame is not an obstacle to the assessment; it is material within it. In households where suicide is understood as a sin and a shame upon the family, the client's ideation will be concealed with particular skill, and the protective factors will include the very belief that produces the shame. The therapist asks about both: what the client believes would happen to her soul, and what she believes would happen to her mother. The answers frequently reveal that the religious prohibition is holding the person alive and that the social consequence is pushing her toward death, at the same time.

Confession and concealment. A client who has confessed suicidal thoughts to a religious authority and been told to pray more has been given a penance without an assessment. The therapist's response is not to criticise the authority but to make the assessment: "what did you tell him, and what did he say, and did anyone ask you whether you had a plan?"

The family's reputation after a death. Where a death by suicide carries lasting stigma, the family's grief work includes managing the community's account of the death. This is a real clinical task and it is regularly ignored by models of bereavement developed elsewhere. The therapist helps the family decide what will be said, to whom, and by whom, and treats the family's need for a survivable story as legitimate.

Postvention. After a death by suicide, the family's risk rises: bereaved relatives have elevated rates of suicide, and the risk is highest in the first year and among parents and

partners. The therapist's postvention work with a family includes assessing each member, naming the risk explicitly with the adults, and agreeing who will watch whom in the first three months. It is the case in which the therapist is most likely to be invited to comfort and least likely to assess.

The therapist's own exposure. A death by suicide in a therapist's caseload is an occupational hazard of clinical work and it produces a characteristic response: a review of every session, a conviction of responsibility, and either an increase in defensive practice or a withdrawal from risk entirely. The professional response is a structured review with a supervisor, a written account of the care provided, and support. The unprofessional response is silence, which converts one death into a pattern of avoidance that affects every subsequent client.

> **DEBATE**
> "When a competent adolescent refuses to allow her parents to be told about her suicidal thoughts, the therapist should respect her refusal." Argue for: the young person's developing autonomy, the evidence that forced disclosure ends engagement, and the clinical value of being the one adult who does not betray her. Argue against: the household is the only available supervision, the means are in the house, and the law in most jurisdictions permits or requires parental involvement. Then state the threshold at which you would override the young person, and who you would tell first.

8.9a The therapist's own history of suicide

Many clinicians have lost someone to suicide, have survived an attempt themselves, or both, and the training literature treats this as though it did not exist.

The rule. A therapist with an unprocessed personal history in this area is at risk of two opposite errors: the under-assessment that comes from familiarity ("she sounds like I did, and I am fine"), and the over-reaction that comes from fear ("I cannot take a client with ideation at all"). Both are clinical problems and neither is a character flaw.

The questions to ask yourself, in writing, once. Whose death or attempt is nearest to this case? What do I do when a client describes a method I have thought about? Am I assessing this client or managing my own fear? The answers determine whether the work needs consultation, a change of caseload, or personal therapy.

What is not acceptable. Practising without supervision in this domain while carrying an unexamined history, and disclosing the history to the client in a way that makes her responsible for the therapist's distress. "I lost my own brother, so I understand" is occasionally the most useful sentence in a session and is more often the moment at which the client stops being the patient.

8.10 Evidence note

What the research supports. Asking about suicide does not increase risk, and screening improves detection. Previous attempt is the strongest single predictor. Means restriction reduces suicide rates — the evidence from pesticide regulation, firearm access and medication packaging is among the strongest in the field. Safety planning interventions reduce subsequent suicidal behaviour relative to usual care, and brief contact interventions after discharge reduce repetition. The therapeutic alliance is associated with reduced suicidal ideation in treatment studies.

What is weaker. Risk assessment tools have modest predictive value at the individual level and should not be used as a substitute for clinical formulation; the field has largely accepted that prediction is not achievable and that formulation, safety planning and continuity of care are the useful targets. The evidence base for no-harm contracts is essentially null and their use is discouraged.

What is not supported. That hospitalisation prevents suicide in every high-risk case — the post-discharge period is the period of highest risk. That a client who denies ideation is safe. That a low score on an instrument settles the question.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same disclosure of ideation in a week when the client's mother is away and the tablets are unlocked, and in a week when the household is stable, are two different risks.

8.11 Key terms

Ideation — thoughts of ending one's life; assessed for frequency, intensity, controllability and content.

Plan — a specified method, place or time; rehearsed plans indicate higher risk.

Means restriction — reducing access to lethal methods; the intervention with the strongest evidence base.

Safety plan — a written, client-authored sequence of steps from warning sign to emergency contact.

Non-suicidal self-injury — self-harm without intent to die, performed for relief, punishment, communication or control.

Postvention — the support and risk management provided after a death by suicide.

Formulation — the clinical reasoning that links the assessment to a level of risk and a set of actions.

8.11a Three sentences that are remembered

The sentence that opens the assessment. "I ask everybody this, because it matters and because people are usually relieved to be asked: have you had thoughts of ending your own life?" The framing does two things — it normalises the question and it names the relief, which is usually accurate.

The sentence that survives a denial. "All right. And if it came back, who would you tell?" A denial is information, not a conclusion, and the follow-up question keeps the door open without an interrogation.

The sentence that ends a session safely. "Before you go: what is the plan for tonight, and who is in the house with you?" Asked at the door, it is heard as care rather than as assessment, and it produces information that the earlier questions did not.

8.12 Self-test

1. Give the correct wording for asking about suicidal ideation, and explain why the four common versions fail.
2. State the ten elements of the assessment, and the follow-up question most often forgotten.
3. What are the actions attached to each of the three levels?
4. Why is a no-harm contract not a safety plan?
5. Give the six steps of a safety plan.
6. In the Suleiman case, what four actions most reduced the risk, and why?
7. What is non-suicidal self-injury, and what are its four functions?
8. What is postvention, and why is the family at elevated risk?

Answers. (1) "Have you had thoughts of ending your own life — and if so, how often, and what have you thought about doing?" The common versions use euphemism, vagueness, leading phrasing, or a closed question, each of which invites denial or produces an unusable answer. (2) Ideation, plan, intent, means, previous attempts, protective factors, substance use and impulsivity, family history, current stressors and recent losses, disclosure and support network; the forgotten question is what will make it worse in the next two weeks. (3) Low — document, plan, review; moderate — plan, means restriction, notify with consent, increase frequency, refer; high — do not leave the person alone, urgent psychiatric or emergency care, inform family, document contemporaneously. (4) Because a contract extracts a promise from someone who cannot guarantee it and produces no actions, whereas a plan specifies who does what, when, with what means removed. (5) Warning signs; things I can do alone; people and places that distract; people I can ask for help; professionals and services; making the environment safe. (6) The individual assessment, which produced the information; telling the mother with Amina's knowledge and a limited content; the removal of the medication to another house; and the locked roof door — the last two being means restriction, the intervention with the strongest evidence. (7) Self-harm without intent to die, performed to relieve intolerable feeling, to punish, to communicate, or to control. (8) The support and risk management of those bereaved by suicide; because bereaved relatives, particularly parents and partners, have elevated suicide rates, highest in the first year.

8.13 Reflective task

Write the note you would want a coroner to read about a client you were worried about. Then write the note you actually keep. The difference between the two is the measure of your documentation practice.

8.14 Further reading

- Stanley, B. and Brown, G. on safety planning — the source of the six-step structure used here.
- Jobes, D. on the collaborative assessment and management of suicidality — the best-developed clinical approach to formulation and alliance.
- Hawton, K. and colleagues on self-harm in adolescents — the epidemiology and the evidence on repetition.
- The WHO suicide prevention materials, particularly on means restriction at population level.
- Any current national guideline on the assessment and management of suicide risk in your jurisdiction.

Next: Week 9 — Risk II: intimate partner violence and child protection.

Week 9 — Risk II: intimate partner violence and child protection

Learning outcomes. By the end of this week the student can (1) distinguish situational couple violence from coercive control and explain why the distinction changes the treatment; (2) screen each partner separately with direct questions, and document a positive screen; (3) identify the factors associated with lethal risk and act on them; (4) explain to a couple why conjoint work is contraindicated and what is offered instead; (5) build a safety plan with a victim that does not require her to leave; and (6) discharge the statutory duty to report a child protection concern correctly and in the right order.

The distinction on which everything depends

	Situational couple violence	Coercive control
Pattern	Mutual, low-level, escalates under stress; both partners can be aggressive	One partner dominates systematically; the violence is one instrument among many
Fear	Situational, episodic	Chronic; the victim is afraid most of the time
Mechanisms	Arguments that escalate	Surveillance, isolation, financial control, threats, sexual coercion, rules, humiliation
The victim's view	"We both lose control"	"I have to be careful all the time"
Couple therapy	May be appropriate after assessment	Contraindicated; risk increases after conjoint sessions
Response	Communication and de-escalation work; individual safety plan	Safety planning, advocacy, protection; individual work only

Why the distinction is not academic. Conjoint work in a coercive control case does three harmful things. It treats the violence as a communication problem, which the perpetrator welcomes. It gives the perpetrator access to the victim's disclosures, because everything said in the room is available to him afterwards. And it produces retaliation: the literature and the practice literature agree that the period after a conjoint session is a period of elevated risk, and the retaliation is frequently about something said in the session.

The family therapist's structural problem. Couple and family therapy is the modality most often requested by couples in which control is present, because the request itself is part of the control: "we need help communicating" is a formulation in which the violence does not appear. The therapist who accepts the referral at face value will conduct twelve sessions of communication training in a household where one partner is deciding what the other may say.

9.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	The two patterns; how the referral conceals the second	Two referral letters, students identify which is which
10–25	Five concepts: control, lethality, separation, the structural problem, the child as victim	Lecture
25–40	Screening: who, when, how, and what to do with a positive	Demonstration, then paired practice of the four questions
40–55	The lethality factors, in plain language	Card sort; each student ranks five cases
55–70	Ending conjoint work, and what to offer instead	Role-play of the conversation with a couple
70–80	The reporting duty: threshold, route, order of operations	Local rules quiz
80–90	The Okafor case	Case, with students deciding what is reported and to whom

9.2 Five concepts that organise the week

Concept 1 — Coercive control is a pattern of domination, not a series of incidents. The incidents are the visible part; the substance is the regulation of the other person's daily life — what she wears, whom she sees, whether she works, whether she has money, whether her phone is checked. The clinical implication is that an absence of recent violence is not an absence of danger, and that the therapist must ask about control, not only about hitting.

Concept 2 — Lethality is assessable, and the factors are specific. Strangulation, threats to kill, access to a weapon, stalking, separation, pregnancy, escalation in frequency or severity, a history of violence towards others, and a partner who has been violent during sex. Each raises the risk of homicide substantially. The therapist does not need a scored instrument to ask about them, and should know the local one if it exists.

Concept 3 — Separation is the period of highest danger. The weeks after a woman leaves, or after she says she will, are the weeks in which the risk of homicide peaks. This inverts the assumption of many helpers, who treat leaving as the solution; leaving is a process with a peak, and the safety planning must cover the leaving rather than begin after it.

Concept 4 — The therapy setting is a site of risk, not a sanctuary. Appointment times are known, waiting rooms are public, text messages are visible, and a partner who is controlling will want to attend every session. The practice must be designed with this in mind: separate contact details, notes that cannot be read at home, a protocol for what to say if the partner telephones, and a receptionist who has been trained.

Concept 5 — The child is a victim of the control, not only a witness. Exposure to the control of one parent by another is a form of harm in its own right, recognised in child protection frameworks in most jurisdictions. The child's presence changes the therapist's duties, the risk formulation and the plan.

9.2a Screening: who, when, how

Who. Every adult seen in a couple or family case, at least once, alone. Not "where indicated", because indication is what the screening is for.

When. In the first two sessions, before any conjoint work, and again if the presentation changes — a new injury, a new fear, a partner who has become more attentive to the therapy.

How. Alone, in a room the partner cannot see into, with an explanation of why the question is being asked. The four questions, in this order:

1. "Has your partner ever hit, kicked, pushed, slapped or otherwise hurt you?"
2. "Has your partner ever made you afraid — not angry, afraid?"
3. "Has your partner ever forced you to do something sexual that you did not want?"
4. "Do you feel safe going home today?"

The follow-ups that produce the information.

- "What is the worst thing he has ever done?" The answer is frequently not in the first four questions.
- "Has he ever put his hands around your neck, or made you unable to breathe?" Strangulation is under-reported and is the single strongest non-fatal predictor of homicide.
- "Has he ever said he would kill you, or the children?"
- "Does he check your phone, your money, where you go?"
- "Has the violence got more frequent or worse in the last year?"
- "What happens after he has been violent — does he apologise, blame you, promise, or nothing?"

What to do with a positive screen. Thank the client, name what she has described, and ask what she wants. Then the sequence: assess lethality; agree immediate safety for today; discuss what the therapist must do (children, statutory duties); provide information about services without making attendance a condition; record; and plan the next contact. The full safety-planning method is in 9.3a.

What not to do. Do not ask in front of the partner. Do not ask as part of a checklist read aloud to both. Do not respond with "have you considered leaving?" as the first sentence, which is heard as "your judgement has been poor". And do not offer couple therapy as the response to a positive screen.

9.2b The lethality factors, in the language of a session

The list below is the one a family therapist should be able to run through without notes. None of them is a prediction; each of them changes the urgency.

1. Strangulation. Ask: "has he ever put his hands around your neck, or covered your mouth and nose?" A woman who has been strangled is at substantially elevated risk of being killed, and the assault frequently leaves no visible injury and is not reported to the police.

2. Threats to kill. Ask for the words. "I will kill you and nobody will find you" is a different fact from "I was angry".

3. Access to a weapon. A firearm, a machete, a knife kept in the bedroom. Ask what is in the house.

4. Separation, or the threat of it. Ask: "have you told him you want to leave, or has he said he would not allow it?" The answer sets the risk of the next month.

5. Stalking and monitoring. Following, checking the phone, tracking the car, arriving unannounced, calling the therapist. Monitoring is a marker of the partner's need for control and of his capacity to find her.

6. Escalation. More frequent, more severe, or new forms. Ask for a comparison with a year ago.

7. Pregnancy and the postnatal period. Violence frequently begins or escalates in pregnancy, and the risk of homicide is elevated.

8. Sexual violence within the relationship. Ask directly; the question is rarely asked and frequently answered.

9. A history of violence outside the home — towards previous partners, children, strangers, animals.

10. The partner's own instability — substance misuse, a recent job loss, a mental illness, a court case, jealousy that has become fixed. Combined with a weapon and a separation, this is the configuration that appears in the files after a death.

How to use the list in a session. Not as an interrogation. Three or four questions, chosen for what the client has already said, in a tone that says "these are questions I ask

everybody". The client who is asked about strangulation by a therapist who is not shocked will answer, and the answer may be the first time she has said it aloud.

9.2c The practice that is safe to be a client in

Safety is a property of the practice, not only of the therapist's intentions. Six arrangements, each cheap and each routinely absent.

1. **A second contact number that the partner does not know.** A client's contact details must be recorded with a note on who may be telephoned and what may be said. The receptionist who telephones a house and asks for "Mrs Okafor about her appointment" has disclosed the therapy.
2. **A receptionist who has been briefed.** The person at the front desk is part of the clinical service. She needs two sentences: do not confirm whether a person is a client; if a man arrives asking for his wife, say you will check and get the therapist.
3. **A waiting room that is not a corridor.** Where a couple is in a violent relationship, the waiting room is where the confrontation happens. Separate entrances, staggered appointment times, or the client waiting in the car and being fetched.
4. **A protocol for the partner's telephone call.** Written down: what may be said, who says it, and the note that is made afterwards. Usually: "I can confirm that she had an appointment and I cannot discuss anything else."
5. **Notes that cannot be read at home.** If the client takes a receipt, an appointment card or a leaflet home, it will be found. The practice's answer: nothing written goes home unless the client asks for it.
6. **A named person to call at night.** Not the therapist's personal number as a general service, and not a promise of availability. A named duty line, written on a card, with the hours stated.

The test. Ask the receptionist, or the answering service, what she would say if a man telephoned and said, "my wife comes to you, I want to know when her appointment is". If the answer is not rehearsed, the practice is not ready for this week's cases.

9.3 Ending conjoint work: the conversation and the alternative

The therapist has screened both partners, and the screen is positive. The couple is expecting a session about communication. The task is to change the treatment without losing the victim.

The principles. Do not confront the perpetrator with the accusation in front of the victim, because she will pay for it. Do not tell the victim's account to the perpetrator. Do not use the word "violence" unless the victim has used it in front of him, because the disclosure of what she said in the individual session may itself be dangerous.

The script, with both present.

1. "I have met each of you separately, as I said I would, and I have decided that the work I do — seeing couples together — is not the right thing for you at the moment."

2. "The reason is that I do not think this is a communication problem, and couple work can make things worse when it is not."
3. "Here is what I am offering instead: individual sessions for each of you, with me or with a colleague, and a referral for [the relevant service]."
4. "I want to be clear about one thing: I will not be discussing with either of you what the other has said to me."

The predictable responses, and the answers.

- The perpetrator's anger: "why does she get her own sessions?" — "Because I need to see each of you alone to do this work, and that is the same for both of you." (The symmetry is real and is stated without disclosing anything.)
- The victim's fear: she may not want to end conjoint work, because the conjoint sessions are the only place she is safe from him. This is a genuine clinical dilemma. The resolution is not to continue conjoint sessions but to increase the frequency and reliability of individual contact, and to help her build other safe contacts.
- The couple's refusal: "we came for couple therapy". Then the therapist states the limit as a limit on her own practice: "I am not able to do the work you are asking for, because I think it would put one of you at risk. I would rather lose the case than do that."

What is offered instead, and why each element matters.

Offer	Purpose
Individual sessions with the same therapist, for the victim	Continuity and a safe place to plan
Individual work with a different therapist for the perpetrator	Avoids the therapist holding both accounts and being triangulated
Referral to specialist advocacy or a domestic violence service	Practical help the therapist cannot provide: housing, police, legal, money
A referral to a perpetrators' programme where one exists	The only intervention with evidence of effect, and only where attendance is monitored
A child protection referral where children are present	The statutory duty, discharged separately from the couple's treatment

The therapist who continues couple work anyway. The commonest reason is the therapist's belief in the relationship or in her own skill. The second commonest is the victim's request. Neither is a justification, and the note written after an injury will not read well: "continued conjoint work despite disclosure of coercive control in individual session of 4 March."

9.3a Safety planning with a victim

The framing sentence. "I am not going to tell you to leave. Whether you leave is your decision and you know your situation better than I do. What I want to do is make sure that if you ever need to leave quickly, you can."

That sentence is the foundation, because the victim has usually been told by professionals, relatives and her husband what to do, and the therapist who does not add to the list becomes usable.

The plan, in eight parts.

1. **The documents.** Identity card, birth certificates, the children's records, the marriage certificate, bank cards, a copy of any protection order. Kept in a place he does not know, or with a trusted person.
2. **The money.** A small amount saved, a separate account if possible, and knowledge of where to get emergency assistance.
3. **The bag.** A packed bag at a neighbour's or a sister's house: clothes for the children, medication, a phone charger, a copy of documents.
4. **The code word.** One word, texted to one person, meaning "come now" or "call the police". Agreed in advance, with a person who will act.
5. **The route.** Where she goes, how she gets there, and what she does if the first place is not available. Two options, not one.
6. **The children.** Where they are at each hour of the day, who collects them, and what the school will do if he arrives.
7. **The phone.** A second phone or a saved number; the emergency number on speed dial; and the practice's policy about what the therapist will say if he answers her phone.
8. **The review.** The plan is reviewed at each contact, because circumstances change and because the plan that was made in March may be wrong in June.

What the therapist does not do. Does not hold the bag, the documents or the money. Does not promise to be reachable at all hours. Does not contact the perpetrator or mediate. Does not tell the family, the church, or the employer without consent, except where a child protection duty requires it.

The danger of the written plan. A safety plan found in a handbag or on a phone can escalate the violence. The practical solution is that the victim keeps the information in her head or in a form that can be explained away, and the therapist keeps the record; the copy given to the client is minimised and, where necessary, not given at all.

9.4 Child protection: recognition, threshold, duty

The four categories. Physical abuse; sexual abuse; emotional abuse; neglect. In most jurisdictions the definition also includes exposure to domestic violence and, increasingly, exposure to harmful practices. The therapist's task is not to determine whether abuse occurred — that is the job of the investigative authority — but to recognise a concern at the threshold the law sets, and to report it.

The threshold in plain language. A concern is reportable when there is a reasonable suspicion that a child is being harmed or is at risk of significant harm. Not proof; suspicion. The therapist does not need to be right; she needs to be reasonable, and the reasonableness is judged on what she knew and when, not on the eventual outcome.

What raises a concern in ordinary family work.

- A disclosure by a child, however tentative or retracted.
- An injury with an inconsistent explanation, or an explanation that changes.
- Sexualised behaviour in a young child that is developmentally unexpected.
- A child who is frightened of a specific adult, or who will not go home.
- A parent's account of violence towards a child, including "discipline" that leaves injury.
- Exposure to the coercive control described in 9.2 — a child who sees it, hears it, or is used in it.
- Chronic neglect: hunger, untreated illness, absence from school, a child left with inadequate care.
- A child who discloses that an adult has asked him to keep a secret that frightens him.

The reporting route in Nigeria, and the general principle. The Child Rights Act 2003 has been adopted by many but not all states, and state child protection laws vary; the practical route is usually the state ministry responsible for social welfare or the state child protection unit, the police family support unit, or a designated safeguarding lead in a school or hospital. The therapist should have the four local numbers in her practice manual, verified within the past year, and should know which of them operates at night.

The order of operations, which matters more than the route.

1. **Ensure immediate safety.** If the child is in danger now, the emergency services come first and the paperwork second.
2. **Record what was said, in the child's words,** before speaking to anyone else, with the date and time.
3. **Consult** — a supervisor, a designated safeguarding lead, a child protection adviser — where consultation is available within the hour. Consultation does not delay a report that is required.
4. **Report.** The call is made by the therapist, recorded with the time, the name of the person spoken to and the advice received.
5. **Tell the parents, unless telling them would endanger the child.** The default is transparency, and the exception is where a parent is the alleged abuser or the parent's response would expose the child to further harm or to pressure to retract. The decision is recorded with reasons.
6. **Tell the child what will happen,** in words she can hold: who will be spoken to, what she will be asked, and that she has not done anything wrong.
7. **Continue the treatment if possible,** and record the referral and the outcome.

What the therapist does not do. Does not investigate. Does not interview the child repeatedly to establish the facts, which contaminates the evidential process and harms the child. Does not confront the alleged abuser. Does not promise the child confidentiality. And does not accept the family's assurance that "it will not happen again" as a reason not to report.

9.4a The child's disclosure: what to say, and what not to say

What to say. Four things, in this order. "Thank you for telling me." "You have not done anything wrong." "What you have told me is important and I am going to have to tell someone who can help — I will tell you who and what I am going to say before I do it." "I am not going to ask you to tell the story again and again."

What not to say. "Are you sure?" "Why did you not tell me before?" "Did he touch you here?" — the leading question that puts the words in the child's mouth. "Promise me you will tell me if it happens again." And "this will be our secret", which is the sentence that makes the child complicit and which the therapist of all people must not say.

Recording. Verbatim, in quotation marks, including the child's own vocabulary and the questions that were asked. "She said 'he does a bad thing to me at night'. I asked 'can you tell me what happens?' She said 'he comes to my bed and I pretend to sleep'." That record is markedly more useful than "child disclosed abuse".

The child's retraction. Common, and frequently the product of pressure at home. A retraction is recorded, reported and not treated as a resolution. The therapist's note: "child retracted the account on 12 April, having spent the weekend with the maternal grandmother; the original disclosure stands recorded and the referral continues."

The non-abusing parent. Frequently the person who brings the child, frequently disbelieving, frequently the target of pressure from the extended family to "settle it". The therapist's work with her is specific: what she believes, what she has been told by whom, what she is afraid of, and what she will do if the child is disbelieved by the family. Her capacity to protect is the single best predictor of the child's outcome, and supporting it is the most useful clinical act available.

9.4b When the report is made against the family's wishes

The family will be angry, and the anger is understandable: they came for help and received an investigation.

What the therapist says. "I know this is not what you wanted. I am required to report what I am told when a child may be at risk, and I have done it. I am telling you now, before anyone contacts you, because I do not want you to be surprised." Then the specifics: who was called, what was said, what is likely to happen next.

What the therapist does not apologise for. The report itself. An apology — "I am so sorry, I had no choice" — converts a professional duty into a personal betrayal and invites the family to treat the therapist as the source of the problem. The better sentence: "I did what the law requires, and I am still your therapist if you want me to be."

When the family leaves. A proportion will. The therapist records the reasoning, offers the referral, and does not pursue. A family that leaves because a report was made may return, and the file should make that possible: a closing summary, a note that the door is open, and no editorial about the family's decision.

Where the alleged abuser is a client. The therapist may be treating the perpetrator — frequently a father in a family case — and now has a duty to the child that conflicts with

the duty to the client. The resolution is not subtle: the child's safety governs. The therapist reports, tells the client that she has done so and why, and considers whether she can continue to treat him. In many cases she cannot, and the transfer is the correct outcome.

9.4c The four cases that must not be missed

Four presentations in which the child protection duty is engaged and in which family therapists most often hesitate.

The one that is "discipline". A father describes beating his nine-year-old with a cable "because he is stubborn". He is not asking for help with it; he is describing ordinary parenting. The therapist's duty is engaged if the law's threshold is met, and the conversation is not about whether the father loves his son. The therapist says what she must do, and does it.

The one that is "cultural". A family requests a practice that is harmful to a child — a cutting, a beating as initiation, a marriage of a fifteen-year-old. The therapist does not adjudicate the culture. She applies the law, states it plainly, reports where required, and works with whatever part of the family will work with her.

The one where the child is used. A mother reports that her husband's family have taken her son to another state and will not return him. This may be abduction, custody dispute, or child protection matter; the therapist does not decide which, and she does telephone the authorities and record the advice.

The one that is a disclosure by an adult. A client in individual therapy says, in passing, that her brother "does things" to her eight-year-old niece. The duty is engaged for the niece, who is not a client and has never been seen. The therapist reports, tells the client that she is doing so, and works with the client's guilt afterwards.

The common thread. In each of the four, the therapist's hesitation is about the relationship with the adults. The child is not in the room. That is exactly why the duty exists, and why the decision cannot be left to the clinical judgement of the person who is fond of the family.

9.5 Case: the Okafors, and the referral that said "communication"

Referral. Blessing Okafor, 34, a hairdresser, telephones a couple practice: "we need help with communication. My husband has agreed to come." Emeka Okafor, 41, a spare-parts trader, attends the first session. Children: Chidi, 6, and Ada, 2.

Session one, both present. The complaint is that "she is always angry and she shouts in front of the children", and "he does not listen". The therapist, following the practice protocol, explains that she will see each of them alone once before beginning conjoint work, and gives the reason: it is how she gets to know each person. Emeka objects mildly, is reassured, and agrees.

Session two, Blessing alone, and what she discloses.

- Physical violence for nine years, beginning in the first year of marriage. Pushing, slapping, a beating during her second pregnancy that led to a threatened miscarriage.

- **Strangulation**, twice, the most recent eight months ago.
- Threats: "he says he will kill me and take the children to his village, and nobody will ask him any questions."
- Control: he keeps her bank card, checks her phone every night, decides whether she may visit her mother, and has twice arrived at her salon and sat outside.
- Sexual coercion: "sometimes I let him so that he will not be angry."
- Isolation: her mother lives in another state; her closest friend has stopped visiting because of him.
- Threats about the children: "he says if I leave he will make sure I never see them."

The assessment. Coercive control with high lethality indicators: strangulation, a specific threat to kill, a threat to remove the children, monitoring of her movements, escalation during pregnancy, and recent escalation in frequency. There is also exposure of the children to the violence: Chidi has been present at four incidents and has begun wetting the bed.

The therapist's actions in that session, in order.

1. She did not ask "why have you not left". She said: "thank you for telling me. None of this is your fault."
2. She assessed immediate safety: "are you safe going home today?" Blessing said she was, "as long as I do not annoy him".
3. She asked about the children and made the note that establishes the child protection question: present at four incidents, bedwetting since the last one.
4. She explained what she must do. She would not be reporting to the police — that is the client's decision — but she was obliged to report the children's exposure to the state child protection unit, and she would tell Blessing exactly what she would say and when. She was clear that the report concerned the children, not the marriage.
5. She made the report the same afternoon, after a supervision call, and recorded the name of the officer and the advice.
6. She arranged a second individual session in three days, and gave Blessing the practice's emergency number and a code phrase — "I am calling about the hair appointment" — that would tell the therapist that she needed to talk and could not speak freely.
7. She documented the safety plan in the 9.3a form, with the documents left with Blessing's aunt and a bag at a neighbour's house.

Session three — the conjoint session that did not happen. Emeka attends expecting the couple session and is told, with Blessing present, that the therapist will not be conducting conjoint work. The therapist used the script of 9.3, and Emeka's response was as predicted: anger, then a demand to know what his wife had said, then a threat to withdraw from the process altogether.

The therapist's answer, which is the one that matters. "I am not going to tell you what she said. What I will tell you is my clinical view, which is that the two of you working on

communication in this room would not be safe, and I am not willing to do it. I will see you individually, and I will give you the name of a programme that works with men in your situation."

Emeka did not attend individual sessions. Blessing attended eleven over four months.

The child protection process. A social worker visited the home twice. No removal. A case was opened and the children were seen at school. Blessing was asked, by the social worker, whether she wanted to leave; she said she did not, yet. The therapist supported her in that answer in the recorded conversation with the social worker, which is the point at which many therapists instead advocate for the outcome they believe in.

Month five — the escalation the plan anticipated. Blessing telephoned at 9.40 p.m. from her sister's house. He had found the bag at the neighbour's, had been "calm", and then had thrown a chair through the window of the room where Chidi was sleeping. She had left with both children. The therapist's role in that call was three things: confirm immediate safety, direct her to the police station and the social worker, and be on the telephone the following morning when she made the statement. She did not go to the house, did not confront Emeka, did not offer her own home.

The outcome, which is not a happy ending and is a good outcome. Blessing did not return. A protection order was applied for with the help of a legal aid organisation. Emeka was charged and the case is before a court. Blessing attended therapy for a further eight months, and the children were seen by a child therapist. The therapist's own note after the night call reads, in part: "the plan made in session two covered the leaving. It did not cover what he would do when he found the bag. The bag should have been somewhere he had no reason to go."

What the case teaches.

1. **The referral said "communication" and the case was coercive control.** The screening was the only thing that found it, and the screening was a practice protocol rather than a clinical hunch.
2. **The individual session was the instrument of safety, and the conjoint session was the instrument of danger.** Everything the case required came out of the individual sessions; everything that would have been harmful would have happened in the joint one.
3. **The child protection report was separate from the marriage.** It concerned the children's exposure, it was made with the mother's knowledge, and it did not require her to leave or to make a statement against her husband.
4. **The therapist did not tell her to leave, and she left.** The decision was hers, made five months later, and it was made because the plan and the relationship were in place.
5. **The plan had a hole, and the therapist recorded it.** The bag's location was a foreseeable mistake, the note says so, and that note is what a supervisor needs in order to improve the practice's protocol.

9.6 Transcript: "I am not going to tell you what she said"

Session three, minutes six to nineteen, with both present. Annotated.

> **B1 Emeka:** So what did she tell you? Because she has been telling people things. **B2 Therapist:** I am not going to tell you what she said. (pause) I am going to tell you what I have decided. — The refusal and the substitution, in one breath. The refusal alone leaves a vacuum that he will fill with suspicion; the substitution gives the session a subject. **B3 Emeka:** So you have decided something without me. **B4 Therapist:** I have. (pause) I have decided that I will not see the two of you together. — Ownership of the decision. The therapist does not say "the practice's policy requires", which would be true and would invite him to argue with an absent authority. **B5 Emeka:** Why? **B6 Therapist:** Because I do not think this is a communication problem, and because couple work in a situation like yours can make things more dangerous afterwards. — "A situation like yours" rather than "your violence": the therapist has not disclosed the individual session's content and has not accused him, and the meaning is unmistakable. **B7 Emeka:** (to Blessing) You told her I hit you. **B8 Therapist:** (to Emeka, immediately) I am talking to you now, not to her. — The intervention that protects the victim in the room. If the therapist lets that question land, the session has become a place where the violence will be settled afterwards. **B9 Emeka:** (standing) I am not paying for this. **B10 Therapist:** You do not have to. (pause) Sit down if you want to hear what I am offering, or leave now and I will write to you. — The choice, without pursuit and without a threat. The therapist does not stand, does not raise her voice, and does not move towards the door. **B11 Emeka:** (after a pause, sitting) What are you offering. **B12 Therapist:** Individual sessions with me, or with a colleague if you prefer. A referral to a programme for men who want to change how they handle anger and control. And no conjoint sessions until that work has been done. — The offer is real, and the referral is to a programme with an evidence base rather than to "anger management", which is a different and largely ineffective thing. **B13 Emeka:** And her? **B14 Therapist:** She has the same offer. (pause) I want to be clear that I will not be passing messages between you. — The symmetry that is true and that protects the individual work, and the explicit refusal of the mediator role, which is the role a controlling partner will try to install.

What the transcript shows. The therapist never discloses, never accuses and never argues. She states decisions in the first person, protects the victim in the room with one sentence, and offers a real alternative. The one moment of real risk is B7, and the response to it is the shortest turn in the transcript.

9.7 Laboratory: the screening interview, observed

The exercise. Each student conducts a twelve-minute individual screening interview with an actor playing one of four briefed roles, and is then asked two minutes of questions by the observer. The roles are deliberately varied: a woman who minimises, a woman who is not being abused but whose partner is controlling about money, a man who is being abused by his wife and will not say so, and a woman who discloses strangulation in the last minute.

What is assessed. Whether the questions are asked at all; whether they are asked directly; whether the follow-ups are used; whether the lethality factors are covered;

whether the therapist's response to a disclosure increases or reduces the client's willingness to say more; and whether the safety planning begins.

Marks (20).

Criterion	0–1	2–3	4–5
Screening	Not asked	Asked in general terms	All four questions asked directly, with the strangulation follow-up
Response	Shock, advice, or "why did you not leave"	Neutral but passive	Names what was described, states it is not her fault, asks what she wants
Lethality	Not covered	Partly covered	Strangulation, threats, weapons, separation, escalation, children — all covered naturally
Safety	None	Advice given	A plan begun, with the "I am not telling you to leave" frame
Duty	Not mentioned	Mentioned vaguely	The child protection duty stated specifically, with what will be reported and when

The most common failure. The student stops after the first question because the answer is "no". The screening is not a checklist; it is a conversation in which the second and third questions do most of the work.

9.8 Six errors

Error 1 — Screening in front of the partner. The commonest and most dangerous error in the whole of couple work. The screening is individual, always, and the reason is stated so that it does not look like suspicion.

Error 2 — Accepting the couple's formulation. "We need communication help" is the referral, not the case. The formulation belongs to the assessment.

Error 3 — Continuing conjoint work after a disclosure of control. Sometimes because the victim begs for it, sometimes because the therapist believes she can manage the sessions. The sessions become the place where the violence is negotiated and, afterwards, enacted.

Error 4 — Telling her to leave. She has been told by her mother, her pastor and her husband. Her judgement is the only asset she has, and the therapist who overrides it has joined the list.

Error 5 — The report without the conversation. The call is made and the family learns of it from a social worker at the door. The relationship, and frequently the treatment, ends there.

Error 6 — The child interview conducted to establish the facts. Repeated questioning contaminates the evidence, harms the child, and does the investigator's job badly. One account, recorded verbatim, then the report.

9.9 Culture, faith and the household's rules about violence

The extended family's jurisdiction. In many of the households this book serves, a marital dispute is settled by elders, and a wife who takes it outside the family is breaking a rule that the family enforces. The therapist's question — "who in the family knows, and what did they say?" — is therefore not background; it is the risk assessment. A family council that has decided the matter is closed is a family council that will return the wife to the house.

Bride price and the language of ownership. Where a marriage involved a bride price, the language of the household may include the idea that the wife has been "paid for", and the husband's family may treat a separation as a debt. The therapist does not need to argue with the custom; she needs to know that the client may be hearing an obligation where the therapist hears a crime, and to make the distinction explicit: "the family may speak about what was paid. That does not change what is allowed to be done to you."

Religious teaching on submission. A client may hold a theology in which endurance is a virtue and divorce is a sin, and she may hold it sincerely. The therapist does not argue with the theology. She asks three questions: what does your faith say about what is being done to you; what does it say about your children watching; and who in your church would help if you told them? The answers frequently produce an internal permission the therapist could never have supplied.

The church as a resource and as a hazard. A congregation can provide practical help — a place to stay, money, childcare — and can also direct a woman back to a violent husband in the name of reconciliation. The therapist's task is to find out which, before recommending that the client tell her pastor.

The man's own shame. The perpetrator in many of these cases is not a stranger to shame; he is a man who has failed at work, at money, at status, and whose violence is the one arena in which he is not powerless. This is an explanation and not an excuse, and it matters clinically because a programme that addresses shame has a better chance than one that addresses anger.

The child's discipline. Physical discipline is normative in many households, and the boundary between discipline and abuse is set by the law and by injury, not by the therapist's cultural preference. The Child Rights Act and state laws define the threshold; the therapist applies it and does not negotiate it in the session, while recognising that the parent who is reported for a beating will experience the report as an assault on his parenthood.

> **DEBATE**
> "A therapist who tells a woman to leave a violent marriage is doing her job; one who does not is colluding." Argue for: safety is the first duty, the therapist has information the woman cannot have, and neutrality in the face of violence is complicity. Argue against: the woman's judgement about her own risk is more accurate than the therapist's, leaving is the period of greatest danger, and the decision must remain hers if the plan is to be used. Then write the sentence you would actually say, and the case in which you would depart from it.

9.10 Evidence note

What the research supports. The distinction between coercive control and situational violence is well established in the specialist literature and in practice guidance, and conjoint treatment is contraindicated where control is present. Strangulation is a strong predictor of subsequent homicide. Separation is a period of elevated risk. Specialist advocacy and safety planning improve outcomes for victims, and the evidence on perpetrator programmes is modest but consistent for court-mandated attendance. Screening increases identification; the evidence that screening alone improves outcomes is weaker, which is why screening must be paired with a response.

What is weaker. Prediction of lethality at the individual level remains limited. The evidence base on couple therapy in situational violence is small and contested. Child protection interventions have mixed evidence, with outcomes depending heavily on the quality of the service rather than the model.

What is not supported. That leaving resolves the risk — it often raises it in the short term. That a couple who present voluntarily are unlikely to have violence in the relationship. That a perpetrator's attendance at an anger management course changes behaviour. And that a child who is "too young to understand" is unaffected by exposure.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same disclosure of strangulation in a first session and in a twelfth changes the case entirely, because in the twelfth the therapist has already conducted six conjoint sessions.

9.11 Key terms

Coercive control — a pattern of domination through surveillance, isolation, threats and violence.

Situational couple violence — mutual, episodic aggression arising in conflict, without a pattern of control.

Lethality factors — the specific features associated with elevated homicide risk, including strangulation, threats to kill, weapons and separation.

Safety plan — an agreed set of preparations and contacts enabling a victim to act quickly if needed.

Advocacy — specialist practical support from a domestic violence service.

Threshold — the level of concern at which a report is required; reasonable suspicion of significant harm.

Exposure — a child's experience of violence between adults in the household, treated as harm in many frameworks.

9.11a The note that protects the next clinician

Five lines, written after every case in which violence or a child protection concern has appeared.

> Assessed individually on [date]. Client disclosed [strangulation on two occasions, most recent eight months ago; threats to kill; monitoring of phone and movements]. Lethality factors present: strangulation, threats, separation threat, escalation. Conjoint work contraindicated; couple informed [date], with reasons; individual work offered to both. Child protection referral made to [agency], [time], officer [name], advice received: [ ]. Safety plan agreed and recorded. Emergency contact protocol explained. Review [date]. Supervision [date].

The five lines exist for a reason that is not defensive: the next clinician, the social worker, the court and the therapist herself in six months all need to know what was known, when it was known, and what was done.

9.12 Self-test

1. Give four differences between situational couple violence and coercive control.
2. Why is conjoint work contraindicated in coercive control, and what are the three harms it produces?
3. State the four screening questions and three follow-ups.
4. Name six lethality factors and the one most strongly associated with homicide.
5. What is the "I am not going to tell you to leave" frame, and why does it work?
6. Give the eight parts of a safety plan.
7. What is the threshold for a child protection report, and what are the seven steps of the order of operations?
8. In the Okafor case, what did the therapist refuse to do, and why did each refusal matter?

Answers. (1) Pattern (mutual and episodic versus systematic domination); fear (situational versus chronic); mechanism (escalating arguments versus surveillance, isolation, money, threats); treatment (conjoint may be appropriate versus contraindicated). (2) Because it treats violence as a communication problem, gives the perpetrator access to the victim's disclosures, and produces retaliation after the session. (3) Hit, kick, push; made you afraid; forced sexual acts; safe going home today. Follow-ups: worst thing he has done; hands around your neck; threats to kill; escalation; monitoring. (4) Strangulation, threats to kill, access to a weapon, separation, stalking, escalation, pregnancy, sexual violence, history of violence outside the home, perpetrator instability; strangulation is the strongest non-fatal predictor. (5) An explicit refusal to direct the client, which preserves her judgement and makes the therapist usable; it works because she has been instructed by everyone else and the therapist's refusal distinguishes her. (6) Documents, money, bag, code word, route, children, phone, review. (7) Reasonable suspicion of significant harm. Order: immediate safety; record verbatim; consult; report; tell the parents unless that endangers the child; tell the child; continue treatment and record. (8) She refused to disclose Blessing's account, to accuse Emeka in the session, and to conduct conjoint sessions — protecting the victim from retaliation, preserving the confidentiality that made the disclosure possible, and removing the setting in which the violence would have been negotiated.

9.13 Reflective task

Write two hundred and fifty words on this question: a woman tells you she is not ready to leave and asks you not to raise it again. What do you say, and what do you record? Then answer honestly which of the two — saying it or recording it — you would find harder, and why. The difficulty is usually about helplessness, and helplessness is the feeling that produces either control or withdrawal in a therapist. Neither is the work.

9.14 Further reading

- Stark, E. on coercive control — the foundational account, and the source of the distinction this week rests on.
- Campbell, J. and colleagues on the danger assessment — the empirical basis for the lethality factors.
- The WHO clinical guidance on responding to intimate partner violence — practical and internationally adaptable.
- Any current national or state guidance on child protection reporting and the Child Rights Act, read alongside the state law that actually applies where you practise.
- A domestic violence advocacy service's own materials — read what the advocates give to women, because your client will read them too.

Next: Week 10 — The legal framework: courts, statutes and the therapist's place in them.

Week 10 — The legal framework a clinician must know

Learning outcomes. By the end of this week the student can (1) describe the four marriage systems operating in Nigeria and the dissolution route attached to each; (2) identify the five areas of law that bear on family therapy and state what the therapist may and may not do in each; (3) apply the welfare principle and explain why a treating therapist should not conduct a forensic custody evaluation of her own client; (4) route a legal question out of the session correctly, with a written referral question; and (5) explain why documentation is a legal instrument.

Orientation, not legal advice

This week teaches legal literacy, not law. Nothing here is advice, statutes vary between states and are amended, and the therapist who relies on a textbook rather than on a lawyer in a live case has made a category error. The purpose of the week is narrower and entirely practical: that the therapist can recognise a legal question when it appears in a session, knows which area of law it belongs to, knows what she may answer and what she must not, and knows who to send it to with a question that a lawyer can answer.

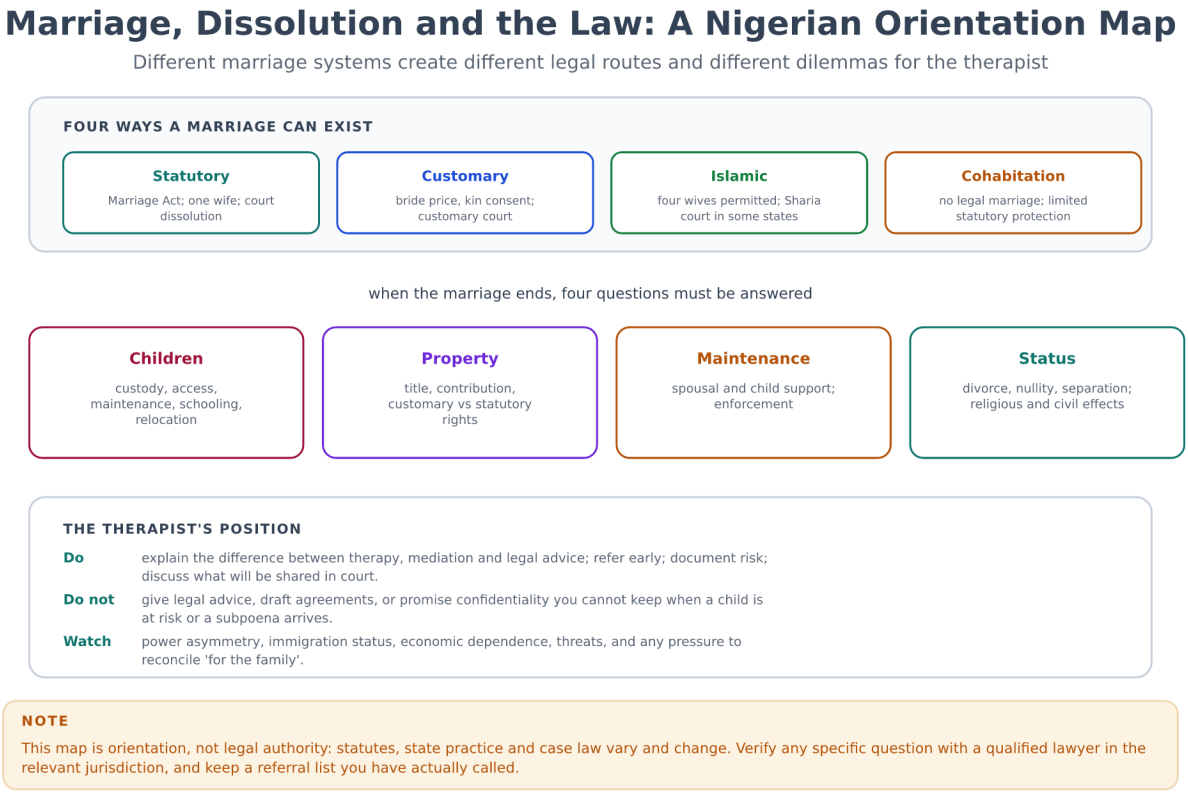


Figure 10.1 — Marriage systems and dissolution questions in Nigeria

Marriage in Nigeria exists in four overlapping forms: statutory marriage under the Marriage Act, customary marriage, Islamic marriage, and cohabitation without a legal marriage. A family may be married under more than one system at the same time, and the therapist who assumes a single legal framework will mis-hear the case.

System	What the therapist needs to know	What to refer
Statutory (Marriage Act)	Monogamous; dissolution by petition to the High Court under the Matrimonial Causes Act, with the two-year rule and recognised grounds	Grounds, timing, procedure, financial orders, custody
Customary	Varies by locality and by community; dissolution through customary process and customary courts; frequently no written record	Validity, dissolution, property consequences, the status of children
Islamic	Governed by Sharia law and Sharia courts in the states that operate them; dissolution by pronouncement, khul' or judicial process	Validity, divorce, maintenance (nafaqah), custody (hadanah)
Cohabitation	No legal marriage; limited statutory protection; the parties may believe they are married	Property, paternity, maintenance, inheritance

Why the plurality matters clinically, not only legally. A woman may believe she is married under the statute and discover that her husband's family has dissolved a customary marriage she did not know was the operative one. A man may marry a second

wife under Islamic law and be committing bigamy under the statute. A couple may have had three ceremonies and one legal marriage. The therapist's question is always the same: **which marriage, under which law, and what has been filed?**

10.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	Why the therapist needs legal literacy and must not give legal advice	Two cases: one where the therapist answered, one where she referred
8–25	The four systems and the dissolution routes	Diagram on the board; students place six families on it
25–40	Five areas of law and the boundary in each	Table, with a "may/may not" sort
40–55	The welfare principle; the custody report; the role conflict	Two reports, one by a treating therapist and one by an evaluator, compared
55–70	Routing a legal question: the five questions and the referral note	Pairs write a referral question from a case vignette
70–82	The Adewale case	Case, with students deciding what the therapist does
82–90	Documentation as evidence	One file note rewritten in legal-proof form

10.2 Five concepts that organise the week

Concept 1 — Legal literacy is a competence; legal advice is a different profession. The therapist must know enough to recognise, to route and to record. She does not opine on whether a marriage is valid, whether a divorce will be granted, whether custody will be awarded, or what a court will do. The distinction is not modesty; it is the boundary of the licence, and crossing it exposes the client to wrong advice and the therapist to liability.

Concept 2 — Legal pluralism means the family may be operating in two systems at once. Statutory, customary and Islamic frameworks coexist, and the same family may have obligations under more than one. The clinical consequence is that a client's account of "what the family has decided" may be legally real in one system and irrelevant in another. The therapist does not resolve this; she establishes which forum is operating and refers the question.

Concept 3 — The welfare principle governs children's cases, and it is not the same as the parents' wishes or the therapist's view of the parents. Every custody and maintenance decision is made by reference to the child's best interests. The therapist's contribution is evidence about the child, not advocacy for a parent. This is why the therapist's clinical opinion — "she is a good mother" — is not what the court needs; the court needs facts about the child's care, stability, schooling and relationships, and an assessment conducted for the purpose.

Concept 4 — Clinical and forensic roles are incompatible in the same case. A treating therapist cannot conduct a custody evaluation of her own client: the alliance compromises the neutrality the court requires, the client's disclosures are not gathered

for the purpose, and the therapist's opinion is shaped by months of one-sided narrative. The correct answer to the request is a factual letter and a referral to an independent evaluator (Week 6, section 6.7).

Concept 5 — Records are legal documents from the moment they are written. They can be subpoenaed, read aloud in court, quoted in a complaint, and inspected by a regulator. The note that records reasoning, dates, consent and actions is the therapist's evidence that she acted properly; the note that records impressions and speculation is the other side's evidence that she did not.

10.2a The five areas of law, with the boundary in each

1. Marriage and dissolution. The therapist may: ask which system or systems the marriage was contracted under, whether any petition or customary process has been started, and what the client wants. The therapist may not: advise on grounds, validity, timing or the likely outcome. Refer: to a family lawyer, with the question written down.

2. Children — custody, access, maintenance, guardianship. The therapist may: provide factual information with consent, support the child, and help the parents reach agreements that are not legally binding. The therapist may not: evaluate her own client's parenting for a court, recommend an outcome, or write a letter of support. Refer: to an independent forensic evaluator, and to a lawyer for maintenance and custody procedure.

3. Violence — criminal and protective. The Violence Against Persons (Prohibition) Act 2015 applies in the Federal Capital Territory and in the states that have adopted it; state laws vary in the protections they provide. The therapist may: explain her own reporting duties, support a client through a police statement, and refer to legal aid. The therapist may not: advise whether to report a crime, mediate a criminal matter, or promise an outcome.

4. Mental health legislation. The Mental Health Act 2021 reformed the framework nationally, and its adoption and implementation vary by state. It matters clinically because it governs involuntary assessment and treatment, and because a family may attempt to use it against a member. The therapist may: explain the process in general terms and support a client through an assessment. The therapist may not: initiate an involuntary process except through the statutory route and with legal advice.

5. Records and data protection. The Nigeria Data Protection Act 2023 and the professional codes impose duties about storage, access, retention and disclosure. The therapist may: hold records, share them with consent, and disclose where compelled or where a duty requires. The therapist may not: disclose without consent or legal authority, keep records indefinitely without a policy, or store clinical material in a form that can be read by a family member.

10.2b When litigation is pending: the therapy contract

The most demanding configuration in this week's material: the family is in court and the therapy is running at the same time. Four additions to the ordinary contract, agreed at the start and recorded.

1. The therapy is not part of the case. "This work is not evidence. If a court asks for my records, I will tell you and I will do what the law requires, and I will not be writing a report for either side." Stated at the start, it prevents the client recruiting the therapist as an ally, which is the commonest failure.

2. The court timetable is not the clinical timetable. The therapist asks for the dates of hearings and filings and writes them in the file. Sessions immediately before a hearing are different sessions — the client is mobilised, the children are frightened, and the material is often the material of the courtroom rather than of the family.

3. Nothing is said about the other party that cannot be said in front of them. This is a constraint on the therapist, not on the client: the therapist will not collude in building a case, and will name the collusion when it is invited ("you are asking me to agree that he is a bad father, and I am not going to do that").

4. The children are not witnesses. The therapist will not ask a child to report on a parent, will not carry messages, and will not tell a child what is happening in court beyond what the parents have agreed to tell them. Where the parents cannot agree, the therapist helps them compose a single short explanation, because the child who is told two stories is being asked to choose.

10.3 The plural system in the consulting room

The five configurations the therapist will meet.

1. **One marriage, one system.** The couple married under the statute and will divorce under it. The path is legible and the therapist's task is to keep the clinical work separate from the litigation timetable.
2. **One marriage, two systems.** A statutory marriage plus a customary ceremony, which many families treat as the "real" marriage and the registry as the formality. The dissolution question then becomes which forum the family will use, and the answer may differ between the husband's family and the wife's.
3. **Two marriages, sequentially.** A customary marriage followed by a statutory one, or the reverse. The legal position of the first wife may be entirely different from what she believes.
4. **Two marriages, concurrently.** An Islamic second marriage alongside a statutory first marriage, which may constitute bigamy under the statute — a criminal question, not a therapeutic one.
5. **No marriage.** Cohabitation, frequently described by the family as marriage, with children. Property, maintenance and inheritance questions arise, and the client may have no legal status at all.

What the therapist does with the configuration. Four questions, asked without legal opinion. Under which law was this marriage contracted, and is there a certificate? Has anything been filed or pronounced — a petition, a customary dissolution, a pronouncement? Who in the family has been involved in that process, and does the client agree with what they have done? What does the client want — the marriage, the divorce, the children, the property, or clarity?

The clinical significance of the fourth question. Clients frequently do not know what they want from the legal process, because the legal process has been started by somebody else — a husband's family, a brother, an employer. The therapist's work is to help the client separate the question "what is happening to me" from "what do I want", because a client who has not answered the second will be represented by whoever answers it for her.

The woman with no forum. In some of these cases the client has no effective access to any court: no money, no lawyer, no standing in the customary process, and a family that has already decided. The therapist's role then is practical and clinical rather than legal: identify the legal aid providers that exist, connect her with a women's organisation that accompanies clients, and support the psychological work of living with an outcome she did not choose. Doing this well is not a lesser form of help than referring to a lawyer.

10.3a The client who has been to three forums

A configuration that appears often enough to be its own subsection. The client has been to a customary court, to a pastor, and to a police station, and has three different answers. She arrives believing that "nobody will help", which is frequently accurate in the sense that no single forum has authority over the whole of her problem.

What the therapist does first. Builds the chronology, because the client's account will be organised by emotion rather than by date. A single page, with dates, forum, decision and who was present, converts a story of chaos into a set of events. The page is the client's, written with the therapist, and it is the most useful document the case will produce.

What the therapist does second. Establishes which forum is authoritative for which question. A customary court may have decided a matter of family status; a police report may have been closed; a pastor may have pronounced a reconciliation that has no legal effect. The therapist does not decide; she writes the question for each.

What the therapist does third. Names the pattern to the client. "You have been to three places and each of them answered a different question. That is why it feels as though nobody has answered you." Naming it is often the first moment at which the client feels understood, and it is the foundation for the referral that follows.

What not to do. Do not tell the client that her customary process "does not count". It counted, to the people who conducted it, and to her; the legal question about its effect is separate, and dismissing the process confirms that the therapist does not understand her world.

10.4 The welfare principle and the custody report

The principle. In any decision concerning a child, the child's best interests are the paramount consideration. The therapist does not need to know the statutory wording; she needs to know that the child's interests are not the same as either parent's wishes, and that a court will weigh stability, care, education, health, the child's own views where age permits, and the child's relationships.

What a court typically asks for, and what a therapist can provide.

The court wants	The treating therapist can supply	The treating therapist cannot supply
Facts about the child's care	Attendance, presenting problems, the child's own statements, with consent	An assessment of the other parent
The child's views	What the child has said, in context, if the child consents or the law requires	A recommendation about residence
Stability and functioning	Observations from sessions	A prediction of future parenting
Expert opinion	Nothing	An expert opinion — she is not the expert, and she is not independent

The factual letter, which is usually the right answer. On letterhead, with the client's written consent, it states: the dates of contact, the fact of attendance, the presenting problem in the client's words, and — only if the therapist judges it appropriate and the client agrees — a short statement of the clinical picture. It does not contain an opinion about parenting, an assessment of the other party, or a recommendation. It is dated, signed and sent to the court or the lawyer, and a copy goes in the file.

The request to appear. Where the therapist is subpoenaed she must attend and answer. Where she is asked to appear as an expert she must decline, and explain why: an expert opinion requires an assessment conducted for the court, independence from the parties, and a methodology that can be scrutinised. A treating therapist who offers an expert opinion in her own client's case is not being helpful; she is being replaced as a witness by an advocate.

The child's own voice. A child of twelve may express a preference, and the expression is evidence, not a decision. The therapist's task is to ensure the child is not placed in the position of choosing between parents, which is a burden that produces loyalty conflicts and lasting damage. The formulation that helps: "you do not have to choose. The adults will decide, and what you can tell me is what you want them to know about how it is for you."

The report that must never be written by a treating therapist. "In my opinion, the mother is the more capable parent and the children should reside with her." It is a forensic opinion, it is outside the therapist's role, it will be attacked in cross-examination, and it converts months of clinical work into a piece of advocacy that the other side can dismantle by showing the therapist met the father four times.

10.4a Routing a legal question: the five questions and the referral note

The moment it happens. A client says, "can he take the children from me?" or "if he has another wife, is our marriage not void?" or "my brother says I can divorce him for free in the customary court, is that true?" The therapist has two seconds in which her face will answer, and the answer must be a route rather than an opinion.

The five questions, in the session.

1. **"Has anything been filed?"** A petition, a customary process, a police report, a maintenance application. The answer changes the urgency from "sometime" to "within days".
2. **"Do you have a lawyer?"** If yes, the therapist's job is to help the client formulate the question for the lawyer. If no, the therapist's job is the referral, with the specific route.
3. **"What are you being asked to decide, and by when?"** A court date, a family meeting, a deadline for a response. Dates are the therapist's business even where the law is not.
4. **"Who else is a party?"** A husband's family, a co-wife, a child's guardian, an employer, an immigration authority. Multi-party cases need multi-party advice.
5. **"What do you want me to do?"** The client may want an opinion, a letter, a witness, or a person to sit with her. Each is a different request, and only some of them are ones the therapist can meet.

The referral note, which is the deliverable of the whole week. Six lines, written in the session, handed to the client, copied to the file.

> Re: [client], [date]. Thank you for seeing [client], who is considering/ facing [divorce petition / custody application / customary dissolution / maintenance claim]. She would like advice on: (1) the validity and effect of the [customary] dissolution pronounced on [date] in respect of a [statutory] marriage; (2) her position on residence of and access to the two children, aged 12 and 7; (3) whether maintenance can be sought and from which court. She is not able to fund private representation and has asked about legal aid. I am her therapist and am not able to advise on any of these matters. Yours, [name], [qualification], [contact].

Why the three numbered questions matter more than the letter's tone. A lawyer receiving "please advise on custody" will write a general letter that costs money and answers nothing. A lawyer receiving three numbered questions can answer them in fifteen minutes and will.

What goes in the file. The referral note, the date, the client's consent to the referral being made, and the question she asked in her own words. Nothing about the therapist's clinical view of the husband.

10.5 Building the referral network before you need it

Therapists build clinical networks and neglect legal ones, then improvise at the point of crisis. Five contacts, established in advance, each with a name and a number verified in the last year.

1. A family lawyer willing to take occasional pro bono or reduced-fee referrals, and willing to receive the six-line note of 10.4a rather than a telephone call in a panic.

2. A legal aid provider — the Legal Aid Council, a state scheme, a university law clinic, or a women's organisation with a legal desk. Legal aid is frequently available and almost never known about by the client.

3. The police family support unit at the nearest station, with the name of the officer in charge. This is the contact that determines whether a report is taken properly at eleven at night.

4. The state ministry responsible for social welfare, or the child protection unit, with the out-of-hours number if one exists (Week 9).

5. A mediator or a customary process you can respect. Not every case needs a court, and some families will use a mediation process that is cheaper, faster and more legitimate to them than litigation. The therapist should know which local processes are conducted fairly and which are instruments of the husband's family.

The maintenance of the network. One telephone call a year to each contact, and one case referred. A network that is only used is not a network; it is a list of strangers with whom the therapist is unfamiliar at the moment of greatest need.

10.6 Case: the Adewales, and the marriage that was also another marriage

The referral. Bola Adewale, 38, a schoolteacher, attends alone "for stress". She and Yinka, 42, a civil servant, have been married twelve years, with two children, 12 and 7. In session three she says, in passing, "his people have done what they wanted to do. They say we are no longer married."

What had happened. Bola and Yinka married in a church with a marriage certificate — a statutory marriage under the Marriage Act. In the eleventh year, following a dispute about money and Yinka's relationship with another woman, Yinka's family convened and pronounced a customary dissolution. Yinka moved out. He has continued to pay the school fees and has visited the children twice a month. He has told Bola that the marriage is over and that she should "go back to her family". Her brother has told her the same. She has no lawyer, has not been to any court, and believes that what the family pronounced is final.

The legal question, which is not the therapist's to answer. Whether a customary dissolution can dissolve a statutory marriage, and what Bola's position is in respect of the marriage, the children, maintenance and property. The therapist knows enough to know that this is a real question with a real answer, and does not know the answer.

What the therapist did.

1. **She asked the five questions.** Nothing filed; no lawyer; the deadline perceived by Bola was her family's pressure to move out "before the school term"; parties included Yinka's family and hers; and what she wanted was "to know whether I am still married".
2. **She wrote the six-line referral note** with three numbered questions, and telephoned a legal aid organisation she had a contact at, with Bola's consent, on the same day.
3. **She separated the three problems** that Bola had fused: the legal status of the marriage; the practical question of money and children; and the emotional question of whether her husband had ever intended to remain. Only the third was the therapist's, and it had been invisible because the first two were consuming her.

4. **She did not tell her to fight or to accept.** She said what she had said in other cases: "there is a legal question and I cannot answer it; there is a decision about what you want, and that is yours; and there is the grief, which is mine to work on with you."
5. **She recorded all of it,** including that she had made no legal assessment.

What happened. A lawyer at the legal aid organisation advised that a customary dissolution does not dissolve a statutory marriage, and that Bola remained married under the statute; a petition would be required. Bola did not immediately petition. She spent four months in treatment deciding whether she wanted to, and in the fifth month she filed. The therapist supported her through the process and did no legal work at all.

The clinical work, which is the part students underestimate. Bola's presenting "stress" was the experience of having been told by two families what her life was. She had not been asked what she wanted since she was twenty-six. The therapy's central question — "what do you want?" — was answered for the first time in session nine, in these words: "I want to be a married woman or a divorced woman. I do not want to be this."

What the case teaches.

1. **Legal pluralism produces a particular kind of helplessness,** and the helplessness is the clinical problem. The client who does not know whether she is married cannot plan, cannot grieve and cannot decide.
2. **The therapist's ignorance of the law is not a barrier when it is honest.** "I do not know, and here is how we will find out" is a complete clinical intervention.
3. **The family's pronouncement is not the law.** In this case the family's process was real to the family and had no effect on the statutory marriage. The therapist's refusal to accept the family's account as the legal fact changed the case, and she reached that refusal not by knowing the law but by knowing that a question existed.
4. **The three problems had to be separated** before any of them could be worked. This is the single most useful move in the whole week, and it generalises to almost every case in which a family presents with a legal crisis.

10.6a The factual letter, written out

The letter that is almost always the right answer to "will you write something for my case". On letterhead, with the client's written consent, five short paragraphs.

> **RE: [CLIENT NAME], [DATE OF BIRTH]; CONSENT TO DISCLOSURE DATED [DATE]**
>
> I have seen [client] on [n] occasions between [date] and [date], at her request, for [presenting problem in the client's words]. She has attended [n] of [n] scheduled appointments. The clinical picture is of [one or two sentences: anxiety following separation; grief; difficulty sleeping]. I have not assessed her parenting capacity and I make no recommendation about residence or contact. [Client] has told me that [only if relevant and consented: she wishes to see her children; she is attending a support group]. I make no finding as to the truth of what she has told me. If the court requires information about the children's welfare, I would suggest an independent assessment; I am not able to provide one, as I am [client]'s treating therapist and my view would not be independent. Yours faithfully, [name], [qualification], [registration number], [contact].

Why the third paragraph is written that way. "I make no finding as to the truth" is the sentence that keeps a clinical letter from becoming a witness statement. Clients often ask for it to be removed, because it weakens the letter; the therapist explains that its presence is what makes the rest of the letter admissible as an account rather than challengeable as advocacy.

The refusal to add the sentence the client wants. "In my opinion she is a fit mother." The refusal is delivered with the alternative: "I can say what I have observed, and I cannot say that. If you need an opinion, you need someone the court will treat as independent, and here is how to ask for one."

10.7 Transcript: "can he take the children?"

Session five, minutes eighteen to thirty-one. Bola has just been told that she remains married under the statute.

> **D1 Bola:** So can he take the children? His mother says they are his children because they bear his name. **D2 Therapist:** (pause) That is a legal question and I am not going to guess at it. (pause) I can tell you what I know and what I do not know. — The refusal, followed immediately by the structure of the answer. "I am not going to guess" is stronger than "I am not qualified", because it names what the therapist will not do rather than what she lacks. **D3 Bola:** But you must know. **D4 Therapist:** I know that there is a law about this, and that it is not the law his mother is describing. (pause) I do not know what a court would decide, and neither does she. — The two facts the therapist can honestly give: a law exists, and the family's version is not it. The second half is the clinically important one, because it interrupts the family's certainty without substituting the therapist's. **D5 Bola:** So I need a lawyer. **D6 Therapist:** You need a lawyer, and you needed one four months ago, and that is not your fault. (pause) I have a name, and I will telephone today if you want me to. — The acknowledgement that avoids blame, and the concrete offer. The client who is told "you should have got a lawyer" hears an accusation, and the accusation is almost always unjust. **D7 Bola:** I do not have money for a lawyer. **D8 Therapist:** There is legal aid, and I have checked that this organisation takes cases like yours. (pause) I am going to write the questions down for you, so that you are not paying for the first hour while he works out what you are asking. — The specific, prepared answer. Vague reassurance about legal aid is what most clients receive; a checked contact and three written questions is a service. **D9 Bola:** (long pause) If I am still married, then what has he been doing for a year? **D10 Therapist:** (pause) That is the question underneath the legal one, and it is the one we can work on. — The pivot to the clinical material, made explicit so that the client knows the therapist has heard it and has not dropped it. **D11 Bola:** He has been living as if I do not exist. And his family have been telling everyone in the compound that I am the one who refused. **D12 Therapist:** So you have been divorced in your street and married in the law. — The formulation that the rest of the case is built on, offered in one sentence and in the client's own terms. **D13 Bola:** (crying) Yes. (pause) That is exactly it. **D14 Therapist:** Then let us be clear about the three things we are dealing with: what the law says, which is your lawyer's; what you want, which is yours; and what it has been like to live in that gap, which is ours. — The separation of the three problems, stated aloud so that the client has a map she can use outside the room.

What the transcript shows. Seven turns, and in none of them does the therapist answer the legal question. In four of them she gives the client something usable: the existence of a law, the name of an organisation, three written questions, and a formulation of her situation that she recognises. The refusal is not the intervention; the substitution is.

10.8 Laboratory: legal triage and the written question

Part A — triage (twenty minutes, individual). Eight vignettes, each with a legal question embedded in clinical material. The student identifies: the area of law; whether the question is urgent (within seven days) or not; what the therapist may say; what the therapist must not say; and the referral destination.

Vignette	Embedded question
A wife says her husband's family "divorced" her by custom	Validity; effect on statutory marriage

Vignette	Embedded question
A father asks whether he must keep paying school fees after separation	Maintenance obligation
A mother says her husband has taken the children to his village	Custody; possible abduction
A client asks whether she can refuse her husband a second wife	Nothing legal; a marital and religious question
A couple ask whether their church marriage is "real"	Validity; registration
A client's employer demands a report about her	Data protection; consent
A client asks if he can be forced into psychiatric treatment	Mental health legislation
A client asks who will get the house	Property; matrimonial causes; customary law

Part B — the six-line note (twenty-five minutes, pairs). Each pair writes the referral note for one vignette, with three numbered questions, and reads it aloud. The group then marks it against four criteria: the questions are answerable; the note contains no legal opinion; it contains no clinical opinion that is not necessary; and it identifies the client's practical constraint (money, distance, language).

Marks (20).

Criterion	0–1	2–3	4–5
Identification	Wrong area of law	Right area, no urgency	Right area, urgency and forum identified
Boundary	Gave an opinion	Hedged opinion	Refused clearly and gave something usable instead
The note	A name only	A referral	Three answerable questions, practical constraints named
Record	None	Description	Question, consent, date, referral made, advice recorded

The comment to make in the debrief. The best notes in the room will be the shortest. Legal questions are answered by being narrowed.

10.9 Six errors

Error 1 — The reassuring opinion. "He cannot take the children; you are their mother." It is said kindly, it is frequently wrong, and the client will act on it. The error is not the warmth; it is the opinion.

Error 2 — Refusing to engage at all. "I am not a lawyer" said and nothing else offered leaves the client with the same problem and one fewer ally. The refusal must be followed by the route.

Error 3 — Treating the family's pronouncement as the legal fact. Customary processes are real to families and may have no legal effect. The therapist who accepts the family's version confirms the client's helplessness.

Error 4 — The letter of support. "I have known Mrs Adewale for a year and she is a devoted mother." It is not evidence, it will be cross-examined, and it has converted the therapist into a witness for one side.

Error 5 — The forensic evaluation conducted by the treating therapist. The role conflict described in 10.4, made worse by the therapist's belief that she knows the family better than an outside evaluator would.

Error 6 — The clinical note written for the file and read in court. Sarcasm, speculation, unverified allegation recorded as fact, and abbreviations that cannot be explained. The note is written for a reader who will be hostile. Write it that way.

10.9b The four questions a therapist asks her own lawyer

Therapists hesitate to consult a lawyer about their own practice because they assume it is expensive and that the question must be serious. In fact four questions account for most of the need, and each can be answered in a short consultation.

1. "May I disclose this, and to whom?" The records question. The answer depends on the statute, the consent and the order, and it changes with the jurisdiction.

2. "What must I do now that I have been served?" The subpoena question. The answer is almost always: verify, tell the client, obtain advice, produce only what is required, and keep a copy of everything produced.

3. "Am I obliged to report this?" The child protection and risk question, where the threshold is unclear.

4. "If I proceed as I intend, what is my exposure?" The advance question, asked before a decision rather than after it, and the one that most reduces the risk of a complaint.

The practice's arrangement. A retainer or a professional indemnity scheme that includes legal advice, and a named contact. The cost of the annual arrangement is a fraction of the cost of a single defended complaint, and the availability of advice changes the therapist's decisions in the direction of more honesty rather than less.

10.10 Culture, faith and the law the family actually uses

Customary process is not a lesser law; it is a different forum with different rules and real authority. In much of this region, a family's decision about a marriage is made in a room with elders, is binding in the eyes of everyone present, and is never written down. A therapist who treats this as "not real" will lose the family's confidence; a therapist who treats it as legally decisive will misinform the client. The correct position holds both: "what your family decided matters to them and to you. Whether it changes your legal position is a different question, and we will ask someone who knows."

The two forums and the confusion between them. Clients frequently assume that a customary dissolution ends a statutory marriage, that a church ceremony creates a legal marriage, that an Islamic marriage gives a second wife the same standing as the first, and that a father's family owns the children. Each assumption is common, each carries

consequences, and none of them can be corrected by the therapist's opinion. It is corrected by a lawyer's letter and by the client's decision to act on it.

Religion, obedience and the right to leave. A client may believe that her faith forbids divorce and that her suffering is a cross. The therapist does not argue with the belief. She does three things: she finds out what the client's own religious authority teaches about cruelty and about release, which is frequently more permissive than the client assumes; she separates the religious question from the legal one, since a religious prohibition is not a legal incapacity; and she asks the question that opens the case: "if your faith permitted it, what would you want?"

The family's use of the law as a threat. "My husband says he will take the children and I will never see them." Sometimes the threat is knowledge of the law; more often it is a threat with no legal basis, issued to produce compliance. The therapist cannot assess which it is. She can do two things: name the threat as a threat, which reduces its power, and route the client to advice that will tell her the truth.

The honour dimension of a public case. Where the family's standing depends on the marriage being intact, a court case is a public declaration that it is not. Clients may therefore prefer a private, customary or mediated route even where a court would give them more. That preference is rational and the therapist should respect it, while making sure the client knows what she is giving up.

The male client and the law's reputational cost. A father who is seen to be fighting his wife in court may lose standing at work and in his family. The therapist working with such a father should not read his reluctance to litigate as indifference to his children.

> **DEBATE**
> "A family therapist should always advise a client to obtain legal advice before making any decision about the marriage." Argue for: information is a precondition of consent, the stakes are irreversible, and the therapist's silence can be read as endorsement of the family's version. Argue against: the client may not want legal advice, the referral may itself escalate the conflict, and the therapist's role is to support the client's own decision rather than to steer it. Then state the three cases in which you would insist.

10.11 Evidence note

What is known. Legal pluralism is a structural feature of family life in Nigeria and in much of sub-Saharan Africa, and the literature on family law in plural systems documents precisely the confusions described in this week: conflicting adjudications, women's differential access to forums, and the gap between customary practice and statutory protection. Research on help-seeking in domestic violence consistently finds that legal literacy is one of the strongest predictors of a woman taking protective action, and that the absence of accessible advice is a principal barrier. Studies of client experiences of court-involved therapy find that role confusion — treating clinician versus evaluator — is a common source of complaint and of harm.

What is weaker. There is little empirical work on the specific question of how therapists should handle legal plurality in the consulting room, and almost none on outcomes. The guidance in this week is therefore derived from professional ethics, from the structure of the law, and from practice rather than from trial evidence.

What is not supported. That a therapist's clinical opinion about parenting has predictive validity for custody outcomes. That legal advice from a non-lawyer is harmless if it is well-intentioned. And that referral to a lawyer inevitably escalates a family's conflict — in many cases it reduces it, because each party stops guessing.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Legal advice given in week two, before positions have hardened, and in week twenty, after a customary dissolution has been pronounced and acted upon, produce two different families.

10.11a The six documents a family therapist should hold

Legal literacy is partly a matter of paperwork, and the paperwork is cheap.

1. **A consent form** that names the limits of confidentiality, the duty to report, the possibility of records being compelled, and the identity of the client system — reviewed and re-signed when the system changes (Week 2).
2. **A privacy and records policy** stating what is kept, where, for how long, who may see it, and how a request for access is handled.
3. **A disclosure protocol** for requests from lawyers, courts, employers, insurers and the police: who decides, what is checked, what is recorded.
4. **A subpoena procedure,** one page: verify the document, tell the client, contact the supervisor or indemnifier, seek legal advice, produce only what is required.
5. **A complaints procedure** telling the client how to complain about the therapist, because a client who has a route to complain is a client less likely to litigate.
6. **A list of verified contacts** — lawyer, legal aid, police family unit, social welfare, crisis line — with the date each was last checked.

The file that survives scrutiny is not the file with the most detail. It is the file in which every disclosure has a consent behind it, every referral has a question attached, and every decision has a date.

10.12 Key terms

Legal pluralism — the coexistence of statutory, customary and religious legal systems within one jurisdiction.

Matrimonial Causes Act — the statute governing statutory divorce in Nigeria, including the two-year rule and the recognised grounds.

Welfare principle — the rule that the child's best interests are the paramount consideration in decisions concerning children.

Forensic role — the role of an independent expert providing an opinion to a court.

VAPP Act 2015 — the Violence Against Persons (Prohibition) Act, applicable in the FCT and in adopting states.

Mental Health Act 2021 — the national framework for mental health care, with state-level variation in implementation.

Legal aid — publicly funded legal advice and representation; availability varies and is frequently unknown to clients.

10.13 Self-test

1. Name the four marriage systems and the dissolution route for each.
2. Give the boundary, for the therapist, in each of the five areas of law.
3. What is the welfare principle, and what does it require of a therapist asked for a report?
4. Why should a treating therapist not conduct a forensic custody evaluation of her own client?
5. State the five questions that route a legal question.
6. What is the six-line note, and why three numbered questions?
7. In the Adewale case, what were the three problems, and which was the therapist's?
8. Why is the family's pronouncement not the law, and why does that matter clinically?

Answers. (1) Statutory — petition to the High Court under the Matrimonial Causes Act; customary — customary process and customary courts, locality-specific; Islamic — pronouncement, khul' or Sharia court; cohabitation — no marriage, so limited statutory protection. (2) Marriage — may ask which system and what has been filed, may not advise on validity or grounds; children — may give factual information with consent, may not evaluate or recommend; violence — may explain her own duties and refer, may not advise on reporting or mediate a crime; mental health — may explain and support, may not initiate involuntary processes outside the statute; records — may hold and share with consent, may not disclose without authority or keep records indefinitely without policy. (3) The child's best interests are paramount; it requires the therapist to give facts about the child and to refuse an opinion about parenting or residence. (4) Because the alliance and the one-sided narrative compromise independence, the material was not gathered for the purpose, and the opinion will be dismantled in cross-examination. (5) Has anything been filed; do you have a lawyer; what must be decided and by when; who else is a party; what do you want me to do. (6) A short referral letter with three numbered, answerable questions; because a general request produces general advice that costs the client money and answers nothing. (7) The legal status of the marriage; the practical questions of money and children; and the emotional question of whether her husband had ever intended to stay — only the third was the therapist's. (8) Because a customary process may have no legal effect on a statutory marriage, and clinically because the client's belief that her life has been decided by others is the presenting problem; correcting it restores her capacity to decide.

10.9a The therapist in the witness box

Most family therapists will give evidence at some point, usually in a custody or protection matter, and the experience is unlike anything else in the work.

The four rules. Answer the question asked. Not the question behind it, not the one you would prefer, and not the narrative you have prepared. Say "I do not know" when you do not know, which is more credible than an improvisation and is frequently the honest answer. Do not defend your clinical decisions beyond the point of explanation. A therapist who argues with counsel is winning an argument and losing the case. Take your notes, and refer to them, because a witness who says "I would need to check my notes" and then reads them accurately is more reliable than one who reconstructs from memory.

The three questions that will be asked. "You only ever saw one side of this family, did you not?" "You are not a psychiatrist, are you?" "You formed the view that my client was the problem because that is what you were told." Each is answerable honestly and none requires a defence. The honest answers — "yes, I saw both parents but mostly the mother", "no, and I have not offered a psychiatric opinion", "I formed my view from what I observed and I recorded what I was told" — are stronger than any attempt to sound more authoritative than the therapist is.

The preparation. Read the file in full; write a chronology; identify the limits of your knowledge; discuss with your supervisor what you will say about the decisions you made; and remember that the court is not asking whether you are a good therapist. It is asking what you know.

10.14 Reflective task

Take one of your own family cases from placement, or the Adewale case, and write three hundred words separating the clinical question from the legal one. Then answer: which of the two did you spend more time thinking about, and why? Most trainees find that the legal question is more anxiety-provoking because it has a right answer and they do not know it — which is exactly the anxiety that produces either an opinion or a refusal, and neither is the work.

10.15 Further reading

- The Marriage Act, the Matrimonial Causes Act and the Child Rights Act, read in the original and not in summary — they are shorter than students expect.
- The Violence Against Persons (Prohibition) Act 2015, with attention to the states that have adopted it.
- The Mental Health Act 2021 and the state-level implementation guidance where you practise.
- Any current text on family law in Nigeria, for the reference section of the practice manual.
- The Nigeria Data Protection Act 2023 and your professional body's record-keeping guidance.

Next: Week 11 — Records, subpoenas and the court.

Week 11 — Records, subpoenas and court

Learning outcomes. By the end of this week the student can (1) write a record that serves three readers — her future self, a colleague taking over, and a court; (2) distinguish fact, observation, clinical opinion and hypothesis in a note; (3) handle an access request from a client, a partner, an employer or a court; (4) respond to a subpoena in the correct order and with the correct advice; and (5) give evidence as a witness of fact without becoming an advocate.

The record is two things at once

It is the clinical memory of the treatment, and it is a legal document that may be read by people who have no interest in the therapist's good intentions. The two functions are compatible, and the note that serves the first serves the second: a note that records what was observed, decided and done is both the best clinical note and the best evidence. Notes fail when a therapist writes for one reader only — the impressionistic note written for herself, or the defensive note written for a regulator — and the failure shows in both places.

Write for three readers.

Reader	What they need
The therapist in six months	What was the problem, what was tried, what happened, what is planned
A colleague taking over tonight	The risk, the current plan, who knows, what to do if telephoned
A court	Facts with dates, sources, decisions with reasons, consent, and no speculation

The four registers, which must never be mixed in a single sentence.

1. **Fact.** "Client attended on 4 March. Her left forearm was bandaged."
2. **Observation.** "She was tearful at the start of the session and calm by the end. She checked her phone four times."
3. **Clinical opinion.** "In my view the presentation is consistent with an acute stress response following discovery of the affair."
4. **Hypothesis.** "Possible that the husband's drinking is a factor; not yet explored."

A note that says "client is a manipulative woman who is clearly exaggerating her husband's behaviour" contains no register at all: it is a character judgement, it is unsupported, and it will be read aloud in a witness box.

11.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The three readers; two notes on the same session, compared	Read both, vote on which is defensible

Minutes	Content	Teaching form
8–22	Five concepts: memory, hostile reader, four registers, minimum necessary, privilege	Lecture
22–40	Note structure, with a worked example written live	Therapist writes a note on the board from a vignette
40–52	The multiperson record: whose file, whose secret, where it goes	Three configurations, students place the material
52–65	Access requests and the subpoena sequence	Simulation: a letter arrives, what do you do in the next hour
65–80	Giving evidence: the four rules, the three questions	Mock cross-examination with a colleague as counsel
80–90	The Eze subpoena	Case, students decide what is produced and what is withheld

11.2 Five concepts that organise the week

Concept 1 — The record is the treatment's memory, and memory is a clinical instrument. Continuity of care depends on the next clinician being able to reconstruct what happened. A file of one-line entries ("seen; discussed anxiety; better") is not a record of a treatment; it is a record of attendance, and it cannot support a handover, a supervision, or a defence.

Concept 2 — Assume a hostile reader. Not because the reader is hostile, but because the only readers who matter in a dispute are the ones looking for inconsistency. Write as though the sentence will be read aloud by someone who wants it to mean something else, because one day it will be.

Concept 3 — The four registers must be labelled. Facts stated plainly, observations stated as observations, opinions introduced as opinions, hypotheses marked as hypotheses. The failure to distinguish them is the single commonest reason a clinical record damages the clinician who wrote it.

Concept 4 — Minimum necessary is the rule for every disclosure. To a court, to a school, to a physician, to a partner: disclose what is required for the purpose and nothing else, and record what was disclosed and why. A file sent in full to a school because the school asked for "the records" is a breach, whatever the school's intention.

Concept 5 — Privilege and confidentiality are different, and privilege is not the therapist's. Confidentiality is the professional duty not to disclose. Privilege is the client's legal protection against disclosure in proceedings. In most jurisdictions privilege is qualified: it can be waived by the client, overridden by statute, or displaced where the court orders production. The therapist cannot promise privilege, which is why the consent conversation must mention that records can be compelled.

11.2a The note, written out

The structure for a family session. Six headings, five to eight minutes of writing, and the note is done.

> **Attendees:** Mrs Eze, Mr Eze (first 20 minutes only), therapist. Children not present. **Presenting this session:** Mr Eze raised the school's report of Chidi's absences; Mrs Eze stated she had not been told of the meeting. **Risk screen:** No disclosures of harm this session. Direct enquiry about safety at home — client states no violence in the last month; no suicidal ideation on direct enquiry. **Interventions:** Reviewed the previous week's agreement about school communication; both parents agreed a written message each Friday; therapist modelled the message and both rehearsed it. **Response:** Mr Eze left the session at minute 20 saying he had to return to work. Mrs Eze remained and stated that she had stopped telling him about school matters "because it always ends in a quarrel". **Plan:** Weekly sessions continue. Mrs Eze to attend alone next week, as agreed in the room. School's safeguarding lead to be told only that the family is attending; consent for that limited contact recorded.

The same session, badly written. "Good session. Couple engaged. Father defensive and clearly controlling; mother tearful and possibly depressed. Will continue." Four judgements, no facts, no plan, no consent, and each judgement is an assertion the therapist cannot support from the session.

The six things every note should contain. Date and attendees; the reason for the session; the risk screen, with the questions asked; what the therapist did; what the client(s) did in response; and the plan, with who was told what.

The eight things that must never appear.

1. **Character judgements** — "manipulative", "attention-seeking", "a difficult woman".
2. **Speculation about third parties** — "I suspect the grandmother is behind this".
3. **Unverified allegation recorded as fact** — "mother is an alcoholic" when the source is the husband.
4. **Other clients' information.**
5. **Secrets** — an individual disclosure recorded in a file to which the other partner may have access.
6. **Jokes, irony, nicknames,** and anything the therapist would not want read aloud.
7. **Abbreviations that cannot be expanded** by a reader who is not the author.
8. **Opinions about the law or the likely outcome of a case.**

The note written late. It happens, and the honest entry is "note written on [date] from memory of session on [date]". An undated, unmarked reconstruction presented as contemporaneous is the error, not the lateness.

11.2b The multiperson record: whose file is it?

The question that generates more complaints than any other in couple and family work.

The three configurations.

Configuration	The file	The individual disclosures
Individual client, family as context	The client's file	In the file; the family has no access
Couple as client, one file	One file, both named	Individual disclosures recorded in a **restricted section**, accessible only to the therapist and supervisor
Family as client, children included	One file with the children named as clients	Children's disclosures recorded separately, with a note of who may see them

The restricted section: how it works in practice. A separate, clearly marked section of the record — in an electronic system, a separate note type with restricted access — containing material given in individual sessions that the other party has not been told. The therapist's clinical record is complete; the disclosure risk is contained. Where the practice has no electronic system, the equivalent is a sealed envelope referenced in the main file, with the practice policy stating what it contains and who may open it.

Why this matters more than it appears. In a couple case the partners may each have a right of access to the data held about them under data protection law, and the other partner's disclosures are data about them in part. The therapist who has recorded the wife's account of the husband in the joint file, in a form that identifies him, will find herself in the position of having to disclose it or to justify withholding it. The restricted section and the practice's policy are what make that possible.

What the note should say when restricting. "Individual session [date]: material recorded in restricted section per practice policy [reference]; client informed that this material will not be shared with [partner]; no information affecting safety withheld from the file." Three clauses, and they answer the question a regulator will ask.

11.2c The electronic record

Most practices now keep records electronically, and the electronic record creates five problems that the paper record did not.

1. The audit trail remembers everything. Every version, every deletion, every time a note was opened and by whom. A therapist who "tidied" a note six months after a session will be asked why, and "for clarity" is not an answer that survives the question.

2. The shared device. A practice laptop used at home, a phone with the notes app, an email with clinical content in it — each is a disclosure waiting to happen. The policy is simple: clinical records are kept on the clinical system, nothing clinical is in personal email, and no clinical material is on a personal phone.

3. The copy that leaves the building. A client who is sent a summary by email has a copy that the therapist cannot retrieve. Where a summary is sent, it is written for that purpose, marked as such, and recorded.

4. The template that writes the note. Templates improve completeness and produce homogeneity: every note says "client engaged well" because that is the first option in the

dropdown. The discipline is to use the template's headings and the therapist's own sentences.

5. The access request that is a download. Where a client requests their data, the practice must be able to produce a coherent record rather than a raw export that includes other clients' names. The restricted section of 11.2b is, in an electronic system, an access permission — and the permission must be tested before it is needed.

11.3 Access requests: four kinds, four answers

1. The client's own request. Under data protection law the client is generally entitled to a copy of the data held about them, subject to exceptions. The therapist's task: verify identity, establish whose data is involved, redact third-party information, and respond within the statutory period. Where the therapist believes access would cause serious harm, the statutory exception may apply and legal advice should be taken before refusing.

2. The partner's request in a couple case. The partner is also a client in a jointly-held file, which is why the restricted section exists. The therapist discloses what is about that partner and does not disclose the other partner's individual material. The reasoning is recorded.

3. The employer's, insurer's or school's request. Usually limited to attendance, or to a report with written consent. The therapist discloses the minimum and records it. An employer who asks for "the notes" receives attendance only, and is told why.

4. The court's or the police's demand. Weeks 10 and 11's core business, and the subject of 11.4.

The letter that answers an access request. Five lines: what has been received, what will be provided, what will be withheld and on what basis, when it will be sent, and who to contact. The therapist does not argue in the letter; she states and records.

The request to "just tell me how she is doing". The commonest of all, made by a relative at a wedding. The answer is one sentence, delivered warmly: "I am not able to talk about that, and I know it is because you are worried about her."

11.3b The client who asks to see her file

The request arrives in three forms, and each is answered differently.

"What have you written about me?" The most straightforward, and the one that is frequently met with evasion. The answer: "I will show you." Reading the record together — the notes, the plan, the supervision entries — takes twenty minutes and does three things: it demonstrates that the therapist has been paying attention, it allows the client to correct factual errors, and it removes the sense that something is being written behind her back.

"I want a copy." A data protection request, answered within the statutory period, with third-party information redacted and a short covering letter. The therapist does not require a reason, does not negotiate, and does not express hurt. The one legitimate

caution is where access would cause serious harm, which is a legal exception to be taken on advice and not a feeling to be acted on.

"My husband wants to see what you wrote about him." If he is a client in the same file, he may be entitled to what is about him. What is about his wife is not his, and the restricted section exists for that reason. The therapist replies in writing, stating what will be provided and what will not, with the basis.

The clinical value that is usually missed. Reading the file with a client is one of the few interventions that reliably produces a correction of the therapist's misunderstanding. Clients routinely say, at line three, "that is not what I meant" — and the correction is worth more than the twenty minutes.

11.4 The subpoena: what it is and what to do in the first hour

What it is. A court order requiring the production of documents or the attendance of a witness. It is not a request, it is not optional, and ignoring it is contempt. It is also not, in itself, an order that the therapist disclose everything in the file: it defines what is required, and the therapist's obligation is to comply with what it requires and nothing beyond.

The five checks, in the first hour.

1. **Is it valid and addressed to me?** Names, addresses, the court, the case number, the date, and the signature or seal. A photocopied letter from a lawyer is a request, not an order, and the two are answered differently.
2. **What exactly does it require?** Documents, attendance, or both; the date; and the scope — "all records relating to" is broader than "your notes of 4 March" and both must be read precisely.
3. **Has the client been told?** Unless the order prohibits it, the client is informed immediately, in writing, and the therapist explains what will be produced.
4. **Who is advising me?** The supervisor, the indemnity provider, the professional body and, where necessary, the therapist's own lawyer. The therapist does not respond alone to a legal process.
5. **What is the deadline, and is there time to object?** Where the scope is excessive or the material privileged, the advice may be to seek to limit the order. That application is made by a lawyer and it is made before the deadline, not after.

The order of operations once advice is taken.

- Produce only what is required, in the form required.
- Keep a complete copy of everything produced, indexed, and mark the date it was produced.
- Where something is withheld on advice, say so in writing and on what basis — a short privilege log.
- Record the process in the file: what was received, when, what advice was taken from whom, what was produced, when, and to whom.

- Tell the client what was produced, unless prohibited, and record that you did.

What not to do. Destroy or alter records, which is a serious offence and the end of the therapist's credibility. Produce the entire file because it is simpler. Ring the other side's lawyer to negotiate scope without advice. Or promise the client that nothing will be produced.

The extra care in family cases. The file frequently contains the other party's individual material and sometimes a child's. The therapist produces what the order requires about the client she treated, and where the order is broad enough to capture other people's disclosures, she raises the scope with her adviser. This is not obstruction; it is the discharge of duties owed to people who are not parties to the dispute.

11.4a Privilege, and the promise a therapist must not make

The distinction. Confidentiality is a duty the therapist owes. Privilege is a right the client holds, protecting communications from disclosure in legal proceedings. The therapist's obligation is to keep confidence; the client's privilege may be waived by the client and may be overridden by statute or by the court.

The four situations in which privilege will not save the file.

1. **Where the client waives it,** frequently by putting their mental state in issue — a client who claims psychological injury, or who relies on the therapy as evidence of good parenting, has opened the door.
2. **Where a statute creates an exception,** child protection being the commonest.
3. **Where the court orders production** after considering any claim of privilege.
4. **Where the therapist is the subject of a complaint or proceeding,** in which case the record is the evidence the therapist herself needs. The codes generally permit disclosure for the purpose of defending a complaint.

The consent conversation, revised. The Week 2 sentence — "this is confidential except for the limits I am about to describe" — should include one further clause in any case with a legal dimension: "and if a court ever orders me to produce my records, I will tell you, I will seek advice, and I will produce what the order requires."

What the therapist must never say. "Nothing you tell me can ever be used in court." It is false, it is frequently said to reassure, and it is the false promise on which a client later bases an allegation of betrayal.

11.4b The request from the school

The commonest third-party request in family work, and the one most often answered carelessly.

What the school actually needs, and what it does not. The school needs to know that the child is receiving support, whether there are practical implications for school life, and — where the school is a referrer — whether to continue the referral. It does not need the content of the therapy, the parents' accounts of each other, or a diagnosis the therapist is not qualified to give.

The three permitted answers. Nothing, where the family does not consent, except where a safeguarding duty requires otherwise. Attendance only, where the family consents to that and nothing more: "the family is attending and I have their consent to tell you that." A short agreed summary, written with the family, showing them the text before it is sent.

The safeguarding exception, handled carefully. Where the school's involvement is a child protection matter, the therapist may be required to share information with the safeguarding lead. That sharing is specific, recorded, and — unless telling the parents would endanger the child — done with the parents' knowledge.

The request that arrives by email at 4 p.m. on a Friday. The answer is not a reply. It is an acknowledgement that the request has been received, that consent will be checked on Monday, and that the therapist will respond in writing. Most breaches in small practices happen in the twenty minutes before a weekend.

11.5 Giving evidence

The role. A witness of fact reports what she saw, heard and did. She does not advocate, does not offer opinions outside her expertise, and does not answer the question she would have preferred. Where the therapist has also been appointed as an expert — rare, and never in her own client's case (Week 10) — the rules are different and the distinction must be stated at the outset.

The four rules, in the box.

1. **Answer the question that was asked.** Briefly, then stop. The commonest error is the therapist who, anxious to be understood, continues into material that creates a new line of attack.
2. **Do not guess.** "I do not know" and "I would need to check my notes" are complete and creditable answers. An improvisation under pressure is how witnesses are destroyed.
3. **Distinguish fact from opinion, and say which you are giving.** "I observed X. My opinion, for what it is worth, is Y, and I should say I am not a psychiatrist."
4. **Do not defend the treatment.** Counsel will invite the therapist to justify a decision. The answer is an explanation, not a defence: "I made that decision because at the time I had these three pieces of information, and I recorded it here."

The three questions that will come.

"You only ever saw one side, didn't you?" Answer honestly: how many sessions each party attended, and what the therapist did not have.

"You are not a doctor, are you?" Answer honestly, and state what the therapist is qualified to say — which is usually about the therapy, not the diagnosis.

"That is your opinion, and it is only an opinion." Agree, and say what it rests on: the sessions, the notes, the recorded statements, the supervision. The opinion that is supported by evidence survives this question; the opinion that is a feeling does not.

The preparation that matters. Read the whole file, not only the notes about the session in question. Write a chronology. Mark the three decisions you might be challenged on and be able to explain each in three sentences. And rehearse with a colleague playing counsel, because the first time the question is asked should not be in court.

The day itself. Take the file and the order. Arrive early. Dress as the court expects. Speak to the bench, not to counsel. Answer slowly. Ask for a break if you need one. And remember that the discomfort of the witness box is not evidence that the therapist did something wrong.

11.5a The report written before the hearing

Where the court asks for a report, the structure below produces something usable and honest. It is a report of fact and clinical opinion within the therapist's competence, not a custody evaluation (Week 10).

The six sections. The instruction: who asked for the report, what was asked, and the date. The sources: sessions, dates, who attended, documents seen, and what was not available. The history: brief, factual, and drawn from the record rather than from memory. The observations: what the therapist saw, in behavioural language. The clinical opinion: clearly labelled, bounded by the therapist's competence, and stating its limits. The limits: what the therapist cannot say — parenting capacity, the other parent, prognosis, the child's best interests.

The four sentences that keep a report inside its role. "I have not assessed [the other parent]." "I make no recommendation as to residence or contact." "This report is based on my contact with [client] over [n] sessions and on no other source." "I am willing to attend court and to answer questions about the treatment I provided."

What the report must not contain. A recommendation disguised as an observation ("she was consistently the more attentive parent, which I would expect to be relevant to residence"), the other party's alleged behaviour presented as fact, and any material from an individual session of a partner who has not consented.

11.6 Case: the subpoena in the Eze matter

Background. Mrs Ngozi Eze, 38, was in treatment for eight months following her husband's departure (Week 6, section 6.7, where she asked the therapist to be a character witness and was refused). She is now in a custody dispute over two children, 11 and 6.

The document. A subpoena duces tecum arrives by courier, addressed to the therapist, requiring "all notes, records, correspondence and reports relating to the treatment of Ngozi Eze" and her attendance on a date three weeks away. It is issued at the request of the husband's solicitor.

The five checks. Valid and properly addressed; scope extremely broad; client not yet told; supervisor available, indemnity provider contacted; deadline leaves eleven days to seek advice on scope.

What is in the file, and why it matters.

1. The treatment notes across eight months, including Mrs Eze's account of the marriage, which is highly critical of her husband and includes allegations that have never been tested.
2. Three entries in which she describes her mother-in-law supervising the children and says she "does not trust her".
3. A note recording that Mrs Eze told the therapist, in an individual session, that she had once left the children with a neighbour overnight while she travelled to another city for a funeral. The therapist recorded it as a fact reported, not investigated.
4. A letter the therapist wrote at Mrs Eze's request to the children's school, confirming that the family was attending therapy, with no clinical detail.
5. A supervision note recording the therapist's view that Mrs Eze was "fragile but improving".
6. Nothing about the husband, who was never a client.

The advice received. Produce items 1, 3, 4 and 5 in response to the order, with third-party identifiers redacted where they are not the subject of the proceedings (the neighbour's name, the mother-in-law's medical details, which appear in one sentence about her own health). Item 2 is producible, with the mother-in-law's health information redacted. Item 5's reference to "fragile" is producible and the therapist should expect to be asked about it. Nothing is withheld except the redactions, and the redactions are logged with reasons.

What the therapist did.

1. Told Mrs Eze by telephone the same day, in the words: "your husband's lawyer has asked the court to order me to produce my records. I have to comply. I am going to tell you exactly what is in the file and what will be produced, and I am going to take advice first." She then read the relevant entries to her, over two sessions.
2. Wrote to Mrs Eze confirming the conversation, the scope of the order, what would be produced, and the date.
3. Took advice from her indemnity provider and, through them, a lawyer, on the redactions and on whether to seek to limit the scope. The advice was to produce with redactions and not to contest the scope, because the material was plainly relevant and a contested application would delay the hearing and damage the therapist's position.
4. Produced an indexed bundle, paginated, with a covering letter stating what was produced, what was redacted, and the basis for each redaction.
5. Attended and gave evidence.
6. Recorded all six steps in the file, with dates and names.

The evidence, and the two hard moments.

The first. Counsel read out the note about the overnight stay with the neighbour. "You recorded that my client's wife left the children overnight with a neighbour, did you not?" The therapist answered: "I recorded what she told me. I did not investigate it and I made no finding." Counsel: "So you cannot say whether it is true." Therapist: "That is correct."

The second. Counsel read out the supervision note: "fragile but improving". "You considered her fragile." Therapist: "I wrote that in a supervision note eight months ago, in the context of her grief and her financial position. I also wrote 'improving', and the treatment continued for five months after that note. I can explain the context if the court wishes."

The outcome of the hearing, which is not the point of the case. Residence was ordered with the mother, with defined contact for the father. The therapist's evidence neither rescued nor destroyed the case, which is what evidence from a treating clinician usually does.

What the case teaches.

1. **The order was complied with, not resisted, and the compliance was careful.** The therapist's protection was not argument but accuracy: an indexed bundle, logged redactions, and a recorded process.
2. **The client was told before she was surprised.** The two sessions spent reading the file to her were the clinical work of the week, and probably prevented the treatment from ending.
3. **The notes she had written were the evidence.** The "fragile but improving" note was the only moment of real difficulty, and it was survivable because it was a supervision note with a date and a context, not a character judgement in the clinical record.
4. **The file contained material about people who were not parties,** which is the ordinary condition of family work and the reason the redaction log exists.
5. **Nothing was destroyed, altered or withheld without a reason on the record.** Had any of those three happened, the case would have been about the therapist rather than about the children.

11.7 Transcript: fifteen minutes of cross-examination

Condensed from the hearing, with annotation.

> **X1 Counsel:** You are not a psychiatrist, are you? **X2 Therapist:** No. I am a family therapist, registered with [body]. **X3 Counsel:** And you are not qualified to say whether my client's wife is a fit mother. **X4 Therapist:** I have not said that, in my notes or in this court. — Fourteen words. The answer corrects the premise without arguing with counsel, which is the difference between a witness and a combatant. **X5 Counsel:** You formed a view, though. **X6 Therapist:** I formed a clinical impression of a woman in grief, which is what I was treating. (pause) I recorded it and I have explained what it was based on. — The concession (yes, a view) followed by the limitation (of a particular thing), which is more credible than a denial. **X7 Counsel:** You never spoke to my client. **X8 Therapist:** I did not. He declined to attend. I wrote to him once, at his wife's request, offering an appointment, and I have the letter in my file. — The fact, plus the document, plus the correction of the implication that the therapist excluded him. **X9 Counsel:** Would it not have been better to insist? **X10 Therapist:** I cannot compel an adult to attend therapy. (pause) I can record that I invited him. — The answer to a "would it not have been better" question, which is counsel's favourite form: it invites the witness to criticise herself. The witness states what was possible. **X11 Counsel:** You recorded that she left the children with a neighbour overnight. That is a safeguarding matter, is it not? **X12 Therapist:** I recorded what she told me. I did not treat it as a safeguarding concern, because it was one night, with a known neighbour, and she told me herself without being asked. (pause) If I had thought a child was at risk I would have reported it, and I would have recorded that too. — The factual answer, the reasoning, and the statement of the duty — three elements in three sentences. **X13 Counsel:** With respect, that is your judgement. **X14 Therapist:** It is. And it is the judgement I am employed to make, and it is written down with the reasons. — The final turn of the transcript, and the one students should memorise: the witness owns the judgement, states its basis, and does not apologise for having made it.

What the transcript shows. The therapist's longest answer is forty words and most are under fifteen. She concedes what is true, corrects what is false, and never argues. The single most damaging thing that could have happened in that box was a therapist who sounded as though she were the client's advocate — and the discipline that prevented it was grammatical: she spoke about what she observed, in the past tense, and left the conclusions to the court.

11.8 Laboratory: the note lab and the subpoena drill

Part A — the note lab (thirty minutes). Students are given a ten-minute transcript of a family session containing: a risk disclosure, an individual aside, an argument between the parents, and a plan. Each writes the note in eight minutes. The notes are exchanged and marked against the "three readers" standard by a partner, who must answer three questions from the note alone: what is the risk, what was decided, and what would I do if the client telephoned tonight?

Criterion	0–1	2–3	4–5
Fact and register	Judgements mixed with facts	Mostly factual	Every statement in the correct register
Risk	Not recorded	Recorded as a statement	Questions asked, answers given, level and plan recorded

Criterion	0–1	2–3	4–5
The individual aside	In the main note	Omitted	In the restricted section, with the reason recorded
Plan and consent	Absent	General	Specific, dated, with who was told what and the consent
Handover	Unusable	Adequate	A colleague could take over tonight

Part B — the subpoena drill (thirty minutes). Each student receives one of four documents: a subpoena duces tecum, a letter from a solicitor requesting records "as a matter of urgency", a request from a school for "the records of the family", and a request from a client's employer. In twelve minutes they must write: the first five actions, the letter or response, and the file entry. The group then compares the four responses, which is the point of the exercise — the same instinct produces four different correct answers.

The four correct answers, in summary. Subpoena: verify, tell the client, take advice, produce what is required, log. Solicitor's letter: this is a request, not an order; consent or a court order is required; reply in writing. School: attendance only, with consent, unless a safeguarding duty requires more. Employer: attendance only, with consent, and a note explaining the limit to the employer.

Marks for Part B (10). Correct identification of the instrument; the client informed; advice taken; minimum necessary applied; the file entry complete.

11.8a The four-minute handover note

The note that matters most is the one written when a therapist goes on leave, becomes ill, or leaves a post. It is four minutes' work and almost never done.

The four lines per case.

1. **Who and why** — the client system, the presenting problem, and the agreed focus.
2. **The current risk** — the last assessment, the date, the plan, and who knows.
3. **The live issues** — the secret being held, the referral pending, the court date, the person who has not been told something.
4. **What to do first** — the call to make, the appointment to keep, the file to read.

What belongs at the top of the bundle. A single page listing every client with a risk flag, a court date, or an open protection matter, in date order. It is the page a covering colleague reads on the first evening.

Why it is an ethical duty and not an administrative one. Continuity of care is a client's interest. The therapist who leaves without a handover has transferred her caseload and abandoned its clinical responsibility, and the case in which this matters most is always the one with the highest risk — because that is the case with the most to record and the greatest reluctance to reduce it to four lines.

11.9 Six errors

Error 1 — The note that records a conclusion without its basis. "Client is depressed." The next clinician cannot tell whether this was an assessment, a diagnosis, or an impression, and neither can a court.

Error 2 — The individual disclosure in the joint file. It is the commonest documentary failure in couple work and it is created by an administrative habit rather than a decision.

Error 3 — Promising that the file can never be disclosed. It is false, it is said to reassure, and it converts a lawful disclosure into a betrayal.

Error 4 — Producing the entire file in response to a narrow order. It breaches duties owed to third parties and to the other partner, and it is usually done out of anxiety rather than necessity.

Error 5 — Altering or destroying records. It is a criminal offence in most jurisdictions, it is detectable, and it converts a defensible case into an indefensible one.

Error 6 — Advocacy in the witness box. The therapist who argues for the client's case has abandoned the only thing that made her evidence useful, which is that she was not a partisan.

11.9a Ten sentences that have been read back to a therapist in a hearing

Each of these was written in a file and each was quoted. They are collected because the pattern is instructive: none is a lie, and each is an assertion that the therapist could not support when asked.

1. "Client is manipulative." Question asked: "On what evidence?"
2. "Mother appears to prefer the son." Question: "Which sessions, and what did you observe?"
3. "The marriage is characterised by his control." Question: "You never met him, did you?"
4. "Client is not at risk." Question: "How could you know that?"
5. "She is a devoted mother." Question: "That is an opinion, is it not?"
6. "Father is in denial." Question: "Did you assess him?"
7. "Client has borderline traits." Question: "Are you qualified to make that diagnosis?"
8. "The family is enmeshed." Question: "What does that mean in observable terms?"
9. "Client was intoxicated at session." Question: "Was this tested, or observed, and by whom?"
10. "I am certain the disclosure is true." Question: "Certain on what basis?"

The rule that produces all ten errors. The sentence was written to capture a feeling, in a document that requires evidence. The remedy is not fewer opinions but labelled ones: "I observed X. My impression, which I have not tested, is Y."

11.10 Culture, faith and the record

"The family wants to see what you have written." In households where information is held collectively, the idea that a therapist keeps a private record is foreign and suspicious. The request may come from an elder, a husband, or a client herself who has been sent to fetch it. The answer is not a lecture on data protection. It is three sentences: what the record is (the therapist's working note), who may see it (the client, and a court if it orders), and what the therapist will do instead (summarise in the session what she has written, and let the client correct anything inaccurate).

The client's right to her own record. Where a client asks to see what is written about her, the request is usually legitimate and should not be treated as hostility. Reading the record with the client, in a session, is frequently therapeutic: it demonstrates that the therapist has understood, it corrects errors, and it makes the record collaborative rather than mysterious.

Privacy in a shared house. A record kept on a phone in a household where the husband checks the phone is not private. This is an argument for paper in a locked cabinet, for a practice system rather than a personal device, and for the practice policy that nothing clinical is stored on a personal phone. In the settings this book is written for, that is not a technicality; it is a safety matter.

Confession, records and religious authority. A religious leader who asks to see the therapist's notes about a member of the congregation is making a request that has no legal basis and considerable social force. The answer is the same as for any third party, delivered with respect: "I cannot discuss her with you. I can tell you that she is being seen, if she has agreed to that."

The family's own record. Many families keep their own documentation — letters, court papers, a written account of events. Where the therapist is shown it, the fact is recorded, because the file must show the source of information that reached the treatment from outside, and because the document may later be produced by the family and attributed to the therapy.

> **DEBATE**
> "A therapist's notes should be written on the assumption that the client will read them." Argue for: transparency, accuracy, the client's right of access, and the discipline it imposes. Argue against: the clinical record must contain hypotheses and observations not intended for the client, candour would be lost, and the therapeutic relationship would change. Then state your practice, and the note you would write differently if the debate changed your mind.

11.10a The retention, storage and destruction schedule

Three decisions that every practice must make in advance, because each of them is impossible to make well at the moment it arises.

Retention. How long is a record kept after the last contact? The answer is set by the professional body, by limitation periods for claims, and by the needs of former clients who

may return. In most jurisdictions the practical range for adult records is seven to ten years, and for children's records until some years after the child's majority. The rule should be written, applied consistently, and stated in the privacy notice the client receives.

Storage. Who can reach the record; what happens if the therapist is ill, dies, or leaves the country; and where the backup lives. A practice of one is a practice with a single point of failure, and the "professional will" — a named colleague who can access records, contact clients and close the practice — is part of the duty to clients rather than an act of foresight.

Destruction. How, by whom, and with what record. A file that is destroyed is recorded as destroyed, with the date and the method, and the destruction is subject to a check that no request or proceeding is pending. The practitioner who shreds a file during a complaint has committed the offence described in Error 5.

The client's information. What the client is told at the start: what will be kept, for how long, who may see it, what happens on the therapist's death or retirement, and how the client can obtain a copy. Three sentences in the consent conversation, and a paragraph in the written information.

11.11 Evidence note

What is known. Studies of clinical records consistently find high rates of missing information on risk, consent and reasoning, and that record quality is improved by structured templates and by audit with feedback. Research on client access to records finds that reading them is usually experienced as neutral or positive and rarely harmful. In the forensic literature, the quality of the contemporaneous record is one of the strongest determinants of how a clinician's conduct is judged in a complaint or a hearing.

What is weaker. There is little research on the specific formats used in family therapy, and none on the restricted-section practice recommended here, which is derived from the structure of multiperson confidentiality rather than from outcome evidence.

What is not supported. That detailed notes protect a therapist regardless of content — a detailed note containing judgements is more dangerous than a sparse factual one. That the client's access to records is generally harmful. And that destroying or amending records ever improves a position.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same sentence, written in a supervision note and written in the clinical record, has two different lives in a courtroom.

11.12 Key terms

Contemporaneous note — a record made at or near the time of the event; its evidential weight is much greater.

Restricted section — a part of a couple or family file holding material that must not be disclosed to another client in the same case.

Privilege — the client's legal protection against disclosure in proceedings; qualified and waivable.

Subpoena duces tecum — an order to produce documents; distinct from a request by a party.

Redaction — the removal of specified information from a disclosed document, recorded with reasons.

Witness of fact — a witness reporting what she observed; contrasted with an expert witness offering opinion.

Minimum necessary — the principle that disclosure is limited to what the purpose requires.

11.12a The two-minute note review

A habit rather than a technique, and the cheapest quality improvement available: at the end of each working day, the therapist re-reads the notes she wrote and asks four questions of each.

1. Could a colleague act on this tonight?
2. Have I written a judgement where I should have written an observation?
3. Have I recorded the risk questions I asked, or only the answers?
4. Is there anything here I would not want read aloud?

Ten notes take two minutes and question 4 is the one that changes practice. A therapist who asks it daily stops writing the sentences that get read back in a hearing (section 11.9a) within about three weeks.

11.13 Self-test

1. Name the three readers of a clinical note and what each needs.
2. Give the four registers, with an example of each.
3. State the six headings of a family session note.
4. What is the restricted section, and what three clauses describe it?
5. Give the five checks to perform in the first hour after a subpoena arrives.
6. Distinguish confidentiality from privilege.
7. Name the four situations in which privilege will not protect the file.
8. Give the four rules for giving evidence, and explain why "I do not know" is a strong answer.

Answers. (1) The therapist in six months — problem, attempts, outcome, plan; a colleague taking over — risk, plan, who knows, what to do if telephoned; a court — facts with dates, decisions with reasons, consent, no speculation. (2) Fact: "client attended on 4 March"; observation: "she checked her phone four times"; opinion: "consistent with an acute stress response"; hypothesis: "possible that the husband's drinking is a factor, not yet explored". (3) Attendees; presenting this session; risk screen; interventions; response; plan. (4) A

clearly marked part of a couple or family file holding individual-session material that must not be shared with the other client; described by its reference to practice policy, the client's knowledge that it will not be shared, and the statement that nothing affecting safety is withheld from the main file. (5) Validity and addressee; exact scope; whether the client has been told; who is advising; the deadline and whether there is time to object. (6) Confidentiality is the therapist's professional duty; privilege is the client's legal protection, waivable by the client and overridable by statute or court order. (7) Client waiver, frequently by putting mental state in issue; a statutory exception such as child protection; a court order; and proceedings in which the therapist is defending herself. (8) Answer the question asked; do not guess; distinguish fact from opinion; do not defend the treatment. "I do not know" is strong because it is credible, it prevents the improvisation that destroys witnesses, and no court expects a clinician to know everything.

11.14 Reflective task

Write one page on the following before you read further: which of the four registers do you default to when you are tired, and what does that default produce in a record that might one day be read aloud? Most therapists default to opinion, because opinion is what the mind produces when it is summarising quickly, and opinion is the least defensible thing in a file.

Take the last note you wrote in a placement or practice and read it as three people: as the therapist in six months, as a colleague on call tonight, and as counsel cross-examining you. Write a paragraph on what each of them would make of it. Then rewrite one sentence.

11.15 Further reading

- Any current text on record keeping and the legal process in health and social care in your jurisdiction.
- The Nigeria Data Protection Act 2023, and your professional body's guidance on records and access.
- Gutheil, T. on the psychiatrist in court — despite the discipline, the clearest treatment of testifying as a clinician.
- Your indemnity provider's guidance on subpoenas and on responding to requests for records — usually free and usually unread.
- Court rules on the production of documents in family proceedings where you practise.

Next: Week 12 — Money, gifts and the economics of the relationship.

Week 12 — Money, gifts and the business of therapy

Learning outcomes. By the end of this week the student can (1) explain why the fee is clinical material and not an administrative detail; (2) conduct the fee conversation at intake, including the sliding scale and the cancellation policy, in five minutes; (3) manage non-payment clinically rather than avoiding it; (4) apply a decision procedure to any gift and record the outcome; and (5) identify the business pressures that produce unethical practice and state the limits that protect the client.

The subject nobody teaches and everybody practises

Money is the least discussed and most consistently present element of the therapeutic relationship. It is present at the first contact, in every session, in the waiting room, in the client's calculation of whether she can afford to continue, and in the therapist's calculation of whether she can afford to keep her. It carries meaning about worth, entitlement, dependency, shame, power and care — and because therapists are frequently uncomfortable with money, the meaning is usually not discussed at all.

The three-part claim of this week. First, the fee is a clinical instrument: it defines the frame, it makes the relationship professional, and it is one of the ways in which the client's difficulties with entitlement, need and worth appear in the room. Second, the therapist's own relationship with money is a source of error, and it is rarely examined. Third, a practice that is not financially viable will exploit its clients in ways that are rarely deliberate and always structural.

Situation	Clinical meaning, usually	Ethical requirement
A client who always pays late	Helplessness, resentment, a test of whether the therapist will chase	A stated policy, applied consistently, discussed as material
A client who pays in advance for ten sessions	Fear of abandonment; an attempt to secure the therapist	Discuss what the payment means before accepting it
A client who asks for a reduction	Shame; a belief that she is not worth the full fee	A written policy, a decision made once, recorded
A therapist who never chases a debt	The therapist's own discomfort with conflict or with greed	Supervision; a policy applied by the practice rather than the person
A gift after a good outcome	Gratitude; a wish to be remembered; obligation	A decision procedure, a conversation, a record
A gift after a disclosure of shame	An attempt to repair the therapist's view of her; a payment	Often to be refused, with the meaning named

12.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	Money as clinical material: three vignettes, and the therapist's discomfort	Anonymous written answers to "what did your family teach you about money?"
10–22	Five concepts: frame, transference, the therapist's money, sliding scale, viability	Lecture

Minutes	Content	Teaching form
22–38	The intake fee conversation, scripted and rehearsed	Demonstration, then triads
38–52	Non-payment: the sequence, the limit, the ending	Case, with students writing the sentence for month two
52–68	Gifts: the four categories and the decision procedure	Card sort of eight gifts; then the refusal script
68–80	The business pressures: discounts, packages, marketing, billing	Lecture with four examples, students identify the risk
80–90	The Nwankwo case	Case, students decide what is accepted, what is charged and what is said

12.2 Five concepts that organise the week

Concept 1 — The fee is part of the frame, and the frame is the treatment. A therapy has an agreed time, place, frequency, duration and payment. The payment is not the least of these: it is what makes the relationship a professional one rather than a friendship, and it is what allows the client to feel entitled to the therapist's attention. A client who is not paying, or who is paying unpredictably, is frequently a client who does not feel entitled to ask for anything.

Concept 2 — Money carries the client's history, and the therapist should expect it. The client who cannot ask for a reduction and stops attending; the client who insists on paying for the whole course in advance; the client who gives an expensive gift and then asks for a favour; the client who treats the fee as a debt of gratitude. Each is a repetition of something, and each becomes treatable when it is named.

Concept 3 — The therapist's own money is the largest unexamined variable in the room. Therapists under-charge because they are anxious about being seen as commercial, or because they do not believe their work is worth the fee, or because a client's poverty activates guilt. They over-charge, or cling to clients they should refer, when the practice is not viable. Neither is a moral failing; both are material for supervision, and neither is correctable by willpower.

Concept 4 — The sliding scale is an instrument of justice and a machine for resentment. A scale that is written, published internally, applied by rule and reviewed annually serves clients and protects the therapist. A scale that is negotiated case by case, in the room, according to how sorry the therapist feels, produces inconsistency, the client's discovery of another client's lower fee, and a therapist who is quietly furious.

Concept 5 — Viability is an ethical matter. A practice that cannot pay its rent will raise its fees abruptly, take clients it should refer, and cut the supervision that protects clients. The ethical practice is a sustainable one: priced to survive, with the pro bono work built into the budget rather than subtracted from it at the end of a difficult month.

12.2a The intake fee conversation, in five minutes

Six statements, made at the first contact, in the order below, and repeated in writing.

1. **The fee, plainly.** "My fee is X per session, and sessions are fifty minutes." No apology, no explanation of overheads, no "I know it is expensive".
2. **What the fee includes, and what it does not.** The session; correspondence with other professionals where consented; a written report where one is agreed. Not telephone calls between sessions, which are handled by agreement.
3. **When and how it is paid.** "Payment is due at the end of each session, by transfer or in cash. I do not carry change." The mechanics are stated because ambiguity about mechanics is where resentment begins.
4. **Cancellations.** "I require twenty-four hours' notice. If you cancel with less, or do not attend, the session is charged, because the time was held for you." Then the sentence that makes it acceptable: "if you are ill or there is an emergency, tell me and I will not charge you."
5. **The sliding scale, if there is one.** "I have a small number of reduced-fee places. If you need one, tell me now or at any point, and I will tell you honestly whether one is available." The phrase "tell me honestly whether one is available" is important: it makes the scale a resource rather than a favour.
6. **What happens if payment becomes difficult.** "If you ever cannot pay, tell me early. We will find a way forward, and I would much rather know than discover it." Said at the start, this sentence makes the later conversation possible.

A worked fee dialogue.

> **Client:** How much do you charge? **Therapist:** X per session. Sessions are fifty minutes, and payment is at the end of each session. **Client:** (pause) That is — I am a teacher. **Therapist:** Then let us talk about it now rather than later. I have two reduced places at the moment; one is taken. (pause) What would be manageable for you? **Client:** I do not know. I do not want to be a charity case. **Therapist:** You are not. The reduced places exist because I would rather work with a full practice than an expensive empty one. (pause) If we agree a figure, we will review it in six months, and if your situation changes we will change it then.

Why the last sentence matters. A reduced fee without a review becomes permanent by default, and both parties know it. A stated review makes the arrangement a professional one and gives the client a way to increase the fee without having to raise it.

The three questions the therapist should be able to answer without flinching. What is my fee? Why is it that? And what will I do if this client cannot pay it? A therapist who cannot answer the second has not priced her work; she has inherited a number.

12.2b Non-payment: the sequence and the limit

The clinical problem. Non-payment is rarely about money alone. It is frequently about shame (the client who cannot say she cannot pay), about entitlement (the client who believes the therapist owes her), about a test (will you chase me, and does my worth depend on your not asking?), or about a genuine catastrophe. The therapist who chases aggressively and the therapist who never raises it both fail; the first humiliates, the second colludes and then resents.

The sequence, session by session.

At the first missed payment: ask, in the session, without accusation. "I noticed the last two sessions were not paid. Is something happening?" The question is about the client, not the debt.

At the second: name the pattern and the feeling. "This is the second month. I want to say what I notice, which is that you have not mentioned it, and I wonder whether it is difficult to talk about." Then the agreement: a date, an amount, and whether the sessions continue meanwhile.

At the third: the limit. "I am going to be honest with you. I cannot continue to hold sessions that are not paid, and I am not willing to end the therapy because of money without us having tried to solve it. Here are the options: a reduced fee for a period, a payment plan, a pause, or a referral to a service that is free."

At the fourth: the decision, made and recorded. Continuing indefinitely is not generosity; it is a slow resentment that the client will feel, and it ends with the therapist withdrawing at the worst possible moment.

The sentences that work, and the ones that do not.

Say	Not
"I need to raise something about the account."	"You have not paid."
"I would rather we talked about it than that I sent a letter."	"This is a business, you know."
"What would be manageable?"	"I have been very patient."
"If we cannot agree, I will refer you and we will end properly."	"I cannot help people who do not pay."

The clinical gain in the conversation. A client who has never been able to negotiate a debt with anyone — and this is common in clients whose financial lives are chaotic or controlled — has the experience of a relationship in which a difficulty is named, negotiated and survived. That is often worth more than the fee.

Ending for non-payment, done properly. A closing session, a written summary, a referral, an offer to return if circumstances change, and a record of the reasoning. Not a letter from an agency, and not silence.

12.2c The therapist who is owed money, and cannot say so

The countertransference of the unpaid fee is one of the most reliable in practice and one of the least discussed.

The four patterns.

1. **The rescuer.** The therapist decides that the client's poverty is part of the problem and that charging is therefore cruel. Sessions continue, the fee is never discussed, and the therapist begins to feel quietly exploited. The client, sensing it, apologises for existing.

2. **The withdrawing therapist.** The therapist begins to offer shorter sessions, fewer appointments, less attention. The client experiences a loss of interest and cannot say why.
3. **The punitive therapist.** The debt becomes a moral matter. The therapist's tone becomes cool, and the client — who has usually been punished before — recognises the tone before she can name it.
4. **The avoider.** The therapist genuinely does not think about the debt until the accountant raises it, and then sends a letter written in a completely different voice from the one used in the room.

The examination. Before the conversation, three questions in writing: what do I feel about being owed this money? What do I imagine the client feels? And what am I doing differently in the sessions since the debt began? The third question is usually answered with a specific change — the therapist has stopped challenging, or has started ending on time with unusual firmness — and that change is the clinical material.

The rule. The therapist's discomfort must not be paid for by the client, in either direction: neither by waiving the fee so that the therapist does not have to have the conversation, nor by extracting the money in a way that punishes her. The conversation is the only way through, and it is the therapist's to initiate.

12.3 Gifts: four categories and one procedure

Category 1 — the cultural token. Food, cloth, a small item given at a festival or a home visit. Value is modest, the meaning is relational, and refusing it can be experienced as rejection. Default: accept, name the meaning, record it, and do not treat it as a transaction.

Category 2 — the transitional gift. Given at the end of treatment, at a birth, at a bereavement, or after a significant improvement. Default: usually accept if modest; discuss what it means; record it. The discussion frequently produces the most important session of the final month.

Category 3 — the expensive gift. Value beyond a token; jewellery, electronics, a large sum, land. Default: decline, warmly, with an explanation that protects the giver's dignity: "I am very moved that you would do this, and I am not able to accept it. It would change something between us that I need to keep as it is."

Category 4 — the gift with strings. Any gift accompanied by a request, an expectation, or a narrative about what the therapist now owes. Default: decline, and name the string, because a gift that creates an obligation has already begun to distort the relationship.

The procedure, in five steps.

1. **Notice.** Register the gift as a clinical event rather than an administrative one, and do not decide in the

doorway.

1. **Ask what it means.** "What would it mean to you if I accepted this?" and, where relevant, "and what would it mean if I did not?" The answers are almost always more informative than the object.
2. **Decide against the category,** not against the person's need. The therapist who accepts because the client will be hurt has made the client's reaction the criterion.
3. **Say the decision, once, warmly, and without a lecture.** The lecture ("you know, ethically, I cannot") turns a gift into an embarrassment.
4. **Record it.** One line: what was given, what was decided, what was discussed, and what happened to the object.

The gift that must be refused, whatever its value. A gift from a client who is in a coercive relationship and who is spending money her partner controls, because accepting it colludes in the control. A gift from a child client that has clearly been taken from a parent's belongings. A gift that is the client's only asset. And a gift given immediately after a disclosure of shame, because the therapist is being paid to stop seeing something.

The gift that is a communication. In many of these cases the gift says something the client cannot: "I am not only a problem", "you have been good to my mother", "I want you to remember me". Naming the communication — "I think this says something about how you want me to see you" — frequently makes the object unimportant, and the client may then be content for the therapist to keep it or return it.

What to do with what is accepted. Food is shared or eaten; cloth may be worn or kept; money is never accepted. Where the practice has a policy that accepted gifts become the property of the clinician, the policy is stated in writing at intake, which prevents the later argument about whether the therapist "kept" something.

12.3a Barter, pro bono, and the client who cannot pay at all

Barter. The exchange of goods or services for treatment. Permissible in narrow conditions: the client initiates; the value is agreed in writing; the arrangement is time-limited; it is supervised; and the therapist records the rationale. Prohibited in practice where the client's service is intimate, where the value is uncertain, or where the exchange would place the therapist in a position of judging the client's work.

The specific hazard of barter. The client's service becomes clinical material. The therapist who accepts tailoring from a client will have views about the tailoring, and the client will know it. What was a fee becomes a performance, and the therapy acquires a second arena in which the client can fail.

Pro bono. Treatment given without fee. It is a professional obligation in most codes, in some form, and it is best organised as a proportion of the practice — one or two places held open — rather than as an occasional decision made under pressure in the room. The client on a pro bono place is told that it is a place, not a favour, and that the arrangement is reviewed annually. That framing protects both parties from the gratitude that otherwise contaminates the work.

The client who cannot pay at all. Three options, in order of preference. A reduced or waived fee for a defined period, with a review. A referral to a free service, with a warm

handover. A short piece of work — six sessions — focused on a single question, rather than an open-ended therapy that will collapse in month three.

The option that must be avoided. Continuing indefinitely without agreement, because the therapist cannot bear to raise it. The client will sense the unspoken arrangement, will begin to apologise for existing, and will leave without a closing. The ending that results is worse for her than the one the therapist was avoiding.

The practitioner's own arithmetic. A practice with four reduced places, each at half fee, has given away two full fees a week. That is a real contribution and it must be in the budget. The practitioner who gives away places ad hoc and then raises everyone's fee to compensate has made the poorest clients pay for the therapist's generosity to other clients.

12.3b The gift from the family, not the client

A distinct problem, and common in family work: the gift comes from a member of the family who is not the identified client — the grandmother, the uncle who pays, the child.

Why it is different. The giver may not be in treatment; the meaning may be about the family's hierarchy rather than about gratitude to the therapist; and accepting from one member can create a coalition that the other members can see.

The procedure. Treat it as the gift from a client, and add two steps. Establish who decided that the gift would be given, because a gift offered by a father on behalf of a family in which the mother is silent is a statement about who speaks. And where the giver is not a client, decline more readily: there is no therapeutic relationship to preserve, and the gift is frequently an attempt to establish one.

The child's gift. A drawing, a card, a stone from the school playground: accept it, thank the child, and where the child is in a family treatment, keep it visible for a session. A child who gives a gift is testing whether the therapist is a person who can be given to, and the answer matters clinically.

12.4 The business: eight pressures that produce unethical practice

1. The package discount. "Ten sessions for the price of eight." It is a commercial instrument in a clinical relationship, it creates pressure on the client to continue when she would have stopped, and it creates pressure on the therapist to keep her coming. Where a package is offered it must be refundable, optional, and never presented as a saving against distress.

2. The productivity target. An employer who requires a fixed number of clinical hours per week, or a fixed income from each clinician, creates an incentive to keep clients in treatment, to shorten sessions, or to accept clients outside competence. The individual therapist cannot change the system; she can record the pressure and refuse the three specific practices that harm clients — retention for revenue, treatment without supervision, and records written to satisfy a billing code.

3. Upcoding and creative billing. Describing a session in terms that maximise reimbursement, or recording a diagnosis that the client does not meet in order to satisfy

an insurer. This is fraud, it is a disciplinary matter, and it also corrupts the clinical record — the diagnosis written for the payer becomes the diagnosis that shapes the treatment.

4. The referral arrangement. A payment, a commission or a reciprocal arrangement for referrals. In most jurisdictions it is prohibited, and in all of them it corrupts the one decision the client depends on: that the referral was made in her interest.

5. Testimonials and advertising. Client testimonials are prohibited or restricted in most codes, because consent to treatment is not consent to be advertised and because the testimonial is a disclosure. Claims of effectiveness that cannot be supported — "guaranteed results", "the only therapist who..." — are misleading conduct, regardless of the therapist's sincerity.

6. The product sold to clients. Books, courses, supplements, retreats. There is nothing inherently wrong with a therapist writing a book; there is a serious problem with recommending to a current client a product from which the therapist profits, without disclosure and without an alternative.

7. The consultancy overlap. The therapist who is engaged by an employer to advise about an employee who is also her client. The two roles have different clients and different duties; the conflict is structural and should be declined rather than managed.

8. The unpaid work that is expected. Reports for courts, letters for schools, telephone calls with social workers: frequently unbilled and frequently essential. A practice that does not price this work either does it resentfully or refuses it at the moment the client most needs it. The remedy is a stated fee for reports and correspondence, discussed at intake (12.2a), and applied without embarrassment.

12.4a The practice that survives without exploiting anyone

Five arrangements, each of which is a decision made in advance rather than in the moment.

Price for survival. The fee is set to cover the practice's costs, the therapist's income, supervision, insurance, continuing development and a margin for the unpaid work. A fee set to be "affordable" by an anxious therapist who has not done the arithmetic becomes a fee that rises unpredictably in year three.

Build the pro bono into the plan. One or two places, budgeted, reviewed annually, publicly described in the practice's information as part of how it works.

Charge for the work that is work. Reports, court attendance, extended telephone calls and meetings with other professionals have stated fees. The client is told in advance; the alternative is a therapist who resents the letter she is writing at ten at night.

Hold a no-show policy that is fair and enforced. Twenty-four hours, with a stated exception for emergency and illness, applied consistently, and discussed clinically when it recurs.

Keep supervision in the budget. It is the last item most practices cut and the first that protects clients. A practice that cannot afford supervision cannot afford to practise.

The annual review. Once a year: what is the fee, what does it cover, what is the pro bono allocation, what is the debt, and what did I do last year that I would not defend to a colleague? The review is the practice's equivalent of the record of consent — the evidence that the arrangements were chosen.

12.4b What the client is told, in writing

The money policy belongs in the written information the client receives at intake, in one page and in plain language. Six paragraphs.

The fee. The amount, the session length, and what it covers. **Payment.** When it is due, and how it is made. **Cancellations.** The notice required, the charge, and the exceptions. **The sliding scale.** That places exist, that they are limited, how they are allocated, and that they are reviewed. **Difficulties.** That the client should tell the therapist early if payment becomes difficult, and that the therapist would rather know than discover it. **Records and confidentiality.** A cross-reference to the consent document, and the statement that the record may be seen by the client and may be compelled by a court.

Why the written page matters more than the spoken conversation. The client under stress remembers about one-third of what is said in a first session. The page is read at home, sometimes twice, and frequently by a partner — which is itself a disclosure the therapist should have anticipated and stated: "you may show this page to whoever you wish; it contains nothing about your treatment."

12.5 Case: the Nwankwos, the fee, the goat and the plot

The family. Chuka Nwankwo, 44, a civil engineer; Ifeoma Nwankwo, 40, a trader in fabric; children 14 and 9. They were referred by their physician after the death of Ifeoma's mother, with whom the household had lived.

The first four months. Weekly couple sessions, then fortnightly. The presenting problem was the household's grief and the conflict between Ifeoma and her husband's sister, who had moved in "to help" and stayed. The work went well by every measure that matters: the conflict reduced, the children were brought for three sessions, and the grief was named.

Month five: the job. Chuka was told his contract would not be renewed. The family's income halved. In session eighteen he said, at the end, "we may need to reduce the sessions". The therapist's response was the sentence she had practised at intake: "thank you for telling me now rather than in three months. Let us talk about what is manageable."

The agreement. Sessions fortnightly rather than weekly; the fee reduced by forty per cent for six months; a review in month eleven. Recorded in the note and in a short letter to the family. The therapist's own note recorded the reason and the review date, which is the difference between a sliding scale and a favour.

Month seven: the goat. Ifeoma arrived with a live goat, tied, on the back of a neighbour's pickup, "for the therapist's family, because of what you did for my mother before she died". The therapist's first instinct was refusal — a live animal in a city clinic — and her second was to accept immediately so as not to shame her.

What she did. She neither accepted nor refused in the doorway. She said: "this is the most generous thing anyone has brought me, and I want to talk about it properly. Can we sit down for five minutes?" Then she asked what the gift meant. Ifeoma's answer: "in my place, when somebody does something for your mother, you do not say thank you with words."

The therapist then made a decision on the category rather than on the feeling: a live animal is not a token and its value is not trivial, and it also cannot be returned without humiliation. She proposed a third option: "I will accept it, and I will not take it home. I will ask you to sell it and to put the money towards the remaining fee — which means you have paid me, in the way your mother would have understood."

Ifeoma refused, then accepted, then cried, and the transaction became the subject of the best session of the treatment: a family that had been unable to receive help without paying for it, and a therapist who had found a way to let them pay.

Month eleven: the plot. Chuka's contract was renewed. He attended alone and said that a relation had given the family a plot of land on the outskirts of the city, and that they wanted to transfer it to the therapist "as a thanks, because the marriage would have ended".

The decision. Declined, immediately and unambiguously, with the explanation that protects: "I cannot accept this. It is worth more than the whole of the treatment, and if I took it, you would always know that I had, and I would always know it too. (pause) What I would like, if you want to mark this, is a letter. Not for the file, for me."

The letter. Chuka wrote two pages. It is not in the file, it is not evidence, and it was, the therapist later said, the only gift in twenty years of practice that she had wanted.

What the case teaches.

1. **The intake sentence made month five possible.** "If you ever cannot pay, tell me early" was said at the start; without it, Chuka would have cancelled the sessions without explanation and the treatment would have ended silently.
2. **The decision was made on the category, not on the feeling.** The goat was a live animal of nontrivial value; the plot was an asset. The therapist's discomfort and the family's generosity were both real and neither was the criterion.
3. **The third option is usually available.** Accepting and refusing were not the only choices; "sell it and put the money towards the fee" preserved the meaning, protected the frame and gave the family a way to pay that their own culture recognised.
4. **The conversation about the gift was the therapy.** Both gifts arrived at moments when the family was working out whether help could be received without debt — which was, in retrospect, the family's central problem and the reason the sister-in-law had stayed for two years.
5. **The refusal of the plot was the more generous act.** The therapist who accepted it would have been, from that day, a person to whom the family owed everything, and the therapy would have ended in a way nobody could describe.

12.6 Transcript: the fee conversation at month two of non-payment

A different case, from the therapist's own practice log. Mrs Bello, 33, has attended eight sessions and has paid for three. Minutes twelve to twenty-four.

> **F1 Therapist:** I want to raise something and I have been putting it off, which I am telling you because it is true. — The confession of reluctance, which disarms the accusation before it lands. **F2 Client:** The money. **F3 Therapist:** The money. (pause) Three sessions have been paid for and eight have happened. — The facts, without an adjective. "Eight have happened" rather than "you owe me for five" — the same information, without the debt-collector's grammar. **F4 Client:** I am sorry. I am so sorry. I will bring it next week. **F5 Therapist:** (pause) You have said that twice. (pause) I am not asking for an apology, and I am not going to pretend I have not noticed. — The pattern named, gently, and the apology declined, because the apology is the thing that prevents the conversation. **F6 Client:** (long pause) My husband controls the account. **F7 Therapist:** (pause) Tell me about that. — Three words. The fee conversation has just become the clinical material, which is what the concept in 12.2 predicted and what the therapist was waiting for. **F8 Client:** He gives me money for the house. If I ask for more he wants to know what for. He does not know I am coming here. **F9 Therapist:** So every session you have attended has been paid for out of — what? **F10 Client:** Food money. I take a little. **F11 Therapist:** (pause) So you have been buying your therapy with your children's food money, and you have not been able to tell me. — The reflection that names the cost without moralising. The client's face, recorded in the note, changed. **F12 Client:** (crying) I did not want you to think I was like that. **F13 Therapist:** Like what? **F14 Client:** Somebody who does not pay. **F15 Therapist:** (pause) We are going to change this today, and it is not going to be you paying more. (pause) First, I want to know whether coming here is safe for you. — The pivot that matters: the fee has revealed a safety question, and the question takes priority over the account.

What the transcript shows. The therapist raised the money, named the pattern, declined the apology, and then followed the material wherever it went. Within eleven minutes of the fee conversation, the case had become a case about financial control — which had been in the room for eight sessions and had not been visible because nobody had mentioned the money.

The outcome. The fee was waived for three months while a safety plan was made; the client's attendance was recorded as "no charge, agreed for safety reasons, reviewed monthly"; and the financial control became the central subject of the treatment. The therapist also changed her intake: the question "who knows that you are coming here?" now appears in the first session of every case.

12.6a The five sentences of the money conversation

Students ask for a script, and a script of five sentences is more useful than a policy document.

1. **"I want to raise something about money, and I have been putting it off."** Every difficult conversation in this chapter begins with the therapist admitting the reluctance, which lowers the client's fear of being judged.

2. **"The facts are these."** The number of sessions, the number paid, the amount outstanding. No adjectives.
3. **"What is happening?"** A question about the client's situation rather than about her conduct, which is where the clinical material lives.
4. **"Here is what I can do, and here is what I cannot."** The options, stated as the therapist's capacity rather than as the client's failure.
5. **"Whatever we agree, we will write it down and we will look at it again on [date]."** The date that converts an arrangement into a plan.

What the five sentences do not include. An apology for asking, an explanation of the therapist's overheads, an account of how hard the therapist works, or a reassurance that the client is not the first person to do this. Each of those makes the exchange about the therapist.

The rehearsal. Practise the five sentences aloud before the session in which they will be needed. Therapists who have not said them aloud once will soften them in the room, and softening is what produces the ad hoc arrangement that becomes a resentment.

12.7 Laboratory: the fee policy and the gift decision

Part A — write the practice's money policy (thirty minutes). One page, six headings: fee and what it covers; payment mechanics; cancellation; sliding scale and how a place is allocated; pro bono allocation; debt and its limits. Then the group exchanges policies and each student applies another practice's policy to three cases. The exercise reliably produces the discovery that most policies cannot be applied by anyone other than their author, which is the definition of a policy that is really a preference.

Part B — the gift sort (fifteen minutes). Eight gifts, sorted into the four categories of 12.3, with a decision and a sentence for each.

Gift	Category	Likely decision
A bowl of fruit at the end of treatment	Token/transitional	Accept, record
A wrapped watch at session four	Expensive, early	Decline
A live chicken, delivered to the clinic	Token with practical problems	Discuss, propose a third option
A child's drawing	Transitional	Accept, display or keep
Money in an envelope after a good session	Never acceptable	Decline, discuss the meaning
A client's business stock as payment	Barter	Discuss, usually decline
A gift from a client in a controlling marriage	Category four — the string is the control	Decline, and assess safety
The offer to renovate the clinic	Expensive, institutional	Decline, or refer to the practice's owner with disclosure

Part C — the refusal script (ten minutes). Each student writes three sentences for declining a watch, and reads them aloud. The group marks them on two criteria: dignity preserved, and no lecture.

Marks (20 total). Policy: applicable by a stranger (5), covers all six headings (5), states limits (5). Gift decisions: correct category for each and a defensible decision with a sentence (5).

The observation to make in the debrief. The best refusal scripts in the room will be the shortest, and every student will initially write one that explains the ethics of gifts to the client.

12.8 Six errors

Error 1 — Never mentioning the fee again after intake. The client who is struggling has to raise it herself, which the client least able to raise it will not do. The therapist must raise it, and the permission to do so was given at intake.

Error 2 — The ad hoc reduction. A lower fee granted in the room, to a client who asked persuasively, with no review and no record. It is discovered, it is resented, and it is unfair to every client who did not ask.

Error 3 — Chasing a debt by letter. The letter is appropriate at the end of a process, not at the beginning. Sent early, it converts a clinical relationship into a commercial one, and the client who was ashamed becomes frightened.

Error 4 — Accepting an expensive gift to avoid embarrassment. The therapist's discomfort is being paid for with a distortion of the relationship, and the client now holds something over her.

Error 5 — Billing a diagnosis the client does not meet. Fraud, and a corruption of the record that outlives the claim.

Error 6 — Subsidising the practice with the therapist's own unpaid labour, silently. It works for a year, then produces resentment, a sudden fee increase, or a withdrawal from the work that looks to the client like abandonment.

12.9 Culture, faith and the meaning of money

The gift that cannot be refused. In many of the households this book serves, a gift is the language in which a relationship is acknowledged, and its refusal is a statement that the relationship is not accepted. A therapist who declines everything on principle will be described as "proud" and will be told, by the family, that she has "refused their hand". The workable practice — clear in the therapist's own mind and sometimes stated aloud — is that tokens are accepted and assets are not, and that the distinction is explained in the family's terms: "a bowl of fruit is between friends; a plot of land would make me something else."

Payment in kind. Where cash is scarce and goods are not — fabric, food, a service, labour — the family may offer payment in kind without embarrassment. The rules of 12.3a apply, and the cultural reality argues for a narrower rather than a wider permission: the

value is usually uncertain, the arrangement is usually permanent once established, and the therapist's judgement about the client's work contaminates the treatment.

The man who pays. In many households the husband pays for his wife's therapy and will want to know what it is for, which raises the Week 3 problem in a financial form. The practitioner's policy: the fee is agreed with the person who pays, the content is not discussed with anyone, and the arrangement is stated at intake — "your husband will be told that you are attending and what it costs, and nothing else, unless you ask me to tell him more."

The client who cannot pay and the therapist who is a member of the same community. The therapist's own obligations — to a church, a mosque, a family, an association — may include expectations of free help, and refusing them can be costly. The professional answer is a stated policy, applied to everyone: "I have two reduced places and both are taken; I can refer you, or you can wait for a place to come free." Said as a policy rather than as a judgement about the person, it is usually accepted.

Wealth, class and the fee. A therapist from a different class position from her client will misjudge what is affordable in both directions. The correct method is to ask, explicitly, and to believe the answer: "what would be manageable for you?" A client who says two thousand naira means two thousand naira, not a figure that the therapist thinks is realistic.

The religious objection to charging. Some clients, and some religious traditions, regard payment for counsel as improper — "you cannot sell the word of God". The therapist's answer is not a lecture on professional economics but an explanation of what the fee pays for, in one sentence: "the fee pays for the hour you have, the training behind it, and the fact that I can be here next week."

> **DEBATE**
> "Therapists should charge what the market will bear, and give pro bono time as they choose." Argue for: viability is an ethical precondition, the market allocates scarce skill, and discretionary generosity is genuine generosity. Argue against: the market allocates to those who can pay, discretionary charity produces inconsistency and resentment, and a profession that prices by the market has abandoned the poorer half of its obligation. Then set out a fee policy you could defend to both a client who cannot pay and a colleague who thinks you are under-charging.

12.10 Evidence note

What is known. Research on psychotherapy fees is limited but consistent on two points: fee arrangements affect the therapeutic relationship, and therapists' discomfort with discussing fees leads to avoidance which clients experience as distance. Studies of gift-giving in therapy find that gifts are common, that clinicians report uncertainty about them, and that gifts are frequently meaningful communications about the relationship rather than attempts to influence. In medical and psychotherapy ethics, financial conflicts of interest are among the most consistently documented sources of harm, largely through subtle changes in clinical decisions rather than overt fraud.

What is weaker. There is very little research on sliding scales, on barter, or on the outcomes of non-payment policies. The guidance in this week is derived from ethics and from practice rather than from controlled study.

What is not supported. That charging a higher fee produces better outcomes. That accepting a gift always damages the relationship — modest, culturally appropriate gifts are usually experienced as affirming. And that a therapist's financial interest does not affect her clinical judgement; the research on conflicts of interest suggests it reliably does, while the clinician reliably believes it does not.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same reduced fee, agreed at intake and agreed in month nine after three unpaid sessions, is a different arrangement with a different meaning.

12.10a The audit of your own practice

Once a year, five questions, written down, kept with the supervision record.

1. **What is my fee, and what does it pay for?** If the answer cannot be given in one sentence, the fee is inherited rather than set.
2. **How many reduced places do I hold, and how were they allocated?** If the answer is "whoever asked", the scale is a favour.
3. **What did I accept last year, and what did I refuse, and would I defend each decision to a colleague?** The question is not whether the decisions were right but whether they were decisions.
4. **How much did I leave unbilled, and what did that do to my attitude to those clients?** The honest answer is frequently "I began to avoid them", which is the clinical cost of unbilled work.
5. **What did I do for money last year that I would not want a client to know about?** A consultancy, an endorsement, a referral arrangement, a discounted package sold to a distressed family. One question, once a year, and it is the one that keeps a practice honest.

What to do with the answers. Take them to supervision, not to a marketing plan. The audit's purpose is not growth; it is the discovery of the places where financial pressure and clinical judgement have begun to converge.

12.11 Key terms

Frame — the agreed arrangements of time, place, frequency, duration and payment that constitute the treatment.

Sliding scale — a published schedule of reduced fees with stated criteria and review.

Pro bono — treatment provided without fee, budgeted as part of the practice.

Barter — exchange of goods or services for treatment, permitted only in narrow documented conditions.

Conflict of interest — a financial or personal interest that could influence clinical judgement.

Upcoding — describing a service or diagnosis inaccurately to obtain higher reimbursement; fraud.

Testimonial — a client's public statement about a therapist; restricted or prohibited in most codes.

12.12 Self-test

1. Why is the fee clinical material rather than an administrative detail?
2. Give the six statements of the intake fee conversation.
3. State the four-step sequence for non-payment, with the sentence for each stage.
4. Name the four categories of gift and the default for each.
5. Give the five steps of the gift procedure.
6. What are the conditions under which barter may be permissible, and the specific hazard?
7. Name five business pressures that produce unethical practice.
8. In the Nwankwo case, what was the third option for the goat, and why was it better than accepting or refusing?

Answers. (1) Because it constitutes the frame that makes the relationship professional, and because it carries the client's history of entitlement, worth, shame and dependency, which becomes treatable when it is named. (2) The fee and session length; what the fee includes; when and how it is paid; the cancellation policy; the sliding scale; and what happens if payment becomes difficult. (3) First missed payment — "I noticed the last two sessions were not paid; is something happening?"; second — the pattern named, "you have not mentioned it, and I wonder whether it is difficult to talk about"; third — the limit, with four options; fourth — the decision, recorded. (4) Cultural token — accept and record; transitional — usually accept, discuss the meaning; expensive — decline; with strings — decline and name the string. (5) Notice; ask what it means; decide on the category; say the decision once, warmly; record it. (6) Client initiates, value agreed in writing, time-limited, supervised and recorded; the hazard is that the client's service becomes clinical material and a second arena in which she can fail. (7) Package discounts; productivity targets; upcoding; referral payments; testimonials and advertising claims; products sold to clients; the consultancy overlap; unbilled work. (8) Accept it and ask the

family to sell it and put the money towards the fee — preserving the meaning of the gift, avoiding an asset transfer, and recognising the family's own form of payment.

12.12a Three sentences to keep in the drawer

"Tell me early." Said at intake and repeated at every review, it is the sentence that prevents the silent ending.

"What would it mean to you if I accepted this?" The gift question that converts an object into a conversation.

"We will write it down and look at it again on [date]." The sentence that turns generosity into a professional arrangement, and the one that protects both parties from the drift into gratitude and resentment.

12.13 Reflective task

Write two hundred and fifty words on your own money history: what you were taught about asking to be paid, what you were taught about accepting gifts, and which of those lessons is most likely to shape your practice. Then name one rule you will write down before you see your first paying client.

12.14 Further reading

- Any current text on the business of private practice in your jurisdiction — read for the arithmetic, not the marketing.
- Pope, K. and Vasquez, M. on gifts, fees and barter in psychotherapy — the standard treatment of the ethical questions.
- The professional code sections on fees, gifts, barter and advertising, read in the original wording.
- The literature on conflicts of interest in medicine — the evidence that financial interest shapes judgement while clinicians believe it does not.
- Your indemnity provider's guidance on fees, debt recovery and court attendance charges.

Next: Week 13 — Ethics of research, assessment and diagnosis.

Week 13 — Ethics of research, assessment and diagnosis

Learning outcomes. By the end of this week the student can (1) obtain valid consent for research with families, including assent from children and the problem of group consent; (2) write about a client without identifying her, and state the limits of disguise; (3) describe the consequences of a diagnosis and decide what she can defend; (4) select and interpret an assessment instrument with attention to its validity for the population in front of her; and (5) explain the ethical difference between the clinician-researcher and the researcher who happens to be a clinician.

Three domains, one theme

Research, assessment and diagnosis are usually taught in three separate weeks and they share a single ethical structure: **each of them gives the therapist power to define a person.** The researcher defines what will be known about a family and who will read it. The assessor defines what will be counted as a problem and how it will be scored. The diagnostician defines what the person is, in a category that will follow her into an insurance form, a school file, a custody hearing and her own understanding of herself.

That power is not a reason to avoid the three activities; a profession that does not research, assess and diagnose is not a profession. It is the reason each of them requires a specific discipline.

Domain	The duty	The characteristic failure
Research	Voluntary, informed, revocable consent; protection of the vulnerable; honest reporting	The family "agrees" because the therapist asked; children's assent treated as unnecessary; the negative result not published
Assessment	Competence to administer; validity for the population; feedback to the client	An instrument normed on another population, interpreted as though it measured the same thing
Diagnosis	Defensible, useful, communicated honestly; benefits weighed against stigma	A diagnosis given for the insurer, the school or the court rather than for the client

13.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–8	The single theme: power to define	Three examples, students identify who was defined
8–22	Five concepts: consent in families, the diagnostic act, cultural validity, the assessed client, the writing therapist	Lecture
22–35	Consent with families: the father problem, the child's assent, withdrawal by one member	Two ethics applications, compared
35–50	Writing about clients: consent, disguise, the small-community problem	Students disguise a real case in twenty minutes; group tries to identify it
50–62	Diagnosis: consequences and defensibility	Four diagnoses, students argue each is or is not defensible
62–75	Instruments: validity, qualification, feedback	Three instruments presented; students ask the five questions
75–85	The clinician-researcher: recruitment, dual role, authorship	Case of a thesis that went wrong
85–90	The Amaka case	Students decide whether the study proceeds

13.2 Five concepts that organise the week

Concept 1 — Consent in a family is not one person's signature. A father can consent to his own participation and cannot consent to his wife's or his children's. A child's assent is required in addition to a parent's consent where the child is old enough to understand, and the child's refusal should be respected even where the law does not require it. A mother who consents under pressure from her husband has not consented. The research consent process in family work is therefore slower, more documented and more individual than in individual research.

Concept 2 — Diagnosis is an act with consequences, and the consequences are distributed. The same diagnosis may give one person access to treatment, mark another as unsuitable for a job, settle a custody question, and change how a family talks about a member for the rest of his life. Diagnosing is not wrong; diagnosing without weighing the consequences is.

Concept 3 — An instrument measures what it was normed to measure, in the population it was normed on. A questionnaire developed and validated on one population, translated and used on another, may measure something different or nothing at all. The ethical problem is not the use of a tool; it is acting on a score whose meaning in this population is unknown.

Concept 4 — The assessment has a client, and it is not always the person being assessed. In family work the assessor must state whose question the assessment answers. A child assessed at a school's request, a couple assessed for a court, a father assessed for an insurer: each has a different client, a different consent requirement and a different limit on what can be reported back.

Concept 5 — The therapist who writes is still a therapist. Case reports, theses and teaching material are written by people who know their subjects personally. The obligations are consent, disguise, proportionality (writing only what the purpose requires), and — in small communities — a willingness not to write at all.

13.2a Research consent with families: six problems and their solutions

Problem 1 — the father signs for everyone. Solution: separate consent forms for each adult; an assent form for each child old enough to read one; a note of who was present when each was signed; and no recruitment in a session where one member is dominant.

Problem 2 — the therapist recruits her own clients. Solution: recruitment by a third party who is not the treating clinician, or a clear statement in the invitation that declining will not affect treatment, made by someone other than the therapist. Where the therapist must recruit, the invitation is written, the client is given a week, and the therapist does not ask again.

Problem 3 — one member wants to withdraw. Solution: a stated policy at the outset. In most family studies the unit is the family, and the withdrawal of one member means the family's data are withdrawn; where the design permits partial participation, that is explained in advance. The member who withdraws must not be told by the researcher that they have "ruined" the study.

Problem 4 — children's assent. Solution: an age-appropriate explanation, a written or verbal assent, and the child's right to stop at any point without anyone being told why. Children are frequently the most reliable participants on this question and the least often asked.

Problem 5 — confidentiality inside the research team. Solution: stated at consent: who will see the data, whether transcripts will be shared, how identifiers will be removed, and where data will be stored. A family consenting to research has not consented to being discussed by a research team in a seminar.

Problem 6 — the participant in crisis. Solution: a written procedure for what happens if a participant discloses risk during data collection: the interview stops, the participant is supported, the clinical pathway is followed, and the researcher's duty is stated in the consent form. A researcher who is also a therapist must know which role she is in at the moment of disclosure, and the participant should have been told in advance.

The document that results. A consent form of two pages for adults, one page for children, an information sheet in plain language, and a procedure for the crisis. Four documents, and none of them is optional.

13.2b Writing about clients: consent, disguise and the limits of disguise

Consent to publish. Where a case is written up, the client's consent is obtained in writing, and it is consent to the specific text, not to "use my case". The client reads the text, or hears it read, and may require changes. Where the client is a child, the parent's consent and the child's assent; where the client has died, the family's consent and additional care, because the dead cannot be embarrassed and the living can.

Disguise, and the four things it cannot protect. Changing the name, the occupation, the town and the ages is standard and insufficient in three situations. Small communities: a therapist in a town of thirty thousand is writing about someone her colleagues will recognise from the presenting problem. Rare presentations: a family with a specific configuration — four children, a father in the merchant navy, a mother with a particular diagnosis — is identifiable no matter how the details are changed. Public events: a case that was in the newspapers is not disguisable. Digital traces: a case that has been discussed online may be found by searching the details.

The proportionality rule. Write only what the teaching purpose requires. A case report does not need the family's immigration status, their religion, their income or the detail of the abuse in order to teach the decision. The most common ethical failure in student case reports is excess detail, which is usually written because the detail is interesting rather than because it is necessary.

The composite case. Where disguise is impossible, a composite — several cases combined into one, with a statement that it is a composite — is acceptable in teaching and unacceptable where the composite is presented as a single case with its own factual integrity. The statement must be explicit: "this case is a composite, constructed for teaching, and does not describe any single family."

The student's thesis, and the supervisor's role. The supervisor is responsible for the ethics application and for reading the case material before submission. A thesis containing a recognisable account of a real family is a failure of supervision before it is a failure of the student.

13.2c The assessed client: whose question does the assessment answer?

An assessment is always conducted for someone. Naming that person determines the consent, the feedback and the limits, and the failure to name them is the commonest ethical error in assessment work.

The four possible clients of an assessment.

Who asked	Consent required	What may be fed back
The person being assessed	Her own	Everything, in a form she can use
The family, about one member	The member's, plus the family's for the referral question	The formulation, not the raw material
A school or employer	The person's written consent, and only for the agreed purpose	What the purpose requires, and nothing more
A court	The person's consent where possible, or the court's order	The report as instructed, with its limits stated

The distortion that follows from not naming the client. The therapist drifts towards the interests of whoever commissioned the work. A child assessed because the school wants a diagnosis is assessed in a way that finds one; a father assessed because his wife's lawyer requested it is assessed in a way that finds what the lawyer is looking for, or — in the therapist's attempt to be fair — in a way that under-reports what is there. Naming the client is the discipline that prevents the drift, and stating it in the report is what allows a reader to judge the work.

The feedback conversation, which is part of the assessment. Three questions: what was measured, what the result means, and what it does not mean. The client who is told a score without the third element will carry a number as though it were a verdict. "Your score is in the range that suggests moderate depression. That tells us the severity is significant enough to treat. It does not tell us the cause, and it does not tell us how long it will last."

The assessment that should not be done. Where the question belongs to a third party and answering it would harm the person assessed — a family who wants an assessment to prove that a daughter is "not really ill", an employer who wants an assessment of an employee's loyalty — the therapist's answer is that the question is not one she can answer, and the reason is stated.

13.3 Diagnosis: what it does, and when it is defensible

What a diagnosis does, in four directions.

1. **To the client:** it names the experience, which can relieve ("there is a name for this"), and it can imprison ("I am a borderline"). The same act produces both, and the way it is communicated determines which.
2. **To the family:** it reorganises the household's explanation. A diagnosis of a child's condition may relieve the mother of blame or transfer it to her; a diagnosis of a father's depression may legitimise his withdrawal or confirm the family's view that he is weak.
3. **To the system:** it opens the door to treatment, insurance, school support and legal protection — and it closes others. A diagnosis on a record follows a person into employment, custody and immigration decisions.
4. **To the therapist:** it organises the treatment and, if it is wrong, organises it in the wrong direction for years. The diagnostic act is a commitment, not a label.

The four questions before making a diagnosis.

1. **Can I defend it?** Is there enough evidence, gathered with the right method, to support it if challenged by a colleague, a regulator or a court?
2. **Is it mine to make?** Does it lie within my competence and my licence? A family therapist may recognise depression and may not be qualified to diagnose a psychotic disorder (Week 7).
3. **What will it do for this client?** Access to treatment, an explanation, a school's support — or stigma, a record, a family's certainty that the problem is located in one person?
4. **What will it do to this family's story?** Diagnosing the identified patient in a family system confirms the family's belief that the problem is in that person, which is frequently the belief that has been preventing change.

The relational alternative, and its limit. In family work the formulation is frequently relational rather than individual: "this family's difficulty is in how it has organised itself around a loss", rather than "this mother is depressed". The relational formulation is often more useful and is not a substitute for a diagnosis where a diagnosis is required for care. The competent practice uses both: a diagnosis where the system requires one, and a formulation that locates the problem in the relationships where it actually lives.

The diagnosis given for a third party. "I need a diagnosis for the insurance." "The school will not act without a diagnosis." "My lawyer says a diagnosis will help in court." Each is a pressure to write a category the client may not meet, for a purpose that is not clinical. The defensible position: a diagnosis is given if it is accurate; where a letter is needed for a purpose, the letter states what is observed and what is needed, without inventing a category. Where the third party will not accept that, the therapist declines and says why.

Communication of a diagnosis, in four sentences. What it is, in plain language; what it is not; what it means for the treatment; and what it does not change about the person. "This is a name for a pattern of low mood that has lasted more than two months and is affecting your sleep, your appetite and your work. It is not a description of who you are,

and it is not permanent. It tells me what treatment is likely to help. It does not change anything about what you are able to do."

13.3a Assessment instruments: six questions before you use one

1. Was it validated for this population? Not translated — validated. A translation with no psychometric work is a new instrument with unknown properties, and reporting its scores as though they meant what they mean in the original population is a methodological and an ethical error.

2. Was it normed on people like this client? Age, education, language, urban or rural, clinical or community sample. A cut-off derived from a clinical sample will classify a community sample as abnormal.

3. Am I qualified to administer and interpret it? Some instruments require specific training; all require competence in interpretation. Administering an instrument one cannot interpret is worse than not using one.

4. What will I do with the result? If the answer is "nothing that I would not do otherwise", the assessment is being done for the therapist's reassurance or for a file. If the answer is "it will change the treatment", the result must be fed back to the client.

5. How will I give the result back? The client is entitled to know what was measured, what the score means, what its limits are, and what happens next. Feedback is part of the assessment, not a courtesy.

6. Where does the raw data go, and for how long? Test data are clinical records: stored, retained and disclosed under the same rules (Week 11). Protocols are not kept in a drawer indefinitely, and they are not shown to anyone outside the treatment without consent.

The family-specific problem. Many instruments are designed for individuals and used on families by summing scores, which produces a number that means nothing. Family assessment requires either an instrument designed for family-level constructs (cohesion, adaptability, communication) or an explicit statement that the scores describe individuals within the family and not the family as a unit.

The one instrument most family therapists do use. The outcome measure, completed at each session or at intervals, which is also an intervention: it gives the client a voice in the treatment and gives the therapist feedback on deterioration. Its ethical requirements are the same as any other — validity for the population, explanation of the purpose, and a stated response when the score indicates deterioration.

13.3b The diagnosis of a child, and the family that receives it

The most consequential diagnostic act in family work, because it lands on a person who did not ask for it, in a system that will reorganise around it.

The four questions, applied to a child. Can I defend it? With a child, the evidence must come from more than one setting — home and school — and from more than one informant, because a child's behaviour in a clinic is a sample of one context. Is it mine to

make? Many child diagnoses require specialist assessment; the family therapist's task is recognition and referral (Week 7). What will it do for the child? Access to support in school, a treatment pathway, an explanation for the parents — or a label that follows him through school records. What will it do to the family's story? A diagnosis of a child's condition can relieve the parents of blame, which is sometimes a kindness and sometimes a way of not examining a household in which the child is frightened.

The conversation with the parents. Five elements: what the assessment found; what it did not find; what the diagnosis means and does not mean; what the parents can do that will help; and who else will be told, and what they will be told. The fifth element is the one most often omitted and the one the parents most need, because the diagnosis will travel — to the school, the church, the extended family — and the parents will be the ones managing its journey.

The conversation with the child. Age-appropriate, brief, and about function rather than category. A nine-year-old does not need the word; he needs to know that "your brain works in a way that makes sitting still hard, and there are things that help, and it is not because you are bad." A fifteen-year-old may want the word, the explanation of what it means, and the assurance that it is not the whole of him.

The label that becomes the family's explanation. "He is like that because of his condition" is a sentence that relieves and closes at the same time. The therapist's work, over months, is to hold both truths: the condition is real and the household's patterns still matter. That is the family therapist's particular contribution to a diagnostic case, and it is the reason a diagnosis alone is never a formulation.

13.3c The diagnosis as a family event

When a diagnosis enters a household it does three things at once, and the therapist should anticipate all three.

It assigns a location to the problem. The difficulty now lives in one person. That is frequently the family's preference and frequently the reason change is difficult: if the problem is in her, the rest of the household has nothing to change. The family therapist's contribution is to accept the diagnosis and to keep the household in the formulation: "the depression is real and it is in her. And a household in which nobody speaks before noon is part of what is keeping it going."

It redistributes responsibility. A diagnosis can relieve a mother of blame ("it is an illness, not my parenting") and can transfer it to her ("it is genetic, from your side"). Both movements happen in the same session and both are heard by the person diagnosed.

It changes the family's language. Members begin to use the diagnosis as an explanation, a weapon and a prediction: "you cannot help it", "it is your condition", "you will always be like this". The therapist listens for the third use, because it is the one that forecloses change, and names it: "when you say she will always be like this, what does that do to her?"

What the therapist does with all three. Assess the diagnosis' accuracy; communicate it in the four-sentence form; then run one session, early, on what the family now believes

about it. That session is frequently worth more than six months of work on the symptoms, because it determines whether the household will support the treatment or organise itself around the label.

13.4 The clinician-researcher: the thesis, the client and the dual role

The MFT 770 thesis requirement means that every student on this programme will conduct research, frequently while seeing clients, and very often on material that arises from their own practice. The dual role is the week's most practical problem.

The four ways the dual role distorts research.

Recruitment bias: the clients who agree are the ones who like the therapist, and they know the therapist's preferences. Consent under influence: the client who is asked by the person she depends on cannot truly refuse. Interpretation bias: the therapist-researcher sees her own clinical decisions confirmed by the data. Role confusion at the moment of disclosure: the participant says something that activates a duty, and the researcher must decide which hat she is wearing, in the middle of an interview.

The four controls.

1. **Recruitment by a third party** wherever possible, and where not, a written invitation delivered by someone else and a stated cooling-off period.
2. **A written statement of what the researcher will do if the participant discloses risk**, agreed with the ethics committee and stated in the information sheet.
3. **Analysis by someone other than the interviewer,** or at least a second coder who has no clinical relationship with the participants.
4. **Supervision of the dual role,** in which the student discusses the specific ethical events of the study as they happen, and a supervisor who reads the transcripts.

The ethics application, in five sections. Purpose, participants, consent, data, risks. The section that is most often thin is risks — not risks to participants' bodies, which are usually minimal in this kind of research, but risks to their relationships and reputations: a couple who both participate may disagree about what has been said; a transcript stored on a laptop may be read by a partner; a quotation may be recognised by the family. Each of these is a risk the application must name and mitigate.

Authorship and honesty. The student's thesis is the student's; the supervisor's contribution is acknowledged according to the convention of the institution. Data are not fabricated, not selectively reported, and not discarded because they do not support the hypothesis. The negative finding is a finding and is reported. A student who has learned to write up an unexpected result honestly has learned the whole of research ethics.

13.4a The seven-step research ethics check

A compressed procedure for the student designing the MFT 770 study, to be run before writing the application.

1. **Whose question is it?** The researcher's, the supervisor's, or a community's. If it is the researcher's alone, the study should say what it gives back.

2. **Who is vulnerable here, and in what way?** Not "vulnerable populations" in the abstract, but the specific people in this sample: the woman whose husband controls the household, the child who is asked about his parents, the couple in acute crisis.
3. **How will consent actually be obtained, in what language, by whom?** If the answer involves the treating therapist, the design has a problem.
4. **What are the relational risks?** What could be said in an interview that would be repeated at home, and what would happen then?
5. **What is the plan if a participant discloses risk?** Written, with names and numbers, and rehearsed.
6. **What will be published, and who could be identified by it?** Including the third parties who never consented.
7. **What does this community get?** A returned report, a training session, a copy of the thesis in the local library, a co-author from the setting. If the answer is "nothing", the study is extractive and the ethics committee may still approve it, which does not make it right.

The two questions that predict trouble. Question 3, because the dual role lives there, and question 6, because it is the one the student has not thought about and the one the supervisor will be asked about later.

13.5 Case: Amaka's study, and the participant who was recognisable

The student. Amaka, a candidate on this programme, is conducting her MFT 770 study on "the experience of forgiveness in couples after infidelity". She has recruited eight couples: three from her own caseload, five through a church announcement.

The design. Semi-structured interviews of ninety minutes with each partner separately, audio recorded, verbatim transcripts, thematic analysis, with the church's permission to announce the study.

The three problems that emerged.

Problem one, at recruitment. One of her own clients, Mr Osagie, offered to participate "because you have done so much for us". Amaka's first instinct was to accept, and the ethics application had specified that clients in current treatment would not be recruited. She declined, and the client experienced it as rejection; the next session was spent on that, which was the right clinical decision and a cost of the design.

Problem two, at interview. During Mrs Osagie's interview (she participated through the church route, not through Amaka), she described her husband's affair in a form of words identical to the account Amaka had heard in the couple session, and added a detail — the other woman's workplace — that Amaka already knew from the clinical file. Amaka stopped the recording and said, "I want to remind you that this is research, and that what you say here goes into the study and not into our sessions. Also, I already know some of this from our work, and I need you to know that I am not going to use anything in the study that I did not hear here, today, in this interview." She recorded that statement in the note and in the study log.

Problem three, at analysis. In the draft chapter, a quotation read: "She worked in the same bank as him. I used to see her at the Christmas party." With the ages, the city and the church, the couple would be identifiable to anyone in their congregation who read the thesis, and the thesis would be in the university library.

What Amaka did.

1. She removed the detail and replaced it with "she worked at his office", which preserved the meaning (proximity) and removed the identifier.
2. She changed the town in every extract, not only in the demographic table.
3. She wrote to both partners with the three quotations she intended to use, and asked whether they were content. Mr Osagie asked for one to be removed. It was removed, and Amaka noted the change.
4. She added a paragraph to the methodology describing the measures taken and their limits: "no disguise can guarantee anonymity in a small community; the participants were told this, and the extracts used were approved by them."

The supervisor's question, which produced the case. "If the other woman read this thesis, what would she know?" Amaka had not considered her — the third party, who has not consented to anything.

What the case teaches.

1. **The dual role has a specific failure point,** which is recruitment. The application's prohibition was right and its cost — a difficult session — was the price of doing it properly.
2. **The moment of overlap between the clinical and the research roles must be handled out loud.** Amaka's statement in the interview was not required by any code; it was the honest management of a boundary that the participant could not have known was there.
3. **Consent to be studied is not consent to be recognised.** Sending the quotations to the participants is unusual and is the highest standard available.
4. **The third party is the person everyone forgets.** The affair partner, the child, the grandparent in the transcript: none of them consented to appear in a thesis, and all of them may be identifiable.

13.6 Transcript: the consent conversation for a case report

A therapist asks a former client for permission to write up the treatment for a teaching chapter. Minutes eight to twenty-two.

> **T1 Therapist:** I am writing a chapter for students about the kind of work we did, and I would like to use your case. (pause) Before you answer, I am going to tell you exactly what that would involve, and I want you to know that you can say no and it will change nothing between us. — The request, followed immediately by the two things that make it consent rather than pressure: the information and the right of refusal. **T2 Client:** What would you write? **T3 Therapist:** The decisions I made and why, and something of what you were dealing with, so that students can learn from it. Not your name, not your town, not your job as it is, not the names of anyone in your family. — The scope, stated positively and negatively, before the document is shown. **T4 Client:** But you would show it to people. **T5 Therapist:** It would be published, yes. (pause) That is the part I want you to think about properly, not answer now. (pause) Anyone could read it, including people who know you. — The risk named plainly, including the one the therapist would rather not mention: the possibility of recognition. **T6 Client:** Would they know it was me? **T7 Therapist:** They might. (pause) I will change the details, and I cannot promise that. Your church, your children's ages, the thing that brought you in — those together are quite specific, and I would rather tell you that than pretend the changes are a guarantee. — The honest answer to the question that decides the case. A therapist who says "no one will know" has obtained a consent that is not informed. **T8 Client:** (pause) If I said no, would you be upset? **T9 Therapist:** I would be disappointed and I would not be upset with you, and I would not mention it again. (pause) That is the difference between asking and pressuring. — The distinction stated because the client cannot know it otherwise, and because the therapist's disappointment is precisely what she must not transmit. **T10 Client:** Can I read it first? **T11 Therapist:** Yes. I will send you the text before it goes anywhere, and you can strike anything you want, and I will tell the editor that you have seen it. — The offer that converts the consent from consent-to-use into consent-to-publish-this-text, which is the standard the codes describe and which few therapists actually meet. **T12 Client:** (pause) And after — could I say later that I have changed my mind? **T13 Therapist:** Before it is published, yes. After, no — it will be out. And if you asked me before publication to stop, I would stop. — The limits of revocability, stated precisely. "I would stop" is a promise the therapist can keep and the client can rely on. **T14 Client:** (long pause) All right. Send it to me.

What the transcript shows. Six of the seven therapist turns give the client something that makes refusal possible: the scope, the risk, the honest answer about recognition, the assurance about disappointment, the right to read the text, and the right to withdraw before publication. Consent obtained without those six things is a signature, not a consent.

What happened afterwards. The client struck one sentence — a detail about her mother — and asked that the town be changed to a different region. Both were done. The published chapter carries a footnote: "details have been changed and the client has read and approved this account."

13.7 Laboratory: the ethics application and the disguised case

Part A — the application (forty minutes). In groups, write the five sections of an ethics application for this study: "the experience of disclosure of HIV status within couples, recruited through a clinic in [state]". Each group must include: the consent documents; the recruitment route; the plan for a disclosure of risk during an interview; the data storage and destruction plan; and the specific relational risks to participants.

Part B — the disguise exercise (twenty-five minutes). Each student writes 250 words about a case, disguised, and reads it aloud. The group then tries to identify the family from the details given. The exercise is uncomfortable and instructive: almost every first draft is identifiable.

Marks (20).

Criterion	0–1	2–3	4–5
Consent	One form	Separate forms	Separate forms for adults and children, with assent and withdrawal stated
Risk plan	None	Risk mentioned	Disclosure procedure specific, with roles named
Disguise	Not attempted	Names changed	Multiple details changed, third parties protected, limits acknowledged
Data	Not addressed	Storage mentioned	Storage, access, retention and destruction specified
Honesty	Overclaims anonymity	Some caution	Anonymous where possible, and the limits stated to participants

The observation for the debrief. No group's first draft will protect the third party — the affair partner, the grandmother, the sibling who appears in one sentence. That omission is the lesson.

13.7a Four research designs and the ethics of each

The design determines which ethical problems will arise, and students choosing a design are also choosing their ethical work.

1. The interview study. Risk: the disclosure of distressing material with no clinical container; the repetition of harm in a research interview with a survivor. Mitigation: the interviewer's training, a stated procedure if distress occurs, a debrief, and the contact details of a service.

2. The survey. Risk: consent that is a click; items that are intrusive in a household where the questionnaire may be seen; the impossibility of withdrawal once data are pooled. Mitigation: plain language, the option to skip items, a statement about who may see the completed form, and a clear withdrawal window.

3. The outcome study using clinical data. Risk: using records for a purpose the client did not consent to; anonymisation that fails in a small clinic; the therapist's interest in a positive result. Mitigation: a specific consent to the use of data, an independent analyst, a pre-specified analysis, and a commitment to report the negative result.

4. The action research or service evaluation. Risk: the conflation of evaluation with research; staff or clients who cannot refuse because the evaluator is their manager or therapist. Mitigation: independent facilitation, anonymised data collection, and a written agreement about what will be reported to whom.

The design choice in a small programme. Most MFT 770 studies will be interview or survey designs with small samples. The ethical burden in a small sample is not statistical; it is the impossibility of true anonymity and the closeness of the researcher to the participants. Both are manageable with honesty, and neither is manageable with a standard consent form.

13.8 Six errors

Error 1 — Recruiting your own clients because they are available. The consent is contaminated, the sample is biased, and the client's refusal becomes a clinical event. The prohibition exists for a reason and the reason is not administrative.

Error 2 — Treating a parent's signature as family consent. Each adult consents for herself; each child assents for himself; and the consent is documented per person.

Error 3 — Promising anonymity that the design cannot deliver. A promise of anonymity to a family in a small community, from a study that will be in a university library, is a false statement made in writing.

Error 4 — Diagnosing what the payer will reimburse. It is fraud, and it is also a clinical decision made by a third party who will never meet the client.

Error 5 — Using an instrument because it is available. Validity for the population is not optional, and acting on a score whose meaning is unknown is acting on nothing.

Error 6 — Writing about clients without telling them, because the writing is "for teaching". Teaching is a purpose, not an exemption; the consent is required, and the requirement is stated in most codes in terms that students frequently have not read.

13.9 Culture, faith and the ethics of knowledge

Who researches whom. A substantial literature on research in African settings documents the problem of extractive research: studies designed elsewhere, conducted on local populations, published in journals the participants will never read, with no benefit to the community studied. The ethical response is not to stop researching; it is to build the study so that the community gains something — findings returned to the clinic or the church that hosted the study, local co-researchers, a dissemination event in the participants' language.

Community consent. In settings where a community leader, a church or a family elder is the gatekeeper, formal consent processes frequently require the gatekeeper's permission in addition to individual consent and never instead of it. A study in which the pastor agrees and the women do not is a study conducted on people who did not agree.

Instruments from elsewhere. Most widely used instruments were developed in North America or Europe. Their use in this region raises the questions of translation, conceptual

equivalence (does the construct exist in the same form in this language?), and norming. A depression scale that asks about "feeling sad" may miss a presentation organised around bodily complaints, which is a well-documented pattern in many settings. The ethical requirement is honesty about the limits, not the abandonment of measurement.

Diagnosis across cultures. The cultural formulation in DSM-5 and the ICD guidance on cultural concepts of distress exist because the same experience is described differently across settings. The practitioner's discipline: describe the experience in the client's terms first, then consider whether a category fits, and record both.

Faith and the diagnosis. A family may hold that the problem is spiritual, and a diagnosis may be heard as a denial of that. The practitioner's position — Week 7's — is additive rather than competitive: the diagnosis and the family's explanation can coexist, and the therapist's task is to work with both without requiring the family to give up its account.

> **DEBATE**
> "Research on families should require the consent of every adult member, and any member's refusal should end the study for that family." Argue for: the family is a unit and the data are relational, one member's refusal will distort what the others say, and the family's privacy is shared. Argue against: an individual's participation is her own decision, a controlling husband could veto his wife's participation, and the rule would silence precisely the people whose voices are least often heard. Then state the rule you would put in your own ethics application.

13.9a Writing the thesis: five sentences the examiner will look for

Not a style guide; a list of the sentences that demonstrate the student has understood the ethics of her own study.

1. **"Participants were recruited by [a third party / letter] and were told that declining would not affect their care."** The sentence that shows the dual role was considered.
2. **"Each adult gave written consent; each child gave written or verbal assent, recorded in the file."** The sentence that shows consent was individual.
3. **"The sample was drawn from a single clinic and anonymity cannot be guaranteed; participants were told this."** The sentence that shows honesty about limits.
4. **"Where a participant disclosed risk during the interview, the procedure agreed with the ethics committee was followed: [what happened]."** The sentence that shows the protocol was not decorative.
5. **"The findings did not support the expected relationship between [x] and [y]."** The sentence that shows the student reported what she found.

The missing sixth, which distinguishes a good thesis from a competent one. A short paragraph on what the study gave back: the report returned to the clinic, the session run for the staff, the plain-language summary for the participants. Students who

write this paragraph have understood that research is a relationship and not a transaction.

13.10 Evidence note

What is known. The evidence on research ethics is largely in the form of guidance and documented failures rather than trials: consent processes that participants do not understand, therapeutic misconception (participants believing that research is treatment), and the systematic under-representation of children's own assent. In assessment, the cross-cultural validity literature consistently shows that instruments do not travel without psychometric work, and that translated tools can produce artefactual differences in prevalence. In diagnosis, research on labelling documents effects on self-concept, on family attributions and on clinicians' subsequent information-gathering — the diagnostic label shapes what is noticed next.

What is weaker. There is little empirical work on how therapists actually obtain consent to publish, and case reports are almost never audited for identifiability. The evidence on the harms of diagnostic labelling in family therapy specifically is thin.

What is not supported. That anonymity is achieved by changing names. That a diagnosis is harmless because it is confidential — the client's own family, school and employer frequently learn it. And that a validated instrument used outside its validation population measures the same construct.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same diagnosis given at the start of a treatment, as a working formulation, and given at the end, as a verdict on a person, has two different effects on a family.

13.11 Key terms

Assent — a child's agreement to participate, required in addition to a parent's consent.

Therapeutic misconception — the participant's belief that research activity is treatment.

Cultural validity — the extent to which an instrument measures the same construct in a different population.

Diagnostic act — the decision to assign a category, with its consequences for access, stigma and identity.

Composite case — a teaching case constructed from several cases, and labelled as such.

Extractive research — research that takes data from a community without returning benefit.

13.11a The five records a research-minded clinician keeps

A clinician who intends to write, teach or research will need material, and the material must be collected lawfully from the beginning.

1. **A consent-to-contact register,** separate from the clinical file, recording who has agreed to be approached about research or teaching and when. It is the single document that makes later recruitment lawful.
2. **A teaching-consent note** in the file, with the specific scope: "agreed to the use of anonymised material from this treatment for teaching and publication, subject to seeing the text; consent recorded [date]".
3. **A research log,** for any study: recruitment, withdrawals, the date of each consent, and every ethical event (a disclosure of risk, a request to withdraw, an adverse reaction) with the action taken.
4. **A data map,** one page: what data exist, where they are stored, who can access them, and when they will be destroyed.
5. **A publication file,** containing the approved text, the participant's written approval, and the correspondence with the journal. It is the evidence that the consent was to that text and not to a general idea.

Why the fifth record is the one students omit. Because consent is usually obtained at the beginning, when the text does not yet exist, and the version published may differ from the version described. The file closes that gap and takes ten minutes.

13.12 Self-test

1. Why is a father's signature not sufficient consent for a family study?
2. Give three things disguise cannot protect, and the rule that follows.
3. What are the four questions before making a diagnosis?
4. State the six questions before using an assessment instrument.
5. What are the four ways the clinician-researcher's dual role distorts research, and the four controls?
6. What is therapeutic misconception, and how is it prevented?
7. In the Amaka case, what did she do at the moment of overlap in the interview, and why?
8. What is a composite case, and when may it be used?

Answers. (1) Because consent is individual: the father consents for himself, the mother for herself, and each child assents; a mother consenting under pressure has not consented. (2) Small communities, rare presentations, public events (and digital traces); the rule is that consent is obtained to the specific text, the limits are stated, and where disguise cannot protect, the case is not published. (3) Can I defend it; is it mine to make; what will it do for the client; what will it do to the family's story. (4) Validated for this population; normed on similar people; am I qualified; what will I do with the result; how will I give it back; where do the data go. (5) Recruitment bias, consent under influence, interpretation bias, role confusion at disclosure; controls are third-party recruitment, a written risk procedure, independent analysis or second coding, and supervision of the dual role. (6) The participant's belief that research is treatment; prevented by explicit information, separate consent, and the researcher's refusal to blur the roles. (7) She stopped the recording and stated aloud that the interview was research, that nothing from

the clinical file would enter the study, and that she would use only what she heard in the interview. (8) A case constructed from several cases for teaching, labelled explicitly, and not presented as a single factual case.

13.12a Three questions the ethics committee will ask

Not the ones students prepare for.

"What will you do if the participant becomes distressed?" The answer must be a procedure, not a sentiment. Who stops the interview, what is said, who is called, and what the participant is offered afterwards.

"How will you know whether they understood the consent?" The answer is a method — a teach-back question, a second conversation, a written summary in the participant's language — not a signature.

"What happens to the data if you leave the programme?" The answer belongs to the institution, not the student: custody, storage and destruction specified in the application and followed at the end.

13.13 Reflective task

Write two hundred words about a family you have already met — in placement, in your own practice, or in this book's cases. Then answer: what would you have to change before you could publish it, and would what remained still teach anything? If the answer is no, you have found the reason that most good clinical writing is never written. If the answer is yes, write the limits into the text: what you changed, why, and what the reader should therefore not assume — because a reader who is told the limits can use the case, and a reader who is not will generalise it into a rule, and rules made from disguised cases have a way of being applied to real families who do not resemble the one described.

13.14 Further reading

- The Declaration of Helsinki and the CIOMS guidelines — short, and the source of most national codes.
- Your institution's research ethics guidance, read before writing the application rather than after the first rejection.
- The DSM-5 cultural formulation and the ICD guidance on cultural concepts of distress.
- Any current text on cross-cultural assessment and test adaptation — the psychometric requirements, not only the translations.
- The literature on research ethics in African settings, particularly on community consent and benefit sharing.

Next: Week 14 — Integration: the decision model and your stance.

Week 14 — Integration: the decision model and your stance

Learning outcomes. By the end of this week the student can (1) apply the seven-step decision model to an unfamiliar dilemma under time pressure and defend each step; (2) identify the three ways the model is misused; (3) write a stance paper that names what she will never do, what she will always consult about, and where her own values are most likely to distort her judgement; and (4) present a case to an examiner in a way that shows reasoning rather than recall.

What integration means at the end of fourteen weeks

The course began with the question of what kind of professional a person becomes when she takes on the codes, and it ends with the same question, now answerable. Everything between — consent, confidentiality, secrets, boundaries, competence, risk, law, records, money, research — is a set of particular decisions that a stance has to hold together. A therapist who knows all fourteen weeks and has no stance will make each decision freshly, under pressure, according to how she feels that afternoon. A therapist with a stance has already decided the large things, which is what leaves her free to attend to the client in front of her.

The course's claim, in one sentence. Ethical practice is not the avoidance of wrongdoing; it is the maintenance of a structure — consent, competence, supervision, records, consultation — that makes good decisions possible when the therapist is tired, frightened, fond of the client, or alone.

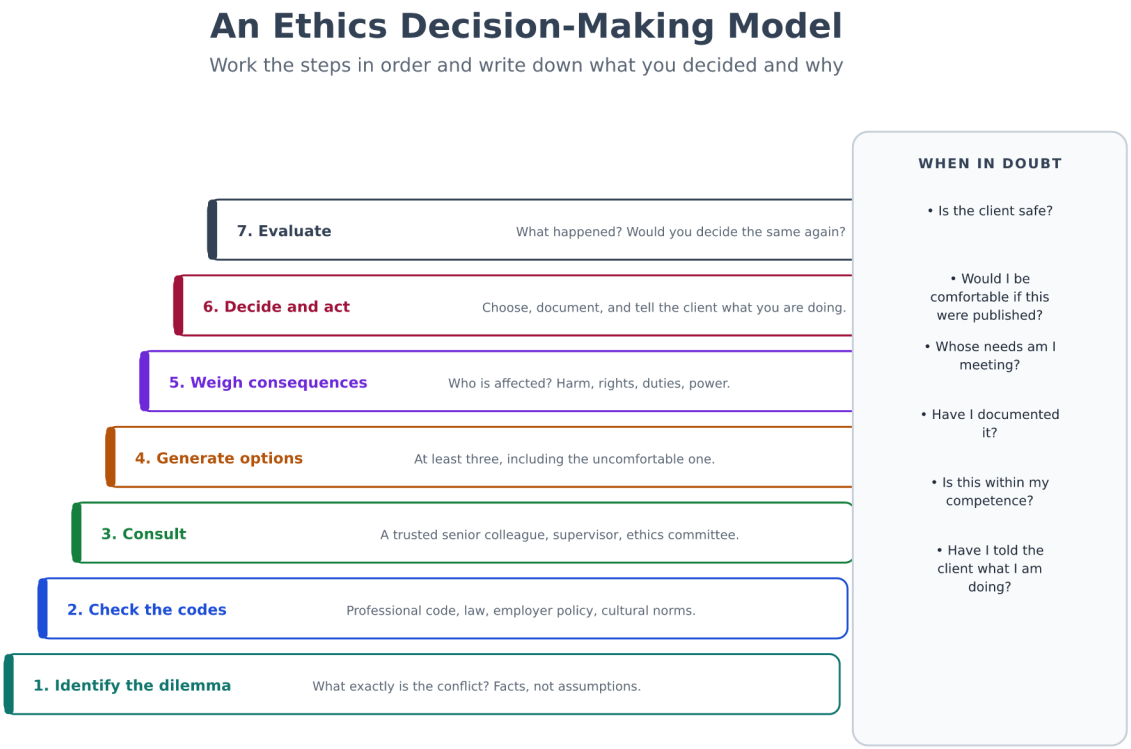


Figure 14.1 — A seven-step decision-making model for ethical dilemmas

14.1 The lecture in ninety minutes

Minutes	Content	Teaching form
0–10	What integration is; the stance and why it is the assessment	Two therapists, same dilemma, different stances
10–30	The seven steps, each with its failure mode	Lecture with a running case
30–45	The model applied end to end to a hard case	Whole-group application, one step at a time
45–55	The three misuses of the model	Three transcripts of reasoning, students identify the misuse
55–70	The stance paper: structure and worked extracts	Two anonymous stance papers, compared and marked
70–85	The viva: what examiners ask and what they are listening for	Demonstration viva with a volunteer
85–90	The closing question: what will you never do?	Written, collected, returned at the end of the programme

14.2 The seven steps, with the failure mode of each

Step 1 — Identify the dilemma. Not the situation: the dilemma. A situation is a mess; a dilemma is a choice between two goods or two harms. The statement to produce: "either I do X, and Y is lost, or I do Z, and W is lost." Failure mode: naming a situation as a dilemma when one option is obviously right — which is a way of avoiding the decision by making it look hard.

Step 2 — Check the codes and the law. Which clauses apply; which statute; what the jurisdiction requires. Failure mode: quoting a code from memory, or from another country. The check is documentary, and it takes ten minutes with the code in front of you.

Step 3 — Consult. A supervisor, a colleague, a lawyer, a specialist. Failure mode: consulting the colleague most likely to agree. The test of a consultation is that it changed something — a fact, an option, or the therapist's confidence in the wrong answer.

Step 4 — Generate at least three options, including the uncomfortable one. Three is the minimum because two options produce a binary in which one side is usually self-serving. The uncomfortable option is the one the therapist has not allowed herself to consider: ending the therapy, reporting, telling the client the truth, refusing the referral, transferring the case. Failure mode: the "three options" that are three versions of the preferred one.

Step 5 — Weigh the consequences for every affected party. The client, the partner, the children, the third party, the therapist, the profession, the future client who will be affected by the precedent. Written as a table, because a table exposes the party that has been left out. Failure mode: weighing only the client in the room, who is usually the most articulate and always the most present.

Step 6 — Decide, act, document. The decision is made, communicated to the client, and recorded with the reasoning, the consultation, the alternatives considered and the review date. Failure mode: documenting the decision without the reasoning, which is the note that reads as an assertion and cannot be defended.

Step 7 — Evaluate afterwards. What happened; what would be done differently; what the case has taught about the practice's structure. Failure mode: skipping step 7 because the case is over. This is the step that converts experience into competence, and it is the step that therapists in busy practices never reach.

14.2a The model applied, end to end

The case below is a composite, constructed to require every part of the course. It is the case used for the practical examination.

The situation. Ngozi A., 41, a nurse, has been in treatment for eleven weeks for depression following her husband's stroke. Her husband, Emeka, 52, is a retired teacher with significant disability from the stroke, cared for at home. Their daughter, Ada, 16, is in the household. In session eleven, Ngozi discloses that her brother-in-law has been sexually touching Ada for some months, that Ada told her in confidence, that Ngozi has not told anyone else, and that she has been "handling it by keeping him away from the house". She says she cannot manage her husband's care alone, that the brother-in-law is her husband's

younger brother and the family's only financial support, and that if this becomes public "they will say I destroyed the family." She is the therapist's neighbour's sister and the family attend the therapist's church.

Step 1 — the dilemma. "Either I report the disclosure, and this woman loses the support that keeps her household functioning and possibly her marriage, or I do not, and a sixteen-year-old remains at risk and the therapist becomes a party to the concealment." Stated as a dilemma, both losses are real. That is the point of step 1.

Step 2 — the codes and the law. The child protection duty is engaged: a child under eighteen has disclosed sexual abuse. The statute in the jurisdiction requires a report on reasonable suspicion, and here there is a disclosure. The code requires the therapist to report and to explain to the client what will be reported. The data protection framework permits the disclosure of the minimum necessary. The report does not require the mother's consent and does not require proof.

Step 3 — consultation. The supervisor, the child protection lead at the local authority, and — because of the dual relationship — the therapist's own supervisor's supervisor, or an ethics consultation. Read what happened in consulation: the child protection officer confirmed the threshold was met and gave the route and the name; the supervisor identified the dual relationship as a separate problem requiring a decision.

Step 4 — the options, including the uncomfortable one.

1. Report to the child protection unit today, tell Ngozi what will be reported, continue the treatment.
2. Report, and transfer the case because of the dual relationship and the threat to the alliance.
3. Support Ngozi to report herself, with a deadline of forty-eight hours, and report if she does not.
4. Do not report, and work with Ngozi to protect Ada within the family.
5. Report, and end the treatment.

Option 4 is unlawful and is stated in order to be rejected in writing, which is what a good record does. Options 1, 2, 3 and 5 are all lawful; they differ in what they protect. Option 3 is attractive and is wrong here: forty-eight hours of delay for a child in a household where the abuser has access.

Step 5 — consequences for every affected party.

Party	Option 1	Option 2	Option 5
Ada	Protection; investigation; possible family disruption	Same	Same
Ngozi	Told first; supported; may lose the therapist	Told; loses the therapist at the moment of crisis	Told; loses the therapist and the treatment
Emeka	Loses his brother's support; may learn of the abuse in a way he cannot control	Same	Same

Party	Option 1	Option 2	Option 5
The brother-in-law	Investigated	Same	Same
The therapist	Manages dual relationship and the alliance	Removes the dual relationship, abandons the client	Ends the case
The practice	A precedent for the dual-relationship question	Cleaner	—

Step 6 — the decision and the record. Report today; tell Ngozi in the session, in the words of Week 9; continue the treatment for the present, with the dual relationship managed and declared to the supervisor; review the decision to continue at the next supervision; record the seven steps in the file with the consultation names and times.

Step 7 — afterwards. At the review, three questions. Did the report protect Ada? Did the treatment survive? And the structural question: what in this practice allowed a therapist to be treating a family from her own church without a policy that would have raised the question at intake?

What the case demonstrates. The model does not make the decision comfortable, and it does not remove the losses. It makes the decision defensible, which is all a decision-making model can do. The therapist who reports is not choosing the child over the mother; she is choosing to comply with a duty that exists because adults in these situations frequently cannot protect the child themselves — including the mother, who is not a villain in this case and is not able to do what the law requires of the professional.

The teaching point students resist. The mother is not the problem. A model that turns her into the problem makes the case easier and the work worse. The step-5 table exists to keep her in the weighing.

14.2b The three misuses of the model

Misuse 1 — using it to justify a decision already made. The therapist reaches the conclusion in the first thirty seconds — usually the one that protects the relationship — and then walks the seven steps backwards. The signature is a step 4 that contains three versions of the same option, and a step 5 table in which the therapist's convenience does not appear because it has not been named.

Misuse 2 — skipping step 4. Two options, both bad, and the therapist chooses the less bad and calls it a decision. The uncomfortable third option is frequently the one that requires a conversation the therapist is avoiding — telling the client the truth, ending the treatment, refusing the fee, admitting the error.

Misuse 3 — treating the model as a substitute for a stance. A therapist with no position on the large questions will run the seven steps for every decision and produce technically defensible answers that do not add up to a practice. The model is a procedure for the difficult case; the stance is what handles the ordinary ones, which are most of them.

The test of a well-used model. After the decision, the therapist can state the option she rejected and why, the party she most regretted, and the part of the decision she found hardest. A therapist who reports no difficulty has probably not done step 5.

14.2c The six questions that find the dilemma

Before the seven steps, a filter. Six questions, used at intake and at every review, that identify the cases in which an ethical question is present before it announces itself.

1. **Who is not in this room, and would they be affected by what happens here?** The absent party is the commonest source of ethical error in family work: the affair partner's spouse, the child who is not brought, the husband who declined, the employer who is paying.
2. **What would I be uncomfortable reading aloud in a hearing?** The question that locates the weak point without requiring a theory of ethics.
3. **Am I the only person who knows this?** An isolated fact is a structural risk (Weeks 3 and 4).
4. **What have I been asked to do, by whom, in whose interest?** The referral question, the report, the character letter, the "just a quick word with the school".
5. **What am I avoiding asking?** The question the therapist has noticed and postponed. It is almost always the one that will become the dilemma.
6. **Is my practice built to catch this?** Policy, supervision, screening, records. If the answer is no, the dilemma is not a case problem; it is a practice problem that has produced a case.

Why the filter comes first. The seven steps are expensive in time and attention, and they are used on perhaps a dozen cases a year. The six questions take two minutes and are used on every case, which is where most of the protection lies.

14.3 The stance paper

The assessment for this week, and the document that outlasts the course.

What it is. Five hundred words in three sections, written in the first person, dated, and revisited at the end of every year of practice.

Section 1 — what I will never do. Six to ten statements, specific enough to be checkable. Not "I will always act ethically", but: "I will never see a client's partner without the client's knowledge"; "I will never accept a gift of money"; "I will never write an opinion about a parent I have not assessed"; "I will never continue a treatment in which I am the only person who knows something that would change it."

Section 2 — what I will always consult about. The list is shorter and it is the more important one, because the things on it are the ones the therapist will be tempted to handle alone. Typical entries: any case involving a child and a disclosure; any sexual boundary question; any decision to continue after a serious error; any case in which I notice that I am the only person the client trusts; any request I am inclined to grant as a favour.

Section 3 — where my own values are most likely to distort my judgement. The honest section, and the one that makes the other two credible. Examples of what a real entry looks like:

> I was raised in a household where a woman who left her husband was a failure, and I notice that I work harder with couples who are separating than with those who are not. I will consult whenever I find myself arguing for the marriage. I am uncomfortable with displays of anger, and I will notice when I am softening a client's anger on her behalf. I am frightened of being thought incompetent, and I will notice when a referral feels like a humiliation rather than a clinical decision. Money was never discussed in my family, and I will use the practice's policy rather than my own judgement at the moment the conversation becomes difficult.

Why the third section is examined. Because a therapist who cannot name her own distortions will not notice them in the room, and because the examined stance paper is the only place in the programme where the student is required to be specific about herself rather than about clients.

How it differs from a values statement. A values statement says what the therapist believes. A stance paper says what the therapist will do, refuse, consult about and watch for. The first is a position; the second is an instrument.

The revision. Once a year, in writing: which of the statements in section 1 did I break; which of the entries in section 3 was accurate; and what new entry belongs there. The revision is the record of professional development that no CPD log records.

14.3a The worked stance paper, in part

An extract from a student's paper, included because the specificity is instructive and because students consistently write something more abstract on the first attempt.

> **WHAT I WILL NEVER DO.**
> I will never treat a member of my own family, or a close friend, or the child of either, and I will say so at the point of referral rather than after the first session. I will never conduct a session without knowing whose client system I am working with. I will never let a couple's file contain a secret that either partner can demand to read. I will never give a diagnosis I have not written the reasoning for. I will never sign a letter I would not read aloud in court. **What I will always consult about.** Any disclosure of abuse. Any case in which I want to break a rule because the client's situation is exceptional — my exceptions are the least reliable part of my judgement. Any case in which I have begun to dislike the client. Any case in which I have become the only person the client talks to. Any fee arrangement I have made in the room rather than from the policy. **Where my values distort me.** I grew up in a home where conflict was forbidden, and I have discovered in role-play that I interrupt anger. I will ask my supervisor, every month, whether I have done it. I was raised in a church where divorce was a failure, and I have already noticed myself looking for reasons the marriage should survive in a case where the wife has not asked me to. My father was a violent man, and I am alert to the risk that I will see violence where there is only frustration, which is the opposite error and equally harmful. I am uncomfortable charging people who remind me of my mother, and I will use the sliding scale rather than decide in the room.

What makes this a good paper. Every statement is checkable, most are testable in a role-play, and the third section contains at least one entry that the student would rather not have written. That last feature is the marker of authenticity: a stance paper with no uncomfortable entry has been written for the examiner.

14.4 The viva: what is being examined

The form. A case presented by the candidate, usually one from her own practice or placement, followed by questions. Twenty to thirty minutes. The examiner has read the case summary; the candidate has the file.

What the examiner is listening for. Five things, in order of importance.

1. **Whether the candidate knows whose client it is.** The first question is almost always about the client system, and it is asked because candidates who cannot answer it are not safe to practise.
2. **Whether the candidate can name the ethical question without prompting.** The blank look followed by "there wasn't really an ethical issue" is the commonest failure, and it is a failure of recognition rather than of knowledge.
3. **Whether the reasoning is visible.** Steps taken, alternatives considered, consultation sought. The candidate who says "I decided to report" scores less than the one who says "I considered three options, and here is why I rejected two".
4. **Whether the candidate can distinguish what she knows from what she believes.** "I formed the impression" is acceptable; "I knew" is not, unless it was known.
5. **Whether the candidate's stance is present.** Not a speech about values — the ordinary consistency of a person who has decided the large things.

The case to bring. One in which the candidate made a decision she can defend, and one thing she would do differently. The case in which everything went well teaches the examiner nothing and the candidate less. The candidate who brings a case she got partly wrong, and can say what she learned, is marked higher than the candidate who brings a success.

The three questions that are asked in some form every time.

- "Who was your client?"
- "What was the ethical question, and when did you first notice it?"
- "What did you not do, and why?"

Preparing. Write a one-page summary: the family, the presenting problem, the decision point, the options, the consultation, the outcome, the reflection. Read the file. Know the dates. Rehearse the answers aloud to a colleague, because the first time these questions are answered should not be under examination.

The ethics of discussing cases in a viva. Consent from the client is not usually required for an examination conducted under confidentiality, and the candidate should nonetheless disguise the case and should say so: "the details have been changed." Examining boards treat the material as confidential; the candidate must treat it the same way outside the room.

14.4a Five answers that score badly, and their replacements

Poor answer	Why it fails	Replacement
"I felt it was the right thing to do."	No reasoning	"I weighed three options; the deciding factor was X; here is what I recorded."
"The code says I had to."	No judgement	"The code required a report; the judgement was in what I told the client and when."
"I did not want to damage the relationship."	The therapist's comfort as the criterion	"I considered the alliance, and I judged that the duty outweighed it; here is how I protected the relationship in the telling."
"It was a difficult case."	Says nothing	"It was difficult because two duties conflicted; the conflict was between X and Y."
"I would do the same again."	No learning	"I would do the same on the report, and differently on the timing — I would have raised it in session eight."

The pattern in all five. The poor answer describes a state; the good answer describes a decision. The examiner is not asking whether the candidate is a good person. She is asking whether the candidate can show her work.

14.4b The four sentences of the professional apology

Every therapist will make an error that harms a client. What happens next is a test of the whole course.

The four sentences. "I made a mistake and I want to tell you what it was." (Specific, owned, no passive voice.) "I am sorry, and I am not asking you to reassure me." (The apology without the request for absolution, which is what makes it usable.) "Here is what I have done about it." (The correction: the supervision, the referral, the record, the change.) "Here is what I will do next, and what you can decide." (The return of agency to the client, including the right to complain.)

The apology that fails. "I am sorry if you feel..." — an apology for the client's feeling. "Mistakes happen." — a generalisation that removes the therapist. "But you have to understand the position I was in." — an explanation that asks the client to carry the therapist. And an apology made so that the client will comfort the therapist, which is a request and not an apology.

What the codes require, and what they do not. The codes require honesty, the correction of the record where appropriate, and the client's right to complain. They do not require self-flagellation, and a therapist who collapses will make the client responsible for her.

The follow-up. The mistake is recorded, presented in supervision, and — where it reveals a structural problem — changes something in the practice. The client is told what changed. That last sentence is the one that converts an error into a demonstration of how a professional relationship handles failure.

14.5 The final supervision

The last supervision of the training year, in which the candidate presents her integration case. What should happen in it, and what the supervisor is looking for.

What the candidate brings. The case; the decision; the record of the decision; the stance paper; and one thing about her own practice that the training has changed.

What the supervisor asks.

- "What would you do differently if you met this family again tomorrow?"
- "Which part of your stance paper did this case test?"
- "What did you do that you did not tell me about at the time, and why?"
- "Who will you consult about this kind of case when I am no longer your supervisor?"

The last question is the one that matters. The purpose of supervision during training is not to make the candidate dependent on a supervisor; it is to build the habit of consultation into a professional life that will have no one appointed to provide it. A candidate who can answer the fourth question precisely — a named person, a peer group, a paid consultation, a specialist — is ready to qualify.

What the supervisor writes. A short assessment of the candidate's readiness that names the strengths and the specific development need going forward. Not a character reference, and not a summary of the case: a statement about what this clinician can be trusted with and what she should not yet do alone.

14.5a The case summary the candidate brings

One page, six headings, written the week before the viva. A worked example, drawn from a completed training case.

> **THE FAMILY.**
> The Oyelarans: Tunde, 46, a mechanic; Kemi, 43, a trader; children Segun, 15, and Bisi, 9. Referred by the school following Segun's attendance. **The presenting problem.** Segun's school refusal, which the parents attributed to "bad friends" and the school to "the home". The first session produced three different accounts and no agreement about the problem. **The decision point.** Session four: Kemi disclosed, alone, that Tunde had been drinking heavily since losing his workshop, that there had been two episodes in which he had pushed her, and that Segun had witnessed the second. She asked me not to tell him that she had told me, and not to raise it in the family sessions, because "it will make it worse". **The options I considered.** (1) Continue the family work and hold the disclosure, working individually with Kemi. (2) End the family work and see both adults individually. (3) Name the violence in the family session without attributing the disclosure. (4) Refer the family to a specialist domestic violence service and end the treatment. **What I did and why.** I did not continue the family sessions as though nothing had been said — option 1 would have made me a participant in the concealment, and Segun's witnessing made the children's exposure a child protection question. I did not confront Tunde with the disclosure (that would have exposed Kemi). I screened for lethality (no strangulation, no threats to kill, no weapon; two pushes; escalation with the drinking); I made a child protection referral regarding the children's exposure, told Kemi what I would say before I said it, and arranged individual work with her and a separate referral for Tunde to a substance use service. I did not end the treatment. I recorded the decision, the consultation (supervisor, [date]) and the review date. **What I would do differently.** I would have screened both partners individually in sessions one and two, as the practice protocol requires, rather than in session four when Kemi volunteered it. The protocol existed; I followed it late. And I would have asked Kemi directly whether she had told anyone else, because her sister had known for three months and was the person who had been keeping Segun overnight. **What the case taught me about my practice.** I avoid asking about violence in the first session because I am afraid of what I will then have to do. That is a stance problem, not a knowledge problem, and it is now in my stance paper.

Why this summary works. It states the dilemma without dramatising it, shows the rejected options with reasons, records the consultation, names the mistake without being destroyed by it, and connects the error to the therapist's own pattern rather than to the difficulty of the case.

14.6 Transcript: twelve minutes of viva

A candidate answering the three inevitable questions. Annotated.

> **Q1 Examiner:** Who was your client? **A1 Candidate:** The parents were the referrers and the school was the referrer of record. I treated the couple as the client system and I saw each of them individually under the policy I had stated in session one — which, as I will say, I did late. Segun was a client for three sessions, with his parents' consent and his own assent, and I was clear with all of them that what he told me would be shared with them only if he was at risk. — The answer names the system, the consent, the policy and the exception, in one turn. The examiner now knows most of what she needs. **Q2 Examiner:** When did you first notice there was an ethical question? **A2 Candidate:** In session four, when Kemi told me. (pause) And honestly, I should have noticed in session one, when the three accounts of the problem did not fit and nobody in the family would look at anybody else. — The honest answer with the retrospective recognition, which is what the examiner is testing: not whether the candidate noticed early, but whether she can tell the difference between noticing and understanding. **Q3 Examiner:** What was the question? **A3 Candidate:** Whether to keep the disclosure, which would have let the family work continue and would have made me part of the concealment, or to act on it, which meant the risk that Kemi would lose the only support she had. (pause) And underneath that, whether the children's exposure was a child protection matter, which it was. — Two duties named, and the second question identified as the one that decided the case. **Q4 Examiner:** Why did you not confront the husband? **A4 Candidate:** Because Kemi would have paid for it. (pause) The evidence is that conjoint confrontation in coercive situations escalates afterwards, and I had no way of predicting what he would do in the house that night. — The answer that shows the candidate has read the material and has applied it to a specific person's safety, rather than reciting a rule. **Q5 Examiner:** Did you consider referring the whole family out? **A5 Candidate:** I did, as option four. I rejected it because Kemi had disclosed to me and would not have disclosed to a stranger, and because a referral at that point would have been a way for me to avoid the decision rather than a service to her. (pause) I would refer now, if the violence escalated or if Tunde began to attend and I found myself holding both accounts. — The rejected option, the reason, and the condition under which the decision would change. This is the shape of a good answer. **Q6 Examiner:** What would you do differently? **A6 Candidate:** I would screen in sessions one and two rather than four. And I would not have assumed that because Kemi had not told me, nobody knew. Her sister knew. The sister had been keeping Segun overnight for three months, and I had not asked who else was involved. — A specific, small, real error, offered without self-punishment. **Q7 Examiner:** What does that tell you about your practice? **A7 Candidate:** That I delay asking about violence because I am afraid of the answer, and that the delay is itself a clinical decision with consequences. I have written it into my stance paper, and I have changed my intake so that the screening questions are asked in the first session of every couple. — The answer that connects an error to a change in structure, which is what makes a training case worth examining.

What the transcript shows. The candidate never defends, never performs regret, and never claims more than she knows. Her answers are short, specific, and each one contains a decision rather than a feeling. The examiner's follow-up questions get shorter as the viva proceeds, which is what happens when the answers are sufficient.

14.7 Laboratory: the mock viva and the stance review

Part A — the mock viva (sixty minutes). Students in threes: candidate, examiner, observer. Two rounds of twenty minutes, with ten minutes of feedback each. The candidate brings a one-page case summary; the examiner uses the seven questions from 14.4 and 14.6; the observer marks against the rubric below.

Criterion	0–1	2–3	4–5
The client system	Unclear	Named	Named, with consent and policy, and the exceptions stated
Recognition	Did not identify the question	Identified late	Identified, with an honest account of when and why
Reasoning	Conclusion only	Reasoning partly visible	Options, consultation, decision and record all visible
The mistake	Denied or catastrophic	Admitted	Named specifically, with the change it produced
Stance	Generic values	Some personal material	Specific, checkable, connected to this case

Part B — the stance review (thirty minutes). In pairs, students exchange stance papers and apply three tests. Is each statement in section 1 checkable? Convert any that are not. Is section 2 specific enough that a supervisor could hold the student to it? Does section 3 contain at least one entry the student would rather not have written? Papers that pass all three are read aloud to the group, and the group identifies which sentence would be hardest to keep.

The question to end on. "Which statement in your paper will be hardest to keep in your first year of practice, and what will make it hard?" The answers — money, a client's distress, the fear of losing the work, a supervisor who disagrees — are the actual curriculum of the first year.

14.8 Six errors of integration

Error 1 — Treating the model as a decision-maker. The seven steps produce a record of reasoning; they do not choose. The therapist who expects the model to make the decision will run it twice and still be undecided.

Error 2 — The stance paper as an essay about virtue. Written in the language of values, unverifiable, and forgotten by the second month. The paper is a set of commitments, and its test is whether it constrains behaviour.

Error 3 — Learning 700 pages and practising none of it. The student who can recite the codes and has not built a supervision arrangement, a records policy or a fee policy has learned the subject and not the practice.

Error 4 — The belief that a good therapist does not need structure. Structure is what allows the therapist to be fully present: the policy holds the decisions that would otherwise be made in the moment, under pressure, by a tired person who likes the client.

Error 5 — Recognising the ethical question only in retrospect. The commonest failure in the viva and in practice. The remedy is a checklist embedded in the ordinary workflow — screening questions in every first session, a consultation prompt in every complex case, a review date in every decision — because a habit fires where recognition does not.

Error 6 — Practising alone. Not a moral failure and a structural one: the therapist with no supervisor, no peer group and no consultation is the therapist in whom errors go uncorrected for years. The course's final recommendation is a relationship, not a rule.

14.8a The practice that is ready: a checklist of twelve

The course has argued throughout that ethical practice is structural. This is the structure, as a checklist that a graduate can work through in a fortnight and a supervisor can audit in an hour.

Documents. A consent form naming the limits and the client system; a confidentiality and secrets policy; a fee and cancellation policy; a records and retention policy; a disclosure protocol for requests from third parties; a complaints procedure; a list of verified referral contacts.

Relationships. A named supervisor with a written contract; a peer consultation group of at least three; an indemnity arrangement with access to legal advice; a professional body membership in good standing.

Habits. Screening questions in every first session of a couple or family; a supervision slot in the diary that is not moved; a note written within the day; a review date on every significant decision; one annual reading of the code and one annual audit of the practice's money.

The test. For each of the twelve, answer: does this exist in writing, and could a colleague find it without me? A practice in which the answer is yes for eight of the twelve is a practice that will survive its first serious case. A practice in which the answer is no for most of them is not unethical; it is unprotected, and the difference will be discovered by a client.

The first fortnight after qualifying. Build the documents, contract the supervisor, join the group, buy the indemnity. Not because a regulator requires it, but because each of the twelve is a decision the therapist will otherwise make alone, in the moment, with a client waiting.

14.9 Culture, faith and the professional stance

The stance is not culturally neutral, and it does not need to be. The professional obligations described in this book — individual consent, confidentiality against the family, the child's separate interests, the refusal to give legal advice — are the product of a particular history. They are nonetheless the terms on which this profession operates, and a therapist who joins the profession accepts them. The question the student actually faces is not whether the obligations are culturally local but how to hold them inside a communal world without becoming either a foreigner or a hypocrite.

Three ways of holding it.

1. **Translate rather than announce.** Every obligation in this book can be explained in the family's own terms: why an individual session exists, why a record is kept, why a gift of land cannot be accepted, why a report must be made. The therapist who translates is obeying the same rule in the family's language.
2. **Keep the form and the substance separate.** The therapist can eat the food, greet the elder, attend the funeral and say the prayers she says — and still not discuss the client's marriage with the aunt. The forms are the family's; the substance is the profession's.
3. **Be explicit about the limits of your own position.** "I know that in your family the elders would decide this. In my work I am required to let the person decide for herself, and I want to tell you why." Naming the difference is more respectful than either imposing or pretending.

Conscience, and the cases where the therapist's own values will not yield. A therapist may hold a religious conviction about divorce, abortion or same-sex relationships. The professional position is that the conviction is hers and the decision is the client's, and where the therapist cannot provide the service without imposing the conviction, the answer is referral and not persuasion. The two sentences: "I am not the right therapist for this, and I want to tell you that it is about me and not about you", and "here is a name."

Where the law and the client's interest diverge. They do, occasionally: a reporting duty that will harm the child it was designed to protect; a court order that will disclose a client's history to a violent ex-partner; a statutory requirement that the therapist believes is wrong. The professional response is to comply, to say so, to record the objection, and to work for change through the professional body. Civil disobedience is a legitimate act for a citizen and is not available to a clinician in the room, because the person who pays for it is the client.

The closing claim. A stance is not a barrier between the therapist and the family. It is what allows the therapist to be present: the person who has decided what she will not do does not have to decide it again in the session, and the attention that would have gone into that decision goes to the client.

> **DEBATE**
> "A therapist should never impose her values on a client, and a therapist who refers a client because of her own convictions has failed her duty of care." Argue for: the client came for treatment, not judgement, and referral is a rejection that falls hardest on the most vulnerable. Argue against: a therapist who cannot work without imposing is more dangerous in the room than out of it, honesty about limits is a form of respect, and referral with a warm handover is the lesser harm. Then state where your own line is, and what you would say to a client on the other side of it.

14.9a Nothing here is a substitute for the code

One final note on authority. This book has cited principles, statutes and codes, and it is not itself any of them. Where this book and a code disagree, the code governs; where a code and a statute disagree, the statute governs; where a statute and a court order disagree, the order governs. The book's purpose has been to teach the reasoning that makes those authorities usable in a room with a family in it, and a reader who treats it as a rulebook has misread it in the direction of safety, which is at least the right direction.

14.10 Evidence note

What is known. Ethics education research finds that knowledge of codes does not reliably change behaviour, while structured decision supports, checklists and consultation habits do. The literature on clinical judgement documents the gap between self-assessed and observed competence, and the value of feedback and routine outcome measurement. Research on moral distress and moral injury in therapists shows that unaddressed ethical conflict is a major contributor to burnout and to leaving the profession, and that supervision and peer consultation are protective. Studies of decision-making under pressure consistently find that prior commitment — a written policy, a rehearsed sentence — outperforms in-the-moment reasoning.

What is weaker. There is very little evidence on the specific instrument of the stance paper, and none on the seven-step model as a measurable intervention. The claim that they improve practice is derived from the broader literature on implementation and habit rather than from trials of these tools.

What is not supported. That ethical behaviour follows from ethical knowledge. That a therapist's good intentions are protective in the absence of structure. And that experience alone produces ethical judgement — experience without feedback produces confidence, which is a different thing.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same therapist, with the same knowledge, in a practice with supervision and a policy and in a practice without them, is two different clinicians.

14.11 Key terms

Dilemma — a choice between two goods or two harms, as distinct from a difficult situation.

Stance — the set of prior commitments a therapist holds about what she will never do, always consult about, and watch for in herself.

Consultation — seeking the view of another professional on a specific question; distinct from supervision in its form and often in its authority.

Moral distress — the experience of knowing the right action and being prevented from taking it.

Moral injury — the lasting psychological harm that follows acting against one's moral convictions in a professional role.

Implementation intention — a pre-decided response to a specified situation; the mechanism by which policies survive pressure.

Stance paper — the written, dated, annually revised statement of a therapist's commitments and vulnerabilities.

14.11a The one-page summary the course actually asks for

If the fourteen weeks were compressed into a page, it would say six things.

1. Name the client system before you do anything else. Most ethical errors in family work are errors about who the client is.

2. State the frame before it is tested. Consent, confidentiality, its limits, the fee, the cancellation policy, the records. Stated at the start, they are agreements; stated when needed, they are changes of rule.

3. Do not carry what you cannot carry. A secret, a risk, a doubt about competence, an attraction, a debt: each is a signal to consult, and the signal is not a personal failing.

4. Know the four duties that override the frame. Risk to life, risk to a child, statutory reporting, and a court order. Learn them for your jurisdiction, in writing, and review them annually.

5. Write as though it will be read aloud. Because some of it will be.

6. Build the structure that decides the ordinary cases, so that your attention is available for the client in front of you. The structure is not the opposite of relationship; it is what makes relationship possible.

14.12 Self-test

1. State the seven steps, with the failure mode of each.
2. In the Ngozi A. composite, why was option 3 (the forty-eight-hour deadline) rejected?
3. What are the three misuses of the decision model, and how is each detected?
4. Give the three sections of the stance paper and an example of a good entry in each.
5. What are the five things a viva examiner is listening for?
6. What are the three questions asked in some form every time, and why?
7. Give three ways of holding professional obligations inside a communal culture.
8. Why is civil disobedience not available to the clinician in the consulting room?

Answers. (1) Identify the dilemma — naming a situation instead; check codes and law — quoting from memory; consult — consulting the colleague who will agree; generate three options including the uncomfortable one — three versions of the preferred option; weigh consequences for every party — weighing only the client in the room; decide, act, document — documenting the conclusion without the reasoning; evaluate — skipping it. (2) Because it delayed protection for a child in a household where the abuser had access; the deadline was attractive because it preserved the alliance and it subordinated the child's safety to the therapist's discomfort. (3) Justifying a decision already made — detected by a step 4 containing three versions of one option; skipping step 4 — detected

by a binary choice with no uncomfortable option; substituting the model for a stance — detected by a therapist who runs the procedure for every decision and cannot say what she would never do. (4) What I will never do — "I will never write an opinion about a parent I have not assessed"; what I will always consult about — "any case in which I have become the only person the client talks to"; where my values distort me — "I interrupt anger, and I will ask my supervisor monthly whether I have done it". (5) Whose client it is; whether the question can be named without prompting; whether the reasoning is visible; whether knowledge is distinguished from belief; whether a stance is present. (6) "Who was your client?", "what was the ethical question and when did you first notice it?", "what did you not do, and why?" — because they test the three capacities that cannot be faked: defining the system, recognising the question, and reasoning about the rejected option. (7) Translate rather than announce; keep the form and the substance separate; be explicit about the limits of your own position. (8) Because the person who bears the consequence of the therapist's protest is the client, who did not consent to be the occasion of it.

14.13 Reflective task, and the close of the course

Write the five hundred words of the stance paper. Date it. Keep it somewhere you will find it in a year, and when you find it, write the revision: which statements held, which did not, and what belongs in section three now.

Then write one paragraph for yourself alone, not for an examiner, answering this question: what kind of therapist do you intend to be in the situation that frightens you most? The answer will be tested within the first eighteen months. The purpose of fourteen weeks of ethics has not been to make you cautious; it has been to give you a structure you can stand inside when the situation arrives, so that the therapist who is there — tired, fond of the client, unsure — is still recognisably you.

14.14 Further reading

- Beauchamp, T. and Childress, J. on the four principles — the framework used throughout this book, read in the original for its argument rather than its list.
- Kitchener, R. on ethical principles in counselling — the account that underlies the codes' structure.
- Doherty, W. on moral conflicts in family therapy — the best short treatment of the therapist's own values.
- Pope, K. and Vasquez, M. on ethics in psychotherapy and counselling — the standard reference, revised periodically.
- Your professional code, read in full, once a year, with a pencil.
- The literature on moral distress and burnout in therapists — read for the evidence that this subject is a protective factor and not an ornament.

Glossary

Autonomy — the client's right to self-determination. **Beneficence / non-maleficence** — the duty to benefit and the duty not to harm. **Coercive control** — a pattern of domination, isolation and threat, distinct from situational violence. **Confidentiality** — the duty not to disclose information learned in a professional relationship. **Dual relationship** — a second significant relationship with a client or someone close to them. **Duty to warn / protect** — a legal obligation, in some jurisdictions, to act on a serious threat. **Informed consent** — voluntary agreement based on adequate information and capacity. **Mandated reporting** — a legal duty to report suspected abuse or neglect. **Privilege** — the client's legal right to prevent disclosure in court proceedings. **Subpoena** — a legal order requiring documents or testimony. **Welfare principle** — the rule that a child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. In couple therapy, the most defensible approach to individual secrets is: (a) keep everything private (b) declare a policy in advance and apply it (c) tell the partner immediately (d) refuse individual sessions entirely

2. Asking a client directly about suicide: (a) increases risk (b) does not increase risk and is required practice (c) is only for psychiatrists (d) should be avoided in family sessions

3. Couple therapy with a couple in which there is coercive control is: (a) recommended (b) contraindicated until safety is established (c) harmless (d) the only legal option

4. The four principles of biomedical ethics are: (a) autonomy, beneficence, non-maleficence, justice (b) consent, capacity, confidentiality, competence (c) duty, utility, virtue, care (d) respect, honesty, integrity, courage

5. A treating therapist asked to conduct a custody evaluation for their own client should: (a) accept (b) accept if paid (c) decline because of role conflict (d) accept with supervision

6. The welfare principle means decisions about children are governed by: (a) parental preference (b) the child's best interests (c) the court's convenience (d) financial capacity

7. The strongest single predictor of a suicide attempt is: (a) diagnosis (b) previous attempt (c) gender (d) family income

8. A small cultural gift after a bereavement should usually be: (a) refused (b) accepted, discussed and documented (c) accepted silently (d) reported to the police

9. Screening for intimate partner violence should be done: (a) with the couple present (b) with each partner separately (c) by questionnaire only (d) only if asked

10. A defensible ethical decision is one that: (a) everyone agrees with (b) can be explained and documented (c) produces the best outcome (d) follows the client's preference

Answer key

1 b · 2 b · 3 b · 4 a · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. State a confidentiality policy for couple therapy in six sentences a family could understand.
2. List the ten elements of a defensible suicide risk assessment.
3. Distinguish situational couple violence from coercive control, and state the treatment implication.
4. What should you do when you receive a subpoena? Five steps.
5. Why is "I know it is unethical but it was the right thing to do" an inadequate defence?

Essay (choose one, 2,000 words)

1. "The most difficult ethical problems in family therapy are not conflicts between right and wrong but between two rights." Discuss with three cases.
2. A father discloses that his 14-year-old daughter is self-harming and asks you not to tell her mother. Analyse the ethical, legal and clinical dimensions and set out your plan.
3. Critically examine the claim that the professional codes are culturally biased in favour of Western individualism.

Further reading

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). OUP.
- Corey, G., Corey, M. S., & Corey, C. (2019). Issues and Ethics in the Helping Professions (10th ed.).
- Doherty, W. J. (1995). Soul Searching: Why Psychotherapy Must Promote Moral Responsibility. Basic Books.
- Wilcoxon, S. A., Remley, T. P., & Gladding, S. T. (2013). Ethical, Legal and Professional Issues in Counseling (4th ed.). Pearson.
- Child Rights Act (Nigeria), 2003; Violence Against Persons (Prohibition) Act, 2015.

Next: MFT 705 — Research Methods in Marriage and Family Therapy